NHS Birmingham and Solihull

NHS Black Country

Joint ICB Cluster Board

AGENDA

09 November 2026, 14:00 to 16:10hrs
ICB Training Room, Civic Centre, St Peter's Square, Wolverhampton

No.	Item	Presenter	Purpose	Time	Format
Introduction					
1.	**Welcome and Introductions**	Chair	-	14:00	Verbal
2.	**Apologies for Absence and Quoracy**	AD Governance	-		Verbal
3.	**Declarations of Interest**	Chair	Assurance		Verbal
4.	**Minutes from the meeting held on 14th September 2026**	Chair	Approval		Enc
5.	**Action Log**	AD Governance	Approval		Enc
6.	**Public Questions**	Chair	Discussion	14:05	Verbal
7.	**Chairs Report**	Chair	Information	14:15	Enc
8.	**CEO Report**	CEO	Information	14:20	Enc
Responding to Population Needs					
9.	**Insight Item – Cancer Plan**	CMO/CSO	Discussion	14:30	Enc
10.	**Birmingham and Solihull Urgent Treatment Centre – Decision making Business Case**	Senior Responsible Officer for UTC Review	Decision	15:00	Enc
11.	**Population Health and Strategy Committee Escalation Report**	Chair of PH&S CSO	Assurance	15:20	Enc
2026/2027 Planning and Performance					
12.	**Joint Quality and Safety Committee Escalation Report**	Chair of Q&S CNO	Assurance	15:30	Enc
13.	**Joint Resource Allocation and Payer Committee Escalation Report:** • Integrated Commissioning Report • Finance Month 5 and 6	Chair of RA&P CFO	Assurance	15:40	Enc
ICB Governance Arrangements					
14.	**Joint Transition Committee Escalation Report**	Chair of Transition	Assurance	15:50	Enc
15.	**Board Assurance Framework**	AD Governance	Assurance	15:55	Enc
Closing – Public Session					
16.	**Any other business**	Chair	Discussion	16:00	Verbal
17.	**Items for escalation to ICB Boards in Common**	Chair	Discussion		Verbal
18.	**Board Forward Plan**	Chair	Information		Verbal
19.	**Date of Next Meeting:** 11th January 2027- Board Room, 8th Floor, Alpha Tower				



NHS Birmingham and Solihull

NHS Black Country

Employee	Role	Interest Description	Interest Type
Danielle Oum	ICB Cluster Chair	Board member of Waythrough, charity providing support to people with issues around drugs, alcohol, mental health and domestic abuse	Outside Employment
		Board member Fusion 21 Ltd, social enterprise generating social value through efficient procurement frameworks.	Outside Employment
		Chair of Aquarius, Birmingham based charity providing support to people with drug, alcohol and gambling issues.	Outside Employment
		Director of Mostly Moseley Properties Ltd buy-to-let rental company.	Outside Employment
		Chair of emh, Leicestershire-based social housing and care provider operating in the East Midlands	Outside Employment
David Melbourne	ICB Chief Executive Officer	Nil Declaration	
Paul Athey	Chief Finance Officer	Director of Birmingham and Solihull Local Improvement Finance Trust LIMI, public private joint venture company that delivers health and social care infrastructure across Birmingham and Solihull. Appointed as the NHS representative for Community Health Partnerships.	Outside Employment
Dominic Benson	Director of Communications and Involvement (Board Attendee)	Nil Declaration	
Emma Bennett	Partner Member – Local Authorities	Chief Executive Officer - Walsall Metropolitan Borough Council	Financial Interest



Return to Agenda

NHS Birmingham and Solihull

NHS Black Country

Employee	Role	Interest Description	Interest Type
		Partner is Chief Executive Officer City of Wolverhampton Council	Indirect Interest
Pamela Bradbury	Non-Executive Member	Nil Declaration	
Kevin Davis	Non-Executive Member	Group Chief Executive (paid). The Vine Trust Walsall.	Outside Employment
		Non-Executive Director (remunerated). Black Country Housing Group (BCHG)	Outside Employment
		Board member (remunerated governance role). The Oversight Trust Oversight and assurance responsibilities relating to social investment/dormant assets ecosystem.	Outside Employment
		Chair Ladder Apprenticeship Foundation,	Outside Employment
		Chair of the Mercian Multi Academy Trust Board	Outside Employment
		Family relationship: brother-in-law is employed as an Analytics Manager at The Strategy Unit, which undertakes work relevant to NHS and Integrated Care Systems.	Indirect Interest
		Member of the Darlaston Pride in Place Board. The role involves contributing to local regeneration, neighbourhood priorities and partnership activity in Darlaston	Non-Financial Personal Interest
		Family relationship: brother-in-law and is Clinical Director for Primary Care at The Royal Wolverhampton NHS Trust. I am declaring this relationship for transparency given the potential for matters relating to primary care and/or The Royal Wolverhampton NHS Trust to arise through my role with the ICB.	Indirect Interest



Return to Agenda

NHS
Birmingham and Solihull

NHS
Black Country

Employee	Role	Interest Description	Interest Type
Mark Hackett	Non-Executive Member	Interim Chief Executive – Devon and Cornwall ICB Cluster	Outside Employment
		Wife is chair of Sandwell Leisure Trust and previous a Non-Executive on the Board	Indirect Interests
Harry Hayer	Non-Executive Member	Vice Chair, Chair of Remuneration, Nominations and Workforce Committee and Trustee Brunelcare	Outside Employment
		Trustee Trees for Cities	Outside Employment
		Interim Chief Executive Walk Wheel Cycle Trust	Outside Employment
		Regional Advisory Board Member, Canal & River Trust	Outside Employment
		Chair and Board Member – Hearing Dogs for Deaf People	Outside Employment
Phil Johns	Chief Strategy Officer	Member Chartered Institute of Public Finance Accountants	Non-Financial Professional Interest
		Member Healthcare and Financial Management Association	Non-Financial Professional Interest
		Spouse is Director of South Warwickshire Foundation Trust	Indirect Interest
Philip Jones	Non-Executive Member	Trustee and Chair of the Johnson Society, a literary society dedicated to promulgating knowledge about the 18ThC writer, Samuel Johnson.	Outside Employment
Peter McKenzie	Associate Director Governance and Corporate Affairs (Company Secretary)	Nil Declaration	
Sally Roberts	Chief Clinical and Quality Officer (Chief Nursing Officer)	Member of NMC	Non-Financial Professional Interest



Return to Agenda

NHS Birmingham and Solihull

NHS Black Country

Employee	Role	Interest Description	Interest Type
		Brother is Employed by ICB	Indirect Interest
Joanne Roney	Local Authority Partner Member	Managing Director, Birmingham City Council	Outside Employment
Ian Sharp	Medical Advisor to the Board (Chief Medical Officer)	Consultant Maxillofacial Surgeon University Hospitals Birmingham	Outside Employment
Paul Taylor	Non-Executive Member	My wife is a long standing friend of Helen Jives, the wife of Dave Rosser, former CEO of UHB. I see Dave in social circumstances .	Loyalty Interests
		Provision of financial and accountancy support for Showerbox CIO, a charity who aim to provide shower facilities for the homeless in London and Birmingham	Loyalty Interests
		My wife Lindsey Webb is a Non-Executive Director Robert Jones and Agnes Hunt NHS Foundation Trust	Loyalty Interests
		I was approached by David Frith and Peter Spilsbury of the Strategy Unit (SU) in early September 2025 to assist them in their deliberations about the future hosting of the SU. I have worked with Peter Spilsbury and also with the Strategy Unit in the past when I was self-employed	Loyalty Interests
		Mark Hackett was appointed as a Non Executive Member of Birmingham and Solihull, and Black Country ICBs in February 2026. Mark and his wife Penny Venables have been close personal friends for over 10 years.	Loyalty Interests



NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

Meeting Name:			
Public Joint ICB Cluster Board			
Date	Monday 14 September 2026	Time	14:00-17:00
Venue	Bournville Board Room Alpha Tower, Birmingham		

Members

Name	Abv	Role	Organisation
Danielle Oum	DO	**Chair**	Birmingham and Solihull ICB and Black Country ICB
Pamela Bradbury	PB	Non-Executive Member (Deputy Chair- Black Country)	Birmingham and Solihull ICB and Black Country ICB
Paul Taylor	PT	Non-Executive Member (Deputy Chair- Birmingham and Solihull)	Birmingham and Solihull ICB and Black Country ICB
Kevin Davis	KD	Non-Executive Member	Birmingham and Solihull ICB and Black Country ICB
Harry Hayer	HH	Non-Executive Member	Birmingham and Solihull ICB and Black Country ICB
Phil Jones	PJ	Non-Executive Member	Birmingham and Solihull ICB and Black Country ICB
David Melbourne	DM	Chief Executive	Birmingham and Solihull ICB and Black Country ICB
Paul Athey	PA	Chief Finance Officer	Birmingham and Solihull ICB and Black Country ICB
Sally Roberts	SR	Chief Quality and Clinical Officer	Birmingham and Solihull ICB and Black Country ICB
Emma Bennett	EB	Chief Executive	Walsall Council
Philip Johns	PJo	Chief Strategy Officer	Birmingham and Solihull ICB and Black Country ICB

In Attendance

Name	Abv	Role	Organisation
Dominic Benson	DB	Director of Communications	Birmingham and Solihull ICB and Black Country ICB
Peter McKenzie	PM	Associate Director Governance and Corporate Affairs	Birmingham and Solihull ICB and Black Country ICB
Laura Askew	LA	Corporate Support Officer (minutes)	Birmingham and Solihull ICB and Black Country ICB
Joanne Carney	JC	Deputy Chief Officer Strategic Commissioning	Birmingham and Solihull ICB and Black Country ICB
Sally Burns	SB	Director Public Health Birmingham (in place of Jo Roney)	Birmingham City Council

Apologies

Name	Abv	Role	Organisation
Jo Roney	JR	Managing Director	Birmingham City Council
Mark Hackett	MH	Non-Executive Member	Birmingham and Solihull ICB and Black Country ICB
Ian Sharp	IS	Medical Adviser to the Board	Birmingham and Solihull ICB and Black Country ICB

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Meeting Reference:	JICBCB-PUBLIC-20260914	
Agenda number	**Item**	**Action**
1.	**Welcome and introductions** The Chair welcomed attendees and members of the public to the meeting.	
2.	**Apologies for Absence and Quoracy** Apologies for absence were received from the members listed above. The Chair confirmed the meeting was appropriately constituted to proceed with the agenda. **Resolutions:** The Board **NOTED** the apologies received and substitutes who were in attendance.	
3.	**Declarations of interest** The Chair reminded members of their responsibilities to declare any interests relevant to the business of the meeting. No declarations were made in relation to the items under consideration and no management action was required. **Resolutions:** The Board **NOTED** that no agenda-specific declarations were made.	
4.	**Minutes from Previous Board Workshop Session** The minutes from the previous meeting in July 2026 were agreed as a correct record. **Resolutions:** The Board **APPROVED** the minutes as a correct record.	
5.	**Action Log** The Board reviewed the action log and noted that all recorded actions were complete. One matter had transferred into the Strategic Commissioning Committee work programme; no further action was required by the Board. **Resolutions:** The Board **APPROVED** closure of all actions on the action log.	
6.	**Public Questions** A written question had been received concerning provision at the Barberry Centre for people with functional neurological disorder and the ICB's response. It was confirmed that the matter was being considered with specialised services and that a joint written response would be provided through the established process. ***ACTION 005: Provide a written response to the public question on functional neurological disorder provision.*** **Resolutions:** The Board **NOTED** the question and **REQUESTED** that a written response be provided.	**PJo**
7.	**Chair's Report** The Chair's report was taken as read and provided an update on activity and engagement since the previous meeting. **Resolutions:** The Board **NOTED** the Chair's Report.	

DRAFT

8.	**CEO Report** David Melbourne presented the report and advised that organisational transition was nearing completion. He thanked colleagues who had maintained delivery while managing uncertainty. He emphasised that the Board's focus should now move from organisational transition to whether the ICB was improving outcomes for its population, reflecting that with the organisation receiving £2,800 per member of the cluster population it was important to invest effectively. It was noted that current performance measures, including long waits and waiting lists, represented visible points of failure rather than measures that could support a shirt to prevention. Members discussed the need to use the ICB's resources more deliberately, act as an active strategic commissioner, strengthen provider management and improve analytical capability so that data led to decisions. The Board noted moderate confidence in materially improving population health and reducing inequalities, recognising the historic difficulty of this task and the importance of place-based prevention, community insight and the voluntary and community sector. It was noted that the Board would need to make decisions about prioritisation of competing priorities. Frailty was highlighted as a potential early clinical priority to consider. Approximately 84,000 people were recorded as living with frailty across the cluster, although this was recognised as an underestimate. Work was developing around primary care identification, care planning, geriatric assessment and the definition of a clearer service offer. The Board also noted that improvement plans were in place to address the inadequate inspection findings for Dudley Children's Services and for maternity services at Midland Metropolitan Hospital. Assurance was provided that the ICB was engaging with statutory partners to develop these improvement approaches, with focus on the safeguarding front door, school health, health visiting and better place alignment. Maternity remained a high-risk area, supported by leadership changes, stronger corporate oversight and work responding to the Baroness Amos findings. **Resolutions:** The Board **NOTED** the report, the strategic shift towards prevention and active commissioning, and the continuing oversight of Dudley Children's Services and maternity improvement.	

Responding to Population Needs

9.	**Insight Item – Women's Health and Maternity** Sally Roberts introduced the item, which brought together key themes associated with maternity improvement, community insight and women's health pathway redesign. The Board were shown a video from Sandwell and West Birmingham Trust that set out their work to rebuild trust in maternity service through engagement with communities, improved accessibility including investment in translation services, in response to the issues raised in the Baroness Amos review in relation to racism and discrimination. The Board were advised that a Perinatal Improvement Programme had been established in response to national maternity and neonatal reviews which highlighted the need for stronger leadership, governance, accountability, safer care, improved use of data, greater co-production with women and families, and action to reduce inequalities. This strengthened maternity and neonatal improvement arrangements to align with NHS England's emerging Regional operating model. Revised governance, oversight and improvement programmes were in place to support delivery of safer, higher-quality and more equitable services. Whilst the Board was able take reasonable assurance that effective governance arrangements, oversight mechanisms and improvement programmes were in place across the cluster to manage identified maternity and neonatal risks further	

DRAFT

	work was required to fully align assurance frameworks, performance reporting, outcome measures and system-wide accountability across the newly clustered system. SR highlighted that listening to communities and understanding their needs was an integral part of the approach that the Cluster would take as a strategic commissioner. Joanne Carney presented the case for heavy menstrual bleeding as the first pathway for community redesign. The evidence indicated that around 40% of secondary-care gynaecology referrals over six months related to heavy menstrual bleeding, that many women on waiting lists required assessment, diagnostics or treatment rather than surgery, and that women in the most deprived communities were more than twice as likely to experience an unplanned admission. The Board were advised that a pilot programme in Dudley was aiming to improve experience and improve outcomes. Members supported the approach recognising the value of a standardised clinical pathway, stronger system working, simpler access, sustainable workforce arrangements and informed choice. Discussion also covered implementation costs, commissioning and block-contract implications, period poverty, employment impacts and the need to demonstrate how feedback from women and communities changed services. Members asked that learning from the Dudley pilot be accelerated where the evidence supported earlier action, including through analysis of GP and postcode-level data and consideration of alternative delivery models. Sally Burns highlighted that Birmingham City Council would be keen to engage with this to align conversations and offers between the Local Authorities and the Cluster ICB.	
	Resolutions:	The Board • **NOTED** the maternity improvement report for assurance; • **SUPPORTED** the development of Women's Integrated Neighbourhood Services; • **AGREED** that heavy menstrual bleeding should be the strategic priority pathway; and • **NOTED** the contribution of the CLEAR programme in Dudley to delivery of the Clinical Strategy.
2026/2027 Planning and Performance		
10.	**Joint Quality and Safety Committee Escalation Report** Pamela Bradbury presented the report which highlighted that Maternity and neonatal services, urgent and emergency care, children and young people's services and diagnostics remained material areas of focus. It was confirmed that the upcoming Improvement Event in September would focus on urgent and emergency care. The Board welcomed improvement in maternity services at University Hospitals Birmingham, while recognising that maternity remained a high-risk area and that assurance should increasingly demonstrate outcomes rather than activity alone. The Board noted that the previously expected recovery of the 26-week diagnostic wait for endoscopy at University Hospitals Birmingham would not be achieved by September. Members challenged whether the ICB should use different commissioning or contractual approaches where recovery was not sufficient. In response to a question, it was confirmed the anti-racism agenda and cultural competence as key areas for improvement were confirmed as active matters across partnership and improvement forums.	
	Resolutions:	The Board • **ACCEPTED** the report for assurance;

DRAFT

		• **SUPPORTED** continued focus on maternity, urgent and emergency care, diagnostics and children's services; and • **ENDORSED** continued monitoring of the associated improvement priorities
11.	**Joint Resource Allocation and Payer Committee Escalation Report** Paul Taylor presented the report, supported by Paul Athey. It was reported that the ICBs' financial position was expected to remain balanced at year end, subject to the management of identified risks. Three principal performance concerns remained: urgent and emergency care and ambulance handovers, 62-day cancer treatment performance, and growth in the total elective waiting list despite reductions in the longest waits. A revised performance report was expected to be presented to Resource Allocation and Payer Committee in October, which would provide a greater focus on recovery actions, expected timescales and the evidence supporting the level of assurance. Members considered when stronger contract management should be used, distinguishing provider underperformance against agreed activity from circumstances where demand genuinely exceeded plan. The Board discussed the developing financial framework for anchor contracts and place-based allocation from 2027/28. The first phase would establish current spend, compare it with weighted population need and develop incentives for population health improvement without destabilising existing services. The Board supported transparency on place-level expenditure and meaningful involvement of place and voluntary, community, faith and social enterprise partners. Place Prevention fund proposals were expected to progress through the Strategic Investment Committee before returning to the Board.	
	Resolutions:	The Board • **ACCEPTED** the report for assurance; • **NOTED** the financial position and areas of performance concern; and • **ENDORSED** continued development of the anchor and place financial framework, performance reporting and prevention proposals.
12.	**Winter Planning Update** Paul Athey presented the report. The Board were advised that the ICB's Winter Plan brought together provider plans for submission to NHS England by 30 September 2026. Modelling had identified a potential peak gap of 412 acute beds before a range of mitigating actions, which were expected to reduce this by 335, leaving a residual gap of 77 beds at the time of the paper being written. The ICB's priorities in developing these mitigations were prevention, frailty and discharge. A total of £1 million had been identified to strengthen vaccination activity. Further measures included a proactive frailty model, targeted primary care support, improvement of the call-before-convey model and paramedic training. Most of the domains identified in an NHS England self-assessment requirements were rated green. PA advised that gaps remained in relation to some provider returns regarding fit testing, PPE and infection prevention but were expected to be resolved before submission. In view of the further work required the Board were asked to delegate authority jointly to the Chair and Chief Executive to approve and sign the final Winter Plan and Board Assurance Statement, subject to receipt of the completed assurance pack. Members commented on the need to work to improve childhood and staff vaccination rates and PA advised that targeted options and potential provider incentives were under development to support this. Equality and quality impact assessments were required and would be completed once the final plan was available. Fortnightly implementation meetings chaired by The Deputy Chief Officer – Operations (the system Winter Director)	

DRAFT

	for would monitor delivery before winter, with daily operational calls to follow. Measures for some newer interventions still required development. The plan aimed to remove an average of 150 people per day from the urgent care pathway from 1 October through vaccination, alternative pathways, improved flow and discharge measures.		
	Resolutions:	The Board • **NOTED** the plan, dependencies, NHS England expectations and residual risks; • **APPROVED** delegation of authority jointly to the Chair and Chief Executive to approve and sign the final Winter Plan and Board Assurance Statement, subject to receipt of the completed assurance pack; and • **REQUESTED** completion of impact assessments and continuing implementation monitoring.	
ICB Transition Arrangements			
13.	**Joint Transition Committee Escalation Report** Harry Hayer presented the report and advised that the transition programme was in its final stages, although the Committee would continue to meet while remaining matters were completed. The Board noted that the committee had received significant assurance on the majority of the elements of the programme, with outstanding elements including the remaining transition arrangements for the Commissioning Support Unit. A proposal regarding the future role and closure of transition arrangements was expected at the November Board meeting. ***ACTION 006: Bring a proposal on the future role and closure of transition arrangements to the Board.***		**HH/PMc**
	Resolutions:	The Board **NOTED** the Committee's overall assurance position and the intended November update.	
14.	**Joint Remuneration Committee Escalation Report** Harry Hayer presented the report alongside the transition update. The Committee's narrower remit was noted, including its July consideration of a business case and the subsequent receipt of NHS England approval for the redundancy business cases.		
	Resolutions:	The Board **ACCEPTED** the report for assurance.	
15.	**Strategic Commissioning Committee Escalation Report** Kevin Davis presented the report, supported by Philip Johns on the strategic commissioning model. The report proposed revised terms of reference and the successor name Population Health and Strategy Committee. The change was intended to sharpen the front end of the commissioning cycle by connecting population need, inequalities, future demand, service utilisation, community insight and strategic opportunities before resource decisions were made. The Committee had taken partial assurance on 2026/27 planning because further work was required on place engagement, neighbourhood readiness, analytical capacity and repeatable evidence. Members welcomed links with local authority population health arrangements and emphasised avoiding duplication and ensuring residents were engaged consistently. The neighbourhood health plan was expected to be considered by the new Committee before the November deadline. NHS England feedback on data, insight and potential contractual levers would be addressed. A discussion with other committee chairs regarding redistribution of delegation responsibilities may mean that some slight alterations to the Terms of Reference for the new committee are needed.		

DRAFT

	ACTION 007: ToR to be finalised		**PMc**
	Resolutions:	The Board • **APPROVED** the revised Terms of Reference and the name Population Health and Strategy Committee; • **ACCEPTED** the escalation report; • **NOTED** the areas of partial or developing assurance; and • **REQUESTED** that alignment with other committee's delegations under the Scheme of Reservation and Delegation be addressed	
Governance			
16.	**Board Assurance Framework** Peter McKenzie presented the report. The Board Assurance Framework had been reviewed through committees and no changes to principal risk scores were recommended. The highest-rated risks continued to concern safe and effective services. Financial assurance had improved and transition risk had reduced, with full assurance reported for some financial controls, substantial assurance for transition and organisational matters, and moderate assurance across other principal risks. Members questioned whether risks remaining at high scores over time required a review of risk appetite, mitigations or escalation. It was confirmed that committee review and deeper analysis would be used where actions were not changing the risk position. The governance team was thanked for streamlining inherited risk arrangements and improving alignment with executive and committee review. The Board also asked that community and patient voice remained visible within the framework.		
	Resolutions:	The Board • **ACCEPTED** the Board Assurance Framework for assurance, • **AGREED** that principal risk scores should remain unchanged, and • **REQUESTED** continued review of static risks and the visibility of community and patient voice.	
17.	**Any other business** No additional business was raised.		
18.	**Items for escalation to ICB Boards in Common** No reserved matters were identified for escalation.		
19.	**Board Forward Plan** The Board was advised that the forward plan remained under review.		
20.	**Annual Reports 2025/2026** The annual reports for Birmingham and Solihull ICB and Black Country ICB were presented for information.		
21.	**Meeting effectiveness against our values** Pamela Bradbury led the reflection. Members considered that the meeting had been open, honest, constructive and evidence based. The discussion demonstrated a focus on people, community listening, partnership working, breaking silos and using evidence to support different commissioning approaches. The Board recognised that bold decisions would be required within finite resources.		
Date of the next meeting: 09 November 2026 – Training Suite, Civic Centre, Wolverhampton **Date of next workshop**: 12 October 2026 – Bournville Board Room Alpha Tower, Birmingham			

DRAFT

Return to Agenda

NHS Birmingham and Solihull

Joint ICB Cluster Board

NHS Black Country

Action Log

Unique Identifier / Action ID	Date Action Raised	Action Title	What is the Action(s) Required?	Person(s) Responsible for the Completion of the Action	Deadline Date for completion of the Action	Action Status Red: Deadline Date is over 2 months Amber: Deadline Date was from the Previous month Green: Action is Due this Month Blue: Completed Pending Approval of Closure Purple: Closed	Action Update / Additional Information	Status
Action005	14 September 2026	FND Response to public	Provide a written response to the public question on functional neurological disorder provision .	Phil Johns	09 November 2026	Blue: Action has been completed		**Open**
Action006	14 September 2026	Transition committee closure	Bring a proposal on the future role and closure of transition arrangements to the Board.	Harry Hayer / Peter McKenzie	09 November 2026	Green: Action is due this Month	**Item on the agenda**	**Open**
Action007	14 September 2026	PHS Terms of Ref	ToR to be finalised	Peter McKenzie	09 November 2026	Blue: Action has been completed		**Open**

NHS Birmingham and Solihull

NHS Black Country

		Agenda No: 7
Report to:	Joint ICB Cluster Board	
Date of Meeting:	9 November 2026	
Title:	Chair's Report – November 2026	
Presenting Officer:	Danielle Oum, Chair	
Author(s) of Report:	Danielle Oum, Chair	
To be Withheld from Publication?	**Yes** ☐ **No** ☒	**(If Yes) Rationale:**
Relevant Conflicts of Interest	N/A	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☒	Digital Transformation	☒	Treatment to Prevention	☒

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham and Solihull	☒

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☒

Purpose:

To share key developments and highlights from the cluster Chair in reference to the Birmingham, Black Country and Solihull integrated care systems.

Decision Required			
Approval	☐	**Accept as Assurance**	☐
Endorsement	☐	**Noted**	☒

Report Journey:

None

Executive Summary

This report highlights key activity and engagements involving the Chair during this period in our three priority areas 1) delivery 2) transformation and 3) transition.



Return to Agenda

Appendices
None

Next steps
None

Assessments:					
Impact Assessments Completed		**Yes**	**No**	**N/A**	**Details (N= explain why: Y= approval date)**
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Impact Area	↑ Risk	⇆ Risk	↓ Risk
Quality and Safety	☐	☒	☐
Regulatory	☐	☒	☐
Finance	☐	☒	☐
People	☐	☒	☐
Reputational	☐	☐	☒

Return to Agenda

Chair's report

November 2026

1. Delivery

West Midlands Ambulance Service (WMAS) – I was privileged to be able to join an ambulance crew from WMAS on a shift. Thanks to Sarah and Jordan, the paramedics who I spent the day with and who showed outstanding patient-focus, resilience and compassion as well as a keen appreciation of the strengths and areas for improvement across the healthcare system.

Colleagues in the ambulance service are under constant pressure and they have first-hand experience of where enhanced community services can play a greater role in the urgent and emergency care system. My aim was to see what a typical shift looks like for paramedics in the region and to better understand the realities of frontline care and the pressures faced by ambulance staff.

Spending time alongside our clinical colleagues in their environment helps strengthen our understanding of the experiences of both staff and patients, which ultimately helps us make more informed decisions and enhances how we work in partnership across our systems.

2. Transformation

Cervical screening – Cancer is one of our seven clinical priorities and improving screening rates is one of the most impactful things we can do to prevent cancer, detect it earlier and improve outcomes.

It has been good to see our Mobile Health Hub back out on the roads of the Black Country, making cervical screening more accessible and bringing services closer to our communities. Cervical screening can help prevent cervical cancer by finding high-risk HPV and cell changes early, so they can be monitored or treated before cancer develops.

The hub visited community sites across Wolverhampton and Sandwell, including gyms, community centres and faith venues. A similar unit continues to run in Birmingham too.

These new ways of delivering vital screening services can help reach people who might otherwise not take up cervical screening, identifying high-risk HPV and cell changes earlier and helping to improve outcomes for our communities. This also supports the delivery of our clinical strategy, cancer strategy and the 10 year health plan which aims to bring care closer to communities, improve prevention and detect conditions earlier.

3. Transition

Merger timeline - There has been further national clarification about the future shape of Integrated Care Boards. The Government's Rewiring the State policy set out an intention for ICBs to align with strategic authority footprints by the end of this Parliament. NHS England has subsequently determined that formal decisions on future ICB mergers and boundaries

should wait until there is greater certainty about those arrangements. On the current timetable, our Birmingham and Solihull and Black Country cluster arrangements will therefore continue throughout 2027/28, with any changes to ICB footprints expected from April 2028.

Importantly, this has minimal impact on our direction of travel. While the organisational timetable has changed, our strategic ambition has not. We will continue developing as a strategic commissioner for our population of 2.9 million people, focused on improving population health, tackling inequalities and using evidence and insight to guide investment and improvement. We will also continue strengthening our relationship with the West Midlands Combined Authority and other partners, recognising the increasingly important connection between health, economic growth and wider prosperity.

The national policy direction therefore reinforces, rather than changes, the course we have already set. Our existing footprint already aligns with the vast majority of the West Midlands Combined Authority area, although there remains uncertainty about the future strategic authority arrangements affecting neighbouring ICBs. With the planned abolition of NHS England, decisions on future ICB mergers and boundaries are expected to transfer to the Secretary of State, with further national engagement ahead of final decisions. **For us, the immediate priority is therefore to maintain momentum: making the cluster work effectively, continuing our transformation and organisational change, and building the strategic commissioning model we will need regardless of the final organisational footprint.**

4. Fit for the future

Staff engagement – Our in-person all-staff event on 15 September was an important opportunity to bring colleagues together at a time of significant organisational change. There was real value simply in creating the space for people to connect face-to-face, hear directly from leaders and focus not only on the changes taking place around us, but on the organisation and culture we want to build together.

The event also reinforced an important principle for our transition: organisational change is about more than structures, processes and reporting lines. It is about people, relationships and culture. As we continue developing our cluster arrangements, we want to create a stronger sense of shared purpose around our responsibility to the 2.9 million people we serve across Birmingham, the Black Country and Solihull, while recognising the experience, identity and strengths that colleagues bring from our existing organisations.

That feels particularly important given the longer national timetable for any future changes to ICB footprints. We should not wait for the final organisational architecture before building the relationships, behaviours and ways of working we will need for the future. The 15 September event was a positive step in that journey, helping us look beyond transition as something to manage and towards the organisation we want to become.

Regional ICB Chairs Network – I would like to thank Paul Taylor for representing us at the Midlands ICB Chairs Network recently, where Chairs discussed implementation of the 10 Year Health Plan and, in particular, how we turn the three shifts from ambition into practical change. Our contribution emphasised that achieving the shifts from hospital to community,



treatment to prevention and analogue to digital will require us to change the way we commission, with a much stronger focus on population health, outcomes and ensuring that every pound we spend secures the greatest possible health benefit for the communities we serve.

We were also able to share some of the practical ways in which that approach is already taking shape across Birmingham, the Black Country and Solihull. These include strengthening place and neighbourhood working and developing neighbourhood anchors to support care closer to home; backing prevention through our clinical strategy and recurrent £16 million Place Prevention Fund; and using digital, data and technology to redesign care and give people greater control over their health. Importantly, these shifts are connected: our ambition is not simply to move individual services, but to create a model which improves health, tackles inequalities and enables more decisions to be made with communities and partners closer to where people live.

Fit and Proper Person Test (FPPT) I can confirm to the Board that, in line with both ICBs relevant policies, a review has been undertaken of all Board members compliance with the Fit and Proper Person Test and that all Board Members meet the requirements. The ICB submitted details of compliance with the FPPT to NHS England at the end of June 2026, and this has now been countersigned by the Regional Director in line with the NHS England Framework.

NHS Birmingham and Solihull

NHS Black Country

			Agenda No: 8
Report to:	Joint ICB Cluster Board		
Date of Meeting:	9 November 2026		
Title:	Chief Executive's Report		
Presenting Officer:	Chief Executive		
Author(s) of Report:	Chief Executive		
To be Withheld from Publication?	**Yes** ☐ **No** ☒	**(If Yes) Rationale:**	
Relevant Conflicts of Interest	None		

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☒	Digital Transformation	☒	Treatment to Prevention	☒

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☒

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☒

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☒	**Digital First**	☐
Financial Discipline/ 2% productivity	☒	**Advancing Health Equity**	☒	**Integrated Partnership**	☒

Purpose:
To provide the Board with an overview of the items presented to the Board and highlight any other local, regional or national developments.

Decision Required			
Approval	☐	**Accept as Assurance**	☐
Endorsement	☐	**Noted**	☒



Return to Agenda

Report Journey:

Not applicable

Executive Summary

As we enter the second half of the year, there is progress in NHS delivery, but we know the next five months, spanning winter, need careful navigation to ensure that the communities we serve get the services they deserve. We invest around £2,800 per person on behalf of approximately three million people, and our role is clear: put money where it delivers, move it when it does not, and build what works into next year's contracts. Across the Board agenda this month we test each of those areas. The urgent treatment centre's decision is about commissioning a model of care that will improve access for people across Birmingham and Solihull to core pathways. The cancer insight and the neighbourhood health readiness assessment show that the gap begins in screening, referral and neighbourhood working in our most deprived communities. Our committees give qualified assurance on performance, a break-even forecast with £5.7m of net risk, and a warning that our analytics and procurement capacity is fragile. NHS England's regional review recognised our direction and was clear that confidence depends on delivery this winter. With merger now not expected before April 2028, we must build and deliver within the current organisational form we have, in partnership, and with the flexibility to move resources at a pace that has not always been characteristic of the NHS.

Whilst not on the agenda, the Board, at its development session in October, received a summary of the implications of the Thirlwall Inquiry. A detailed action plan will be considered at the December Quality & Safety Committee and reported back to the Board in January 2027.

Appendices

None

Recommendations & Next Steps

The Board is asked to:

1. Note the report.
2. Formally approve the process for considering the implications of the Thirlwall Inquiry across the conurbation.
3. Approve the process by which assurance on the single-sex services and facilities guidance and the non-ideological extreme violence guidance will be sought through the Joint Quality and Safety Committee.

Other matters are picked up through the Board agenda.

Assessments:

Impact Assessments Completed		Yes	No	N/A	Details (N= explain why: Y= approval date)
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Return to Agenda

Implications	Impact
Quality and Safety	YES
Legal and Regulatory	YES
Finance	YES
People	YES
Reputational	YES

Introduction – from establishing the new organisation to delivery.

In September I said that it would be the last report in which the Board received transition as a headline. The Transition Committee's final report confirms that the programme has achieved its objectives, that Day One was implemented without material operational failure, and it asks the Board to establish a Joint Remuneration and People Committee in its place. There are areas around transition still requiring attention as the Committee took only partial assurance: the impact of change on disabled colleagues, where further analysis is needed, and the CSU stranded costs and hosting costs for the Office of Pan-ICB Commissioning, which we have provided for at the worst case. Around 30 colleagues are expected to remain at risk once phase two of the management of change concludes. I share the Committee Chair's thanks to colleagues who did this work while carrying real personal uncertainty.

As the Chair's report sets out, NHS England has decided that formal decisions on ICB mergers should wait for greater certainty about strategic authority footprints across the country, so our cluster arrangements will continue through 2027/28, with any change expected from April 2028. The Resource Allocation and Payer Committee has opened a new risk on the cost of running two statutory ICBs for longer, and the Board Assurance Framework asks us to review where organisational risk is now scrutinised. My own conclusion is that whilst the timetable has changed, the task has not, and we should get on with the changes required to ensure that we secure the health outcomes our population deserve within the resources we have been allocated.

It is clear the NHS will have to do this with less financial room to manoeuvre than we would like. The Budget contained little new for the NHS; planning guidance for 2027/28 has not been issued, which speaks to the challenging financial settlement ahead. That does not weaken the case for commissioning at the point of cause rather than paying at the point of failure; it strengthens it, because late intervention is the cost we can least afford. It does mean that we need to be clear on the impact of the decisions and track those – analysis and elegant strategies alone are not enough.

NHSE Regional review: confidence in the direction, conditional on delivery.

NHS England's System Review Meeting on 16 October recognised our ambition to act as strategic commissioners for the whole population, not only commissioners of healthcare; a clinical strategy focused on the conditions and groups that most affect life expectancy and inequality; and growing collective leadership across two systems that have historically operated apart. It described the anchor model as innovative, with the potential to become a leading example, and welcomed our decision to invest available flexibility in services rather than hold it within the ICB.

It was equally clear about what comes next: delivery on finance, urgent care, ambulance handovers, elective recovery and constitutional standards through the second half of the year, and a medium-term financial plan showing how prevention, neighbourhood health and anchors will reduce the underlying deficit, with a clear line of sight from our priorities to measurable outcomes. I accept both points as a credible strategy alone does not necessarily lead to delivery – the rest of this report focusses on that delivery requirement.

Understanding our population: how health inequalities begin.

The neighbourhood health readiness assessment contains the single statistic that best explains the challenge the local NHS faces. A man in Sandwell can expect 52.7 years of life in good health and a woman 51.3, against 63.2 and 62.9 in Solihull. That is a decade of healthy life separating communities a few miles apart, and no one organisation can close it alone. The ICB holds 93% of the NHS budget across the conurbation and as such must play a significant role with partners in developing the conditions to close that gap.

The cancer insight item shows the same gap from another angle. Just over half of cancers are diagnosed at stage one or two, 55.1% in Birmingham and Solihull and 56.2% in the Black Country, and that has not moved since 2020, against an ambition of 75% by 2035. The reasons sit upstream, urgent suspected cancer referral rates run around 22% below England in Birmingham and Solihull and 12% below in the Black Country, and our own oversight data shows cervical screening at around 68% and 64% against an 80% standard. Uptake falls as deprivation rises, and emergency presentation, the costliest and least survivable route to diagnosis, is concentrated in the same places.

Once people reach us, diagnosis is getting faster, but treatment is not, and 62-day performance remains well short of the 80% we must reach by March 2027. The Resource Allocation and Payer Committee judged diagnostics to offer only limited assurance and noted that diagnostic capacity now drives both elective and cancer recovery. Our cancer strategy proposes one cancer control centre across BBCS, a diagnostic bottleneck programme, a navigator for every patient with suspected cancer and tumour-site deep dives. Our job is now to set these into contracts with our providers that help shift that early diagnosis dial.

The early evidence on that question is encouraging, if small. As the Chair's report describes, the Mobile Health Hub is back in gyms, community centres and faith venues across Wolverhampton and Sandwell; in its first three months it identified eight high-risk results among women the standard offer was not reaching. Around 13,000 cancers are diagnosed each year across the conurbation we cover, so each percentage point of stage shift is 130 people diagnosed earlier. That is the measure that we should focus on during the debate.

Neighbourhood health: early progress must accelerate.

The Population Health and Strategy Committee assessed our overall neighbourhood readiness as 'early stage.' Our strengths, based on the analysis, are urgent community response, reablement and pooled Better Care Fund arrangements; our weaknesses are integrated neighbourhood teams, devolved budgets, clear organisational ownership and consistent measurement of outcomes. Walsall, East Birmingham and Solihull are ahead as wave one national sites. The Committee approved seven common Place principles and a phased model in which the ICB keeps its statutory commissioning and payer accountability unless authority is expressly delegated, and it will receive progress against a six- and eighteen-month plan from January.

Two of the Committee's judgements deserve the Board's attention. It did not accept full assurance on our ten-year demand model, because provider plans still focus on existing two-to-three-year programmes and do not yet show how residual demand and cost will be mitigated; this cuts to one of the challenges from NHS England in our System Review Meeting – how we optimise healthcare utilisation and value. Secondly, the analysis of urgent care utilisation found material variation between neighbourhoods without it being explained; we must move beyond diagnosis alone and identify what we will do differently because of that analysis.

Where we have invested upstream, the measures are starting to move. We have funded 20,000 additional hypertension checks across the six Places, clopidogrel resistance testing for South Asian communities and wider access to continuous glucose monitoring, and the proportion of people with



hypertension managed to NICE guidance has risen across the cluster from 70.5% to 73.7% in two years. With the recurrent £16m Place Prevention Fund, these are the investments most likely to be questioned under financial pressure and the last we should give up, provided each comes back with evidence that it worked and has an impact.

Urgent care: transparent, evidence-led decision.

Deferring the urgent treatment centre decision in September was the right judgement, and it carried a commitment to decide today. The recommended Option A Plus would create four urgent treatment centres alongside the emergency departments at Queen Elizabeth, Heartlands, Good Hope and Midland Metropolitan University Hospital; retain and improve community centres at Solihull, Erdington and Selly Oak; add locality hubs at Summerfield, for respiratory and diabetes care, and Washwood Heath, for rapid access heart failure care; and commission GP out-of-hours services in three locality lots. Walk-in and NHS 111 access remain. More than 1,600 people contributed to consultation, and the independent post-consultation review confirmed the original appraisal. There will be no immediate change to services; the model is assessed as affordable within the existing envelope, and full implementation is planned from April 2028.

When the executive team considered this analysis, we had three tests in mind that we felt the recommendation met. Does the proposal improve access for those who most need urgent care and are least able to travel for it? Does it give patients, and clinicians acting for them, fewer doors to choose between rather than more? Is it deliverable?

Our role as payer: securing delivery, standards and value.

The Resource Allocation and Payer Committee gave qualified assurance on commissioning performance overall. Diagnostics and community services offered limited assurance, urgent care, elective care, cancer and mental health partial assurance. There is real progress: Category 2 ambulance responses are within the national standard in both systems, four-hour performance is 73%, up from 71%, and three in four clinically urgent general practice appointments now happen on the same day. But four-hour performance is 67% in Birmingham and Solihull against 78% in the Black Country, and the Committee was clear that recovery depends on clearer trajectories and stronger evidence that recovery actions are working.

Elective performance is more challenged with 18-week performance around 65% against a year-end target of 68%, 1,685 people had waited more than 52 weeks in August against a plan of 1,242, and the year-on-year fall in our waiting list has slowed from 6.4% in April to 1.2% in August. Paul Athey is leading a review of every major contract with provider operational and finance leaders, concluding by 30 November and allowing for the consultant strike announced on 26 October. Where a provider cannot deliver agreed activity, the funding attached to it will move to providers that can. Learning disability and inpatient autism also remain above trajectory, at 95 adults against 80 and 16 children and young people against seven, and formal contractual escalation is under way.

For winter, the NHS chief executives across our conurbation agreed on 23 October to add one or two things to existing plans rather than a long list, starting with single points of access, where acceptance of ambulance referrals is the lowest in the region. We have set four standards: one definition and dataset for acceptance, a core offer in every single point of access, 24/7 cover everywhere, and a reason code for every decline. These measures plus the investment in call before you convey training at West Midlands Ambulance Service plus a package of winter measures from flu vaccination to increasing the focus on the frailest in our population contribute to capacity over winter.

Both ICBs forecast break-even at month six, with favourable year-to-date positions of £0.942m in the Black Country and £0.483m in Birmingham and Solihull, and full delivery of efficiency plans worth £97.2m and £130.6m. Net risk after mitigation is £5.7m, £3.1m in Birmingham and Solihull and £2.6m in the Black Country, driven by prescribing price concessions, continuing healthcare and Right to Choose activity,



and £3.4m of Birmingham and Solihull schemes remain high risk. On 23 October, every NHS organisation in the conurbation confirmed its commitment to its board-agreed financial plan set at the start of the year. Beyond the investment package we put in place in October, the ICB has little headroom, so where there is slippage, as there may be in the flu incentive scheme, the money will be recycled into elective and urgent care rather than lost. We will continue to work with colleagues as we are only too aware of the additional costs in the healthcare supply chain because of geo-political events. West Midlands Ambulance Service for example face an additional bill for the diesel in their ambulances of circa £750,000 because of the recent fuel price increases.

The Committee agreed that anchor allocations and initial contract envelopes will be based on GP-registered populations, the best fit with how we are funded and contract, while recognising that council's plan on a resident basis. The physical health programme moves to a formal Programme Board from 26 November, ahead of contract award from April 2027 subject to gateway and maturity assessment, with outpatient reform incentives built in. For mental health and learning disability, both providers met the Stage 1 gateway, and the Population Health and Strategy Committee approved a twelve-month shadow arrangement before readiness is reassessed. I thank both mental health trust chief executives for their support.

Commissioning quality: safe services, patient voices.

Our highest-rated principal risk, at 16 against a target of eight, is that the ICB fails to use its levers to ensure consistently safe and effective services across every Place. That is the commissioner's safety role and Lady Justice Thirlwall's report tests it directly. NHS England has asked every ICB Board to consider the Inquiry at its public meeting. We should ask ourselves not just whether we commission neonatal services well on paper, but whether events like those the Inquiry examined could occur in services we commission, and whether concerns would be heard and acted on in time.

The immediate actions fall to trusts: confirmation that the Sudden and Unexpected Death in Childhood protocol applies in neonatal units, CCTV focused on insulin storage in every neonatal unit by 30 November, and full access control for insulin by 31 March 2027. Our role is to know, not assume, that each has happened, and I have asked Sally Roberts, as Chief Clinical and Quality Officer, to bring confirmation for every trust with a neonatal unit through the Joint Quality and Safety Committee and then to the January Board. Every Board member must also be compliant with safeguarding training at the appropriate level, and our own compliance will be reported in January.

The extension of Martha's Rule into emergency departments, explicitly including corridors and waiting rooms, gives us a further source of intelligence about where flow is failing, and we should treat it as such.

Commissioning for inclusion: meeting needs, closing gaps in care.

All six local area partnerships have had their Local SEND Reform Plans approved, and five have indicative High Needs Stability Grant allocations totalling around £140m. Two partnerships are judged to be making progress, three are still developing and one is at an early stage, and delivery ratings will be published. Three issues land with health: joint commissioning is a recurring weakness; we will name a senior responsible officer for health in all six partnerships; and future deficit support for councils depends on therapy capacity. We are working with local authority partners to ensure that the NHS responds to each of these areas.

Two pieces of national guidance ask for assurance rather than acknowledgement. The interim guidance on non-ideological extreme violence asks who owns the risk when a person presents a risk of serious violence below the Prevent threshold, and I am seeking assurance through our safeguarding partnerships that such a person cannot fall between agencies. The guidance on single-sex services and facilities requires a coordinated process across the ICB and our providers, with local equality impact assessment and staff and trade union engagement, so that decisions are lawful, proportionate and documented and



everyone continues to have provision that protects their privacy, dignity and safety. Finally, our success in a national bid to strengthen safeguarding against violence against women and girls in primary care should strengthen specialist capacity that already works.

Adapting our capabilities to deliver our purpose.

Strategic commissioning depends on three capabilities: knowing our population, buying well, and hearing what people tell us. Our committees have flagged the first two as fragile. The Population Health and Strategy Committee increased its risk on analytics continuity because unresolved national hosting and data service arrangements are beginning to impede population health work. The Resource Allocation and Payer Committee reported that procurement capacity remains constrained following the end of CSU support, and the urgent treatment centre business case names the same constraint as a risk. These are not back-office matters, an ICB that cannot analyse need or run a compliant procurement cannot invest to make a difference and the executive team will develop plans to mitigate these risks.

On the third, the Committee supported a broader approach to patient experience, drawing together national data, local engagement, general practice intelligence and voluntary sector insight. In September I asked that community insight should come to this Board with at least one decision altered. The urgent treatment centre proposal, reshaped by consultation into a model with locality hubs and retained walk-in access, is the clearest example yet. Further developing our insight capability to better understand our population and then influence effective use of the services that we commission is an area we need to further develop.

Delivering our purpose: better health, better care, better value.

This report has been written around a single proposition we hold around £2,800 per person on behalf of three million people, and we answer to them for what it buys. Today's agenda tests each part of that role: deciding the shape of urgent care; understanding why cancer is found too late and healthy life is a decade shorter in some of our communities; paying for what works and moving money when it does not; knowing, not assuming, that services are safe; and building the capability to do all of this well. The merger timetable has moved, but our role has not changed and it remains clear. NHS England has told us the direction is right.

We will continue to work on gaps identified by our Committees, being flexible in the way we work to ensure that the organisation can respond to the challenges I have set out above.

November 2026



NHS Birmingham and Solihull

NHS Black Country

		Agenda No: 9
Report to:	**NHS Birmingham and Solihull and NHS Black Country ICB Board**	
Date of Meeting:	**9 November 2026**	
Title:	**Cancer insight: what the data tells us and how we will respond to the National Cancer Plan**	
Presenting Officer:	**Sonia Ashraf / Phil Johns**	
Author(s) of Report:	**Jo Tolley / Sonia Ashraf**	
To be Withheld from Publication?	**Yes ☐ No ☒**	Not applicable
Relevant Conflicts of Interest	None identified in the source paper.	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☐	Digital Transformation	☒	Treatment to Prevention	☒

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☐

ICB Focus					
Delivery	☒	**Transformation**	☐	**Transition**	☐

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☒	**Digital First**	☒
Financial Discipline/ 2% productivity	☐	**Advancing Health Equity**	☒	**Integrated Partnership**	☒

Purpose:

This session gives the Board insight into cancer across Birmingham, Solihull and the Black Country. It brings together the National Cancer Plan (published 4 February 2026), the BBCS Clinical Strategy and our local data, and sets out how the joint Cancer and Diagnostics Programme will respond. The Board is not asked to approve a plan at this meeting. The aligned Cancer and Diagnostics Strategy, with trajectories and a full data pack, will return to the Board for approval.

Decision Required			
Approval	☐	**Accept as Assurance**	☒
Endorsement	☐	**Noted**	☐



Return to Agenda

Report Journey:

Presented to the joint ICB Board on 9 November 2026. The aligned Cancer and Diagnostics Strategy, trajectories and full data pack will return to the Board for approval on a date to be agreed.

Executive Summary

Cancer is being diagnosed too late, referral and screening rates remain below England, and inequalities are greatest in deprived and some ethnic minority communities. Faster diagnosis has improved, but 62-day treatment performance remains below the March 2027 ambition and is constrained by diagnostic capacity. The proposed response is four shifts delivered through five workstreams, with immediate focus on a cancer control centre, diagnostic bottlenecks, navigation for patients and tumour-site deep dives.

Appendices

Appendix A – Summary of Data, Insight and Delivery Plan

Recommendations & Next Steps

The Board is asked to note the headlines; discuss the four shifts and four proposed priorities; note the principal workforce, diagnostic capacity and transformation funding risks; and agree the future assurance route and return of the full strategy for approval.

Assessments:

Impact Assessments Completed		Yes	No	N/A	Details (N= explain why: Y= approval date)
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	.
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	Late diagnosis and long waits carry risk of harm. Harm reviews and learning through the Patient Safety Incident Response Framework apply to long-waiting patients.
Legal and Regulatory	Cancer measures feed the NHS Oversight Framework and affect ICB and provider segmentation.
Finance	Several programmes depend on West Midlands Cancer Alliance and Community Diagnostic Centre funding. Late-stage cancer is significantly more expensive to treat.
People	Delivery depends on diagnostic, oncology and nursing workforce, and on extended roles across primary and community care.
Reputational	Failure to improve access, diagnosis and treatment, particularly for communities experiencing inequalities, may affect public and partner confidence.



Return to Agenda

Birmingham, Black Country, and Solihull ICB Cancer Insight
9 November 2026

1. Headlines

- **We are finding cancer too late, and this has not changed since 2020.** 55.1% of cancers in Birmingham and Solihull and 56.2% in the Black Country are diagnosed at stage 1 or 2 (June 2026), against an ambition of 75% by 2035.
- **We refer and screen fewer people than England.** Urgent suspected cancer referral rates are about 22% (BSol) and 12% (Black Country) below England, and screening uptake is below England in all three programmes.
- **The gap is widest in our most deprived and some ethnic minority communities,** and emergency presentation, which is closely linked to deprivation, remains too common.
- **Diagnosis is faster, but treatment is not yet.** BSol meets the 28-day Faster Diagnosis Standard (85.2%); the Black Country is at 79.6%. 62-day performance is 72.1% and 71.8% against 80% by March 2027, held back by diagnostic capacity.
- **Our response is four shifts, delivered through five workstreams in the Cancer and Diagnostics Delivery Plan 2026-30:** find cancer sooner, unblock diagnostics, treat and support people differently, and close the gaps in variation and inequality.
- **Early community work is promising.** The cancer bus and the mobile cervical screening unit are reaching people the standard offer misses, including one early breast cancer found and treated and eight high-risk cervical results.

2. What the evidence tells us

Appendix A provides a summary of insight data and local context along with the developing approach and delivery plan for the response.

3. Outline of Board Session

Time	Item	Lead
2 min	Introduction and framing	Ian Sharp
20 min	Presentation: the national and local context, what the data tells us, our response and four proposed priorities, and risks	Sonia Ashraf / Phil Johns
15-20 min	Questions and discussion, structured around four questions set out below	Chair



Return to Agenda

4. Decisions required/ Assurance Offered

The Board is asked to:-

ASK 1	**Note the headlines** Note the headlines on cancer outcomes, access and inequalities across the two systems.
ASK 2	**Discuss our response and proposed priorities** Discuss whether the four shifts are the right response, and give a steer on four proposed priorities for the next 12 months: • one cancer control centre across BBCS, with a real-time view of delays • a diagnostic bottleneck programme covering radiology, endoscopy and pathology • a cancer navigator for every patient with suspected cancer • tumour-site deep dives, starting with lung, colorectal, prostate and breast
ASK 3	**Note the principal risks** Note the principal risks: workforce (red), diagnostic and Community Diagnostic Centre capacity (red), and continued transformation funding (amber).
ASK 4	**Agree the assurance route** Agree how and how often the Board wishes to receive assurance on cancer, and that the full strategy returns to the Board for approval.

4. Key risks

Risk	RAG	Summary
Workforce	**Red**	Recruitment and retention across diagnostics, oncology and nursing. The largest dependency, affecting every workstream.
Diagnostic and CDC capacity	**Red**	Without growth in CT, MRI, endoscopy and pathology capacity, the 62-day and 6-week diagnostic targets will not be met.
Transformation funding	**Amber**	Screening, prehabilitation, psychosocial support and pathway work depend on continued West Midlands Cancer Alliance funding.

Overall programme RAG: Amber. Full risk and dependency tables are in appendix D.



5. Implications

Area	Implication
Quality and safety	Late diagnosis and long waits carry risk of harm. Harm reviews and learning through the Patient Safety Incident Response Framework apply to long-waiting patients.
Health inequalities	Screening uptake and early diagnosis are lowest in deprived and some ethnic minority communities. Every measure will be reported by deprivation and ethnicity.
Finance	Several programmes depend on West Midlands Cancer Alliance and Community Diagnostic Centre funding. Late-stage cancer is significantly more expensive to treat.
Workforce	Delivery depends on diagnostic, oncology and nursing workforce, and on extended roles across primary and community care.
Regulatory	Cancer measures feed the NHS Oversight Framework and affect ICB and provider segmentation.
Governance	Delivery is overseen by a cluster-wide Strategic Cancer Executive, with assurance through provider contracts and the West Midlands Cancer Alliance.

6. Next steps

Owner	Action	By
Cancer & Diagnostic Team	Align the Cancer and Diagnostic strategic commissioning strategies across the cluster	Before strategy returns to Board
Cancer & Diagnostic Team	Update the cluster-wide cancer risk register	30.11.26
Strategic Cancer Executive	Agree trajectories for early-stage diagnosis, screening and emergency presentation	Before strategy returns to Board
Cancer & Diagnostic Team	Bring the aligned strategy and full data pack back to the Board for approval	Date to be agreed



Appendix

Cancer insight: supporting detail and sources

ICB Board, 9 November 2026. To be read with the cover sheet and presentation.

Section	Contents
A. The data	Stage at diagnosis, incidence and referrals, screening, inequalities, performance
B. Context	National Cancer Plan, NHS 10-Year Health Plan, BBCS Clinical Strategy, delivery plan and governance
C. Delivery plan	Five workstreams: leads and deliverables; research and innovation; workforce
D. Risks and dependencies	Risk and dependency tables with RAG ratings
E. Early evidence	Cancer bus and mobile cervical screening results
F. Outcome measures	How progress will be measured and reported
G. Source documents	References

A. The data

A1. Stage at diagnosis

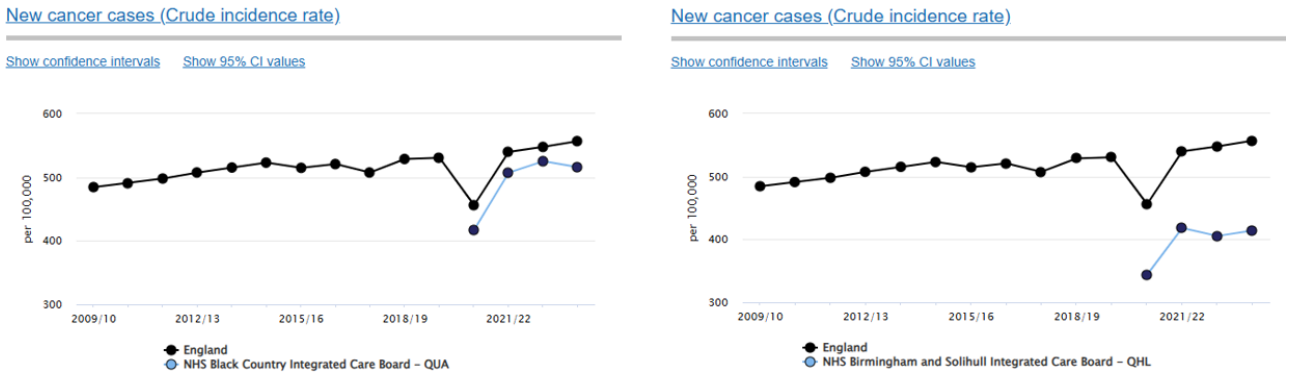
Cancer outcomes in both ICB areas are worse than the England average, and the gap is explained largely by late diagnosis. Late-stage diagnosis is associated with worse survival and is significantly more expensive to treat. The proportion of cancers diagnosed at stage 1 or 2 has not changed materially since February 2020 in either system.

12-month rolling average	Feb 2020	Jun 2025	Jun 2026
BSol: early-stage proportion	56.8%	57.4%	55.1%
BSol: new cancer diagnoses (observed)	496	539	554
Black Country: early-stage proportion	55.6%	56.0%	56.2%
Black Country: new cancer diagnoses (observed)	535	543	556

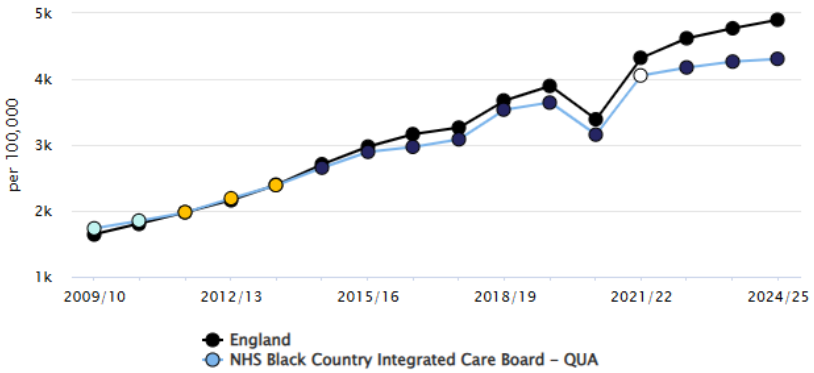
Source: Rapid Cancer Registration Data (RCRD), 12-month rolling average.

A2. Incidence and urgent suspected cancer referrals

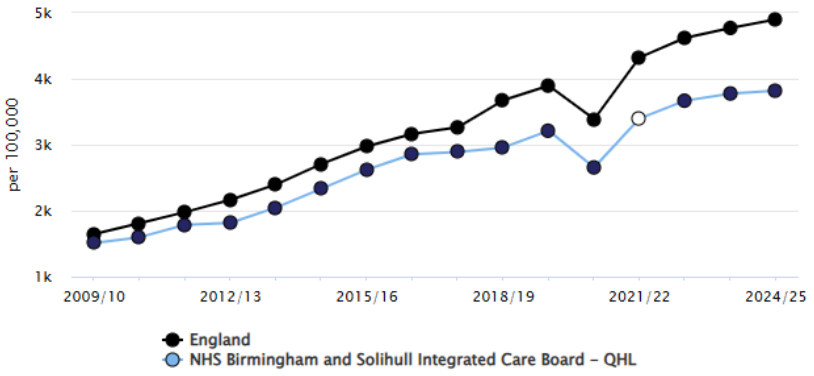
Crude cancer incidence is below England in both systems (about 415 per 100,000 in BSol and 517 in the Black Country, against about 557 for England in the latest year). Birmingham's younger population accounts for part of this, so lower crude incidence should not be read as lower need. Urgent suspected cancer referral rates are also below England: in 2024/25 about 3,800 per 100,000 in BSol and 4,300 in the Black Country, against about 4,900 for England. Conversion from referral to cancer diagnosis is around 5%.



Crude cancer incidence rate per 100,000: Black Country (left) and BSol (right) against England. Source: OHID Fingertips.



Urgent suspected cancer referrals per 100,000: Black Country against England. Source: OHID Fingertips.



Urgent suspected cancer referrals per 100,000: BSol against England. Source: OHID Fingertips.

A3. Screening

Screening uptake is below England in both systems for all three national programmes, and lowest in areas of high deprivation and in some ethnic minority groups.

Programme (latest year)	BSol	Black Country	England	Standard
Breast, women 50-70 (2024/25)	c.63%	c.68%	c.72%	70% acceptable
Bowel, people 60-74 (2024/25)	c.62%	c.64%	c.70%	60% acceptable
Cervical, 25-49 (2023/24)	c.61%	65.9%	c.68%	80%
Cervical, 50-64	83.5% (2023/24)	72.2% (2024/25)		80%

Figures marked 'c.' are read from the OHID Fingertips charts below and rounded. Highlighted figures must be confirmed against source before issue.

Changes over time:

- Breast screening has recovered since the pandemic in both systems: in BSol from 59.4% (2022/23) to 64.3% (2024/25, women 53-70), and in the Black Country from 63.6% to 66.7%. Both remain below the 70% target and England.
- Bowel screening in BSol fell from 64.8% (2022/23) to 62.9% (2023/24), a fall of 1.9 percentage points. It remains above the 60% acceptable standard but around 8 points below England.
- Cervical screening for people aged 25-49 is the largest gap in BSol, at around 7 points below England.

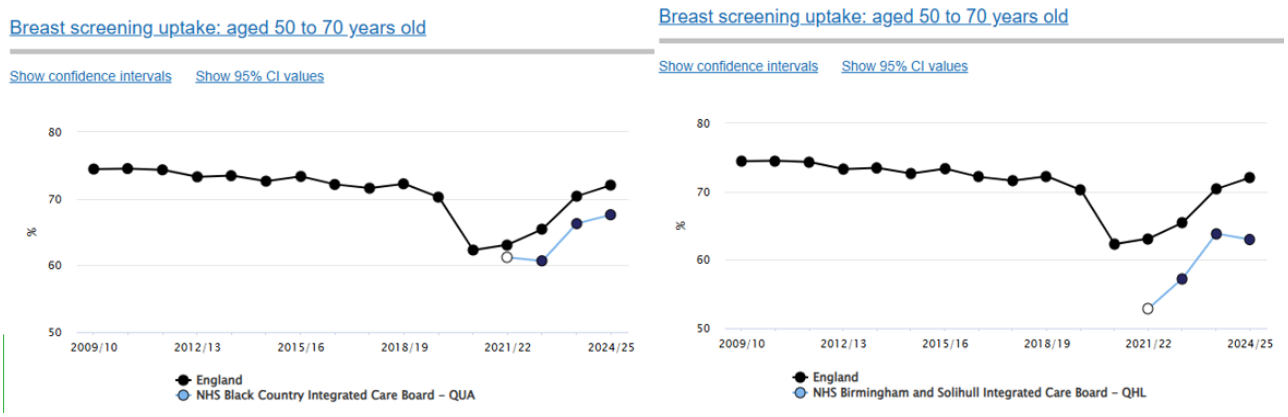


Fig 1. Breast screening uptake, women 50-70: Black Country (left) and BSol (right) against England.

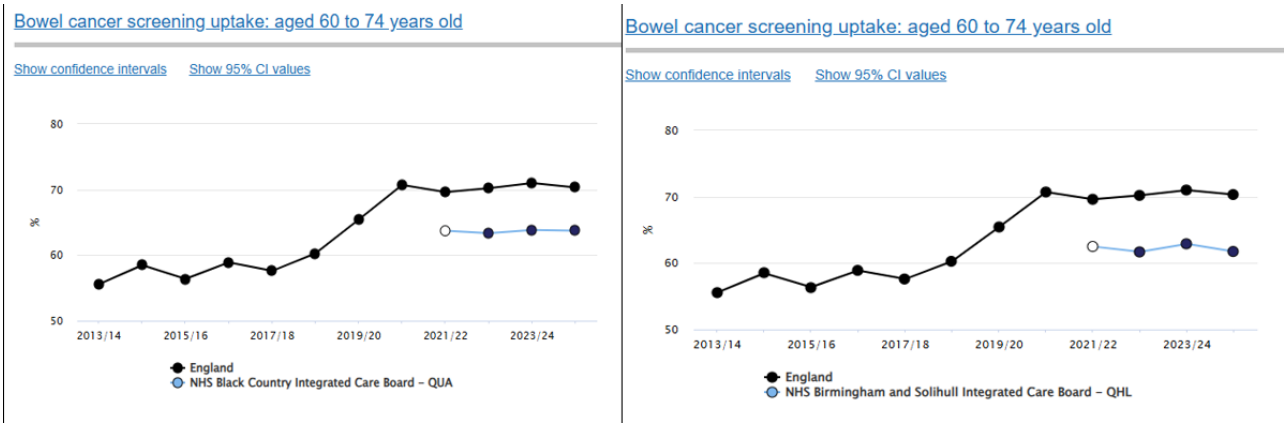


Fig 2. Bowel screening uptake, people 60-74: Black Country (left) and BSol (right) against England.

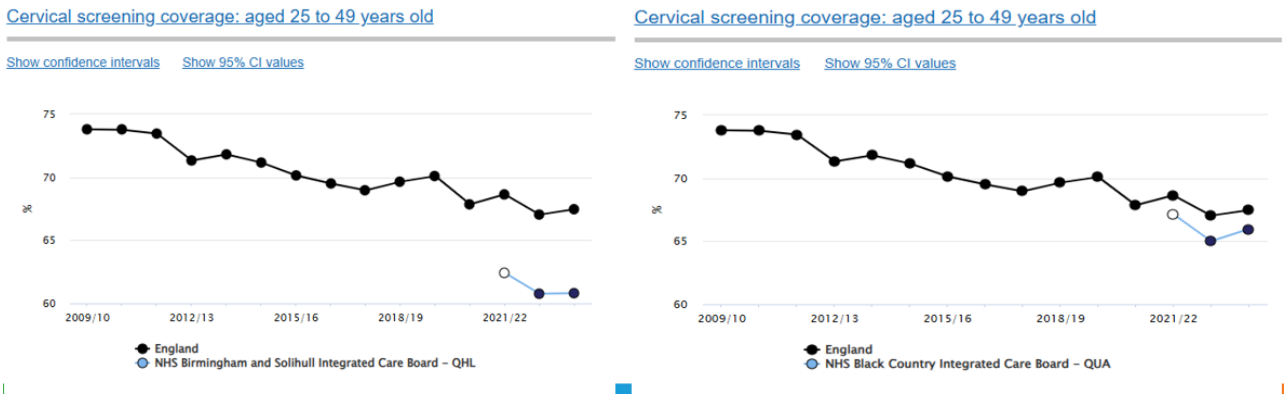


Fig 3. Cervical screening coverage, people 25-49: BSol (left) and Black Country (right) against England.

A4. Inequalities and community insight

- 35.4% of women invited do not attend breast screening appointments. Core20PLUS5 data shows attendance is typically lowest among Black Caribbean women.
- Pakistani women are markedly less likely to attend cervical screening (Core20PLUS5 figure of 61% to be confirmed and defined).
- Cancer Bus patient survey: 41% had not returned their bowel screening kit; 15% had not attended cervical screening; 12% had not responded to a breast screening invite.
- 70% of patients with cancer who have an acute admission die within 12 months (published data, Dr Ashing Lillis et al: full reference to be added).
- Emergency presentation with cancer is strongly associated with deprivation and remains too common across our footprint.

A5. Cancer and diagnostic performance

Standard	BSol	Black Country	Target
28-day Faster Diagnosis Standard (Aug 2026)	85.2%	79.6%	85% (delivery plan)
31-day decision to treat to treatment (Aug 2026)	94.7%	93.0%	96%
62-day referral to treatment (Aug 2026)	72.1%	71.8%	80% by March 2027
6-week diagnostic waits, DM01 (Jul 2026)	64.4%	81.5%	86% by March 2027
Longer-term diagnostic standard			98% within 6 weeks and no waits over 13 weeks by 2029

Source: national cancer waiting times and DM01 returns. Targets as stated in the Cancer and Diagnostics Delivery Plan; to be confirmed against 2026/27 planning guidance.

Performance against the 62-day standard is improving but remains well short of target. Diagnostic performance is furthest from standard in endoscopy, non-obstetric ultrasound and the physiological sciences. Cancer measures are included in the NHS Oversight Framework and contribute to ICB and provider segmentation.

To support 62-day delivery the programme is considering:

- Diagnostic capacity: CT, MRI, endoscopy, reporting radiologists, histopathology turnaround, Community Diagnostic Centre use, and seven-day reporting where needed.
- Earlier first appointments.
- A cancer flow control centre using a real-time dashboard of breaches, diagnostic queues and MDT, theatre, SACT and radiotherapy delays.
- A patient navigation model in which each patient has a cancer navigator or key worker, with digital tracking. Navigators book tests, chase results, escalate delays and support vulnerable patients.

B. Context

B1. National Cancer Plan

The National Cancer Plan for England, Delivering World Class Cancer Care, was published on 4 February 2026. It is the first dedicated national cancer plan since 2015 and sets a ten-year

direction. Its headline ambition is that three in four people diagnosed in 2035 will be cancer-free or living well five years later. It commits to meeting the cancer waiting time standards by the end of this Parliament, expanding diagnosis and care in the community, using digital and AI, and tackling cancers linked to deprivation.

The plan reflects the three shifts in the NHS 10-Year Health Plan: from hospital to community, from treating sickness to prevention, and from analogue to digital.

B2. BBCS Clinical Strategy

The outcome measures in the delivery plan (appendix F) reflect those in the BBCS Clinical Strategy: stage at diagnosis, screening uptake, faster diagnosis, emergency presentation, survival and equity.

B3. Cancer and Diagnostics Delivery Plan 2026-30

The Cancer and Diagnostics Delivery Plan is the strategic commissioning response for the Birmingham and Solihull and Black Country (BBCS) Cancer and Diagnostics Programme from 2026 to 2030. It aligns delivery of the Clinical Strategy with national cancer priorities, diagnostic transformation programmes and West Midlands Cancer Alliance (WMCA) investment programmes. The programme is aligning the cancer and diagnostic strategies across the two ICBs; the delivery plan will translate the National Cancer Plan into improvement over the next three years, and the aligned strategies will set out the plan for the following five.

B4. Governance

Delivery is overseen by a cluster-wide Strategic Cancer Executive, with delivery embedded across the ICBs, providers, primary care and regional partners. Assurance runs through contracts with individual providers. The WMCA supports providers to deliver and sustain cancer performance. The programme is also developing how cancer and diagnostic improvement can be embedded in anchor organisations.

C. Delivery plan by workstream

Each workstream has an ICB strategic commissioning lead and a GP cancer lead, and is developing a project plan with timelines.

Workstream	ICB lead(s)	Shift (presentation)
1. Cancer and diagnostic performance and access	Ann-Marie Wright / Jo Tolley	Unblock diagnostics
2. Prevention, early diagnosis and screening	Tracy Jones / Kafilla Munir	Find cancer sooner
3. Modernising treatment and reducing variation	Nurgis Shafiq	Treat and support differently
4. Pathway transformation	Melody Bridges / Nurgis Shafiq	Unblock diagnostics; close the gaps
5. Care quality and patient experience	Jo Tolley / Ann-Marie Wright	Treat and support differently

C1. Performance and access

- Single system cancer control centre across Birmingham, Solihull and the Black Country (proposed priority).

- Diagnostic bottleneck programme across radiology, pathology and endoscopy (proposed priority).
- 100% cancer navigator coverage for patients with suspected cancer (proposed priority).
- Tumour-site deep dives into lung, colorectal, prostate and breast pathways (proposed priority).
- Prioritising first appointments to support the 62-day standard.

C2. Prevention, early diagnosis and screening

Aim: increase the proportion of cancers diagnosed at stage 1 and 2, contributing to the ambition of 75% early-stage diagnosis by 2035. To do this both systems need to increase urgent suspected cancer referrals and raise screening uptake to at least the England average, and improve case-finding through new technologies and surveillance.

Deliverable	Status and next step
Targeted Lung Health Check / lung cancer screening	Procurement and commissioning across the remainder of BSol, Wolverhampton and Walsall
Mobile cervical screening	Four-day service in BSol since December 2025; extending to the Black Country (October 2026 per slides; by end of 2026/27 per draft paper)
Self-referral chest X-ray, East Birmingham	Pilot began August 2026; evaluate before wider roll-out
Community and primary care Cancer Champions; cancer bus outreach	In place; extending through neighbourhoods
Screening uptake programmes: bowel, breast, cervical	Ongoing, targeted at low-uptake practices and communities
Prevention: smoking, obesity, alcohol, HPV, skin cancer	Working at place level with local authorities
Prostate cancer risk assessment and targeted PSA	Including support for the WMCA prostate case-finding pilot
Neighbourhood cancer and diagnostics offer	Scoping with developing neighbourhood teams
Surveillance and case-finding: liver (HCC), Barrett's oesophagus, post-menopausal bleeding	WMCA and CDC pathway funding received for liver

C3. Modernising treatment and reducing variation

Deliverable	Status and next step
Comprehensive psychosocial support model	Mapping complete; WMCA-funded post being recruited to develop the model and business case
Regional oncology sustainability programme: workforce, demand and capacity, SACT models	Supported by provider chief medical officers and the WMCA deputy medical director

Deliverable	Status and next step
Clinical trial participation, especially in deprived and minority ethnic populations	Working with the West Midlands Innovation Hub on funding and targeted approaches
More treatment closer to home	Linked to the oncology programme and the neighbourhood strategy
GIRFT recommendations	Providers implementing through tumour-specific groups
Acute and supportive oncology	Primary care referral to local SDEC; automated notification to oncologist or CNS after an acute admission; advance care planning

C4. Pathway transformation

Challenged pathways across the cluster: colorectal / lower GI, urology, gynaecology, breast, and head and neck.

Deliverable	Pathways supported
TQuest and symptomatic FIT, including a FIT-negative pathway in the Black Country	Colorectal
Reflex testing for iron-deficiency anaemia	Colorectal referral quality
Beacon clinical decision support (IPP): upper GI, lower GI, gastroenterology	Colorectal recovery
Gynaecology tertiary pathway review	Gynaecology
Sarcoma, paediatric and aligned non-specific symptoms pathways	Upper and lower GI, gynaecology, urology
Straight-to-test: five pathways in place, more in development	Urology, lower GI, gynaecology, head and neck
Community Diagnostic Centre pathways (post-menopausal bleeding and unscheduled bleeding on HRT in place)	Gynaecology and others
Breast pain pilot; direct access breast pathway at MMUH	Breast
Colon capsule endoscopy for low-risk suspected colorectal cancer	Colorectal

Further proposals:

- Capsule sponge endoscopy for Barrett's oesophagus surveillance and case-finding, including through community pharmacy.
- Extend the mobile cervical screening unit into a mobile women's health hub, and develop a men's health van with point-of-care PSA testing and direct referral for mpMRI.
- Further cancer bus tours through neighbourhoods; extend self-referral chest X-ray after evaluation.
- Research and innovation: £6m NIHR funding to the University of Birmingham for multi-cancer detection testing in symptomatic patients with vague abdominal symptoms; Dxcover liquid-biopsy test for the brain cancer pathway; Nonacus GALEAS urine test for bladder cancer; genomic testing. These aim to improve referral quality and conversion.

C5. Care quality and patient experience

Deliverable	Status and next step
Improvement plans from the National Cancer Patient Experience Survey	Developing primary care plans with GP clinical leads
Personalised stratified follow-up	In place in most providers; further work at SWBH and UHB
Prehabilitation across BSol and the Black Country	WMCA funding received; projects under way, including a patient app
Cancer Care Reviews in primary care	Strengthening
Patient safety	Harm reviews for long-waiting patients, with learning through PSIRF

C6. Cross-cutting themes and workforce

Themes running through the plan: reducing health inequalities; prevention and population health; diagnostic transformation; innovation and digital; workforce sustainability; community-based care; research and clinical trials; data-driven performance management.

A different cancer workforce is proposed: primary care cancer nurses linking primary and secondary care; nurse-led secondary care roles such as LATP prostate biopsy; dentists and optometrists supporting oral cancer detection and prevention advice; and endoscopy activity shifting to trained allied health professionals, including colon capsule and capsule sponge endoscopy in the community.

D. Risks and dependencies

D1. Risks

Risk	RAG	Description	Potential impact
Workforce capacity and sustainability	**Red**	Diagnostics, screening, pathway, genomics and oncology delivery depends on recruiting and retaining specialist clinical, diagnostic and managerial staff.	Standards missed, longer waits, reduced resilience, transformation at risk.
Diagnostic capacity	**Amber**	62-day, FDS and 6-week standards rely on use and expansion of CDCs, endoscopy, imaging and diagnostics.	Delayed diagnosis, backlog pressure, 2027 and 2029 targets missed.
Transformation funding	**Amber**	Major programmes depend on WMCA funding, national screening programmes and business case approval.	Prevention, screening, psychosocial and pathway work reduced or delayed.
Provider delivery and collaboration	**Amber**	Requires coordinated delivery across the ICBs, UHB, Black Country providers, primary care and specialised commissioning.	Slower redesign, continued variation, system benefits not realised.
Rising demand	**Amber**	Rising incidence, an ageing population and diagnostic demand may exceed planned capacity.	Continued performance pressure; slower early-diagnosis gains.

Risk	RAG	Description	Potential impact
National dependencies	**Green**	Some actions depend on national cancer manuals, GIRFT, specialised commissioning and screening programme decisions.	Delays to timelines and benefits.
Digital and innovation delivery	**Amber**	Beacon/IPP, NHS App, TQuest and other digital programmes need successful implementation and adoption.	Lower productivity gains; slower pathway improvement.
Inequalities and screening uptake	**Amber**	Early-diagnosis gains depend on higher engagement from underserved populations.	Inequalities persist; stage shift not achieved.

D2. Critical dependencies

Dependency	RAG	Commentary
Continued WMCA transformation funding	**Amber**	Supports prevention, screening, pathway redesign and care quality programmes.
Community Diagnostic Centre delivery	**Red**	Underpins diagnostic capacity growth, pathway redesign and earlier diagnosis.
Workforce recruitment and retention	**Red**	Largest system-wide dependency, affecting every workstream.
Provider collaboration across BBCS	**Amber**	Pathway standardisation, mutual aid and GIRFT implementation.
Primary care engagement	**Amber**	Earlier diagnosis, screening, Cancer Champions and referral optimisation.
National screening programme developments	**Green**	Bowel, cervical and lung screening ambitions.
Regional genomics and oncology programmes	**Amber**	Precision medicine, workforce sustainability and treatment transformation.
Digital infrastructure and data	**Amber**	Performance monitoring, pathway redesign and patient-facing digital services.

Overall programme RAG: Amber. The plan aligns with national and regional priorities. Subject to the dependencies above being managed through the Cancer and Diagnostic governance structure, most planned milestones remain achievable.

E. Early evidence

E1. Big Red Cancer Information Bus

The bus promotes the national screening programmes and lung and liver screening, raises awareness of signs and symptoms, tackles myths and barriers, offers health checks and clinical advice, and gives access to GPs, health professionals and screening practitioners.

- Over 2,000 health checks on board: 14 escalated to primary care and 1 to NHS 111.
- 2,850 information bags distributed.
- 20 skin lesions referred; 2 breast urgent suspected cancer referrals, with 1 early breast cancer found and treated; 1 bowel cancer referral.

E2. Mobile cervical screening unit (first three months)

- 236 people educated on cervical, breast and bowel screening.
- 130 cervical screens completed: 13 HPV positive, 8 high-risk with possible pre-cancerous cells.
- 10 co-produced, translated resources and 4 patient information videos for primary care.
- Evaluation under way with Birmingham City University.

F. Outcome measures

Outcome	Measure
Stage at diagnosis	% of cancers diagnosed at stage 1 or 2
Screening uptake	Bowel, breast and cervical coverage by practice, PCN and ethnicity (ward-level data via local authorities)
Diagnostic speed	% meeting the 28-day Faster Diagnosis Standard
Emergency presentation	% of cancers first presenting as an emergency admission
Survival	1-year and 5-year age-standardised survival
Equity	Gap in screening uptake between most and least deprived quintiles
Lung screening	Participants in lung cancer screening; uptake by deprivation and ethnicity
Vaccination	% of eligible young people receiving HPV vaccination; hepatitis B vaccination in high-risk groups

These reflect the outcomes in the BBCS Clinical Strategy and will be reported for both Birmingham and Solihull and the Black Country. A full data pack covering these measures, treatment variation and patient experience is in development.

G. Source documents

1. Department of Health and Social Care. National Cancer Plan for England: Delivering World Class Cancer Care. February 2026.
2. NHS 10-Year Health Plan for England.
3. BBCS Clinical Strategy.
4. BBCS Cancer and Diagnostics Delivery Plan 2026-30 (spreadsheet held by the Cancer and Diagnostics team).
5. Rapid Cancer Registration Data, NHS England.
6. OHID Fingertips: Cancer Services profiles (incidence, referrals, screening).
7. NHS England cancer waiting times and DM01 diagnostic waiting times statistics.
8. Core20PLUS5 screening data; Cancer Bus patient survey; mobile cervical screening unit monitoring.

NHS Birmingham and Solihull

NHS Black Country

		Agenda No: 10
Report to:	Joint Cluster Board	
Date of Meeting:	09 November 2026	
Title:	Decision-Making Business Case – Birmingham & Solihull UTC & GP Out of Hours Review	
Presenting Officer:	Dr Barbara King	
Author(s) of Report:	Claudia Sharples	
To be Withheld from Publication?	**Yes** ☐ **No** ☒	**(If Yes) Rationale:**
Relevant Conflicts of Interest	N/A	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☒	Digital Transformation	☐	Treatment to Prevention	☐

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☒

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☐

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☐	**Digital First**	☐
Financial Discipline/ 2% productivity	☐	**Advancing Health Equity**	☐	**Integrated Partnership**	☐

Purpose:
This document is provided for decision-making purposes, seeking ICB Public Board endorsement to proceed to the implementation and mobilisation phase of the programme and to note the proposed implementation arrangements and next steps.

Decision Required			
Approval	☒	**Accept as Assurance**	☐
Endorsement	☒	**Noted**	☐

Return to Agenda

Report Journey:

- 20 November 2024 - NHS England Stage 1 Strategic Sense Check
- 20 January 2026 - NHS England Stage 2 Assurance
- 5 February – 16 April 2026 - Formal Public Consultation
- 24 September 2026 - Independent Post-Consultation Options Review Workshop
- 15 October 2026 - UTC Programme Board
- 26 October 2026 - Executive Board
- 9 November 2026 - ICB Public Board (Decision Required)

Executive Summary

The Birmingham and Solihull UTC and GP OOH Strategic Review was undertaken following publication of the NHS England Urgent Treatment Centre Principles and Standards and recognition that current urgent care provision across Birmingham and Solihull is variable, confusing for patients and does not consistently meet national standards. Birmingham and Solihull serves approximately 1.3 million residents with significant variation in deprivation, health need and access to services.

The review has included extensive clinical engagement, NHS England assurance, independent review by the West Midlands Clinical Senate, public consultation and scrutiny engagement. Public consultation ran from 5 February to 16 April 2026, generating over 1,600 contributions from patients, carers, staff and stakeholders. Feedback highlighted the importance of maintaining local access, face-to-face care, walk-in services, improved diagnostics and clear navigation whilst supporting improved consistency of urgent care provision.

The preferred model was identified through a robust appraisal process that included options development, weighted options appraisal, public consultation, stakeholder engagement, scrutiny review, quality and equality assessments, financial modelling and implementation planning. Option A achieved the highest appraisal score and was subsequently subject to an independent post-consultation review. Following consideration of consultation feedback and additional evidence, the Programme Board recommended a refined Option A Plus model, which retains the strengths of the original preferred option whilst enhancing locality-based urgent care provision and responding directly to issues raised through consultation. This option was considered to provide the best balance of clinical quality, accessibility, deliverability, workforce sustainability and affordability.

The recommended model **Option A Plus** comprises:

- Four co-located UTCs at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital.
- Three community UTCs at Solihull, Erdington and South Birmingham/Selly Oak.
- Enhanced locality urgent care arrangements at Summerfield and Washwood Heath.
- GP Out of Hours services commissioned through three locality-based lots covering South/Central, North/West and Solihull/East.
- Continued walk-in and NHS 111 access routes supported by improved diagnostics, clinical pathways and integration with wider urgent and emergency care services.

Option A Plus achieved the strongest overall balance across the agreed appraisal criteria, including workforce sustainability, deliverability, patient navigation and compliance with national UTC standards, whilst retaining significant community-based provision and responding directly to consultation feedback through the locality-based GP OOH model.



Return to Agenda

Appendices

Decision-Making Business Case – BSoL UTC & GP OOHs Review
Appendices:

1. *UTC & GP OOH Strategic Review DMBC*
2. *Supplementary Locality Analysis*
3. *Consultation Plan*
4. *Healthwatch Report*
5. *Public Consultation Document*
6. *Summary Consultation Document*
7. *Easy Read Consultation Document*
8. *Consultation Survey*
9. *Independent Consultation Report*
10. *HOSC Feedback*
11. *Detailed Financial Modelling*
12. *ADEPT Estates Report*
13. *BSoL Digital Strategy*
14. *Digital & UTC Self-Assessment Tool*
15. *QEIA*
16. *Travel Time Analysis*
17. *Programme Risk Register*
18. *Implementation Gantt Chart*

Recommendations & Next Steps

The Joint Cluster Board is asked to:

1. Endorse the recommendations contained within the Decision-Making Business Case.
2. Support Option A Plus as the preferred future configuration for UTC and GP Out of Hours services across Birmingham and Solihull.
3. Note the planned procurement, mobilisation and implementation programme, subject to approval by the ICB Public Board.

Following Board approval, the programme will progress to:

- Detailed implementation planning.
- Procurement of community UTC and GP Out of Hours services.
- Co-located UTC mobilisation with Acute Trust partners.
- Workforce, estates and digital readiness activities.
- Benefits realisation and evaluation arrangements.
- Ongoing stakeholder engagement and communications

Assessments:

Impact Assessments Completed		**Yes**	**No**	**N/A**	**Details (N= explain why: Y= approval date)**
	QIA	☒	☐	☐	March 2025 (For WM Clinical Senate) Updated January 2026 (For pre-consultation business case)

					Updated October 2026 as per outputs from Public Consultation.
	EQIA	☒	☐	☐	March 2025 (For WM Clinical Senate) Updated January 2026 (For pre-consultation business case) Updated October 2026 as per outputs from Public Consultation.
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	The preferred model will standardise UTC provision in line with NHS England standards, improve access to diagnostics, strengthen clinical governance and support more effective streaming between UTCs and Emergency Departments. An Equality & Quality Impact Assessment has been completed and consistently updated as part of the programme.
Legal and Regulatory	The programme has fulfilled NHS service change requirements including public consultation, equality duties, HOSC engagement and NHS England assurance. Formal public consultation and scrutiny processes have informed the recommendation.
Finance	Financial modelling estimates annual UTC costs of approximately £24.7m under Option A. The model is considered affordable within the existing financial envelope, although final tariff and contracting arrangements will require confirmation during implementation.
People	Implementation will require workforce transition, recruitment and mobilisation activities. Workforce planning, TUPE/COSOP considerations and provider workforce assurance arrangements will form part of procurement and mobilisation.
Reputational	The review has attracted significant public and stakeholder interest. Maintaining transparency, demonstrating how consultation feedback has informed the recommendation and delivering effective communication throughout implementation will be key to maintaining stakeholder confidence.

Return to Agenda

DECISION-MAKING BUSINESS CASE – UTC & GP OUT OF HOURS REVIEW

9th November 2026

1. Why this matters now

1.1 Urgent Treatment Centre (UTC) and GP Out of Hours (OOH) services across Birmingham and Solihull currently operate through a variable model that does not consistently meet NHS England Urgent Treatment Centre Principles and Standards. Variation in service offer, diagnostics, clinical leadership and access arrangements has contributed to patient confusion, inconsistent access and increasing pressure on Emergency Departments.

The UTC Strategic Review has been undertaken to develop a clinically sustainable, equitable and financially affordable urgent care model that aligns with national policy, supports the NHS 10 Year Health Plan, improves patient experience and delivers more consistent urgent care services across Birmingham and Solihull.

A Decision-Making Business Case (DMBC) has now been completed following consultation, scrutiny, NHS England assurance, options appraisal and further post-consultation review and seeks endorsement of the recommended Option A Plus model prior to submission to the ICB Public Board on 9 November 2026.

2. What the evidence tells us

2.1 The review has demonstrated:

- Current UTC provision varies significantly across Birmingham and Solihull and does not consistently meet national UTC standards.
- Emergency Departments continue to experience sustained pressure and significant levels of UTC-amenable activity.
- Public consultation generated over 1,600 responses and identified strong support for maintaining local access, face-to-face care and walk-in services whilst improving consistency and access to diagnostics.
- Consultation findings did not identify a clear preference for either option but demonstrated support for elements of both models.
- The original weighted options appraisal scored Option A higher than Option B (434 versus 410).
- A post-consultation independent options review confirmed that the original appraisal remained valid.
- Quality, Equality and Health Inequalities assessments identified no impacts that would outweigh the benefits of the recommended model provided mitigations are implemented.
- Financial, workforce, digital and estates assessments support progression of the preferred option.



Key improvements

Subject to Board approval, the recommended model will deliver a more consistent, accessible and sustainable urgent care offer across Birmingham and Solihull, with significant investment in facilities and services. The key improvements will be:

- **More consistent urgent care:** seven Urgent Treatment Centres (UTCs) operating to consistent national NHS standards, seven days a week for at least 12 hours a day, providing a clearer and more consistent offer for patients across Birmingham and Solihull.
- **Significant investment in urgent care facilities:** four hospital-based UTCs at Queen Elizabeth Hospital Birmingham, Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital, including new facilities and investment in the urgent care estate.
- **Better integration of urgent and emergency care:** locating UTCs alongside major emergency departments will help patients with urgent but non-life-threatening conditions get to the right clinical team, while allowing Emergency Departments to focus on people requiring emergency care.
- **Retaining and improving community UTC provision:** UTCs at Solihull Hospital, Stockland Green Primary Care Centre in Erdington and Katie Road in Selly Oak will be retained and improved to meet consistent national standards.
- **Urgent care shaped around local population needs:** New locality-based urgent care hubs in East and West Birmingham will provide targeted specialist services aligned to local health needs. Summerfield will deliver enhanced respiratory and diabetes pathways, while Washwood Heath will provide a rapid access heart failure service. Together these services will strengthen preventative care, support people with long-term conditions during periods of deterioration, and provide alternatives to hospital attendance through multidisciplinary community-based care closer to home.
- **A redesigned GP out-of-hours service:** patients across Birmingham and Solihull will continue to access GP out-of-hours care through NHS 111, with remote consultations, face-to-face appointments and home visits provided according to clinical need.
- **Clearer and simpler access for patients:** people will be able to walk into a UTC or access an appointment through NHS 111, with consistent diagnostic capabilities and clearer pathways into other NHS services where further care is needed.
- **Responding to public consultation:** the recommended model brings together elements of the options consulted on, reflecting feedback from more than 1,600 participants, particularly the importance people placed on accessible local services alongside improved quality and consistency of the offer.
- **A phased and managed transition:** implementation will take place over time, with no immediate change to existing services or how patients access them. Detailed implementation will be developed with providers, patients and communities.
- **Strengthening the role of community anchor organisations:** As the board are aware UTCs would fall within the proposed scope of NHS Anchor arrangements. As this process pre dates these arrangements the ICB will work alongside partners in UHB to lead on the procurement of the new UTCs. UHB and SWBH colleagues have been engaged in working up likely activity numbers and patient flows and in the proposed clinical model and will work with us in developing the likely UTC+ offer and



on additional services to impact on urgent care in east and west Birmingham in the preferred option linked into their wider services.

3. The local picture

3.1 The recommended Option A Plus model provides:

- Four co-located UTCs at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital.
- Three community UTCs at Solihull, Erdington and South Birmingham/Selly Oak.
- Enhanced locality urgent care services at Summerfield and Washwood Heath.
- GP Out of Hours services commissioned through three locality lots:
    - South/Central
    - North/West
    - Solihull/East

The model retains community-based provision whilst increasing compliance with national UTC standards, improving access to diagnostics, supporting Emergency Department streaming and responding directly to consultation feedback regarding local access and community-based urgent care.

4. Decisions required/ Assurance Offered

DECISIONS:-

The Board is asked to make the following decisions:-.

DECISION 1	**DECISION 1** Endorse the Decision-Making Business Case (DMBC) for the Birmingham and Solihull UTC and GP Out of Hours Strategic Review.
DECISION 2	**DECISION 2** Support Option A Plus as the preferred future model for urgent care services across Birmingham and Solihull.
DECISION 3	**DECISION 3** Note the proposed implementation, procurement, mobilisation and benefits realisation arrangements following Board approval.

ASSURANCE:-

The Board is asked to accept the following level of Assurance:-.



ALERT	Matters of Concern or Escalation, and gaps in Assurance where there is minimal or limited Assurance N/a

ADVISE	Matters of reasonable assurance including Actions Commissioned and Progressed For Information N/a

ASSURE	The Joint Cluster Board is asked to take significant assurance that the Birmingham and Solihull UTC and GP Out of Hours Strategic Review has completed the required business case development, assurance and consultation processes and that the Decision-Making Business Case provides a robust evidence base for decision making. Assurance is provided through: • Completion of NHS England Stage 1 and Stage 2 assurance processes. • West Midlands Clinical Senate Clinical Review • Public consultation and scrutiny requirements have been completed. • Findings from consultation, scrutiny and locality analysis have informed the final recommended model. • Equality, quality, access and health inequalities impacts have been assessed. • Financial, workforce, estates and implementation assessments have been undertaken. • A comprehensive risk register, benefits realisation framework and implementation plan are in place. • UTC Programme Board approval 15th October 2026 & ICB Executive Board – 26th October 2026.

5. Associated Risks

The Committee should note the following risks.

Risk	Likely consequence
Insufficient procurement capacity and specialist support available to deliver the	Delays to procurement timelines, increased risk of non-compliance with

UTC and GP Out of Hours procurement programme.	procurement regulations and delayed implementation of the approved model.
Digital integration requirements and future CSU/GPIT support arrangements are not fully established.	Delays to mobilisation, inability to achieve full interoperability between services and reduced effectiveness of urgent care pathways.
Delays to delivery of co-located UTC estate developments.	Delayed implementation of the preferred model and postponement of anticipated patient and system benefits.
Community estate, planning permission or landlord approval issues impact delivery of community UTC and locality urgent care facilities.	Inability to establish services in the required locations, increased costs and programme delays.
Delay in development and mobilisation of locality urgent care services through the East and West Anchor Organisations.	Reduced consistency of urgent care provision across Birmingham and Solihull and delayed realisation of benefits associated with the preferred model.

6. Proposed Next Steps

Subject to Committee decisions, The following actions are proposed over an 18 month initial period.

Owner	Action	By
Programme Manager	Obtain ICB Public Board approval of the DMBC.	November 2026
Programme Team	Finalise procurement approach, service specifications and procurement documentation.	Jan-Mar 2026
Acute Trust/ICB Commissioning Team	Mobilise co-located UTCs	Nov 26 - Jul 2027
Programme Team	Mobilise services, including workforce, estates, digital and operational readiness activities.	Mar-May 2026
Procurement	Undertake procurement and contract award processes.	May 27-Mar 2028
Programme Team	Implement the approved UTC and GP Out of Hours model through a phased transition programme.	1st April 2028
Programme Team	Monitor delivery, benefits realisation and service outcomes through established governance arrangements.	Post April 2028



Return to Agenda

Appendix: Source Documents

This paper draws on the following source documents:

- Decision-Making Business Case – BSoL UTC & GP Out of Hours Review
- Appendix 1: CLEAN_BSOL UTC Strategy PCBC V1.6 FINAL_260125
- Appendix 2: Supplementary Locality Analysis
- Appendix 3: Consultation Plan
- Appendix 4: Healthwatch Report
- Appendix 5: Public Consultation Document
- Appendix 6: Summary Public Consultation Document
- Appendix 7: Easy Read Consultation Document
- Appendix 8: Consultation Survey
- Appendix 9: Independent Consultation Report
- Appendix 10: HOSC Feedback
- Appendix 11: Annex 1 Detailed Financial Modelling Option A
- Appendix 12: ADEPT BSoL Estates Report
- Appendix 13: BSoL Digital Strategy Full Version_29.09.2022
- Appendix 14: BSoL Digital & Enhanced Self-Assessment Tool_Oct26
- Appendix 15: UTC & GP OOHs Review QEIA_Option APlus_Oct26
- Appendix 16: UTC Travel Time Analysis Summary_v4_Option APlus
- Appendix 17: BSoL UTC & GP OOOHs Risk Register_v7_Oct2026
- Appendix 18: UTC & GP OOHs Review Implementation GANT Chart

NHS
Birmingham and Solihull

NHS
Black Country

Decision-Making Business Case

Urgent Treatment Centre and GP Out of Hours Review

Birmingham and Solihull

November 2026

AI Assisted Content.
Human Reviewed.



BSol ICB Changing UTC & GP Out of Hours provision: decision-making business case

Decision-Making Business Case Approval:

Area	Chair Name & Title	Signature	Date	Comments
UTC Programme Board	**Philip Johns**		**15 October 2026**	
ICB Executive Board	**David Melboune**		**26 October 2026**	
BSoL & BC ICB Public Board	**Danielle Oum**		**9 November 2026**	

Document Amendment History:

Modified By	Date Modified	Version	Summary of Changes Made
Claudia Sharples	*08/07/2026*	*1.0*	*Initial draft*
Claudia Sharples	*28/09/2026*	*1.1*	*Amendments to initial draft*
Claudia Sharples	*09/10/2026*	*1.2*	*Inclusion of Consultation and Engagement Chapter*
Claudia Sharples	*12/10/2026*	*1.3*	*Inclusion of Finance & Risk Chapter*
Claudia Sharples	*14/10/2026*	*1.4*	*Inclusion of Estates and updated Appendices*
Claudia Sharples	*19/10/2026*	*1.5*	*Amendments following commentary from UTC Programme Board*
Clauda Sharples	*26/10/2026*	*1.6*	*Updated to include locality description and commentary from ICB executive board*

Contents

1 EXECUTIVE SUMMARY 8

1.1 Purpose of this Decision-Making Business Case and Need for Change 8
1.2 Current services and proposals for change 8
1.3 Consultation, engagement and consideration of feedback 10
1.4 Options review and recommendation 11
1.5 Impact analysis 12
1.6 Stakeholder engagement and assurance 13
1.7 Next steps 13
1.8 Conclusion 13

2 PURPOSE AND Case for Decision 14

2.1 Purpose of this decision-making business case 14
2.2 Decision Required 14
2.3 Background to Urgent Treatment Centre strategic review 15
2.4 The scope of the decision 15
2.5 Structure of pre-consultation business case 16

3 STRATEGIC CONTEXT 16

3.1 Local geography and demographics: Birmingham and Solihull 16
3.2 NHS localities in Birmingham and Solihull 18
3.3 10-year Health Plan for England 18
3.4 NHS Medium term planning framework 2026/27 to 2028/29 19
3.5 National principles and standards for UTCs 19
3.6 National capital investment in UTCs 20
3.7 Local strategies 20

4 CURRENT ARRANGEMENTS AND PERFORMANCE 21

4.1 Current hospital emergency department provision and performance 21
4.2 Current provision of UTC services in Birmingham and Solihull 22
4.3 UTC presenting conditions. 26
4.4 Current UTC activity 27
4.5 Data provision and data quality 29
4.6 GP out of hours (OOH) services 29
4.7 Key Implications for preferred option 32

5 CASE FOR CHANGE 33

5.1 Deprivation in Birmingham and Solihull & Population Need 33
5.2 Health inequalities and access 34
5.3 Current demand and Changing patterns of Urgent Care 36
5.4 Current UTC model and service limitations 37
5.5 Opportunities to shift activity from EDs to UTCs 38
5.6 Opportunities to improve urgent care pathways and shift activity 39
5.7 What this means for the future model 39

6 Future URGENT CARE MODEL & Service configuration 40

6.1 Vision for Future Urgent Care Model 40
6.2 Principles Underpinning the Future Model 41
6.3 Overview of Preferred Service Configuration 41
6.4 Co-located Urgent Treatment Centres 44
6.5 Community Urgent Treatment Centres 46

6.6 Enhanced Locality Urgent Care Service 48
6.7 GP Out of Hours 53
6.8 Workforce, Estates and Digital Enablers 55
6.9 Access, Choice and Inclusion 56
6.10 Key Implementation enablers 56

7 Consultation, Engagement and Response 57

7.1 Overview 57
7.2 2. How we listened 57
7.3 What we heard 61
7.4 Health Overview and Scrutiny 63
7.5 How we responded 65
7.6 Next steps 66

8 Options Appraisal and Recommendation 67

8.1 Purpose and approach 67
8.2 Options Considered 70
8.3 Comparative Assessment of the Options 77
8.4 Comparative Assessment – Service Model and Clinical Configuration 79
8.5 Comparative Assessment – Activity, Capacity and System flows 81
8.6 Comparative Assessment – Estates 82
8.7 Comparative assessment – Finance 83
8.8 Comparative assessment – Access and Health Inequalities 84
8.9 Comparative assessment – Workforce 85
8.10 Comparative assessment – Deliverability 86
8.11 Weighted Options Appraisal 86
8.12 Overall Comparative Assessment 88
8.13 Recommended option – Option A as amended 89

9 Deliverability ASSESsment 91

9.1 Capacity, Demand and Activity Modelling 91
9.2 Demand modelling 92
9.3 Option A demand modelling 92
9.4 Option A capacity modelling 94
9.5 Relationship to the Recommended Option A Plus Model 94
9.6 Financial modelling 95
9.6 Implications for procurement and implementation 101
9.8 Conclusion 102

10 ENABLERS 102

10.1 Workforce Overview 102
10.2 Multidisciplinary working and workforce resilience 103
10.3 Workforce Considerations from Consultation 104
10.4 Workforce Implications of the Recommended Model 105
10.5 Workforce risks and mitigation 107
10.6 Workforce Conclusion 111
10.7 Estates and sustainability 111
10.8 Digital 118

11 Quality, Equality, Access and Health Inequalities 120

11.1 Introduction 120
11.2 Quality impact of the recommended model 120

11.3 Potential adverse impacts on quality ... 121
11.4 Equality and health inequalities ... 122
11.5 Impact on population groups ... 123
11.6 Travel and accessibility ... 124
11.7 Public transport, parking and affordability ... 126
11.8 Maintaining inclusive access ... 127
11.9 Impact of consultation and scrutiny ... 127
11.10 Role of Quality, Equality, Access and Health Inequalities in Decision-Making ... 128
11.11 Mitigation of identified impacts ... 129
11.12 Conclusion ... 130

12 BENEFITS, Risks & Assurance ... 131

12.1 Benefits Overview ... 131
12.2 Benefits framework ... 131
12.3 UTC benefits and measures ... 132
12.4 GP Out of Hours benefits and measures ... 134
12.5 System-wide outcome measures ... 135
12.6 Right place, first time and patient experience ... 135
12.7 Benefits realisation and monitoring arrangements ... 136
12.8 Monitoring access, equality and health inequalities ... 137
12.9 Consultation and scrutiny feedback informing implementation ... 137
12.10 Post-implementation evaluation ... 138
12.11 Benefits Realisation Conclusion ... 139
12.12 Risks and Mitigations ... 139
12.13 Assurance Undertaken ... 143

13 IMPLEMENTATION PLANNING ... 144

13.1 Implementation approach and timeline ... 144
13.2 Procurement considerations ... 145
13.3 Contracting approach and procurement routes ... 145
13.4 Current Contractual Arrangements and Transition Approach ... 146
13.5 Mobilisation and transition ... 147
13.6 Implementation planning and assurance ... 147
13.7 Urgent Treatment Centre Designation ... 148
13.8 Co-located UTC Implementation ... 148
13.9 Implementation conclusion ... 150

14 Decision, APPROVALS AND NEXT STEPS ... 150

14.1 Decision Requested ... 150
14.2 Governance and approval process ... 150
14.3 Actions following approval ... 151
14.4 Next steps ... 151
14.5 Conclusion ... 151

15 APPENDICES ... 152

15.1 Appendix 1: CLEAN_BSOL UTC Strategy PCBC V1.6 FINAL_260125 ... 152
15.2 Appendix 2: Supplementary Locality Analysis ... 152
15.3 Appendix 3: Consultation Plan ... 152
15.4 Appendix 4: Healthwatch Report ... 152
15.5 Appendix 5: Public Consultation Document ... 152
15.6 Appendix 6: Summary Public Consultation Document ... 152
15.7 Appendix 7: Easy Read Consultation Document ... 152

15.8 Appendix 8: Consultation Survey 152
15.9 Appendix 9: Independent Consultation Report 152
15.10 Appendix 10: HOSC Feedback 152
15.11 Appendix 11: Annex 1 Detailed Financial Modelling Option A 152
15.12 Appendix 12: ADEPT BSoL Estates Report 152
15.13 Appendix 13: BSoL Digital Strategy Full Version_29.09.2022 152
15.14 Appendix 14: BSoL Digital & Enhanced Self-Assessment Tool_Oct26 152
15.15 Appendix 15: UTC & GP OOHs Review QEIA_Option APlus_Oct26 152
15.16 Appendix 16: UTC Travel Time Analysis Summary_v4_Option APlus 152
15.17 Appendix 17: BSoL UTC & GP OOOHs Risk Register_v7_Oct2026 152
15.18 Appendix 18: UTC & GP OOHs Review Implementation GANT Chart 152

Birmingham, Solihull & Black Country Integrated Care Board

Changing how Urgent Treatment Centre & GP Out of Hours services are provided across Birmingham and Solihull

Decision-Making Business Case

1 EXECUTIVE SUMMARY

1.1 Purpose of this Decision-Making Business Case and Need for Change

1.1.1 This Decision-Making Business Case (DMBC) sets out the evidence and recommendation for the future configuration of Urgent Treatment Centre (UTC) and GP Out of Hours (OOH) services across Birmingham and Solihull. It brings together the strategic case for change, clinical and activity evidence, public consultation and scrutiny feedback, equality and access considerations, financial and estates analysis, workforce implications, implementation requirements and the further review undertaken after consultation.

1.1.2 The review was initiated following publication of NHS England's national UTC principles and standards in October 2023, alongside recognition that current provision across Birmingham and Solihull is variable and does not consistently deliver the intended UTC model. Differences in clinical leadership, service offer, access arrangements and diagnostic availability have contributed to a complex urgent care landscape. Patients and stakeholders have also described confusion about where to seek help, while emergency departments and the wider urgent and emergency care system continue to experience pressure.

1.1.3 The review also supports the wider ambition of delivering care closer to home, reducing avoidable attendance at Emergency Departments and ensuring patients are supported to access the most appropriate service first time. The future model strengthens the interface between UTCs, GP Out of Hours, NHS 111, Pharmacy First, primary care and community services, supporting a sustainable left shift of activity into lower acuity settings where clinically appropriate.

1.1.4 Birmingham and Solihull serves a population of approximately 1.3 million people, with significant differences in deprivation, health need and access to transport and services across localities. The future model therefore needs to provide consistent, safe and sustainable urgent care while retaining appropriate local access, reducing avoidable barriers and making best use of available workforce, estates and funding.

1.2 Current services and proposals for change

1.2.1 The DMBC recommends Option A as amended, referred to in this business case as Option A Plus. The proposed model has been developed to provide greater consistency in service standards, improved clinical resilience, enhanced workforce sustainability and a more integrated urgent care pathway across Birmingham and Solihull. The model aims to improve patient experience and outcomes whilst ensuring finite workforce and financial resources are utilised as effectively as possible.

1.2.2 The model combines co-located UTCs at acute hospital sites with community UTCs, enhanced locality urgent care services and a revised approach to GP OOH commissioning. Alongside a network of co-located and community UTCs, the model retains a strong neighbourhood focus through locality-based services at Summerfield and Washwood Heath, supporting people with long-term conditions to access preventative care, evidence-based treatment and specialist support closer to home, including during periods of acute deterioration.

1.2.3 The recommended Option A Plus model incorporates a number of refinements arising from the options appraisal, consultation and implementation planning process. The table below provides a summary of the key components of the proposed model and how these will be delivered across Birmingham and Solihull.

- Four co-located UTCs at Queen Elizabeth Hospital Birmingham (QEH), Birmingham Heartlands Hospital (BHH), Good Hope Hospital (GHH) and Midland Metropolitan University Hospital (MMUH).
- Three community UTCs at Solihull, Erdington and South Birmingham/Selly Oak, developed to meet national UTC requirements and provide local access for appropriate urgent presentations.
- Enhanced locality urgent care services at Summerfield and Washwood Heath, replacing the current UTC model with targeted community-based services designed around identified population health needs. These services will support people living with long-term conditions to access preventative care, evidence-based treatment and specialist support both when clinically stable and when experiencing an acute deterioration. Delivered through revised clinical pathways and multidisciplinary community teams, the services will help more people receive care closer to home, reduce avoidable Emergency Department attendance and admission, and strengthen neighbourhood-based urgent care provision. Initial pathway development is focused on respiratory disease, diabetes and heart failure, with detailed service specifications to be developed through implementation planning.
- GP OOH services reprocured under a common overarching specification and divided into three locality lots: South/Central, North/West and Solihull/East. Detailed service standards, operating arrangements and capacity requirements will be confirmed through procurement and mobilisation.

Option A Plus
• North Birmingham UTC - which was previously Warren Farm UTC - would move permanently to Good Hope Hospital. It will provide walk-in and care pre-booked via NHS 111 for non-life-threatening illness and injuries. • Solihull, Erdington and South Birmingham UTCs would stay where they are, but will be improved to meet national NHS standards. They will provide walk-in and care pre-booked via NHS 111 for non-life-threatening illness and injuries. • Summerfield and Washwood Heath UTCs will close. Instead, enhanced locality urgent care services will be delivered from the same sites as part of wider health and wellbeing hubs. These services will support people with long-term conditions through prevention, early intervention, specialist assessment and ongoing community-based support, helping more people receive care closer to home and reducing the need for hospital attendance where appropriate. • GP out-of-hours (OOH) services is to commission provision through three locality-based lots covering South/Central, North/West, and Solihull/East. Each lot will be required to deliver a consistent overarching clinical and service specification, whilst allowing flexibility to develop locality-specific arrangements for face-to-face appointments and home visiting services. The service will continue to work closely with NHS 111, providing telephone assessment as the primary access route, with face-to-face appointments or home visits offered where clinically required. • This approach represents a refinement of the original Option A proposal, which proposed recommissioning GP OOH services as a single Birmingham and Solihull-wide service. This is intended to strengthen responsiveness to local population needs, support integration with wider locality urgent care services, and enable greater flexibility in service delivery while maintaining consistent standards of care across Birmingham and Solihull. • Additional UTCs will open at Birmingham Heartlands Hospital, Queen Elizabeth Hospital Birmingham and Midland Metropolitan University Hospital in Smethwick. These services would provide urgent care for minor illness and minor injury, with the agreed clinical model, diagnostics and pathways developed alongside the relevant acute Trusts. Co-location is intended to support appropriate streaming and diversion from ED, while maintaining clear routes for patients who require emergency assessment.

1.2.4 The recommendation does not propose changes to the provision of Emergency Departments themselves. It does, however, depend on effective clinical and operational interfaces between EDs, co-located UTCs, community services, NHS 111 and GP OOH. Detailed specifications will need to define opening hours, walk-in and booked access, diagnostic provision, referral pathways, digital arrangements and the support available to people who cannot readily use telephone or online routes.

1.3 Consultation, engagement and consideration of feedback

1.3.1 A public consultation ran from 5 February to 16 April 2026. It generated 1,452 survey responses, alongside approximately 182 contributions through events and focus groups, giving around 1,633 recorded contributions across the consultation activity. The findings were considered alongside Healthwatch and other engagement evidence, feedback from local

authority Health Overview and Scrutiny Committees (HASC/HOSC), equality analysis, travel-time analysis and the clinical, activity, workforce, estates and financial evidence.

1.3.2 The consultation did not demonstrate an unequivocal public preference between the two options. In the survey, 50% of respondents rated Option A as meeting needs very or extremely well, compared with 38% for Option B. Views varied by locality and engagement method. Feedback highlighted the value of familiar local services, walk-in and face-to-face access, diagnostics, clear navigation and the ability to receive care close to home. Concerns included longer or more complex journeys, public transport and parking, digital exclusion, reliance on NHS 111, the future of services at Summerfield and Washwood Heath, and the need for a clear explanation of how GP OOH would operate.

1.3.3 The ICB has considered this feedback conscientiously rather than treating consultation as a vote. Relevant themes have informed the description and further development of the recommended model, particularly the locality-based GP OOH procurement approach and the need to specify inclusive access routes, face-to-face care, communications, transport mitigations and monitoring. HASC/HOSC feedback has also informed the post-consultation review and the need to explain clearly how the final model responds to concerns raised.

1.3.4 The consultation has limitations, including under-representation of some groups and localities and differences in participation across engagement methods. These limitations are acknowledged in the business case and have been considered alongside other evidence, rather than treating consultation responses as representative of the whole population.

1.3.5 The final recommendation reflects feedback received throughout the consultation process and is not solely a replication of the original preferred option. Consultation findings informed the enhancement of the locality-based GP Out of Hours model and reinforced the importance of maintaining accessible locality urgent care provision as implementation progresses.

1.4 Options review and recommendation

1.4.1 The pre-consultation appraisal shortlisted Option A and Option B against five established criteria: health outcomes and reducing inequity of access; workforce sustainability and staff development; clarity for patients and the public; deliverability; and impact on access, including travel by car and public transport and the sustainability of access. The original weighted scores were 434 for Option A and 410 for Option B.

1.4.2 Following consultation, an independent Olovus-facilitated Options Review Workshop took place on 24 September 2026. The Programme Board reviewed the options, consultation and scrutiny feedback, relevant legal duties, access pathways, service specifications and activity modelling, and considered whether a hybrid approach was needed. Option A Plus was agreed as the recommended option, subject to further development of the GP OOH specification in response to consultation and scrutiny feedback.

1.4.3 A subsequent commissioner-only review revisited the original criteria and evidence to test whether the original scores remained appropriate. This was not a new options appraisal or re-scoring exercise. The review concluded that the original scores remained appropriate and that relevant elements of the potential hybrid approach could be incorporated into Option A. The resulting recommendation is Option A Plus, with the three community UTCs retained, enhanced locality urgent care arrangements at Summerfield and Washwood Heath, and GP OOH procured through three locality lots.

1.4.4 Option B was considered in its revised form, including the retention of Solihull as a community UTC and the closure of five other existing community UTCs, with community urgent care

primarily provided outside core GP hours through NHS 111. The business case acknowledges that some supporting modelling was developed using earlier assumptions and should be interpreted with those limitations in mind. Option C was considered in response to consultation and scrutiny feedback; relevant elements have been incorporated into the recommended approach where considered deliverable.

1.5 Impact analysis

1.5.1 **Clinical quality and capacity:** The proposed model supports a more consistent urgent care offer across Birmingham and Solihull through a combination of co-located UTCs, community UTCs and specialist locality services. The enhanced locality urgent care services at Summerfield and Washwood Heath provide additional opportunities for early intervention, preventative care, specialist community management and avoidance of unnecessary Emergency Department attendance and hospital admission. The model is intended to support people with long-term conditions through revised clinical pathways and enhanced multidisciplinary support, enabling more patients to receive care within their own homes and communities wherever clinically appropriate. The proposed model is intended to standardise the UTC offer, improve access to diagnostics at co-located sites and support more appropriate streaming between UTCs and EDs. Activity modelling informs the strategic decision, but the current model does not fully quantify every element of Option A Plus, particularly the enhanced locality arrangements at Summerfield and Washwood Heath. Detailed capacity, activity and service requirements will therefore need to be validated during implementation planning.

1.5.2 **Access, equality and health inequalities:** Travel-time analysis indicates that the proposed configurations improve modelled access for many residents, although the degree of improvement varies by mode of travel and locality. The analysis does not capture every practical barrier, including affordability, interchange, parking costs, mobility, caring responsibilities or unfamiliarity with services. The recommended model retains three community UTCs and proposes enhanced locality arrangements, but residual access risks remain and require mitigation and monitoring.

1.5.3 **Workforce:** Safe delivery depends on recruiting, retaining and supporting an appropriately skilled multidisciplinary workforce across co-located UTCs, community UTCs, enhanced locality urgent care and GP OOH. Workforce requirements, opening hours, clinical leadership, training, supervision and transition arrangements will need to be confirmed through specification development, procurement and mobilisation.

1.5.4 **Finance:** The financial modelling indicates broadly similar modelled UTC costs for the two options, with Option A estimated at approximately £24.7 million and Option B at approximately £24.1 million on the assumptions used. These figures should not be interpreted as confirmed cash-releasing or system-wide savings. The analysis identifies material dependencies around tariffs, activity flows, ED costs, diagnostics, transition and the final GP OOH model.

1.5.5 **Estates and implementation:** Co-located UTC developments are linked to acute-site capital schemes and require coordination with provider estates, workforce, digital, diagnostics and operational plans. Community and enhanced locality arrangements require confirmation of premises, planning and landlord requirements, operating hours and any additional revenue costs. Site-level implementation plans and the outstanding estates assessment will be incorporated when available.

1.5.6 **Digital and service navigation:** Digital tools and NHS 111 can support access, assessment and referral, but must not create avoidable barriers for people who cannot use digital or telephone

services. The final service specifications and communications will need to set out clear access routes, accessible information, interpretation and translation support, and routes to face-to-face care where needed.

1.6 Stakeholder engagement and assurance

1.6.1 The review has been informed by engagement with patients, carers, staff, existing UTC providers, primary care, community services, acute Trusts, ambulance services, NHS 111, local authorities, Healthwatch and voluntary, community, faith and social enterprise organisations. The West Midlands Clinical Senate reviewed the proposals in 2025, and its findings informed the clinical case and options development. NHS England assurance has also informed the programme: the Stage 1 strategic sense check took place on 20 November 2024 and the Stage 2 assurance process was completed on 20 January 2026.

1.6.2 The DMBC brings together the resulting evidence with consultation findings, the post-consultation review, equality and travel analysis, activity and workforce modelling, and the developing finance, estates and implementation plans. It identifies outstanding dependencies and risks rather than assuming that all implementation details have been resolved. In particular, finance and estates risks, detailed site arrangements, workforce requirements, procurement and mobilisation plans, and final service specifications must be completed and governed through the relevant approval and assurance processes.

1.6.3 The proposals have been subject to extensive assurance through NHS England, independent options appraisal, public consultation, Clinical Senate review, Quality and Equality Impact Assessment processes and scrutiny by local authority committees. Collectively, these processes provide assurance that the recommended model is evidence-based, proportionate and capable of implementation.

1.7 Next steps

- Programme Board consideration of the DMBC on 15 October 2026.
- ICB Executive Board consideration on 26 October 2026, with any resulting amendments incorporated where appropriate.
- Submission to the ICB Public Board on 9 November 2026 for the formal decision on the recommended model.
- Subject to approval, progress detailed implementation planning, procurement of community UTC and GP OOH services, confirmation of the contracting route for co-located UTCs, site-specific estates and operational planning, workforce transition and mobilisation.
- Develop the benefits realisation and evaluation arrangements, including monitoring of activity, access, patient experience, quality, equality and health inequalities, and the impact on the wider urgent and emergency care system.

1.8 Conclusion

1.8.1 The evidence presented throughout this DMBC demonstrates a compelling case for change and supports implementation of Option A Plus as the preferred future model for Urgent Treatment Centre and GP Out of Hours services across Birmingham and Solihull. The proposed model delivers improved compliance with national standards, improves consistency of urgent care provision, maintains local access for communities and supports the wider transformation of urgent and emergency care services.

1.8.2 The DMBC represents the culmination of the programme's business case development process, building on the strategic and pre-consultation work previously approved by the Integrated Care Board. For completeness and to provide a clear decision-making audit trail, the original Pre-Consultation Business Case (PCBC) is included as Appendix 1 – BsoL UTC Strategy PCBC to this document and should be read in conjunction with the supporting consultation, appraisal and assurance documentation.

1.8.3 The model is not without risks or outstanding implementation dependencies. Successful delivery will require effective procurement, workforce mobilisation, digital interoperability, estates development and contractual transition arrangements across multiple organisations. Key programme risks include procurement capacity, digital and IT dependencies, delivery of co-located UTC estate developments, workforce availability and implementation timescales. These risks have been assessed through the programme governance framework and mitigation plans are in place to support successful mobilisation, implementation and benefits realisation. The ICB will need to confirm the final financial position, estates and site arrangements, service specifications, workforce requirements and procurement and mobilisation plans before implementation.

2 PURPOSE AND CASE FOR DECISION

2.1 Purpose of this decision-making business case

2.1.1 This decision-making business case (DMBC) presents the case for the future configuration of Urgent Treatment Centre (UTC) services across Birmingham and Solihull. It brings together the strategic case for change, the evidence developed throughout the review, the findings from the public consultation and stakeholder engagement, and the outcome of further analysis undertaken following consultation. The document provides the basis upon which Birmingham and Solihull Integrated Care Board (BSol ICB) is asked to approve the preferred option for implementation.

2.1.2 The decision-making business case demonstrates how BSol ICB, as the NHS commissioner of Urgent Treatment Centres, has fulfilled its statutory duties in relation to service change, including public consultation under the NHS Act 2006, consideration of the consultation findings, equality duties, and compliance with NHS England assurance requirements. It provides the evidence to support a recommendation for implementation of the preferred option.

2.1.3 This case is intended for decision-makers within BSol ICB, NHS England, partner organisations and other stakeholders with an interest in the future provision of urgent care services. It demonstrates how stakeholder feedback, clinical evidence, financial analysis and equality considerations have informed the recommended option and supports the formal decision-making process.

2.2 Decision Required

2.2.1 The ICB Board is asked to consider the evidence presented in this DMBC and approve the implementation of Option A as the future model for Urgent Treatment Centres and GP Out of Hours services across Birmingham and Solihull, subject to the conditions and further work identified within the business case.

2.3 Background to Urgent Treatment Centre strategic review

2.3.1 The NHS and local authorities are faced with increasing demands for services and the associated financial pressures. In that context it is even more important that the NHS continues to change the way it works to respond to the needs of the people who use it, adopting best practice and providing evidence-based services. It is the role of the commissioner to ensure that they regularly review the services commissioned to ensure they meet the needs of their populations, effecting service reconfigurations when needed.

2.3.2 The urgent care system in Birmingham and Solihull is complex, and the number of different entry points can be confusing to people, at a time when they need it to be simple. The role of the Integrated Care Board as system leaders and healthcare providers is to remove this confusion and present the community with a straightforward option that is easy to navigate. This will require changes to the structure of the existing services, including enhancing some services and reducing or stopping others when they are no longer appropriate. The ICB has finite resources available with which to deliver sustainable services. The preferred model described within this business case aims to improve services for patients, better serve the public and deliver a more effective and sustainable urgent care system.

2.3.3 In particular the review addresses the gaps between current national standards for Urgent Treatment Centres and the existing provision in Birmingham and Solihull. It does this in the context of increasing attendances at hospital emergency departments (EDs) including attendances that could be treated more appropriately in a different care setting. Achieving this shift of activity will improve patient and public experiences in emergency departments and provide capacity to allow EDs to improve their wating times performance. The evidence presented within this business case demonstrates that the preferred option best addresses these challenges while reflecting the feedback received during consultation.

2.4 The scope of the decision

2.4.1 This business case recommends the preferred option for the implementation of future commissioning arrangements for Urgent Treatment Centre (UTC) and GP Out of Hours (GP OOH) services across Birmingham and Solihull.

2.4.2 The business case also considers the interfaces with related urgent care services, including NHS 111 and locality-based urgent care services, where these are relevant to the future model and patient pathways.

2.4.3 The decision does not propose changes to Emergency Department provision. However, it considers the interface between UTCs and Emergency Departments, including how the future UTC model can support appropriate streaming, diversion and patient flow, ensuring that patients who do not require emergency care can be assessed and treated in the most appropriate setting**.**

2.5 Structure of pre-consultation business case

2.5.1 The case follows a structure aligned with NHS England guidance (NHS England, 2022), as follows:

- **Strategic context** – national and local policy, population needs, strategies and governance
- C**urrent arrangements and case for change** – current urgent care provision, activity, performance and the evidence supporting change;
- **Future service model** – the proposed configuration of UTC, GP Out of Hours and locality urgent care services;;
- **Options appraisal and recommendation** – the options considered, appraisal methodology, evidence and rationale for the preferred option;
- **Capacity, quality and impact** – activity and demand modelling, clinical considerations, workforce, equality, health inequalities and access
- **Financial and implementation case** – affordability, key financial considerations, benefits, risks, implementation requirements and transition arrangements
- **Consultation, assurance and decision-making** – consultation and stakeholder feedback, external assurance, governance and the decision requested from the ICB.

3 STRATEGIC CONTEXT

3.1 Local geography and demographics: Birmingham and Solihull

3.1.1 The map below shows the wider metropolitan area of Birmingham with surrounding areas including Solihull. Birmingham itself is a city and metropolitan borough in the West Midlands, the second-largest city in Britain. There are a number of population centres within the city which incorporates the Royal Town of Sutton Coldfield. The population of Birmingham **(Office for National Statistics, 2023)** was 1.1 million people in 2021, having grown by 7% since 2011. The average age is 34, compared to 40 for England as a whole.

3.1.2 The ethnic composition of Birmingham is diverse, with 49% identifying as White, 31% identifying as Asian, Asian British, or Asian Welsh and 11% identifying as Black, Black British, Black Welsh, Caribbean or African.

Birmingham metropolitan area



Source: BlueandWhiteStripes

3.1.3 The table sets out ethnicity across Birmingham and Solihull by NHS locality **(Birmingham City Council, 2021)**, **(Office for National Statistics, 2021)**:

Birmingham and Solihull ethnicity

National Census 2021 by NHS locality	White	Asian	Black	Mixed	Other	Total
Central	47%	36%	6%	5%	6%	100%
North	71%	13%	9%	5%	2%	100%
West	27%	41%	22%	5%	6%	100%
East	37%	46%	8%	4%	5%	100%
South	69%	13%	9%	6%	4%	100%
Solihull	82%	11%	2%	3%	2%	100%
Birmingham & Solihull	**54%**	**28%**	**10%**	**5%**	**4%**	**100%**

Source: ONS 2021

3.1.4 The population of Birmingham is projected to grow to 1,230,000 (7.8%) from 2018 to 2038,

10.2% for Solihull. Predicted population growth as set out in the table below shows an estimated 29.5% increase in the over 65 population from 2018 to 2038 **(Birmingham City Council, 2018)**.

Birmingham population growth by age group

Age Group	Base Year 2018	Projection Year 2028	Projection Year 2038	2018-2028 number	2018-2028 percent	2018-2038 number	2018-2038 percent
Children 0-15	259,800	254,600	256,800	-5,200	-2.0%	-3,000	-1.2%
Working age 16-64	733,600	764,800	781,600	31,200	4.3%	48,000	6.5%
Pensioner 65+	147,900	166,600	191,600	18,700	12.6%	43,700	29.5%
Birmingham	**1,141,400**	**1,186,000**	**1,230,000**	**44,600**	**3.9%**	**88,600**	**7.8%**

Source: ONS 2020, analysis by Birmingham City Council

3.1.5 Activity modelling to inform capacity planning assumes 5% growth mitigated by 2% demand management resulting in a net 3% growth in line with the population growth set out above. Mitigations include directing patients to other community services including Pharmacy First. Detailed service specifications will seek to address the particular needs of older people given the forecast growth in that age category.

3.2 NHS localities in Birmingham and Solihull

3.2.1 Birmingham and Solihull ICB plans and delivers healthcare across six localities: Central, North, East, South, West and Solihull. Each locality has a population of approximately 250,000–300,000 people.

3.2.2 The locality structure is relevant to the UTC review because the future model is intended to reflect differences in population need, existing provision, access, deprivation and patterns of urgent care use across Birmingham and Solihull.

3.2.3 Locality-level evidence, including travel-time assessments, consultation feedback and health inequalities analysis, has therefore been considered alongside system-wide evidence in developing and assessing the preferred service model.

3.3 10-year Health Plan for England

3.3.1 The Government's 10 Year Health Plan for England (2025) sets out a strategic direction for the NHS based around three shifts: from hospital to community, from analogue to digital and from sickness to prevention. The plan emphasises greater provision of care within neighbourhoods and communities and the development of integrated services that reduce unnecessary reliance on hospital care.

3.3.2 The future urgent care model supports this direction by combining enhanced urgent care provision integrated with acute hospitals with community-based UTC and locality urgent care provision. This provides a framework for treating patients in the most appropriate setting while maintaining access to acute clinical expertise where required.

3.4 NHS Medium term planning framework 2026/27 to 2028/29

3.4.1 The NHS Medium Term Planning Framework 2026/27 – 2028/29 identifies reducing emergency department crowding, improving urgent and emergency care flow and expanding community-based urgent care as key priorities.

3.4.2 The future UTC model supports these objectives through the development of co-located UTCs, enhanced streaming and diversion from Emergency Departments, and continued community-based urgent care provision.

- Reduce ED crowding:
- Neighbourhood health model:
- Ambulance handover:
- Mental health crisis response:
- Discharge delays:
- Vaccination uptake:

3.5 National principles and standards for UTCs

3.5.1 National principles and standards for operation of Urgent Treatment Centres were updated in October 2023 **(NHS England, 2023)**. The following is a verbatim excerpt:

> *"UTCs, wherever they are located, should be able to treat patients of all ages for minor ailments and injury, with senior clinical leadership, in line with these standards. As a priority, UTCs are expected to:*
>
> 1. *open 7 days a week, 12 hours a day as a minimum, typically increasing to 24 hours a day when co-located*
> 2. *see both booked and walk-in patients*
> 3. *see both minor injuries and minor ailments*
> 4. *see patients of all ages, including children under 2*
> 5. *have a named senior clinical leader supported by an appropriate multi-disciplinary workforce*
> 6. *have a basic consistent investigative and diagnostic offering on site (with clear protocols if not on site)*
> 7. *accept appropriate ambulance conveyance*
> 8. *have access to patient records and the ability to send a post event message (PEM)*
> 9. *report as a Type 3 daily on Emergency Care Data Set (ECDS)*
> 10. *have an up-to-date directory of services (DoS) profile*
> 11. *clearly communicate to the public what the service is for via consistent urgent treatment centre nomenclature, to ensure the service provision is understandable and accessible to all."*

3.5.2 The document states that *"ICBs should determine where UTCs will be most effective; this may be co-located with the local emergency department (ED) and increasingly acting as the front door of ED, or a standalone service either on or off a hospital site."*

3.5.3 Integrated care boards (ICBs) should consider UTCs as part of their strategy for urgent care, taking into account the wider provision of emergency and primary care services for their population.

3.5.4 ICBs, in their commissioning of UTCs, will need to:

- Consider local activity, demand management and patient flow and throughput;
- Establish the urgent care needs of their local population, and the capacity they require;
- Where there are high rates of referral from UTCs into local EDs, ICBs should review relevant care pathways across UEC to maximise use of UTCs where clinically appropriate. This should form part of the ongoing review of ED case mix and re-attendance;
- Ensure that UTCs meet the updated UTC principles and standards, and where there are gaps support UTCs to identify the actions required;
- It is for ICBs to determine where UTCs will be most effective. Where type 3 or 4 A&E services have yet to determine their future function as either a UTC or alternative service (subject to local consultation and following proper procurement process where appropriate) current provision of extended GP access and plans for Enhanced Services (DES) should be considered. In this review, ICBs should also consider implications for patient transportation, to ensure appropriate transport of patients to and from UTCs;
- ICBs, working across their integrated care system (ICS) and with other non-NHS partners, should use data to consider opportunities to reduce health inequalities through the commissioning and specification of UTCs. This is of particular importance given the disproportionately high use of Type 3 services nationally by those from the highest levels of deprivation.

3.6 National capital investment in UTCs

3.6.1 One-year capital allocations for 2025/26 were announced following the government's 2024 Autumn Spending Review. This included over £370 million to support urgent and emergency care, including UTC development. University Hospitals Birmingham NHS Foundation Trust bid for capital for UTC development at three of its hospitals in Birmingham, at Good Hope, Heartlands and Queen Elizabeth.

3.7 Local strategies

3.7.1 The preferred option future urgent care model is aligned with the strategic direction established through the Birmingham and Solihull Integrated Care Strategy and Joint Forward Plan.

3.7.2 The local strategy identifies the need to improve integration across health and care, strengthen neighbourhood working, reduce avoidable emergency admissions and improve urgent and emergency care flow.

3.7.3 The Joint Forward Plan also identifies urgent care transformation, UTC redesign, locality-based integration and increased community capacity as important components of the system's approach to improving patient flow and achieving financial sustainability.

3.7.4 The preferred option therefore supports the wider system direction by strengthening the integration between acute, community and neighbourhood urgent care services.

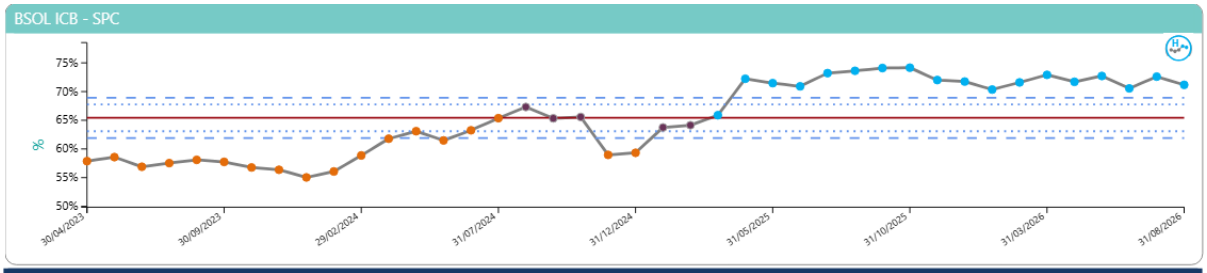
4 CURRENT ARRANGEMENTS AND PERFORMANCE

4.1 Current hospital emergency department provision and performance

4.1.1 Birmingham and Solihull is served by 3 major hospitals with emergency departments: Queen Elizabeth (1,215 beds, 133,000 ED attendances in 2024/25) in Edgbaston, Heartlands Hospital (462 beds 163,000 ED attendances in 2024/25) in Bordesley Green, Good Hope Hospital in Sutton Coldfield (550 beds 109,000 ED attendances in 2024/25). Solihull Hospital (229 beds) does not have an emergency department but does host a minor injuries unit (19,000 attendances in 2024/25) and an Urgent Treatment Centre open 12 hours daily (25,000 attendances in 2024/25). The newly opened (October 2024) Midland Metropolitan University Hospital (700 beds, 85,000 ED attendances in its first twelve months of operation) in nearby Smethwick also serves the west of Birmingham.

4.1.2 Recent Emergency Department (ED) performance against the 4 & 12-hour standard for treatment and discharge is shown in the charts below. Current performance is consistently below the 78% target for four-hour maximum waits. The number of patients waiting over 12 hours is currently well over 4,000 against a year-end target of 3,000.

4.1.3 Birmingham and Solihull emergency department 4-hour performance (Apr26 – Aug26)

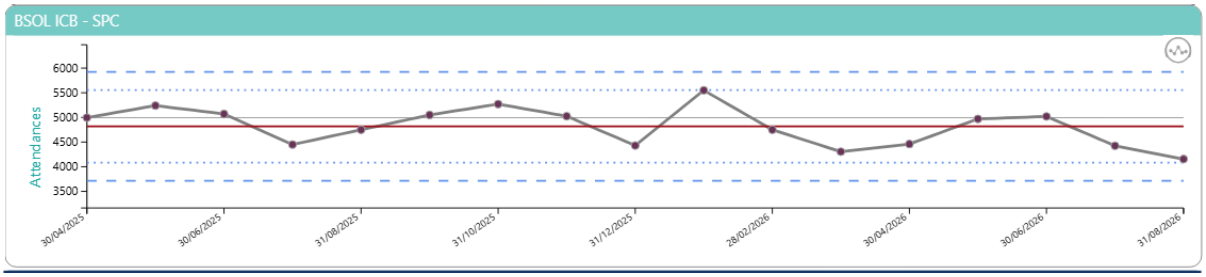


ICB / Cluster level performance - based on Providers within each ICB

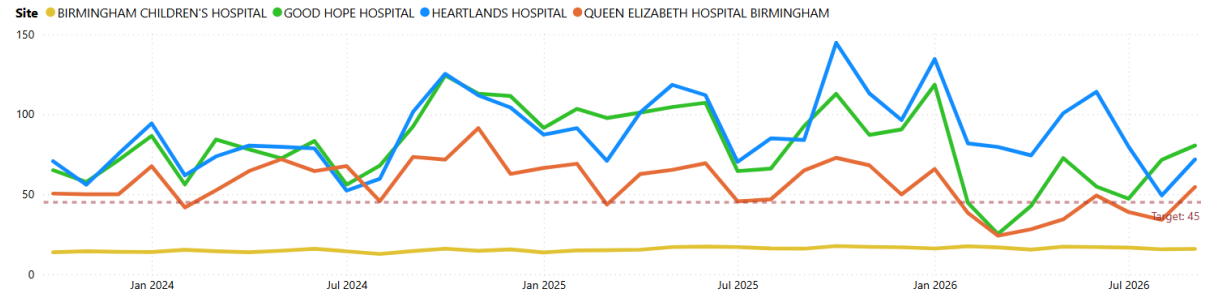
ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BSOL ICB	Actual	71.70%	72.72%	70.55%	72.60%	71.17%							

Source: Bsol

4.1.4 Birmingham and Solihull emergency department 12 hour waits (Apr26 – Aug26)



4.1.5 Birmingham and Solihull ambulance delays (data range below May24 - Aug26) at hospital emergency departments for BSOL are shown in the graph. Handovers at Birmingham Children's Hospital met the 45-minute target consistently. Heartlands, Good Hope and Queen Elizabeth hospitals almost never met the target.



Year	2024								2025										2026						
Site	May	June	July	August	September	October	November	December	January	February	March	April	May	June	July	August	September	October	November	December	January	February	March	April	May	June	July	August
BCH	14.73	15.92	14.35	12.67	14.54	15.97	14.67	15.54	13.66	14.89	15.04	15.36	17.01	17.35	16.98	16.19	16.01	17.62	17.12	16.87	16.12	17.49	16.83	15.38	17.24	17.00	16.69	15.60
BHH	79.62	78.66	52.27	59.69	101.43	125.22	111.80	104.17	87.23	91.33	70.91	101.03	118.34	111.90	70.22	84.93	83.90	144.59	113.03	96.36	134.41	81.73	79.51	74.29	100.62	113.98	79.75	49.27
GHH	72.41	83.27	56.02	67.87	92.32	124.21	112.85	111.35	91.46	103.30	97.60	101.02	104.46	107.17	64.45	66.02	92.51	112.71	87.09	90.48	118.41	44.63	25.18	42.76	72.57	54.82	47.22	71.55
QEH	71.86	64.49	67.58	45.66	73.31	71.60	91.33	62.73	66.42	69.06	43.52	62.61	65.26	69.37	45.57	46.81	64.86	72.76	68.09	49.86	65.76	38.34	24.09	28.17	34.32	49.25	38.85	34.00
Total	**72.18**	**72.14**	**56.64**	**54.97**	**85.51**	**100.75**	**99.80**	**87.48**	**78.26**	**83.17**	**64.93**	**84.45**	**92.48**	**92.38**	**58.62**	**65.39**	**77.58**	**107.08**	**87.38**	**76.31**	**102.86**	**55.86**	**45.49**	**49.32**	**68.84**	**74.42**	**56.09**	**48.87**

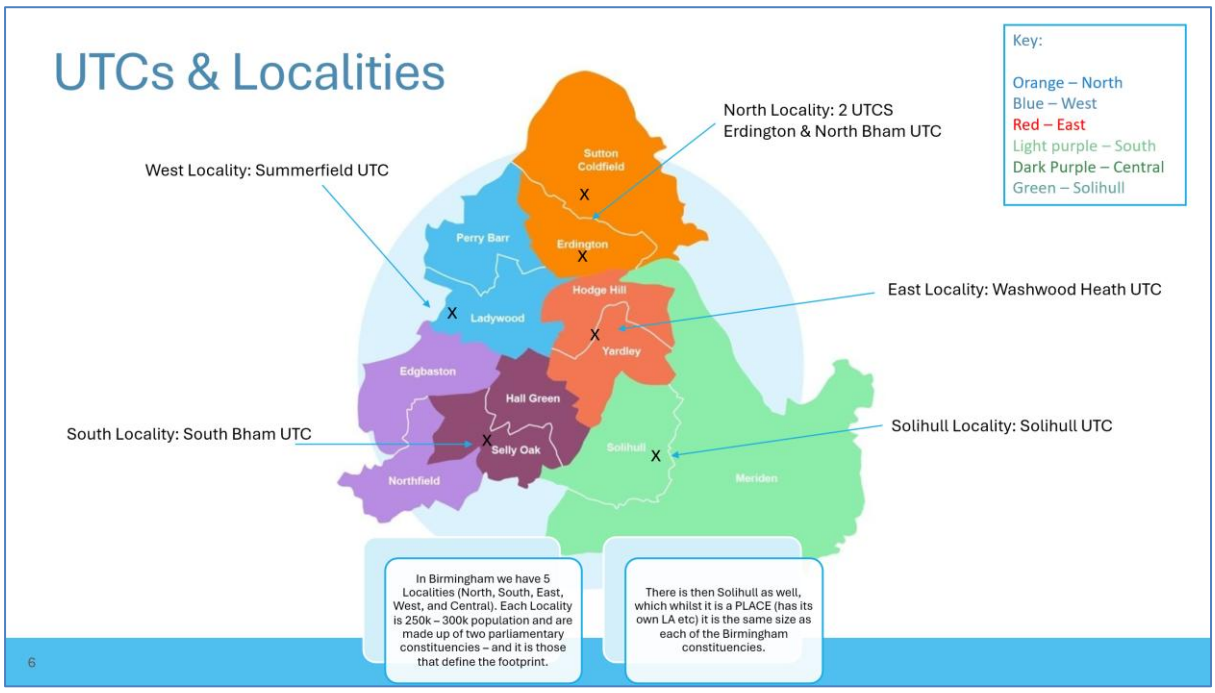
Source: Bsol ICB

4.2 Current provision of UTC services in Birmingham and Solihull

4.2.1 The map below shows current UTC provision across Birmingham and Solihull. There are six localities based on council ward combinations. North locality has two UTCs and central none; the remaining localities have one UTC each.

4.2.2 Alongside this UTC provision within BSol, there are also 6 Locality Hubs, with Solihull locality hub within the same Solihull Hospital location as the Solihull UTC. There are four primary care streaming services, one at each local hospital. There are new workstreams such as Community Pharmacy's 'Pharmacy First' programme.

BSol localities and UTC provision



Source: BSol ICB

4.2.3 The two tables below set out current UTC service provision across the area. Each service is rated at least "Good" by the CQC. Only the recently (February 2025) relocated North Birmingham UTC is co-located with a hospital emergency department, with a number being

in a centre also offering other primary care services. All are operated by commercial operators and are open for 12 hours daily and use a mixture of in-person and telephone treatment and advice. Three are nurse led and three are GP-led. All are accessible by public-transport and have parking.

BSol UTC service provision – North and East

Locality	North	North	East
Service name	**North Birmingham UTC** PREVIOUSLY Warren Farm until February 2025	**Erdington UTC**	**Washwood Heath Urgent Care Centre**
Location	Treatment Centre Ground Floor Good Hope Hospital Rectory Road Sutton Coldfield Birmingham B75 7RR	Stockland Green Primary Care Centre 192 Reservoir Road Erdington Birmingham B23 6DJ	Washwood Heath Health and Wellbeing Centre Clodeshall Road Washwood Heath Birmingham B8 3SN
Co-located services	Good Hope Hospital	Stockland Green Primary Care Centre: GPs etc.	Washwood Heath Health & Wellbeing Centre: GPs, community health teams and diagnostic centre.
Nearest acute emergency department	Good Hope Hospital (on site)	Heartlands Hospital (4 miles)	Heartlands Hospital (2 miles)
Opening hours *(all subject to a daily activity cap)*	12 hours daily x 365 days pa	12 hours daily x 365 days pa	12 hours daily x 365 days pa
Physical patient attendances pa	28,135	5,519	8,121
Patient consultations by telephone pa	7,341	13,265	20,912
Total activity pa	**35,476**	**18,784**	**29,033**
Operator	Operose Health	Badger	Operose Health
Clinical leadership	Nurse-led	GP-led	Nurse-led
Clinical Quality Commission rating (date)	Good (previous location) Feb 2017	Outstanding Nov 2023	Good Dec 2018
Public Transport	Multiple bus routes stop outside the hospital; Sutton Coldfield rail station approx. 15-minute walk with frequent services to Birmingham.	Direct bus access from Birmingham city centre (Route 65) and Outer Circle routes (11A/11C).	Well served by local bus routes; Adderley Park rail station approx. 12-minute walk with onward connections to Birmingham New Street.
Parking	: c.288 visitor spaces in designated car parks; peak-time capacity pressures; Park Mark® accredited; free parking for Blue Badge holders.	c.156 spaces (visitor and staff), including 4 accessible spaces; free visitor parking.	c.103 spaces (visitor and staff), including 8 accessible spaces; free visitor parking.
Accessibility	Step-free access; simplified Blue Badge exit process in place.	Accessible parking available.	Step-free access from accessible parking; wheelchair accessible.
Estate Status		Estatecode condition B; core asset.	Estatecode condition B; core asset.

Source: BSol ICB

BSol UTC service provision – South, West and Solihull

Locality	South	West	Solihull
Service name	**South Birmingham GP UTC**	**Summerfield Urgent Care Centre**	**Solihull UTC**
Location	15 Katie Road Selly Oak Birmingham B29 6JG	First Floor 134 Heath Street Winson Green Birmingham B18 7AL	Solihull Hospital Lode Lane Solihull B91 2JL
Co-located services		Summerfield Primary Care Centre: GPs, dentist, pharmacy, community health and NHS locality hub.	Solihull Hospital (no Emergency Dept)
Nearest acute emergency department	Queen Elizabeth Hospital (2 miles)	Midland Metropolitan University Hospital (1 mile)	Heartlands Hospital (6 miles)
Opening hours *(all subject to a daily activity cap)*	12 hours daily x 365 days pa	12 hours daily x 365 days pa	12 hours daily x 365 days pa
Physical patient attendances pa	26,939	30,314	25,059
Patient consultations by telephone pa	54,369	7,679	11,493
Total activity pa	**81,308**	**37,993**	**36,552**
Operator	South Doc Services Ltd	Operose Health	Badger
Clinical leadership	GP-led	Nurse-led	GP-led
Clinical Quality Commission rating (date)	Good Mar 2017	Good Nov 2021	Outstanding Dec 2023
Public Transport	Strong bus connectivity along the Bristol Road corridor (routes 61, 63, X21, X22); Selly Oak rail station within walking distance with frequent services to Birmingham New Street.	Extensive bus network serving City Hospital and Birmingham city centre; Birmingham New Street is nearest mainline station with onward bus or taxi connections.	Multiple bus routes on Lode Lane; Solihull town centre within 10 minutes' walk; Solihull rail station approx. 15 minutes' walk with frequent services to Birmingham.
Parking	Free on-site parking with additional adjacent pay-and-display; accessible spaces available.	c.71 spaces (visitor and staff), including 5 accessible spaces; free visitor parking.	222 visitor spaces in main visitor car park; accessible parking available; parking charges apply (short-stay and free Blue Badge parking); peak-time capacity constraints.
Accessibility	Accessible parking provision.	Accessible parking available.	: Blue Badge parking and step-free access across site; Park Mark® accredited car parks.
Estate Status		Estatecode condition B; core asset.	

Source: BSol ICB

4.3 UTC presenting conditions.

National Context and Evidence

4.3.1 National guidance sets out a clear clinical specification for UTCs, including the ability to manage a wide range of presentations, deliver diagnostics, and ensure timely treatment, typically within a 1–12 hour timeframe.

4.3.2 National Emergency Care Data Set (ECDS) intelligence demonstrates that UTCs safely and effectively treat a substantial proportion of urgent presentations, with an average patient episode time of 124 minutes. Presenting complaints commonly include:

- Minor trauma and musculoskeletal injury
- Minor wounds, lacerations and burns
- ENT and respiratory symptoms
- Dermatological complaints
- Abdominal and urinary symptoms
- Fever, headache and other minor illness presentations.

4.3.3 This supports the national UTC model as a first-line response for ambulatory urgent care, reducing unnecessary Emergency Department (ED) attendances and improving system sustainability.

Local Population Need and Demand Profile

4.3.4 Analysis of presentations across Birmingham & Solihull UTCs shows strong alignment with the national UTC clinical scope. The most common UTC presentations locally include:

- Respiratory infections
- Skin and subcutaneous infections
- UTI and genitourinary symptoms
- ENT symptoms
- Abdominal pain
- Musculoskeletal pain
- Infectious gastroenteritis
- Cough

4.3.5 Comparatively, Birmingham & Solihull Emergency Departments frequently manage the following conditions:

- Chest pain and shortness of breath
- Abdominal pain and fever
- Limb injury
- Head injury
- Weakness/collapse
- Rash
- Seizure and near syncope

4.3.6 The top presenting conditions to UTCs are well-aligned with the national DoS Dx codes and dispositions, which recommend UTC attendance for minor illness and injury within 1-12 hours. Conditions such as respiratory infections, skin conditions, musculoskeletal pain, ENT issues,

abdominal pain, fever, and vomiting are all included in the national UTC scope. There is a clear overlap between lower-acuity ED presentations and conditions safely managed within UTCs. Evidence demonstrates that many patients currently self-presenting to ED could be treated within UTCs, supporting the need for:

- Co-located UTCs at ED front doors
- Robust clinical streaming and redirection
- Directory of Services (DoS) – driven referral pathways as per the National Minimum Standard UTC profiles
- Extended workforce capability

4.4 Current UTC activity

4.4.1 UTC activity UTC activity for April 2025 to March 2026 is shown in figure 4.4.1 below. Over half of UTC activity (52%) is by telephone consultation rather than in person, although this varies considerably by site.

Figure 4.4.1 BSol UTC activity 2025/26

Locality	UTC	Provider	Face to Face consultations	Telephone consultations	Total activity	% phone
North	Erdington	Badger	10,819	10,510	21,329	49%
Solihull	Solihull	Badger	11,656	10,705	22,361	48%
West	Summerfield	Operose	27,918	8,340	36,258	23%
North	Warren Farm	Operose	4,622	6,288	10,910	58%
East	Washwood Heath	Operose	7,409	20,505	27,914	73%
South	South Birmingham	SouthDoc	24,195	38,769	62,964	62%
TOTALS			86,619	95,117	181,736	52%

Source: Midlands & Lancs CSU

4.4.2 Figure 4.4.2 below summarises activity at the UTCs compared to commissioned capacity. It also identifies the 95th percentile attendance levels that can inform capacity planning. If workforce and physical capacity can accommodate daily demand at this 95th percentile level, then it will be sufficient to meet daily demand on 95% of days across the year.

4.4.3 The analysis shows that actual activity is above (105%) commissioned activity. Across all of the localities, capacity for over 700 consultations daily is required. Although there is a broadly balanced model, there is a slight reliance on virtual consultations.

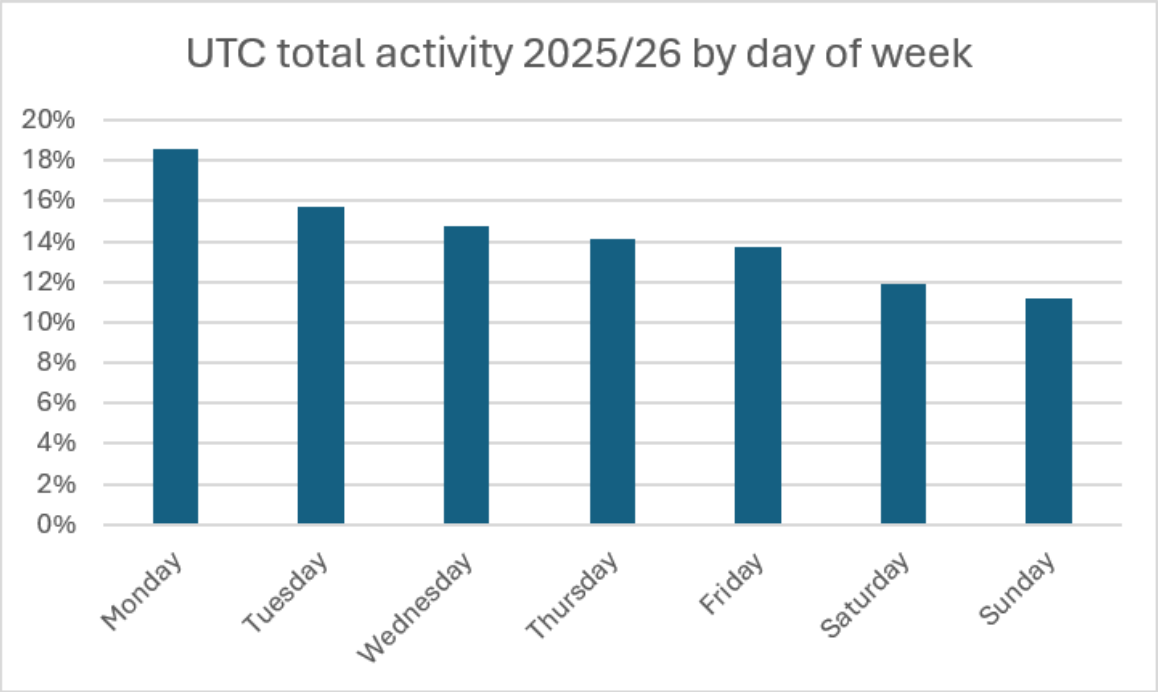
Figure 4.4.2 BSol UTC capacity and demand

UTC demand and commissioned capacity		Activity 2025/26			Commissioned Capacity 2025/26
		Face to Face	Telephone	Total	
Erdington UTC	Annual 2025/26	10,819	10,510	21,329	31,390
	Daily average 2025/26	30	29	58	86
	95th percentile 2025/26	73	62	98	114%
Solihull UTC	Annual 2025/26	11,656	10,705	22,361	36,135
	Daily average 2025/26	32	29	61	99
	95th percentile 2025/26	50	57	99	100%
South Birmingham UTC	Annual 2025/26	24,100	39,073	63,173	85,045
	Daily average 2025/26	66	107	173	233
	95th percentile 2025/26	93	161	247	106%
North Birmingham UTC	Annual 2025/26	4,666	6,292	10,958	25,915
	Daily average 2025/26	13	17	30	71
	95th percentile 2025/26	22	32	48	68%
Washwood Heath UTC	Annual 2025/26	7,430	20,579	28,009	31,025
	Daily average 2025/26	20	56	77	85
	95th percentile 2025/26	30	79	103	121%
Summerfield UTC	Annual 2025/26	28,071	8,378	36,449	38,690
	Daily average 2025/26	77	23	100	106
	95th percentile 2025/26	92	38	118	111%
OVERALL	Annual 2025/26	86,742	95,537	182,279	248,200
	Daily average 2025/26	238	262	499	680
	Sum of 95th percentile	360	429	713	105%

Source data: System Co-ordination Centre; Analysis BSol ICB

4.4.3 Figure 4.4.4 The table below shows activity by day of the week. Demand is higher on Mondays with activity then gradually reducing through the working week (if demand were evenly spread it would be at 14% each day).

Figure 4.4.4 BSol UTC activity by day of week



Source: System Co-ordination Centre, Analysis by BSol ICB

4.5 Data provision and data quality

4.5.1 Data is submitted by the third-party UTC providers and by the acute hospitals for Emergency Departments. While daily attendance and consultation numbers are provided, detailed information forming the Emergency Care Data Set (ECDS), such as postcode of residence, is not always made available for UTCs. The apportionment of activity to localities is therefore based on the data available pro-rated up to the overall attendance numbers. Hospital detailed data is provided so it can be allocated to localities directly.

4.5.2 Data analysis provided by UHB indicates a significant proportion of activity currently provided in ED could be provided in a UTC setting within the community or managed through an appropriately staffed front door UTC.

4.6 GP out of hours (OOH) services

Service Overview

4.6.1 Badger Healthcare delivers the GP Out of Hours (OOH) service for Birmingham and Solihull; the GP OOH service provides access to urgent primary care for patients during periods when GP practices are closed. The model is designed to ensure that patients with urgent but non-emergency conditions are assessed and treated in a timely and appropriate setting. The service operates:

- Weekdays: 6:30pm – 8:00am
- Weekends & Bank Holidays: 24 hours

4.6.2 The service offers a combination of clinical telephone assessment, face-to-face consultations at the OOH base, and home visiting for patients who are clinically unable to travel. These elements together form the core urgent primary care function during out-of-hours periods.

4.6.3 In addition to its core urgent care role, the service also provides out-of-hours clinical support to HM Prison Services and St Giles Hospice. It also hosts the COVID Medicines Delivery Unit (CMDU), a function established nationally to identify and treat high-risk patients with antiviral medicines where clinically appropriate.

Service Locations

4.6.4 The primary GP Out of Hours (OOH) hub is located at 1-2 Bourne Road, Aston, Birmingham (B6 7RD). This site provides telephone triage, face-to-face appointments, and coordination of home visiting activity. In addition, face-to-face GP OOH appointments are available from a number of locality sites across Birmingham and Solihull during weekends and bank holidays, including Erdington Stockland Green PCC (North), West Heath Hospital (South), Washwood Heath Health and Wellbeing Centre (East), Sandwell Health Campus (West), Moseley Hall Hospital (Central) and Solihull Hospital (Solihull). However, the Aston hub remains the operational centre of the service and the only location providing a full range of OOH functions throughout the week.

4.6.5 Whilst a number of locality-based sites are utilised to support face-to-face appointments at weekends and bank holidays, the current configuration does not fully meet the needs of the Birmingham and Solihull population. The majority of sites operate only during limited periods, typically between 08:00 and 20:00 on weekends and bank holidays, and do not provide a consistent seven-day locality-based offer. As a result, patients requiring face-to-face OOH care during weekday evenings are predominantly directed to the Aston hub, creating a concentration of provision in the north of the system.

4.6.6 This means that patients living in the south, east, west, central and Solihull localities may experience significantly longer travel times, limited public transport options during out-of-hours periods, and reduced access to timely primary care intervention. Consequently, the current geographical distribution of OOH provision does not provide equitable access across the Integrated Care System footprint. Patients in some localities remain disproportionately reliant on telephone assessment, may decline appointments due to travel requirements, or may instead access Urgent Treatment Centres or Emergency Departments when travel to Aston is not feasible.

4.6.7 The current model therefore represents a partial locality-based approach rather than a fully operational distributed OOH service. Whilst the existing weekend and bank holiday appointment sites improve local access during specific periods, they do not provide comprehensive locality-based coverage throughout all OOH operating hours. The absence of a fully developed locality model limits the ability of the OOH service to function as an accessible, place-based component of the wider urgent and emergency care system. A more distributed model would be required to deliver equitable access aligned to population need and support the strategic ambition of providing urgent care closer to home.

4.6.8 The table below sets out the current locations from which face-to-face GP OOH appointments are delivered. Whilst these sites provide additional locality-based access, the majority operate on weekends and bank holidays only and do not constitute a fully operational locality-based service across all GP OOH hours.

GP Out of Hours Sites for Face to Face Appointments on Weekends and Bank Hoildays

Locality	Face to Face Appointment Sites	Operating Hours
North	Bourne Road, Aston	Monday to Friday 1830-0100 Weekends and Bank Holidays 24hr
North	Erdington Stockland Green PCC	Weekends and Bank Holidays 0800-2000
South	West Heath Hospital	Weekends and Bank Holidays 0800-2000
East	Washwood Heath Health and Well Being Centre	Weekends and Bank Holidays 0800-2000
West	Sandwell Health Campus	All day 0900-2100 Weekends and Bank Holidays 24hr
Central	Moseley Hall Hosptial	Weekends and Bank Holidays 0800-2000
Solihull	Solihull Hospital	Weekends and Bank Holidays 0800-2000

Activity and Utilisation

4.6.9 Service review and benchmarking for 2025/26 data indicates that the service is operating significantly below its commissioned capacity. Although a change to the Directory of Services (DoS) in April 2024 was intended to increase patient flow into the OOH service, activity has not risen to the expected level. The majority of activity continues to be managed by telephone, with only a small proportion converting to face-to-face consultations or home visits.

4.6.10 The activity mix for 2025/26 is:

- **86.4% telephone consultations**,
- **11.9% face-to-face appointments**, and
- **1.6% home visits**.

4.6.11 **Operational and Delivery Challenges**

4.6.12 A number of operational improvements agreed between the ICB and Badger Healthcare have been subject to significant delay. Extensions to face-to-face appointment availability were achieved only partially and several months later than planned. Implementation of video and photographic consultation capability was delayed by six months. Requests for an updated and comprehensive home-visiting policy have not yet resulted in a finalised document, despite repeated commissioner engagement.

4.6.13 These delays reduce the service's ability to respond flexibly to demand and limit the extent to which the OOH model can meet system expectations for urgent primary care.

Integration with Wider System

4.6.14 The OOH service plays an important role in supporting NHS 111, ambulance services and other urgent care pathways. However, operational inconsistencies have reduced the service's effectiveness as part of the broader urgent and emergency care system. Expected support for "call-before-convey" pathways has not been fully realised, and limited face-to-face availability

contributes to avoidable flow into urgent treatment centres and emergency departments. The CMDU function, although operational, is not being utilised to its full potential, and opportunities to accept referrals from neighbouring systems have not yet been maximised.

Strategic Alignment

4.6.15 The current GP OOH model does not fully align with national expectations for Integrated Urgent Care (IUC), nor with the BSol strategy for a coherent, place-based urgent care offer. There is a growing need for a more integrated and responsive OOH service that can:

- reduce unnecessary pressure on urgent treatment centres and emergency departments,
- provide timely alternatives to ambulance conveyance,
- operate consistently across localities.

Summary

4.6.16 The current GP OOH service model is no longer optimally designed or deployed to meet the needs of the BSol population or the requirements of an integrated UEC system. The combination of operational delays, underutilisation, and fragmented pathways demonstrates the need for a redesigned, modernised service model aligned to strategic priorities and capable of delivering better value and improved access for patients.

4.7 Key Implications for preferred option

4.7.1 The strategic context indicates that the future urgent care model needs to respond to a number of key requirements.

4.7.2 First, the model needs to provide consistent and clinically appropriate UTC services that meet the NHS England UTC Principles and Standards, while recognising the different roles that co-located and community-based services can play within the wider urgent and emergency care system.

4.7.3 Second, the model needs to support a shift towards care closer to home and within neighbourhoods, while maintaining strong integration with acute services for patients who require enhanced diagnostics, specialist clinical input or access to emergency care. This requires clear interfaces between UTCs, Emergency Departments, Same Day Emergency Care, primary care, GP Out of Hours, NHS 111 and community services.

4.7.4 Third, the future configuration needs to reflect the different population needs and circumstances across Birmingham and Solihull. Population size, deprivation, health inequalities, transport and accessibility vary across localities and therefore need to be considered alongside overall system-level requirements.

4.7.5 Fourth, the model needs to be clinically, operationally and financially sustainable, recognising the challenges associated with workforce availability, service capacity, estates, diagnostics and the wider financial position of the health and care system.

4.7.6 Finally, the future model needs to support the wider strategic objectives of improving patient flow, reducing avoidable pressure on Emergency Departments and ensuring that patients receive care in the most appropriate setting.

4.7.7 These requirements have informed the development and appraisal of the options considered within this DMBC and provide the strategic context for the proposed future service

configuration set out in Chapter 6.

5 CASE FOR CHANGE

5.1 Deprivation in Birmingham and Solihull & Population Need

5.1.1 Birmingham and Solihull is a diverse population with significant variation in levels of deprivation, health need and access to services across the Integrated Care System.

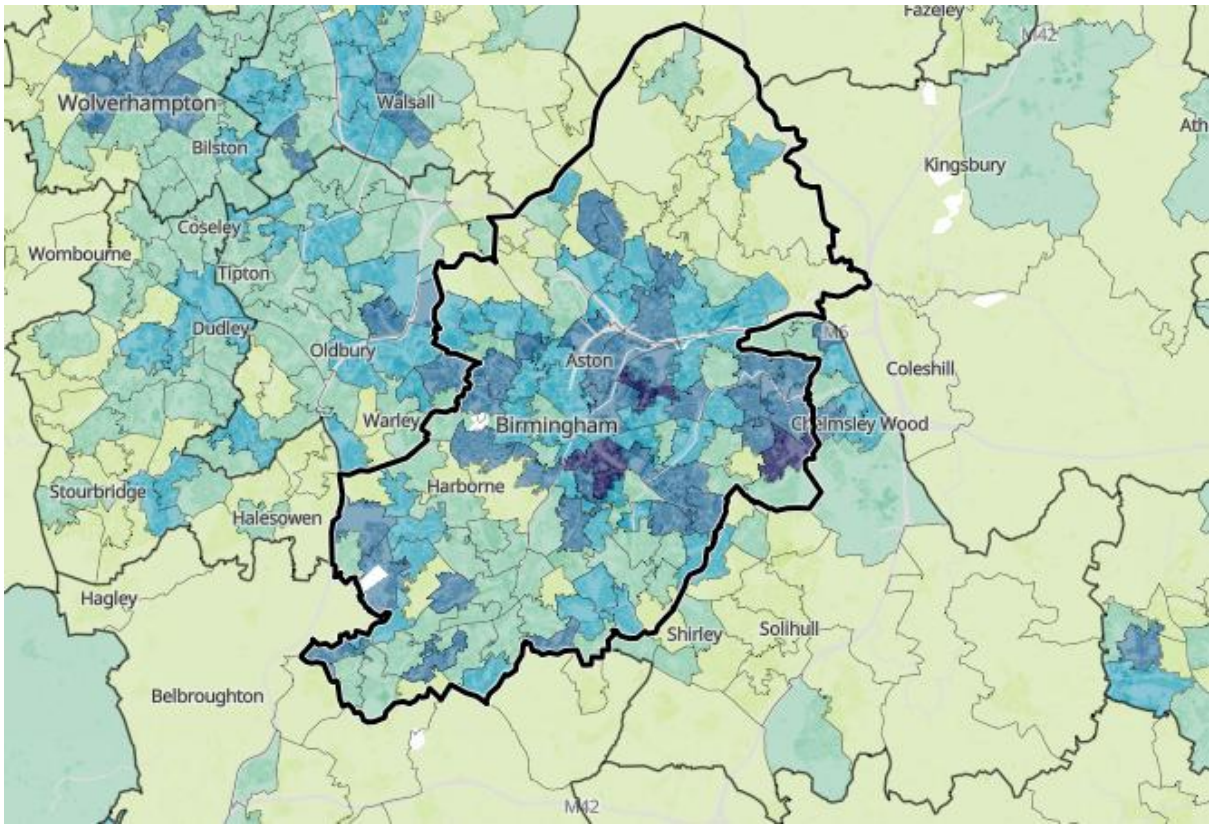
5.1.2 Deprivation is particularly concentrated in parts of Birmingham, with areas of high deprivation located across a number of the city's localities. Solihull has a lower level of deprivation overall but also contains areas experiencing significant socioeconomic disadvantage. The distribution of deprivation is therefore an important consideration when assessing the accessibility and configuration of urgent care services across Birmingham and Solihull.

5.1.3 The population profile has implications for the way urgent care services are designed and accessed. People living in areas of higher deprivation are more likely to experience poorer health outcomes and greater levels of health need, and the existing DMBC evidence identifies higher use of emergency services among more deprived populations.

5.1.4 The future configuration of urgent care therefore needs to consider not only the overall availability of services across Birmingham and Solihull, but also whether services are accessible to communities experiencing the greatest levels of health need. This includes consideration of travel time, public transport, availability of face-to-face care, digital exclusion and the ability of patients to navigate services appropriately.

5.1.5 Deprivation should therefore be considered alongside population health, patterns of urgent care use and the wider evidence on health inequalities when determining the future configuration of UTC and related urgent care services.

Birmingham and Solihull household deprivation 2021



Source: ONS census 2021 map of household deprivation: darker blue is more deprived

5.2 Health inequalities and access

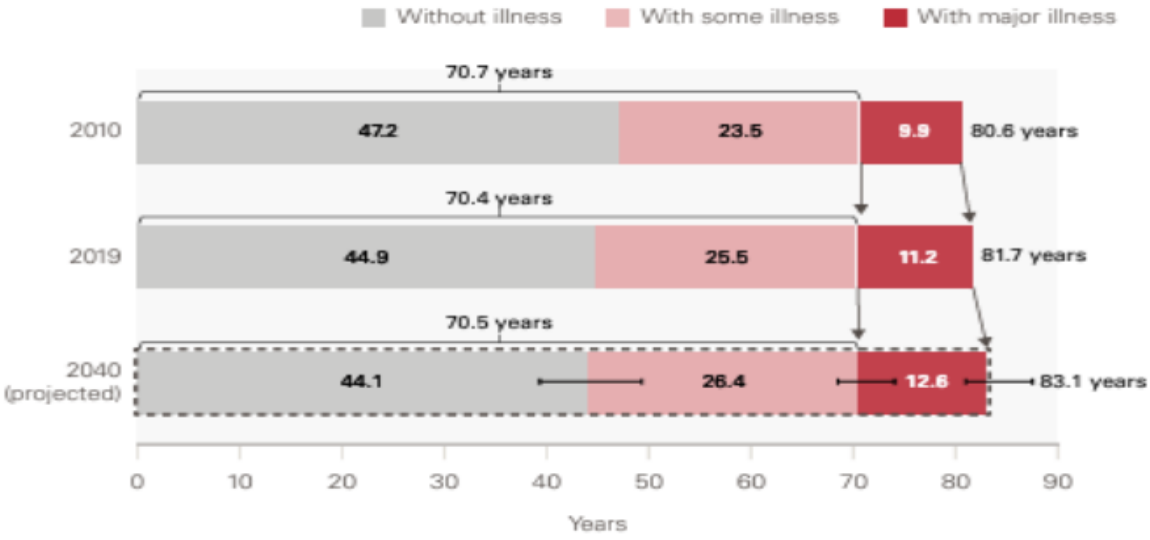
5.2.1 Health inequalities are an important consideration in the future configuration of urgent care services across Birmingham and Solihull. Population health need varies considerably across the area, and people experiencing poorer health outcomes may also experience greater need for urgent and emergency care.

5.2.2 The health profile of Birmingham and Solihull demonstrates significant variation between populations. Census 2021 data indicates that 9.6% of Birmingham's population reported being disabled and limited a lot, with a further 10.4% reporting being disabled and limited a little. In Solihull, the equivalent figures were 7.0% and 9.8% respectively. Self-reported health also differs, with 7.4% of Birmingham residents reporting bad or very bad health compared with 4.8% in Solihull.

5.2.3 There are also significant differences in life expectancy. The existing analysis identifies life expectancy for men in Birmingham as approximately three years below the England average and life expectancy for women approximately 2.1 years below the England average. Within the population, there is a substantial gradient in health outcomes, with people living in the poorest neighbourhoods experiencing considerably shorter life expectancy than those living in the most affluent neighbourhoods.

England life with illness

Figure 1: Average years of life people spend in different states of ill health, England, 2010, 2019 and projected for 2040



Source: Analysis of linked health care records and mortality data conducted by the REAL Centre and the University of Liverpool.

Notes: The black capped bars represent uncertainty intervals. The chart shows expectancy, which is a summary measure. In reality illness and death are distributed across all ages.

Source: REAL Centre and University of Liverpool

5.2.4 These differences are relevant to urgent care because levels of emergency service use are not evenly distributed across the population. Almost half of the population (49.4%) is in the most deprived fifth of the population of England. The most deprived of the local population use emergency services a third more than the least deprived population. Local benchmarking by BSol ICB shows this results in 77,000 more emergency attendances each year than the rate at which the least deprived population access emergency services. To move into the top 10% of areas in the country would require a reduction in emergency attendances of 168,000 annually, a reduction of a third.

5.2.5 The future urgent care model therefore needs to consider equity of access as well as overall capacity. A service may be geographically available across the system but still present barriers to some groups. These can include:

- limited access to private transport or reliance on public transport;
- disability or mobility needs;
- **c**ommunication and language requirements**;**
- digital exclusion or limited confidence using digital services;
- socioeconomic circumstances;
- **t**he need for face-to-face assessment rather than remote consultation; and
- difficulties navigating a complex urgent care system.

5.2.6 The consultation undertaken as part of this review reinforces the importance of these considerations. Among survey respondents, 46% reported a disability, long-term illness or health condition and 41% reported providing care or support to another person because of

long-term ill health, disability or problems associated with old age. The consultation analysis also identified the need to ensure that services remain accessible to people with limited English, people without digital access and people who require face-to-face care.

5.2.7 The consultation evidence should, however, be interpreted as the views of those who participated rather than as a statistically representative sample of the BSol population. The consultation analysis identified some groups as potentially under-represented, including young people, Muslim residents and lower-income households.

5.2.8 The future model therefore needs to incorporate appropriate mitigations for identified access barriers. This includes maintaining appropriate face-to-face routes, ensuring services are accessible to people with disabilities and communication needs, providing interpretation and translation where required, supporting people who cannot access services digitally, and ensuring that information and navigation arrangements are clear.

5.2.9 These considerations are particularly important when assessing the impact of changes to the location and configuration of urgent care services. Travel time and geographical accessibility should therefore be considered alongside population health need, deprivation, service utilisation and lived experience rather than being considered in isolation.

5.3 Current demand and Changing patterns of Urgent Care

5.3.1 Demand for urgent and emergency care across Birmingham and Solihull remains significant, although the pattern of activity across urgent care services has changed over time.

5.3.2 UTC activity has reduced over recent years, with average weekly activity falling from approximately 5,000 attendances per week in 2023/24 to approximately 3,500 per week in 2025/26. The largest reduction has been in face-to-face activity. This change needs to be considered alongside the wider pattern of activity across Emergency Departments, primary care, NHS 111 and other urgent care services, rather than interpreted as a reduction in the underlying need for urgent care.

5.3.3 The 2025/26 UTC/UTCAA baseline comprises approximately 336,762 attendances. Activity varies by site, time of day, presenting condition and patient characteristics. The existing analysis also identifies differences between the demographic profile of UTC users and the wider population. Children are over-represented within UTC activity, while people aged over 45 are under-represented. Females are also over-represented compared with the benchmark population. Ethnicity data is currently incomplete and therefore does not support robust conclusions regarding differences in access by ethnic group.

5.3.4 These patterns are important in considering the future service model. They indicate that demand is not uniform and that patients use urgent care services in different ways depending on age, presenting condition, access requirements and knowledge of available services.

5.3.5 GP Out of Hours activity also demonstrates a predominantly remote model of care. Based on 2026/27 activity to 31 August 2026, approximately 80.2% of activity was delivered by telephone, 18.4% face-to-face and 1.4% through home visits. This is relevant to the future design of GP OOH services, particularly in considering how remote assessment should interface with locally available face-to-face care and home visiting.

5.3.6 The future model therefore needs to respond not simply to the overall volume of demand,

but to the type, acuity, location and accessibility requirements of that demand. Capacity and activity modelling undertaken for the options appraisal is considered in more detail in Chapter 8.

5.4 Current UTC model and service limitations

5.4.1 Despite improvements, urgent care in Birmingham and Solihull remains under pressure, with demand for emergency care high and key performance metrics below Shortcomings of current UTC services.

5.4.2 The 6 UTCs across BSoL deliver a variable and inconsistent service, with no single overarching model of care/delivery. The table below sets out the gaps between current service provision and the eleven national standards. Across the ICB area these include:

- Variability in service provision leading to inequity and patient confusion;
- Limited acceptance of booked and walk-in patients;
- Availability of diagnostics, including lack of access to X-rays;
- Limited acceptance of ambulance conveyances;
- Adopting a GP-led model of care across the system.

Current services against national UTC standards

Urgent Treatment Centres: national standards	North Birmingham UTC	Erdington UTC	Washwood Heath UTC	South Birmingham UTC	Summerfield UTC	Solihull UTC
Locality	North	North	East	South	West	Solihull
Open 7 days a week, 12 hours a day as a minimum, typically increasing to 24 hours a day when co-located	Open 7 days; may close before 12 hrs	Open 7 days; may close before 12 hrs	Open 7 days; may close before 12 hrs	Open 7 days; may close before 12 hrs	Open 7 days; may close before 12 hrs	Open 7 days; may close before 12 hrs
See both booked and walk-in patients	x	x	x	x	√	x
See both minor injuries and minor ailments	Minor ailments only	Minor ailments only	Minor ailments only	Minor ailments only	Minor ailments only	*injuries treated at Solihull MIU*
See patients of all ages, including children under 2	√	√	√	√	√	√
Have a named senior clinical leader supported by an appropriate multi-disciplinary workforce	Nurse-led	√	Nurse-led	√	Nurse-led	√
Have a basic consistent investigative and diagnostic offering on site (with clear protocols if	x	x	x	x	x	*Solihull MIU has diagnostics*
Accept appropriate ambulance conveyance	x	x	x	x	x	*Solihull MIU receives ambulances*
Have access to patient records and the ability to send a post event message (PEM)	√	√	√	√	√	√
Report as a Type 3 daily on Emergency Care Data Set (ECDS)	√	√	√	√	√	√
Have an up-to-date directory of services (DoS) profile	√	√	√	√	√	√
Clearly communicate to the public what the service is for via consistent urgent treatment centre nomenclature, to ensure the service provision is understandable and accessible to all."	Not meeting standards	Not meeting standards	Not meeting standards	Not meeting standards	Not meeting standards	Not meeting standards

5.4.3 Previous engagement identified that the variation between services can contribute to patient confusion about where to go when they have an urgent care need. This is particularly relevant where patients may move between NHS 111, primary care, UTCs and Emergency Departments depending on their presentation and the route through which they access care.

5.4.4 The purpose of the strategic review is therefore not simply to change the location of services. It is to establish a more consistent and integrated urgent care model, with clearly defined roles for co-located UTCs, community UTCs, GP OOH, NHS 111 and other community and neighbourhood services.

5.4.5 The future model also needs to ensure that standardisation does not result in a loss of locally appropriate provision. The consultation and locality analysis demonstrated the importance that communities place on familiar services and accessible face-to-face care. These considerations have therefore informed the development and appraisal of the options.

5.5 Opportunities to shift activity from EDs to UTCs

5.5.1 Urgent and emergency care services across Birmingham and Solihull operate within a system experiencing sustained demand and pressure. Emergency Departments provide care for a wide range of presentations, including patients with conditions that may be appropriately managed through UTCs or other urgent care pathways.

5.5.2 The existing analysis indicates that a proportion of Emergency Department attendances may be suitable for management through UTCs or alternative urgent care pathways. Earlier modelling identified approximately 30% of Emergency Department attendances as potentially UTC-amenable using a locally developed algorithm, while an alternative approach based on national Healthcare Resource Group criteria identified approximately 20%. These estimates use different methodologies and should therefore not be interpreted as a single definitive proportion.

UTC-amenable activity by hospital

UTC-amenable activity by hospital	Total ED attendances 2024/25				*UTC Amenable*	*% UTC Amenable*	*Minor HRGs*	*% Minor HRGs*
	Walk In	Ambulance	Total					
Good Hope	84,177	24,367	108,544		*47,560*	*44%*	*33,475*	*31%*
Heartlands	123,524	39,320	162,844		*61,505*	*38%*	*35,638*	*22%*
Mid Met	61,962	22,709	84,671		*10,948*	*13%*	*N/A*	*N/A*
Queen Elizabeth	97,744	35,753	133,497		*46,253*	*35%*	*26,215*	*20%*
Overall	367,407	122,149	489,556		*166,266*	*34%*	*95,328*	*19%*

5.5.3 The significance of this analysis is that there is potential to improve the way patients are streamed and managed across the urgent and emergency care pathway, rather than assuming that all patients presenting to an Emergency Department require the same level of emergency care.

5.5.4 The future model therefore needs to provide effective alternatives for patients who require urgent but non-emergency treatment, while ensuring that patients requiring emergency or specialist care can access this without unnecessary delay.

5.5.5 This is particularly important at co-located sites, where UTCs can operate as part of an integrated front-door model alongside the Emergency Department. This enables patients to be clinically streamed to the service most appropriate to their needs and supports Emergency Departments to focus their capacity on patients requiring emergency care.

5.5.6 The opportunity is therefore not solely to increase UTC activity. It is to improve the distribution of activity across the urgent and emergency care system, supported by appropriate clinical pathways, diagnostics, workforce and integration between services.

5.6 Opportunities to improve urgent care pathways and shift activity

5.6.1 The case for change identifies an opportunity to improve the way urgent care is organised across Birmingham and Solihull by providing a more consistent and integrated range of services.

5.6.2 The future model provides opportunities to:

- improve streaming and diversion at acute hospital front doors;
- provide nationally specified UTC services for minor illness and injury;
- provide enhanced urgent care for patients whose needs require diagnostics, same-day assessment or closer integration with acute services;
- retain community-based UTC provision where this provides accessible care closer to local populations;
- develop locality-based urgent care services targeted at identified population and pathway needs;
- Deliver the most effective care in the most appropriate setting by strengthening interfaces between UTCs, GP Out of Hours, NHS 111, primary care, community services and pharmacy, supporting a left shift of activity to services such as Pharmacy First where clinically appropriate and ensuring patients receive the right care, in the right place, at the right time;
- reduce avoidable Emergency Department attendances where patients can safely be managed through an alternative pathway;
- improve patient navigation and understanding of available urgent care services; and
- **s**upport more equitable access for people who may experience barriers associated with transport, disability, language, digital exclusion or other circumstances.

5.6.3 The opportunity is therefore to move from a collection of variable urgent care services towards a more coherent system in which each service has a clearly defined role and pathway.

5.6.4 The preferred model also recognises that not all urgent care needs can or should be managed in the same setting. Co-located UTCs provide access to enhanced diagnostics and close integration with acute services, while community UTCs and locality urgent care services provide opportunities for care closer to home and within neighbourhoods. GP OOH provides an additional route for urgent primary care outside core GP opening hours.

5.6.5 The options appraisal considers how effectively each option delivers these opportunities, alongside the associated clinical, operational, workforce, financial, equality and access implications.

5.7 What this means for the future model

5.7.1 The evidence presented in this chapter identifies a number of requirements for the future urgent care model across Birmingham and Solihull.

5.7.2 First, consistency is required. Patients should be able to expect a clearly defined and clinically appropriate service regardless of which UTC they attend, while recognising the different functions of co-located and community services.

5.7.3 Second, services need to be integrated. UTCs cannot operate in isolation from Emergency Departments, NHS 111, primary care, GP OOH, Same Day Emergency Care and community services. Clear interfaces and referral pathways are required to ensure patients are managed in the most appropriate setting.

5.7.4 Third, access and health inequalities need to be explicitly considered. Overall travel-time improvements do not necessarily mean that every population or individual will experience improved access. The future model therefore needs to consider transport, disability, communication, digital exclusion, deprivation and the availability of face-to-face care alongside overall system capacity.

5.7.5 Fourth, the model needs to respond to different types of urgent care demand. This includes lower-acuity minor illness and injury, higher-acuity urgent presentations, patients requiring diagnostics or same-day assessment, and patients who require urgent primary care outside normal GP opening hours.

5.7.6 Finally, the future configuration needs to be clinically, operationally and financially sustainable. Workforce availability, estates, diagnostics, digital infrastructure, activity levels, procurement, transition and affordability all need to be considered alongside the benefits of the proposed model.

5.7.7 These requirements have informed the development of the future service model and the appraisal of Options A and B set out in Chapters 6 and 7.

6 FUTURE URGENT CARE MODEL & SERVICE CONFIGURATION

6.1 Vision for Future Urgent Care Model

6.1.1 The preferred model for Birmingham and Solihull is a networked urgent care model designed to provide patients with access to the right level of care according to their clinical need, while reducing unnecessary reliance on Emergency Departments and supporting care closer to home wherever clinically appropriate.

The model has been developed through the UTC Clinical Reference Group and assured by the UTC Programme Board, it is also underpinned by the NHS England Urgent Treatment Centre Principles and Standards, local activity and demand analysis, multidisciplinary clinical pathway work and consideration of patient and public feedback.

A formal multidisciplinary clinical review was undertaken during development of the Pre-Consultation Business Case (PCBC), including review of common Emergency Department presentations and the clinical criteria for management within UTCs, community services or Emergency Departments. The emerging clinical model was subsequently independently reviewed by the West Midlands Clinical Senate in April 2025. The Senate supported the need for redesign and recommended, as a minimum, that future UTC provision should meet the national UTC specification, alongside consideration of the opportunity to develop co-located UTCs at Good Hope, Heartlands and Queen Elizabeth, & Midland Metropolitan University Hospitals.

The clinical model was subsequently incorporated into the PCBC and has been further refined through the options appraisal, public consultation, equality and access analysis, activity and

capacity modelling and development of the preferred option.

The decision being sought through this DMBC is therefore primarily about the configuration and commissioning of urgent care services within an established clinical framework, rather than a fundamental reconsideration of the underlying clinical model.

6.2 Principles Underpinning the Future Model

6.2.1 The future model is designed around the following principles:

- Right care, right place, first time – patients should receive assessment and treatment in the setting best able to meet their clinical needs.
- Clinical safety – services must have the workforce, diagnostics, clinical leadership, governance and escalation arrangements required to manage the presenting level of acuity safely.
- Consistent standards – all UTCs will operate to the NHS England national UTC Principles and Standards.
- Care closer to home – patients who can be safely assessed and treated in community settings should not need to attend an acute hospital.
- Effective clinical streaming – patients presenting to acute hospitals should be assessed and directed to the most appropriate service, with rapid escalation to ED where required.
- Integrated urgent care – UTCs will operate as part of a wider urgent and emergency care pathway, rather than as stand-alone services.
- Accessible care – the model will retain walk-in access while strengthening NHS 111 pathways and providing face-to-face, virtual and home-based care according to clinical need.
- Improved access to diagnostics and specialist advice – appropriate diagnostic and referral pathways will support more patients to be assessed and treated without requiring an ED attendance.
- Equity and inclusion – the model must recognise the different barriers experienced by older people, disabled people, people with communication or language needs and people experiencing digital exclusion.
- Workforce sustainability – the service model must be deliverable through an appropriately skilled multidisciplinary workforce and support recruitment, retention and development.
- Financial sustainability – the future configuration must operate within the available financial envelope and make effective use of existing estates and capital investment.

6.3 Overview of Preferred Service Configuration

6.3.1 Option A establishes a network of four co-located UTCs and three community UTCs, supported by enhanced locality urgent care services and a reconfigured GP Out of Hours service.

The four co-located UTCs are located at:

- Queen Elizabeth Hospital Birmingham
- Birmingham Heartlands Hospital
- Good Hope Hospital
- Midland Metropolitan University Hospital

The three community UTCs are located at:

- Solihull
- Erdington/Stockland Green
- South Birmingham/Selly Oak

Enhanced Locality Urgent Care Services at:

- Washwood Heath; and
- Summerfield.

A reconfigured GP Out of Hours service, commissioned through three locality-based lots:

- South/Central;
- North/West; and
- Solihull/East.

6.3.2 The model therefore provides different levels of urgent care rather than seeking to make every site deliver the same service. The co-located UTCs provide the highest level of UTC capability, community UTCs provide a standardised community urgent care offer, locality services provide targeted specialist community capacity, and GP OOH provides urgent primary care outside core GP hours.

6.3.3 This distinction is important because the preferred model is not simply a redistribution of the existing six UTCs. It represents a different configuration of services, with each service having a defined role within the wider urgent care pathway. The intention is that these services operate as complementary components of a single urgent care system, with patients able to move between services according to their clinical need.

BSol UTC clinical model

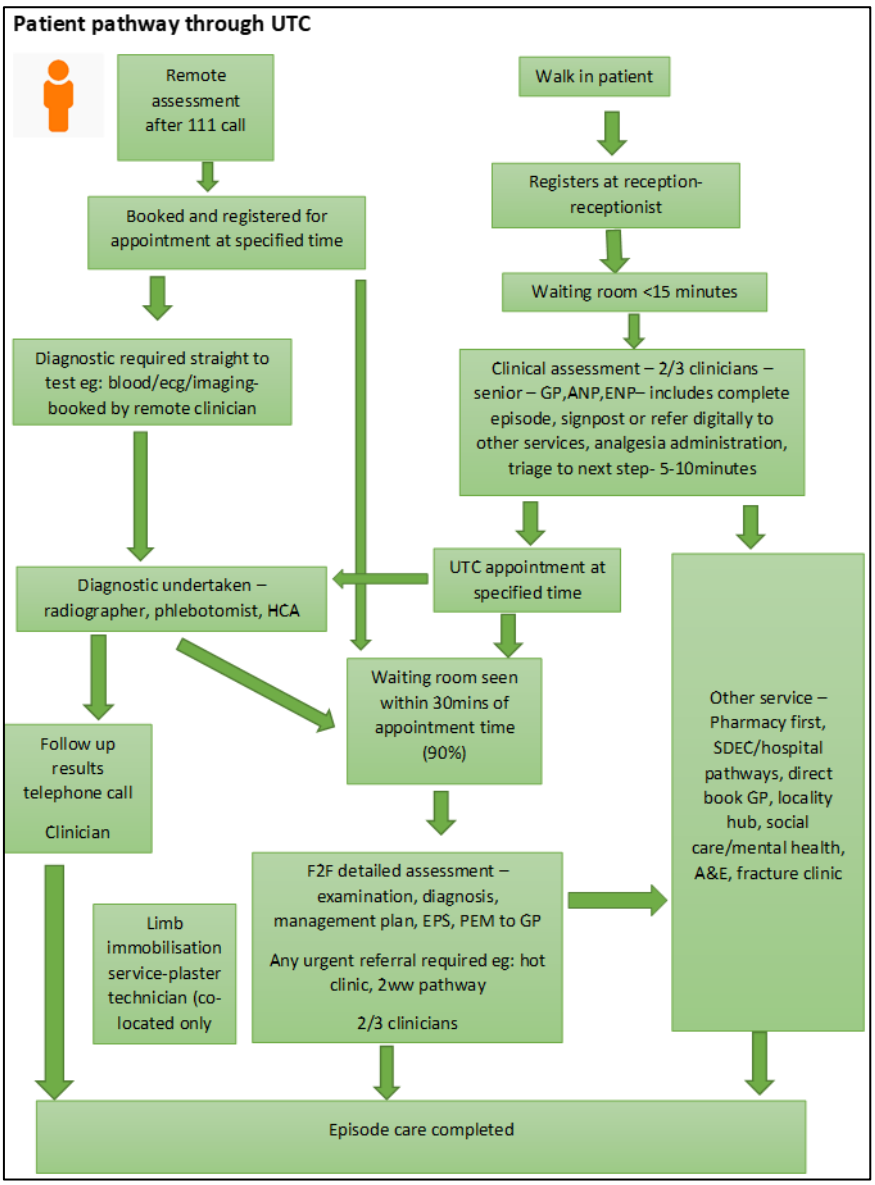
6.3.4 The table below sets out in more detail the patient disposition for the various streams.

BSol UTC clinical model – patient disposition

Patient presents with urgent care need: contacts NHS 111 or self-presents	Recognises that the most effective use of services is to ensure access through NHS 111 but that patients will self-present. Triage then needs to be undertaen in line with national UTC standards.
UTC / primary care disposition out of hours	Revised directory of services to ensure more patients are directed to a UTC in line with national standards and agreed clinical model
Remote assessment of patient by telephone or videa	The clinical model development group were clear that for a safe and effective service this was a critical step to ensure patients are directed and managed into the correct service avoiding duplication and reducing the number of contacts required.
Front door streaming assessment and face to face triage	At the front door this should be done for all patients including ambulance conveyances only excluding those with life-threatening conditions. After assessment this should include referring patients to other services such as to a local pharmacy.
Patient presents with urgent care need: contacts NHS 111 or self-presents	Recognises that the most effective use of services is to ensure access through NHS 111 but that patients will self-present. Triage then needs to be undertaen in line with national UTC standards.
Co-located UTC: Likely fractures / non-weight-bearing / requiring imaging	Seeing between 40-50% of current A&E activity - booked for - injuries including fractures, pathways to imaging in place, reporting expertise including radiographer, plastering, pathways for trauma follow-up, self-presentation to A&E where not life-threatening.
Community UTC: Illness requiring examination or simple diagnostics, near-patient testing: virology, bloods, ECG.	Booked activity post triage, patient requiring further examination / investigations to support management including infections, exacerbations of COPD, non-cardiac chest pain possible pulmonary embolism, swollen legs possible DVT / cellulitis, fever with no sepsis markers. Walk-in activity will assess and treat referring on where appropriate to specialist services or with clear pathways to direct to imaging and follow-up in co-located UTC. Will also take referrals from SPOA and ambulance referrals for non-life-threatening conditions.
Other service most appropriate: referred or signposted eg SDEC / UCR via SPOA or Pharmacy First	Where most appropriate based on initial history and triage, patients should be managed by an alternative service. This particularly applies to Pharmacy First and any patient with one of the 7 conditions suitable for Pharmacy First and no exclusions should be directed there and not seen in the UTC.
A&E life-threatening conditions only	Clinically agreed conditions such as suspected sepsis, significant breathing difficulties, myocardial infarction, stroke, significant trauma, signficant head injury requiring imaging, meningitis, subarachnoid haemorrhage, rapidly deteriorating patients.

6.3.5 The diagram below sets out the patient pathway through the UTC service.

BSol UTC clinical model – patient pathways



6.4 Co-located Urgent Treatment Centres

6.4.1 Role and Purpose

The four co-located UTCs will provide the highest level of UTC capability within the preferred model and will operate alongside the Emergency Departments at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital.

The co-located UTC will act as an important front door to urgent and emergency care, providing clinical assessment and streaming for appropriate patients presenting to the hospital, while also receiving patients referred through NHS 111 and other urgent care pathways.

The purpose is not simply to provide an alternative physical location for minor illness and injury. The co-located UTC will provide an enhanced clinical service capable of assessing and managing a broader range of urgent presentations, including patients who may otherwise have been directed to the

Emergency Department.

Patients requiring specialist Emergency Department care will continue to be escalated rapidly to ED. The model therefore strengthens the interface between UTC and ED rather than creating two separate services operating independently.

The co-located model is consistent with the clinical direction supported through the West Midlands Clinical Senate review and makes use of the clinical and diagnostic capability available within acute hospital environments treatment Centres are one part of a fully functioning urgent care system.

6.4.2 Clinical Scope

The co-located UTCs will provide assessment and treatment for:

- minor illness and minor injury across all age groups;
- more complex urgent illness presentations where clinically appropriate;
- more complex minor injuries;
- ambulatory urgent care;
- same-day urgent assessment and treatment;
- appropriate frailty presentations;
- patients who can be safely managed without requiring full ED assessment; and
- patients requiring further investigation or specialist advice but who do not necessarily require admission or full Emergency Department care.

The service will therefore provide a broader clinical offer than the current variation across UTCs and will support a more consistent approach to the management of urgent presentations across Birmingham and Solihull.

6.4.3 Streaming and redirection from Urgent Treatment Centres

A key function of the co-located UTC is the ability to stream patients according to clinical need.

Patients presenting to an Emergency Department, including appropriate non-life-threatening ambulance conveyances, should be assessed and directed to the most appropriate pathway. Where the patient can safely be managed within the UTC, this avoids an unnecessary ED episode.

Where a patient requires Emergency Department assessment, there must be a clear and rapid escalation route between the UTC and ED.

This model is intended to improve patient flow, reduce duplication and support Emergency Departments to focus their capacity on patients requiring emergency and specialist care.

6.4.4 Diagnostics and specialist pathways

A significant benefit of co-location is access to the diagnostic and specialist infrastructure of the acute hospital. The future model will provide access, as clinically appropriate, to:

- X-ray;
- ECG;
- CT;
- MRI and other relevant imaging;

- point-of-care testing;
- pathology;
- enhanced laboratory investigation; and
- specialist clinical advice.

The current service description identifies potential future use of tests including CRP, troponin and D-dimer where clinically appropriate.

The co-located UTC will also establish direct pathways into Same Day Emergency Care, specialist services, frailty and ambulatory emergency care. This will enable patients to move from assessment to the appropriate next clinical intervention without unnecessary duplication or an automatic ED admission pathway.

6.5 Community Urgent Treatment Centres

6.5.1 Role and Purpose

Three community UTCs will be retained and developed at Solihull, Erdington and South Birmingham. These services will meet the national UTC specification while providing a community-based urgent care offer focused on patients whose clinical needs can be safely managed outside an acute hospital.

The community UTCs therefore provide an important balance within the preferred model. While the co-located UTCs provide enhanced urgent care capability at the front door of acute hospitals, community UTCs maintain accessible local urgent care provision and support the principle of treating patients closer to home. The three sites will be upgraded to provide a more consistent service, reducing the variation currently experienced between UTCs.

The community UTCs will not operate as isolated urgent care facilities. They will form part of the wider neighbourhood health model. The service will have defined pathways into:

- Pharmacy First;
- Urgent Community Response;
- the 999/111 Single Point of Access;
- Integrated Neighbourhood Teams;
- community nursing;
- mental health services;
- social prescribing; and
- other community services.

This creates an opportunity to identify patients whose urgent presentation is also an indicator of a wider health or care need and connect them with the appropriate ongoing support. For example, a patient repeatedly presenting with urgent care needs may benefit from proactive intervention through an Integrated Neighbourhood Team rather than continuing to rely on episodic urgent care.

This is an important component of the future model because the objective is not solely to move activity between services. It is to improve the way the urgent care system responds to patients and, where appropriate, connects them to services that can address the underlying need.

6.5.2 Urgent Illness and Minor Injuries

The community UTCs will provide assessment and treatment for a broad range of urgent but non-life-threatening illnesses, including:

- acute infections;
- respiratory illness;
- skin conditions and rashes;
- ear, nose and throat conditions;
- gastrointestinal illness;
- urinary symptoms and infections;
- fever in adults and children;
- Support for people living with frailty; and
- acute deterioration of long-term conditions requiring same-day assessment.

Services will also support prescribing, medicines management, self-care advice and health promotion where appropriate. The intention is that patients receive a clinically appropriate assessment and treatment without needing to travel to an acute hospital where hospital-based intervention is not required.

Community UTCs will also provide assessment and management of appropriate minor injuries, including:

- minor wounds and lacerations;
- minor burns and scalds;
- sprains and strains;
- minor musculoskeletal injuries;
- removal of superficial foreign bodies; and
- basic wound closure.

Where an injury requires diagnostics or specialist intervention that cannot be provided within the community UTC, a defined referral pathway will be used to access the appropriate acute service

6.5.3 Community Diagnostics

Access to appropriate diagnostics is an important part of the future community UTC model. The proposed service includes point-of-care and community-based diagnostics such as:

- pregnancy testing;
- urine testing and culture;
- swab testing;
- glucose;
- haemoglobin;
- electrolytes; and
- ECG where appropriate.

There will also be direct clear referral pathways into imaging services, including X-ray and CT, where required. This approach is intended to avoid unnecessary transfer to an Emergency Department simply because a diagnostic investigation is required.

6.6 Enhanced Locality Urgent Care Service

6.6.1 Role and Purpose

Under the recommended Option A configuration, Summerfield and Washwood Heath will no longer operate as community Urgent Treatment Centres (UTCs). Instead, these sites will form part of an enhanced locality urgent care model, providing targeted specialist assessment, diagnostics, treatment and prevention services aligned to identified population needs and gaps in existing provision.

This distinction is important. The enhanced locality services are not intended to replicate the national UTC specification or simply replace the existing nurse-led UTC model with an equivalent service under a different name. Rather, they will provide a more targeted, specialist-led offer that complements core primary care, community services and the co-located UTCs, supporting patients who can be safely managed outside a hospital setting.

The model will be developed around local population need, activity and service gaps, with the aim of improving access to appropriate community-based urgent care, supporting earlier intervention and reducing avoidable emergency department attendances and hospital admissions.

The proposed locality model will be tailored to the needs of each locality and developed through implementation planning. Current proposals focus on supporting people living with long-term conditions to access preventative care, evidence-based treatment and specialist support both when clinically stable and when experiencing an acute deterioration. Initial pathway development has identified respiratory disease, diabetes and heart failure as priority areas, building on existing services and addressing identified gaps in provision. The detailed service specifications, referral pathways and workforce models will be refined through the next phase of business case development.

6.6.2 Clinical Model

The enhanced locality urgent care services will provide targeted specialist assessment, diagnostics and treatment for patients who can be appropriately managed within the community. Access will primarily be through professional referral and agreed, clinically triaged pathways, rather than unrestricted walk-in attendance.

Referral routes are expected to include:

- General Practice;
- Single Point of Access;
- community and neighbourhood services;
- Urgent Community Response;
- hospital services, including relevant assessment, discharge and specialist teams; and
- other appropriate healthcare professionals.

The intention is to provide additional specialist assessment and clinical intervention beyond that routinely available through core primary care, while complementing existing community provision and supporting the wider urgent and emergency care system.

Potential areas for development include acute respiratory assessment and treatment, specialist

diabetes stabilisation and other targeted pathways identified through locality-level service development. The final clinical configuration, eligibility criteria, referral arrangements, diagnostics, workforce, operating hours and capacity will be established through further modelling and detailed business case development.

6.6.3 West Birmingham Service Offer – Summerfield Primary Care Centre

West Birmingham already has an established and successful adult and paediatric respiratory service within the West Locality Hub, demonstrating positive outcomes in relation to vaccine-preventable admissions and reductions in admissions for ambulatory care sensitive conditions. The current adult respiratory service operates as a proactive, risk-stratified model, using primary care registers and patient records to identify adults with known respiratory disease who would benefit from specialist assessment, treatment optimisation and ongoing review.

The paediatric respiratory service accepts referrals for specialist assessment and diagnostics, including spirometry and fractional exhaled nitric oxide (FeNO) testing, supporting earlier diagnosis and management of respiratory conditions. These services provide strong links to prevention, supported self-management and neighbourhood care. However, neither service currently provides an acute respiratory illness pathway and current funding arrangements for the existing respiratory services are confirmed only until March 2027.

Under the recommended Option A Plus configuration, Summerfield UTC, located at Summerfield Primary Care Centre, would transition from its current walk-in UTC function to a specialist locality assessment, treatment and prevention hub, comprising an enhanced all-age respiratory pathway and a new Intensive Diabetes Stabilisation Service for adults. This configuration would complement the planned co-located UTC at Midland Metropolitan University Hospital (MMUH) and support the wider urgent and emergency care system by providing alternative community-based pathways for patients who would otherwise be at risk of attendance or admission to hospital.

Access would be through professionally referred and clinically triaged pathways, supported by clearly defined eligibility criteria, clinical governance arrangements and escalation processes for patients requiring secondary care intervention. The precise scope, workforce model and capacity requirements would be developed through a detailed business case.

The proposed all-age respiratory pathway would build upon and integrate with the existing adult and paediatric respiratory services whilst introducing additional capability to manage acute respiratory illness and deterioration in existing respiratory conditions. Unlike the current model, which focuses primarily on proactive case finding, diagnostic assessment and long-term condition optimisation, the enhanced pathway would provide rapid specialist assessment, diagnostics, treatment and follow-up for patients experiencing acute episodes who can be safely managed outside an acute hospital setting. Referrals would be accepted via agreed pathways from general practice, community and neighbourhood services, Urgent Community Response teams and secondary care services.

Prevention and early intervention would remain central components of the model through vaccination assessment, treatment optimisation, supported self-management and early identification of deterioration, with strong links to Integrated Neighbourhood Teams and existing respiratory provision. The intended outcomes are earlier intervention, improved continuity of care, reductions in avoidable respiratory-related emergency attendances and admissions, and improved respiratory health outcomes across the population.

The proposed Intensive Diabetes Stabilisation Service would represent a new service offer within the West Locality Hub. The service would provide adults with a time-limited period of specialist assessment, treatment optimisation, education and monitoring, typically over a 12 to 16-week period,

where a level of intervention greater than that available through routine primary care or existing DICE services is required. The service would focus on patients at increased risk of diabetes-related emergency attendance or admission, including those recovering from an acute diabetes admission, experiencing recurrent or severe hypoglycaemia, or requiring intensive support for symptomatic hyperglycaemia or complex treatment adjustments.

Provision would include specialist diabetes nursing or advanced clinical practitioner support, prescribing and pharmacy input, access to dietetic and engagement services, and consultant diabetologist oversight through a multidisciplinary arrangement with SWBT. Referrals would be accepted from general practice, DICE services, hospital diabetes and discharge teams, Same Day Emergency Care (SDEC) services and other community-based professionals. The service would combine face-to-face and remote follow-up, with responsibility for prescribing and treatment adjustment throughout the intervention period prior to planned transfer of care back to the patient's registered practice.

The intended benefits of the diabetes pathway include reductions in diabetes-related emergency department attendances, admissions, readmissions and ambulance conveyances, alongside supporting earlier and safer discharge from hospital, improved treatment stability and enhanced patient confidence in self-management. These benefits would be assessed primarily in individuals recovering from acute diabetes crises or experiencing significant instability in their diabetes management, with longer-term improvements in glycaemic control evaluated separately through established diabetes outcome measures.

Across both pathways, the detailed business case would establish demand, activity baselines, eligibility criteria, workforce requirements, diagnostic capacity, service hours, prescribing costs, funding requirements and implementation arrangements. It would also quantify the anticipated additional benefits achieved over and above current provision.

Key risks include the availability of sustainable specialist workforce and funding, the requirement to secure continuation of existing respiratory services beyond March 2027, potential displacement of existing provision, and patient misunderstanding regarding the change from walk-in to referral-based access arrangements. A comprehensive transition and mobilisation plan would therefore be required, addressing continuity of care, travel and accessibility considerations, engagement with underserved populations, language and communication requirements, public and professional communications, signage and wayfinding, management of patients presenting without referral, and clinical streaming arrangements between Summerfield and MMUH. The business case establishes the strategic direction of travel, with the detailed operating model, investment requirements and implementation plans subject to further development and formal approval.

6.6.4 East Birmingham Service Offer – Washwood Heath Primary Care Centre

The East Locality proposal builds on the existing specialist Heart Failure Ambulatory Service at Washwood Heath Health and Wellbeing Centre and expands it into a broader neighbourhood-based rapid access model, providing earlier intervention, direct access from community and primary care services, and improved alternatives to Emergency Department attendance and hospital admission. The proposal aligns with the wider strategic ambition to provide more care closer to home and reduce reliance on acute pathways.

The current service is a seven-day specialist ambulatory heart failure service operating from 08:00-18:00. Patients requiring intravenous diuretic therapy, medication optimisation and enhanced monitoring are referred predominantly from hospital cardiology teams, inpatient wards, emergency departments and specialist heart failure teams.

The current offer includes:

- Ambulatory IV diuretic (furosemide) treatment as an alternative to inpatient admission.
- Heart failure assessment and monitoring, including blood pressure, pulse, oxygen saturation, weight monitoring and blood testing.
- Advanced Clinical Practitioner review, prescribing and treatment optimisation.
- IV iron (Ferinject) administration and medication optimisation where clinically appropriate.
- Access to diagnostics including echocardiography, ECG, phlebotomy and imaging.

Currently, the service is largely focused on patients with established heart failure who require ambulatory treatment and monitoring. It does not provide a comprehensive rapid access pathway for referrals directly from primary care, community services or urgent care settings.

The East Locality Hub enhancement proposes an expansion into an Integrated Rapid Access Heart Failure Service.

New services will include:

- Rapid Access Heart Failure Clinics providing same-day, 24-hour or 72-hour specialist assessment depending on clinical need.
- Consultant-supported and ACP-led urgent heart failure assessment and treatment.
- Structured post-discharge heart failure reviews within 2-14 days of discharge to reduce deterioration and readmissions.
- Dedicated ACP-led Advice and Guidance service for GPs, PCNs, neighbourhood teams, community teams, frailty services and intermediate care services.
- Virtual monitoring and medication optimisation for patients requiring closer follow-up following treatment changes or clinical deterioration.
- Direct referral routes from primary care, community services, UTCs and hospital teams.
- Expanded medication optimisation and treatment escalation pathways.
- Greater utilisation of existing co-located diagnostics.

Access routes will be significantly expanded compared with the current model and patients may be referred by:

- General Practice.
- Primary Care Networks.
- Community heart failure teams.
- Intermediate care and neighbourhood teams.
- Frailty services.
- Urgent Treatment Centres.
- Hospital specialist teams.
- Inpatient wards and Emergency Departments.

- Post-discharge pathways following hospital admission.

The service will provide a visible alternative route to Emergency Department attendance for appropriate patients with deteriorating heart failure.

Under the proposed model, Washwood Heath Health and Wellbeing Centre becomes the East Locality specialist heart failure hub.

Anticipated Benefits

The proposed East Locality model is expected to deliver improved access to specialist heart failure care by providing earlier assessment and intervention closer to home, reducing the need for patients to attend Emergency Departments or be admitted to hospital unnecessarily. By introducing rapid access clinics, enhanced medication optimisation, virtual monitoring and structured post-discharge follow-up, patients will receive more timely and proactive management of deterioration, supporting improved clinical outcomes, reducing the risk of readmission, and reduced cardiology length of stay.

The enhanced service will adopt a proactive and preventative approach to heart failure management, focusing on the early identification and treatment of patients at risk of deterioration before crisis intervention is required. Patients will receive timely intervention at an earlier stage of disease progression and this anticipatory model will support improved self-management, reduce avoidable exacerbations, prevent unnecessary Emergency Department attendances and hospital admissions, and enable patients to remain independent and well within their own homes and communities for longer.

The service will create a clear alternative to acute hospital pathways for appropriate patients, helping to avoid admissions, reduce pressure on Emergency Departments and release acute hospital capacity. It will also support a more consistent and equitable service offer across the East Locality, reducing unwarranted variation in access to specialist heart failure care and improving the overall patient experience.

In addition, the enhanced model will strengthen integration between primary care, community services, urgent care and specialist hospital teams through a single point of access, specialist advice and guidance, and shared care arrangements. This will support more coordinated care delivery, improve clinical decision-making and ensure patients are managed in the most appropriate setting first time

To support implementation, the locality model will have:

- Clear communications describing which patients should attend Washwood Heath Health and Wellbeing Centre rather than Emergency Departments.
- Consistent referral criteria and referral pathways across GP practices, UTCs, community teams and acute services.
- Improved wayfinding and signage within the site to support direct access to the Heart Failure Service.
- Standardised streaming protocols across UTCs, EDs and cardiology services to direct suitable patients into the rapid access pathway.
- Demand management arrangements including triage thresholds, referral criteria, activity monitoring and phased implementation to ensure demand remains aligned to available workforce capacity

This model positions Washwood Heath Health and Wellbeing Centre as the East Locality specialist heart failure hub, providing a clear alternative to acute admission and supporting the wider

neighbourhood health ambition of delivering more proactive, integrated care closer to home.

The overarching aim of the locality model is to support patients with long-term conditions to receive preventative care, evidence-based treatment and specialist support both when clinically stable and when acutely unwell. Through revised clinical pathways and closer integration between primary care, community services, urgent care and specialist teams, the service will help more people receive care in their own homes and communities, reducing avoidable Emergency Department attendance and hospital admission.

6.7 GP Out of Hours

6.7.1 Role and Purpose

GP Out of Hours (OOH) provides an important component of the future urgent care model, providing urgent primary care for patients whose needs cannot safely wait until their GP practice is next open. It will therefore sit alongside, rather than duplicate, UTC and acute urgent care provision.

The GP OOH model has been refined following the public consultation and subsequent Health Overview and Scrutiny Committee (HOSC) feedback. This refinement is particularly important given the strong and consistent feedback regarding the value of local, community-based care and access to services closer to home.

The consultation identified a number of common priorities across Birmingham and Solihull, including accessible local urgent care, minimising travel, maintaining face-to-face care and ensuring that services are responsive to the needs of individual communities. In relation specifically to GP OOH, respondents recognised the benefits of a coordinated service across Birmingham and Solihull, including consistency and reduced fragmentation, but also expressed concern that a wholly centralised model could reduce local accessibility. The consultation analysis therefore identified support for a hybrid approach, combining central coordination with locally accessible and flexible delivery.

This was reinforced through the supplementary locality analysis [appendix 2 - Supplementary Locality Analysis]. Across the localities, there was a consistent preference for care closer to home and retention of local provision, although the emphasis varied. North and East Birmingham respondents, for example, expressed support for locality-based GP OOH services and stronger links with local hubs and NHS 111. Solihull respondents expressed particular concern that a single Birmingham and Solihull-wide service could be less responsive to local needs and showed stronger support for locally organised OOH provision. Across the localities, home visiting was also identified as an important practical feature of GP OOH, particularly for people who are housebound, older, disabled, living with significant health conditions or caring for young children.

The scrutiny feedback similarly highlighted the need for the future model to consider familiar local services, travel, public transport, parking, digital exclusion, language barriers and continued face-to-face access in practical terms. HASC/HOSC feedback therefore provided an important consideration in refining the GP OOH model to ensure that the benefits of a consistent service specification are balanced with the need for locally accessible care.

6.7.2 Locality-based Commissioning

In response to this evidence, the preferred approach is to procure GP OOH through three locality-based lots:

- South/Central

- North/West
- Solihull/East

Each lot will operate to a common overarching clinical and service specification, while providing locality-specific arrangements for face-to-face and home visiting provision. The locality-based commissioning approach also supports alignment with the proposed UTC model, enabling GP Out of Hours services to work closely with local UTC provision and wider urgent care pathways. This will strengthen integration between services, support consistent patient navigation and facilitate the delivery of care within local systems while maintaining a common Birmingham and Solihull service specification. This approach represents an important refinement to the original Option A proposal, which envisaged GP OOH being recommissioned as a single service across Birmingham and Solihull. The Programme Board agreed that the specification should instead explicitly allow for a locality/lot-based commissioning approach in response to consultation and HASC feedback.

The intention is to retain the advantages of a coordinated BSol-wide service, including consistent standards, NHS 111 access and the ability to manage the service as a coherent part of the urgent care pathway while creating greater opportunity for care to be delivered within local communities.

The three locality groupings are therefore a commissioning and planning framework rather than an intention to create three entirely separate services. Further localisation within these footprints may be considered where this is supported by population need, affordability, workforce availability and the provider market. This provides flexibility to develop arrangements that are responsive to local circumstances while retaining a consistent system-wide specification.

6.7.3 **Service Offer**

Access to GP OOH will continue through NHS 111, which will undertake clinical assessment and determine whether a patient's needs can safely wait until their GP practice reopens or require an urgent primary care response. The service will provide a combination of:

- virtual clinical assessment and treatment;
- face-to-face assessment and treatment; and
- home visits where clinically appropriate.

The balance between these forms of provision will be determined by clinical need. The intention is not to create a predominantly remote service, but to ensure that patients can access face-to-face and home-based care where this is clinically necessary or where travel to a service would create a significant barrier.

This is particularly relevant to groups identified through consultation as potentially experiencing greater barriers to accessing urgent care. Locality analysis highlighted concerns about reliance on telephone-based assessment for people with learning disabilities, hearing impairments, limited English, anxiety or other communication needs. There was also concern that a more centralised model could increase waiting times for home visits where clinicians were required to cover larger geographical areas.

The current service description proposes core out-of-hours coverage from 6.30pm to 8.00am Monday to Friday, with weekend provision from 6.30pm Friday to 8.00am Monday for practices that have opted out of OOH provision. The final service specification, operating arrangements and detailed locality provision will be confirmed through procurement and mobilisation.

6.7.4 **Role within the wider community urgent care model**

6.8.4 Rationale for the amended model

The amended GP OOH model responds directly to the evidence gathered through consultation and scrutiny. It seeks to balance two potentially competing requirements identified through the engagement: System-wide consistency and coordination, including:

- a common clinical specification;
- consistent quality and governance;
- NHS 111 as the principal access route;
- coordinated service management; and
- reduced fragmentation across Birmingham and Solihull.

Local accessibility and community-based care, including:

- provision organised around localities;
- access to face-to-face care;
- home visiting where clinically appropriate;
- reduced reliance on travelling to hospital sites;
- responsiveness to local population needs; and
- stronger integration with community and neighbourhood services.

The proposed three-lot approach is therefore intended to provide a balance between consistency and local responsiveness, rather than representing a move from a single coordinated service to three completely separate services.

This refinement also reflects the Programme Board's consideration of the consultation and HASC/HOSC feedback and enables Option A to incorporate an important element of the localised care model that was valued by respondents who supported Option B, while retaining the wider configuration of Option A.

6.8 Workforce, Estates and Digital Enablers

6.8.1 Delivery of the preferred model will require appropriate workforce, estates, diagnostic and digital infrastructure across the different service types.

6.8.2 Workforce requirements will vary according to the clinical role of each setting. Co-located UTCs will require a broader multidisciplinary workforce reflecting their ability to manage more complex urgent presentations, while community UTCs and locality services will require workforce models appropriate to their defined clinical scope.

6.8.3 Final workforce requirements will need to reflect activity and demand modelling, agreed opening arrangements, clinical acuity, skill mix, recruitment and retention, training requirements and the implications of TUPE/COSOP where applicable.

6.8.4 The future model will also depend on suitable estates and diagnostic capacity. The co-located UTC developments are being progressed alongside the relevant hospital estate programmes, while community and locality services will require appropriate consultation, treatment and diagnostic facilities.

6.8.5 Digital infrastructure will support the integration of services and patient pathways. This includes appropriate use of shared care records, electronic patient records, e-prescribing, digital triage and clinical assessment tools, clinical decision support and referral and pathway management.

6.8.6 Workforce, estates and digital infrastructure are critical enablers of the future urgent care model. Their development will need to remain closely aligned to the proposed service configuration to ensure that the model is safe, sustainable, accessible and capable of delivering integrated care across Birmingham and Solihull.

6.9 Access, Choice and Inclusion

6.9.1 The future model is intended to retain a range of access routes, recognising that different patients and communities will have different needs.

6.9.2 NHS 111 will provide an important route into booked urgent care, while walk-in access will remain available. The Programme Board confirmed that the intention is for both the co-located and community UTCs within Option A to accept walk-in patients, subject to appropriate clinical triage.

6.9.3 The model also recognises that digital and telephone access alone will not meet the needs of all communities. Face-to-face provision, local community locations and appropriate reasonable adjustments will therefore remain important components of the model.

6.9.4 Accessibility considerations include transport and parking, support for people with disabilities, interpretation and communication support, and consideration of people who may experience digital, financial or other barriers to accessing services.

6.9.5 The consultation and scrutiny process identified the importance of clear public information and consistent service naming, particularly where different urgent care services have different functions. The ICB will therefore need to develop clear communications explaining what each service provides, how patients should access it and when NHS 111 should be used.

6.9.6 Detailed equality and health inequality impacts are considered separately in Chapter 11.

6.10 Key Implementation enablers

6.10.1 Successful implementation of the preferred UTC model will be supported by a number of key enablers, including:

- Appropriate premises that are accessible, fit for purpose, capable of supporting future demand and located to maximise access for local populations.
- Integrated digital systems, including access to the Shared Care Record ("Your Care Connected"), electronic patient records and interoperability with GP, community and acute care systems.
- Electronic prescribing;
- Interpretation, translation and accessible communication services to support equitable access for all patients.
- Digital tools to support clinical triage, appointment booking, remote consultation, referral management and real-time sharing of patient information across organisations.
- Clearly defined referral pathways with Emergency Departments, Same Day Emergency Care,

GP Out of Hours, community services, mental health services, locality hubs and other relevant partners to support integrated patient care.

- Veteran-aware accredited provision to support equitable access and tailored care for veterans and the Armed Forces community.

7 CONSULTATION, ENGAGEMENT AND RESPONSE

7.1 Overview

7.1.1 Consultation and engagement have been integral to the review of Urgent Treatment Centre (UTC) and GP out-of-hours (OOH) services throughout the programme.

7.1.2 This has included early patient, public, staff and stakeholder involvement; sustained engagement with local authority health scrutiny; a ten-week formal public consultation; independent analysis of consultation findings; additional locality analysis commissioned following scrutiny feedback; and consideration of this evidence through the ICB's post-consultation decision-making process.

7.1.3 The consultation did not result in a decisive preference for either Option A or Option B. Instead, respondents frequently supported different elements of the two models. In particular, there was evidence of support for the UTC configuration associated with Option A alongside a more localised and integrated approach to GP OOH provision associated with Option B.

7.1.4 This evidence has been considered alongside clinical, equality, access, travel, operational, workforce, estates, financial and deliverability evidence in developing the recommendation within this DMBC.

7.2 2. How we listened

Statutory duties and consultation principles

7.2.1 There are a range of legal and regulatory requirements governing patient and public involvement in NHS service change.

7.2.2 The NHS Act 2006, as amended, places duties on relevant NHS bodies to involve patients and the public in the planning and provision of services, the development and consideration of proposals for changes in the way services are provided, and decisions affecting the operation of services. For ICBs, the relevant public involvement duty is contained within section 14Z45.

7.2.3 Section 23(1) of the Local Authority (Public Health, Health and Wellbeing Boards and Health Scrutiny) Regulations 2013, as amended, requires NHS organisations to consult relevant Overview and Scrutiny Committees on proposals for a substantial development of the health service in the area of the local authority, or a substantial variation in the provision of services.

7.2.4 The involvement and consultation process has also been informed by the Gunning Principles; the Equality Act 2010 and Public Sector Equality Duty; the ICB's Working with People and Communities Strategy; its Service Change Policy; and the Memorandum of Understanding for Proposed Service Change agreed with Birmingham City Council and Solihull Metropolitan Borough Council's Joint Health Overview and Scrutiny Committee.

7.2.5 The consultation was designed around the four Gunning Principles:

- consultation took place while proposals remained at a formative stage and before a final decision had been made;
- sufficient information was provided to enable people to understand and consider the proposals;
- the formal consultation ran for ten weeks, providing adequate time and multiple routes to participate; and
- consultation responses have been independently analysed and considered before any final decision is made.

7.2.6 The detailed consultation methodology, statutory framework and planned assurance arrangements are set out in Appendix 3 – Consultation Plan.

Pre-consultation engagement

7.2.7 Patient and public involvement began before the shortlisted options were finalised.

7.2.8 Initial work undertaken with Healthwatch Birmingham and Healthwatch Solihull explored how people accessed UTCs, experiences of existing services, barriers to access and opportunities for improvement. This drew on feedback from more than 1,000 participants and over 300 engagement records. See Appendix 4 – Healthwatch Report.

7.2.9 Further engagement between March and August 2025 included feedback from NHS England and West Midlands Clinical Senate, an NHS staff survey involving 178 participants; workshops with VCFSE partners; engagement with elected representatives and scrutiny bodies; and a workshop involving 36 patient and public representatives who helped weight the desirable criteria subsequently used in options appraisal.

7.2.10 Recurring themes included the importance of local and accessible care; retaining walk-in access; improved diagnostics; clearer service navigation; better integration between services; and avoiding additional barriers for people who find travel, telephone or digital access difficult.

7.2.11 This engagement informed the development and appraisal of the shortlisted options before formal consultation.

Identifying audiences and targeting activity

7.2.12 The consultation approach was informed by a two-stage process to identify people most likely to use the services, be affected by the proposals or experience barriers to participating.

7.2.13 First, available evidence was reviewed, including UTC utilisation data, travel-time analysis and equality impact information. This identified adults aged 25 to 64, including parents and carers of young children, as significant users of UTC services and highlighted communities potentially more affected by changes to service location or access.

7.2.14 Second, a stakeholder-mapping workshop involving more than 20 engagement leads from NHS trusts, local authorities, VCFSE organisations and ICBs across Birmingham and Solihull and the Black Country brought together local knowledge and population insight.

7.2.15 This enabled activity to be targeted towards current and recent service users and communities potentially more affected or under-represented, including people living in areas of higher

deprivation; people with protected characteristics; minority ethnic and faith communities; disabled people; carers; people experiencing homelessness or insecure housing; refugees and people seeking asylum; Gypsy, Roma and Traveller communities; LGBTQ+ communities; and people experiencing literacy, language or digital-access barriers.

7.2.16 Channel selection was also informed by this work so that participation did not depend on a single digital route.

Formal public consultation

7.2.17 Formal public consultation on Option A and Option B took place for ten weeks between 5 February and 16 April 2026.

7.2.18 The ICB commissioned Olovus, an independent organisation specialising in major NHS service change legal requirements and public consultation, to support the design of the consultation survey and independently analyse and report on the findings.

7.2.19 The survey explored respondents' use and experience of urgent care, views about the proposed models and potential impacts, preferences between the options and the reasons for those views. Free-text questions enabled people to raise concerns, suggest improvements and propose alternatives.

7.2.20 People could participate or access information through a broad mix of channels:

- a dedicated consultation website and online survey;
- paper surveys, dedicated telephone line and email inquiry address;
- targeted face-to-face and online Town Hall events;
- information stands at UTCs, hospitals and community locations;
- targeted leaflet drops;
- VCFSE-led focus groups and engagement;
- posters, printed materials and patient-facing screens;
- an animated film and other explanatory materials;
- press release to local media;
- organic and targeted paid social media;
- ICB, primary care, NHS trust, local authority and VCFSE newsletters, websites, intranets and mailing lists;
- briefings and communications for MPs, councillors, scrutiny bodies and other key stakeholders.

7.2.21 A toolkit was shared with MPs, councillors and communications and engagement leads across partner organisations to support consistent promotion and amplification.



7.2.22 The **detailed public information provided to consultees is contained in Appendix 5 – Public Consultation Document, Appendix 6 – Summary Consultation Document and Appendix 7 – Easy Read Consultation Document. The questions on which respondents were invited to give their views are set out** in Appendix 8 – Consultation Survey.

Accessibility, inclusion and VCFSE involvement

7.2.23 Four VCFSE organisations were commissioned to reach communities less likely to participate through conventional consultation routes and to provide additional qualitative insight.

7.2.24 Accessible formats and participation routes included Easy Read materials, different languages where required, large print, accessible electronic formats and interpreting support.

7.2.25 Participation was monitored while the consultation was live. Where gaps were identified, the approach was adapted. This included targeted activity to increase participation among children and young people, men and Muslim residents, including paid social media and additional VCFSE and faith-based engagement.

7.2.26 During the consultation, monitoring also identified that some people sought additional descriptive information about the options. The presentation of the options within the survey was therefore clarified, including through additional explanatory content and visual representation.

Consultation governance and assurance

7.2.27 The ICB is satisfied that the consultation was robust and proportionate, provided a broad range of opportunities for people to participate and was undertaken in line with relevant NHS service change assurance requirements.

7.2.28 Governance was overseen through the UTC Review Programme Board, supported by regular progress reporting and a formal mid-point review of responses, demographic participation, communications activity and engagement undertaken and planned.

7.2.29 Findings from that review informed additional targeted activity where gaps in participation had been identified.

7.2.30 NHS England reviewed and approved the Pre-Consultation Business Case, draft Consultation Plan and draft Public Consultation Document before consultation commenced.

7.2.31 Oversight was also maintained through the ICB's wider governance arrangements, including the ICB's former Health Inequalities, Communities and Economies Board Sub-committee and the NHS Birmingham and Solihull and NHS Black Country executive board.

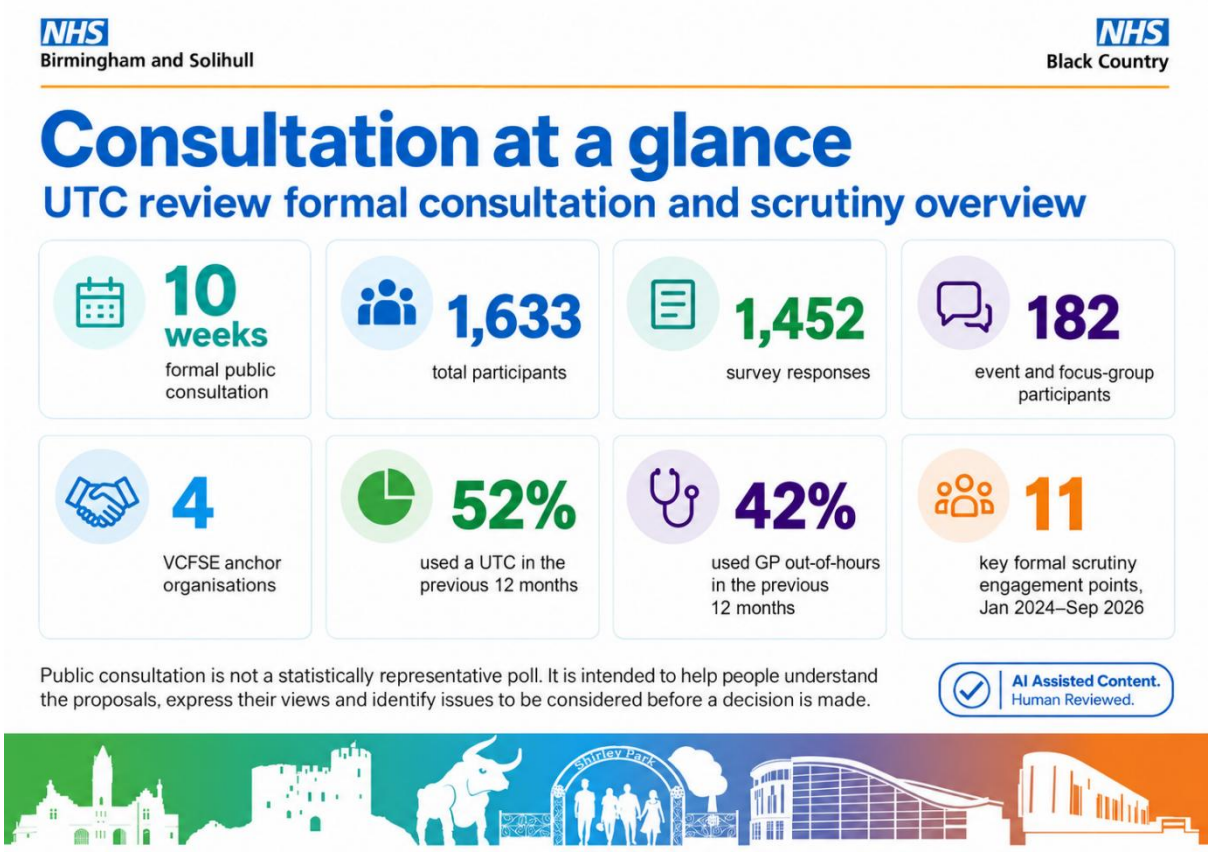
7.3 What we heard

Participation and independent analysis

7.3.1 A total of 1,633 people participated in the formal consultation: 1,452 through the consultation survey and 182 through events and focus groups.

7.3.2 There was significant participation by people with recent experience of the services: 52% of survey respondents had used a UTC in the previous 12 months and 42% had used GP OOH services.

7.3.3 The consultation also included participation from people whose circumstances could make them particularly sensitive to changes in access, including disabled people and people with long-term health conditions, carers and people on lower incomes.



7.3.4 Olovus independently analysed all survey responses and feedback from events and focus

groups, including thematic analysis of free-text responses and specific consideration of equality and inclusion issues.

7.3.5 The respondent profile was not statistically representative of the Birmingham and Solihull population. Respondents were older and more likely to be female and Christian, while younger people, Muslim communities and lower-income households were under-represented. This is consistent with known challenges reaching underserved communities in our system.

7.3.6 Public consultation is not intended to operate as a statistically representative opinion poll. Its purpose is to enable people to understand proposals, express their views and identify issues that must be considered before a decision is made. Consultation findings have therefore been considered alongside population need, service-use data, clinical evidence, equality evidence and travel and access analysis.

7.3.7 The full analysis, including respondent demographics and methodological limitations, is contained in Appendix 9 – Independent Consultation Report.

Overall findings

7.3.8 The consultation produced a nuanced picture rather than a decisive preference for one complete option.

7.3.9 When respondents rated each option independently, 50% rated Option A as meeting local needs very well or extremely well, compared with 38% for Option B.

7.3.10 However, when asked which option came closest to meeting local needs, the result was almost evenly divided: 37.5% selected Option A and 37.2% selected Option B, with 25.2% selecting neither or unsure.

7.3.11 Participants in events and focus groups tended to favour Option B, particularly because of its emphasis on community-based provision, local services and access closer to home.

7.3.12 Across the different methods of participation, respondents frequently supported elements from both options rather than one complete model.

7.3.13 In particular, there was evidence of support for retaining and improving the UTC configuration associated with Option A while adopting a more localised and community-based approach to GP OOH provision associated with Option B.

Key themes

7.3.14 The strongest recurring themes were:

- the importance of familiar, trusted and accessible local services;
- concern about additional travel, public transport, parking costs and navigating acute hospital sites;
- strong support for retaining walk-in urgent care;
- concern about NHS 111 becoming the sole gateway into urgent care;
- support for face-to-face GP OOH appointments and home visits where clinically appropriate;
- the need for simpler and clearer pathways between NHS 111, UTCs, GP OOH, primary care and emergency departments;

- equality impacts associated with travel and reliance on telephone or digital access;
- support for improved diagnostics and consistent national UTC standards; and
- a desire for GP OOH services that combine consistent standards with accessible local delivery.

7.3.15 The detailed evidence underpinning these themes is set out in Appendix 9 – Independent Consultation Report.

Differences across localities

7.3.16 Following scrutiny of the consultation findings, the ICB commissioned Olovus to undertake additional geographical analysis across Central, North, South, East and West Birmingham and Solihull.

7.3.17 The analysis found more similarities than differences. Across all areas, people wanted urgent care that was local, accessible, easy to understand and adequately resourced.

7.3.18 Differences largely reflected the impact of the options on existing local services:

- North Birmingham showed the strongest preference for Option A and strong support for Erdington UTC and North Birmingham UTC at Good Hope Hospital;
- Central and South Birmingham demonstrated significant attachment to the existing South Birmingham UTC at Katie Road in Selly Oak;
- South Birmingham also showed comparatively stronger support for the additional community provision proposed within Option B;
- East Birmingham respondents emphasised the importance of accessible provision at Washwood Heath alongside Birmingham Heartlands Hospital;
- West Birmingham respondents placed particular value on Summerfield UTC and highlighted deprivation, transport and access; and
- Solihull respondents strongly supported retaining and improving Solihull UTC, while also expressing concern about travelling to Birmingham hospital sites.

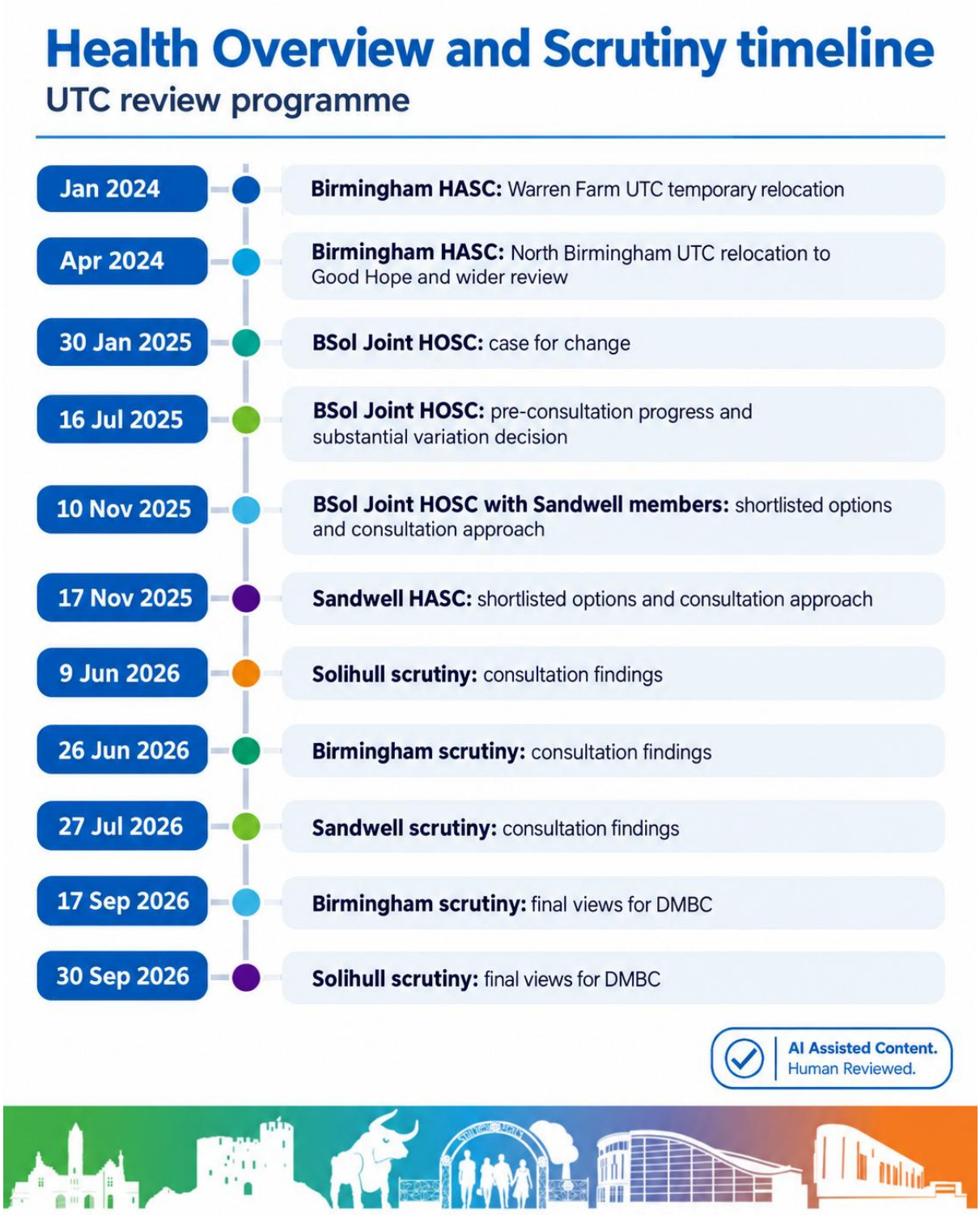
7.3.19 The full locality analysis is contained in Appendix 2 – Supplementary Locality Analysis.

7.4 Health Overview and Scrutiny

7.4.1 Local Authority Health Overview and Scrutiny engagement has taken place throughout the programme, from the temporary changes affecting Warren Farm/North Birmingham UTC through development of the case for change, options development, formal consultation and post-consultation consideration.

7.4.2 Given the geographical impact of the proposals, the ICB sought the establishment of a joint scrutiny arrangement involving Birmingham, Solihull and Sandwell, however it was not possible to convene the three scrutinee committees together at the same time. Prior to consultation, the ICB therefore engaged with the Birmingham and Solihull Joint Health Overview and Scrutiny Committee and Sandwell's Health and Adult Social Care Overview and Scrutiny Committee separately. Following consultation, the ICB engaged each local authority scrutiny committee separately on the consultation findings and issues to be considered in the development of the DMBC. Furthermore, following local elections and committee reconstitution, a number of members had not been involved in earlier stages of the review. Additional context and opportunities for scrutiny were therefore provided.

7.4.3 Key formal scrutiny engagement included:



7.4.4 Key scrutiny themes included consultation reach and representativeness; health inequalities; travel, parking and public transport; walk-in and face-to-face care; digital and language barriers; NHS 111 access; clarity of the options; and how consultation findings would influence the final recommendation.

7.4.5 Scrutiny feedback directly contributed to the commissioning of the supplementary locality analysis and to further consideration of whether a modified approach could better respond to the evidence.

7.4.6 A detailed record of scrutiny feedback and the ICB response is provided in Appendix 10 – HOSC Feedback.

7.5 How we responded

Post-consultation consideration

7.5.1 Following formal consultation, scrutiny and supplementary locality analysis, the evidence was incorporated into the ICB's post-consultation options review.

7.5.2 The review considered whether the evidence reinforced, qualified or challenged the original appraisal of Options A and B and whether a modified approach could respond more effectively to the issues raised.

7.5.3 The detailed options appraisal, legal considerations and rationale for the recommended option are set out elsewhere in this DMBC.

7.5.4 In considering modifications, the ICB has also had regard to whether the emerging proposal remains within the scope of the options on which the public was consulted. Any material development during subsequent commissioning or implementation will similarly need to be considered against the ICB's continuing public involvement, equality and scrutiny duties.

7.5.5 The outcome of this wider process was the development of Option A+ as the recommended model.

How consultation and scrutiny informed the recommended option

7.5.6 The consultation evidence did not point to a clear preference for either Option A or Option B in their entirety. Instead, respondents frequently supported different elements of each model.

7.5.7 In particular, there was evidence of support for the UTC configuration associated with Option A, including retention of familiar community UTC provision and walk-in access, alongside a more localised approach to GP OOH provision associated with Option B.

7.5.8 These findings, together with scrutiny feedback and the supplementary locality analysis, were considered through the formal post-consultation options review and contributed to the development of Option A+.

7.5.9 The detailed service configuration, options appraisal and rationale for Option A+ are set out elsewhere in this DMBC. From a consultation and engagement perspective, the key issue is that the recommended model demonstrates how feedback has been considered, including where it has influenced the model, where concerns remain and where further mitigation or implementation activity is required.

NHS Birmingham and Solihull

NHS Black Country

How consultation feedback was reflected in the recommended model

What we heard	How this was reflected in the recommended model or decision-making
Familiar and trusted community UTCs are important	The recommended model retains some existing community UTC provision alongside hospital-based UTCs.
Walk-in access is important	Walk-in access retained across UTC provision.
NHS 111 should not be the only route into UTC care	NHS 111 remains one access route, but not the sole route.
There was support for more local GP OOH provision	The recommended model includes a more locality-focused approach to GP OOH delivery.
Face-to-face care and home visits remain important	Reflected through the proposed GP OOH commissioning.
Travel and equality impacts are significant	These impacts have informed the options review, equality assessment and mitigation planning.
People can find urgent care pathways confusing	Clear patient-facing communications and service navigation will be required as part of implementation.

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7.5.10 The supporting consultation evidence for these is contained in Appendix 9 – Independent Consultation Report, Appendix 2 – Supplementary Locality Analysis and Appendix 10 – HOSC Feedback.

7.6 Next steps

7.6.1 Subject to the ICB Board decision on 9 November 2026, a communications and stakeholder engagement plan will explain the decision, the reasons for it, how consultation and scrutiny feedback influenced the recommended model, what will and will not change, and the anticipated implementation process.

7.6.2 Further engagement will take place where detailed implementation would benefit from local input, particularly around access, navigation, accessibility and communications.

7.6.3 Post-implementation monitoring will include patient experience, access, equality impacts, travel, use of NHS 111 and GP OOH and any unintended impact on other urgent and emergency care services. A communications campaign will also be put in place to develop understanding amongst the public of the different urgent care options and services they provide.

8 OPTIONS APPRAISAL AND RECOMMENDATION

8.1 Purpose and approach

8.1.1 This chapter sets out the options considered through the Urgent Treatment Centre (UTC) Strategic Review and explains how the evidence has informed the recommended future urgent care model for Birmingham and Solihull.

8.1.2 The purpose of the post-consultation options review was to provide a structured and evidence-based assessment of the options for the future configuration of Urgent Treatment Centres (UTCs) and GP Out of Hours (OOH) services, taking account of the evidence available at the conclusion of the public consultation and subsequent engagement with Health and Adult Social Care Overview and Scrutiny Committees (HASC/HOSC).

8.1.3 The review was not intended to constitute a new options appraisal or to automatically select the option which received the greatest level of public support. Rather, it was undertaken to test whether the original assessment of Options A and B remained appropriate in light of the consultation findings, scrutiny feedback, updated evidence and further development of the service models.

8.1.4 The post-consultation review was undertaken through two stages. The first stage was an independently facilitated options review workshop with the UTC Review Programme Board on 24 September 2026, led by Olovus. The purpose of the workshop was to provide an independent and structured review of the options and the evidence available following consultation. The workshop also provided an opportunity to revisit the descriptions of Options A and B in light of evidence gathered through the consultation and scrutiny process and to consider whether the original rationale underpinning the options appraisal remained valid.

8.1.5 A particular consideration arising from the consultation and subsequent HASC/HOSC engagement was whether elements of Options A and B should be combined into a hybrid model, subsequently referred to as Option C.

8.1.6 The potential hybrid model reflected feedback which identified value in retaining the established community UTC provision associated with Option A while incorporating elements of the more locally distributed GP OOH/community urgent care approach associated with Option B. The Programme Board therefore considered whether this represented a sufficiently distinct option to require separate consideration within the post-consultation review.

8.1.7 Option C was considered specifically in response to the consultation and scrutiny feedback. The review considered its potential benefits alongside its practical implications, including financial and neighbourhood viability, workforce, procurement and service configuration. The review identified that elements of the hybrid approach could be incorporated into Option A without creating a fundamentally different service model requiring a separate weighted appraisal.

8.1.8 In particular, the potential to strengthen the locality-based configuration of GP OOH provision within Option A provided a means of responding to the concerns raised through consultation and scrutiny regarding the local responsiveness of GP OOH services.

8.1.9 The Programme Board concluded that Option C did not represent a fundamentally distinct strategic option. Rather, it combined components already present within Options A and B. Following review, the Board determined that the beneficial elements of Option C could be incorporated into Option A through refinement of the GP OOH model without altering the core UTC configuration. Consequently, a separate weighted appraisal was not considered

necessary or proportionate.

8.1.10 The Programme Board also considered whether the proposed refinement to GP Out of Hours provision remained within the scope of the options presented during consultation. The Board concluded that the amendment did not constitute a materially different strategic option, as it did not alter the core UTC configuration, the location of proposed UTC sites, the proposed closure or retention of community UTCs, or the fundamental model of urgent care provision consulted upon. Rather, it represented a refinement to the commissioning and delivery approach for GP Out of Hours services in response to issues raised through consultation and HASC/HOSC engagement. The Board therefore concluded that the amended model remained within the scope of the consultation and could appropriately be considered through the DMBC.

8.1.11 The second stage was a commissioner-only options review, undertaken following the independently facilitated workshop. This provided an opportunity for the ICB commissioning team and relevant senior leads to consider the implications of the workshop discussions in greater detail and to test the options against the available evidence and the requirements of the DMBC.

8.1.12 The commissioner-only review considered whether the original weighted assessment remained appropriate following the additional evidence and whether any changes to the supporting rationale, service configuration or proposed mitigation were required. This included consideration of the revised descriptions of Options A and B and the implications of the potential hybrid approach.

8.1.13 The established five desirable criteria used within the original options appraisal were retained:

- Health outcomes and reducing inequity of access
- Improving workforce sustainability and enhancing staff training
- Patient and public clarity on where to get the right care at the right time
- Deliverability
- Impact on access, particularly access by car and public transport, and sustainability of access

The post-consultation review did not introduce a separate criterion for public acceptability or stakeholder preference. Instead, consultation and scrutiny feedback were considered across the existing criteria, recognising that the feedback had implications for access, health inequalities, clarity, deliverability and the design of the future service model. This approach was consistent with the agreed methodology for the post-consultation review.

8.1.14 For each criterion, the review considered whether the additional evidence reinforced, qualified or challenged the original assessment. The intention was not to re-score the options simply because further evidence had become available, but to determine whether there was sufficient material evidence to justify changing the original assessment.

8.1.15 The original weighted assessment had resulted in a score of 434 for Option A and 410 for Option B. The subsequent review therefore tested whether these scores remained appropriate, while also considering whether changes to the supporting rationale, mitigation or service design were required. The commissioner-only review subsequently confirmed that the original scores remained appropriate.

8.1.16 The review therefore considered three related questions:

- Does the additional evidence change our assessment of Options A and B?
- Does the consultation and scrutiny feedback identify any mitigation or refinement required to the preferred model?
- Does the potential hybrid model represent a sufficiently different option to warrant separate consideration, or can its beneficial elements be incorporated within an existing option?

8.1.17 This approach enabled the Programme Board and ICB commissioners to distinguish between changing the outcome of the formal options appraisal and refining the design of the recommended model in response to consultation and scrutiny.

8.1.18 The resulting recommendation is therefore Option A as amended, rather than a separately scored hybrid option. The amendment relates to the commissioning and locality configuration of GP Out of Hours services and does not alter the core UTC configuration that formed part of the consultation proposals.

Further Development Prior to Implementation

8.1.19 The Programme Board recognised that, whilst sufficient evidence exists to identify a preferred option, a number of elements require further development through the procurement, mobilisation and implementation phases. These include:

- finalisation of the GP Out of Hours service specification and locality-based commissioning approach;
- further definition of the enhanced specialist locality urgent care services proposed within East and West Birmingham;
- refinement and validation of activity, workforce and financial assumptions where required;
- market engagement and procurement planning to confirm the most appropriate delivery arrangements;
- consideration of whether elements of UTC and GP Out of Hours provision should be procured through linked or separate arrangements; and
- further development of implementation, transition and communications plans.

8.1.20 The Programme Board considered that these matters relate to delivery and implementation rather than the strategic direction of travel, and therefore do not alter the recommendation that Option A as amended should be adopted as the preferred model.

8.1.21 **Fixed points**

The UTC Review Programme Board set fixed parameters to define the boundaries of the Urgent Treatment Centre redesign programme. These parameters reflect national policy requirements, financial constraints, and strategic priorities, and therefore cannot be altered. They ensure alignment with national standards, maximise the benefit of available investment, and maintain overall system sustainability.

Fixed points for options development

Fixed Points	Description
Compliance with national UTC Standards and Principles	All proposed models must meet the national UTC service specification and clinical standards.
National capital funding for co-located UTCs	Proposals must reflect and utilise the national capital funding allocation for UTCs co-located with Type 1 Emergency Departments (Good Hope, Heartlands, and Queen Elizabeth Hospitals).
Operating within existing financial envelope	All models must be developed within the current financial parameters and not result in additional recurring system costs.

8.1.22 As a result of applying these fixed points to the proposals discussed, the clinical sub-group identified seven possible proposals that could, depending on further development and appraisal, constitute of viable solution to the provision of urgent care services across Birmingham and Solihull.

8.1.23 The Programme Board also confirmed that any future model must remain affordable within the existing recurrent funding envelope and must not compromise the delivery of the nationally funded co-located UTC programme. Any proposal requiring retention of all existing UTCs alongside additional locality urgent care provision would not be financially sustainable and would therefore fall outside the agreed programme parameters.

8.2 Options Considered

8.2.1 The two principal options considered through the consultation and subsequent review are summarised in the table below;

	OPTION A (PLUS)
Option A	A locality-based urgent care model will be introduced, aligning with the NHS Long Term Plan's vision for neighbourhood health services and with the national programme of investment in Urgent Treatment Centres (UTCs) located at acute hospitals. This approach aims to: • Reduce pressure on A&E by providing a consistent "front door" for urgent care. • Ensure patients are seen in the right place, by the right team, at the right time. • Standardise all UTCs to meet NHS England's national UTC principles and standards.
Option A	**East Birmingham:** • Locate a UTC at Birmingham Heartlands Hospital. The UTC will act as

	OPTION A (PLUS)
	the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not needing face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • The current Washwood Heath UTC service will close. In its place, the East Birmingham Locality Hub at Washwood Heath Health and Wellbeing Centre will continue to be developed. This includes a Respiratory Same Day Emergency Care service led by Heartlands Hospital consultants and cardiology urgent care. Patients will also benefit from a Community Diagnostic Centre based at Washwood Heath Health & Well-being Centre.
Option A	**West Birmingham:** • Locate a UTC at Midland Metropolitan University Hospital (MMUH). Acting as the 'front door' for A&E, it would triage and assess all A&E walk-in patients and those referred by NHS 111 including trauma cases such as suspected fractures or wounds likely needing to be closed with stitches. UTC staff would provide a virtual consultation service for patients not requiring face-to-face assessment. Patients would be tri-aged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • The current Summerfield UTC service will close. New community urgent care services specific to the needs of the local population will instead be provided via a Locality Hub based at Summerfield Primary Care Centre.
Option A	**North Birmingham:** • Permanently locate North Birmingham UTC (previously known as Warren Farm UTC) at Good Hope Hospital. The UTC will act as the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not needing face-to-face assessment. Patients would be triaged where

	OPTION A (PLUS)
	appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • Erdington UTC based at Stockland Green Primary Care Centre will be upgraded to meet national UTC guidelines, being led by GPs with extended opening hours. It would care for patients referred by NHS 111 through either face-to-face or virtual consultations and would provide triage and assessment for walk-in patients.
Option A	**South and Central Birmingham:** • Locate a UTC at Queen Elizabeth Hospital Birmingham which will act as the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not needing face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • South Birmingham UTC (Katie Road, Selly Oak) will be upgraded to meet national UTC guidelines. It would care for patients referred by NHS 111 through either face-to-face or virtual consultations and would provide triage and assessment for walk-in patients.
Option A	**Solihull** • Solihull UTC at Solihull Hospital will be upgraded to meet national UTC guidelines, being led by GPs with extended opening hours. It would care for patients referred by NHS 111 through either face-to-face or virtual consultations and would provide triage and assessment for walk-in patients.
Option A	**GP Out of Hours** GP Out of Hours services across Birmingham and Solihull will be re-commissioned under a common overarching clinical and service specification, with procurement structured through three locality-based lots: • South/Central • North/West

	OPTION A (PLUS)
	• Solihull/East The service will provide an integrated urgent primary care pathway through NHS 111, with virtual clinical assessment, face-to-face appointments and home visits where clinically appropriate. During core GP opening hours, NHS 111 referrals requiring primary care will be directly booked into GP appointments. The locality-based lot approach represents a refinement of the original proposal to commission a single BSol-wide GP OOH service, following consultation and HASC feedback. It retains the benefits of a coordinated system-wide service, including consistent clinical standards, integrated NHS 111 access and alignment with the wider urgent care pathway, while creating greater flexibility for care to be delivered closer to local communities.

	OPTION B
Option B	A locality model will be introduced outside of core GP hours, aligning both to the 10 Year Health Plan's focus to develop a neighbourhood health service and the NHS's national investment in UTCs based at acute hospitals to reduce pressure on A&E departments and ensuring patients are treated in the right place, by the right people at the right time. All UTCs will be standardised to meet NHS England's national UTC principles and standards.
Option B	**East Birmingham:** • Locate a UTC at Birmingham Heartlands Hospital. The UTC will act as the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not needing face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • The current Washwood Heath UTC service will close. New community urgent care services will instead be provided via Locality Hubs based at Washwood Heath Health & Well-being Centre (WWHWC) outside of core GP Hours.
Option B	**Solihull:** • Solihull UTC at Solihull Hospital will be upgraded to meet national UTC guidelines, being led by GPs and providing multi-disciplinary care with extended opening hours and access to community diagnostics. It would care for patients referred by NHS 111 through either face-to-face or virtual consultations and would triage and assess walk-in

	OPTION B
	patients. • A new community urgent care services will be provided via Locality Hubs based at Solihull Hospital outside of core GP Hours.
Option B	**West Birmingham:** • Locate a UTC at Midland Metropolitan University Hospital (MMUH). Acting as the 'front door' for A&E, it would triage and assess all A&E walk-in patients and those referred by NHS 111 including trauma cases such as suspected fractures or wounds likely needing to be closed with stitches. UTC staff would provide a virtual consultation service for patients not requiring face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • The current Summerfield UTC service will close. A new community urgent care will instead be provided via Locality Hubs based at Summerfield Primary Care Centre outside of core GP Hours.
Option B	**North Birmingham:** • Permanently locate North Birmingham UTC (previously known as Warren Farm UTC) at Good Hope Hospital. The UTC will act as the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not needing face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • The current Erdington UTC service based at Stockland Green PCC will close. A new community urgent care will instead be provided via Locality Hubs based at Stockland Green Primary Care Centre outside of core GP Hours.
Option B	**South and Central Birmingham:** • Locate a UTC at Queen Elizabeth Hospital Birmingham which will act as the 'front door' for A&E, triaging and assessing all walk-in patients and those referred by NHS 111 referrals including trauma such as suspected fractures or wounds likely needing to be closed with stitches. A virtual consultation service will support patients not

	OPTION B
	needing face-to-face assessment. Patients would be triaged where appropriate to the hospital UTC and only those needing specialist care will be triaged to A&E. Opening hours of service needs to be determined, with limited overnight activity but potential for 24/7 and undertaking category 3/4 ambulance assessments prior to dispatch and category 2 prior to conveyance overnight. • South Birmingham UTC (Katie Road, Selly Oak) will close. Community urgent care services instead be provided via Locality Hubs at Moseley Hall and West Heath Hospital outside of core GP Hours.
Option B	**<u>GP Out of Hours</u>** GP Out of Hours services across Birmingham and Solihull will be re-commissioned under a new specification incorporated into the Community Urgent Care offer within the locality model. This new service will: • Provide virtual assessments across Birmingham and Solihull • Deliver out of hours face-to-face consultations and home visits where clinically appropriate • Support NHS 111 in triaging non-critical ambulance calls (Categories 3 and 4) and urgent OOH cases that cannot safely wait until the next routine GP appointment • Integration with urgent community services such as respiratory and cardiology SDEC, end-of-life care, and district nursing (where available)

8.2.2 The table below outlines the clear differences between Option A and Option B, providing a concise comparison to support and ensure clarity of each proposed model;

Locality	**Option A**	**Option B**
East	Locate a UTC at Birmingham Heartlands Hospital Locality Specialist Urgent Care Service at Washwood Heath Health and Wellbeing Centre replaces current nurse led UTC Patients will also benefit from a Community Diagnostic Centre based at Washwood Heath Health & Well-being Centre. GP OOH recommissioned across	Locate a UTC at Birmingham Heartlands Hospital Closure of Washwood Heath UTC. Replaced with new community urgent care service operating outside of core GP hours.

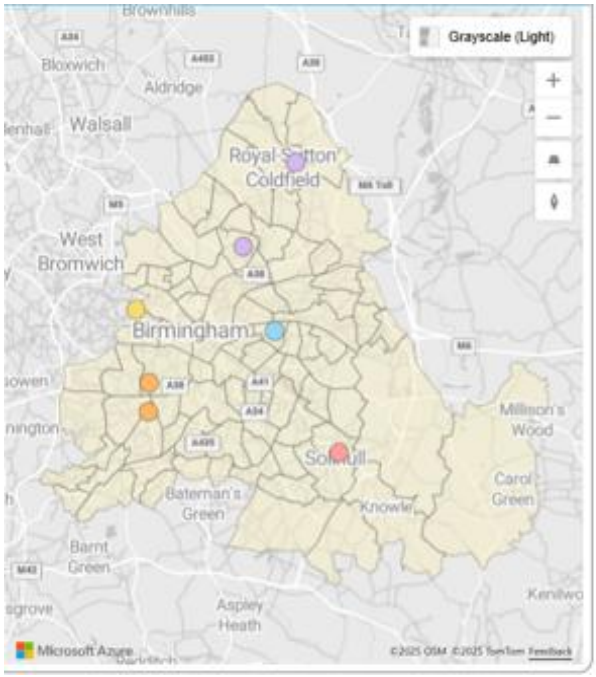
Locality	Option A	Option B
	East/Solihull.	
South and Central	Locate a UTC at Queen Elizabeth Hospital Birmingham South Birmingham UTC (Katie Road, Selly Oak) will be upgraded to meet national UTC guidelines Locality Urgent Care service at West Heath Hospital and Moseley Hall GP OOH recommissioned across South/Central.	Locate a UTC at Queen Elizabeth Hospital Birmingham Community urgent care service one at West Health Hospital for **South** locality, one at Moseley Hall Hospital for the **Central** locality replaces current South Birmingham operating outside of core GP hours.
North	Permanently locate North Birmingham UTC (previously known as Warren Farm UTC) at Good Hope Hospital. Erdington UTC based at Stockland Green Primary Care Centre will be upgraded to meet national UTC guideline Locality Urgent Care Service based at Sutton Cottage Hospital GP OOH recommissioned across North/West.	Permanently locate North Birmingham UTC (previously known as Warren Farm UTC) at Good Hope Hospital. A new community urgent care provided at Stockland Green Primary Care Centre and Sutton Cottage Hospital replaces Erdington UTC operating outside of core GP hours.
West	Locate a UTC at Midland Metropolitan University Hospital Locality Specialist Urgent Care Service at Summerfield replaces current nurse led UTC	Locate a UTC at Midland Metropolitan University Hospital New community urgent care provided at Summerfield Primary Care Centre replacing Summerfield operating outside of core GP hours.

Locality	Option A	Option B
	GP OOH recommissioned across North/West.	
Solihull	Solihull UTC at Solihull Hospital will be upgraded to meet national UTC guidelines GP OOH recommissioned across East/Solihull.	Solihull UTC at Solihull Hospital will be upgraded to meet national UTC guidelines New Community Urgent Care service operating outside of core GP hours.

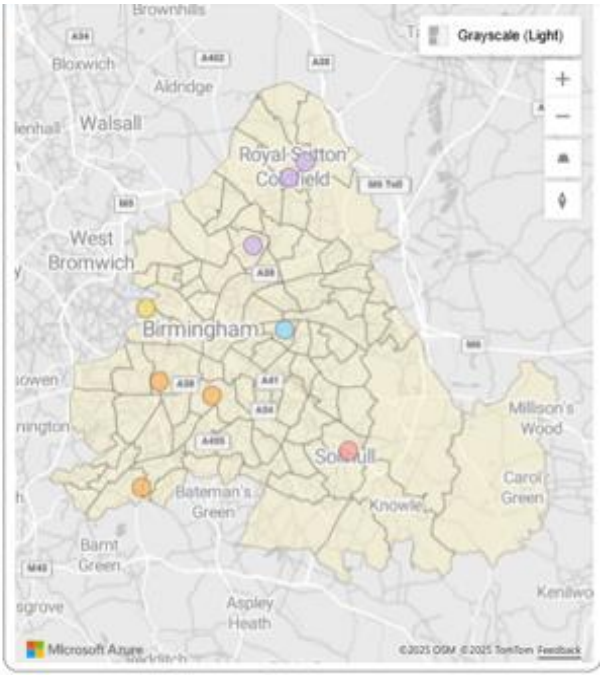
8.2.3 The maps below show the location of sites for each of the two short-listed option. Site markers are colour-coded by which locality the site is aligned to.

Maps showing two considered option locations

Option A



Option B



8.2.4 The two options were subsequently considered against the agreed desirable criteria established through the strategic review. The criteria were intended to assess the extent to which each option could deliver the intended improvements in urgent care while supporting equitable access, workforce sustainability, patient and public understanding and system deliverability. The comparative assessment against these criteria is set out in the following sections.

8.3 Comparative Assessment of the Options

8.3.1 The shortlisted options were assessed against the five desirable criteria established through the earlier options appraisal process. The criteria were agreed by the UTC Programme Board on 17 July 2025 and applied to the shortlisted options in August 2025. They were designed to

assess not only the potential clinical and population benefits of each option, but also its ability to provide equitable access, support a sustainable workforce and be delivered within the wider system context. The five desirable criteria were:

Desirable criterion	**Description**
Impact on health outcomes and reducing inequity of access	To what extent does this option demonstrably improve patient health outcomes and reduce health inequalities related to UTC access for our population, particularly for underserved groups, based on projected data and evidence? Consider deprivation data, how will this serve those with greatest inequalities, core 20 plus 5 groups and specifically: ASD, learning disabilities, mental health, those needing translation, digital exclusion groups
Improving workforce sustainability and enhancing staff training	How effectively does this option contribute to a sustainable, resilient, and highly skilled UTC workforce, ensuring adequate recruitment, retention, and ongoing professional development, while also supporting staff well-being? Workforce - consider wellbeing, sustain and retain, developmental roles and career progression, clinical supervision and training, reduction in long waits- reduced patient and staff frustration
Patient and public clarity on where to get the right care at the right time	How clearly and simply will the proposed service model enable patients and the public to understand where and when to access the most appropriate urgent care for their needs, minimising confusion and supporting effective navigation? Supporting and education of patients system wide, care navigation support for most vulnerable (overlap with 1)
Deliverability	Is this option truly practical and feasible to implement within the projected timescales and budget, considering all associated risks, resource requirements, and interdependencies with other services? Consider phasing of roll out, financial envelope and priorities, system and national, future proofing, population growth and increasing demand across all services, impacts of not delivering

Desirable criterion	Description
Impact on access, particularly access by car and public transport, and sustainability of access	How will this option impact patient access to UTC services, specifically considering the implications for travel time, parking, and public transport availability for all communities, including those who are car-dependent or reliant on public transport? (Add a note about sustainability) Environmental impact/ sustainability, analogue to digital - remote consultation, more care at home, longer term impacts of AI

8.3.2 The original weighted appraisal resulted in Option A scoring 434 compared with 410 for Option B as shown in the below table. The post-consultation review did not constitute a new appraisal or re-scoring exercise. Instead, the Programme Board considered whether the consultation, scrutiny feedback and additional evidence materially changed the rationale underpinning the original scores. The established criteria were therefore retained to provide a consistent basis for the final decision.

Birmingham and Solihull ICB - UTC Review Programme
Desirable Criteria Scoring

NHS Birmingham and Solihull

Options scoring	Weight	Option A average score	Option A weighted score	Option B average score	Option B weighted score
Impact on health outcomes and reducing inequity of outcomes	27	4	106	5	133
Improving workforce sustainability and enhancing staff training	18	5	88	4	70
Patient and public clarity on where to get the right care at the right time	22	4	90	4	90
Deliverability	17	5	83	3	50
Impact on access, particularly access by car and public transport, and sustainability of access	17	4	67	4	67
Total weighted score	100	22	434	20	410

8.3.3 The subsequent assessment considers each criterion in turn, drawing together the quantitative modelling, equality and health inequalities evidence, consultation findings, workforce and estates considerations, financial analysis and implementation requirements. This enables the relative strengths, limitations and risks associated with each option to be considered alongside the overall weighted assessment.

8.4 Comparative Assessment – Service Model and Clinical Configuration

8.4.1 The consultation demonstrated a strong preference for urgent care that is accessible, local and easy to navigate. Respondents and stakeholders particularly valued familiar community UTC locations and continued access to face-to-face care, while feedback on Option B identified the potential benefits of a more locally distributed urgent care model.

8.4.2 Subsequent HASC/HOSC engagement reinforced the importance of clarity regarding what services would be available at each location, how UTCs would operate alongside A&E, and how patients would access walk-in, NHS 111 and other urgent care services.

8.4.3 Consultation analysis demonstrated that respondents placed significant value on retaining local face-to-face urgent care provision, particularly at existing community UTC locations. There was also a strong emphasis on maintaining accessible walk-in services and minimising the need for longer journeys to access care. At the same time, consultation responses, stakeholder engagement and HASC/HOSC discussions identified support for GP Out of Hours provision that is responsive to local population needs and delivered closer to communities. These themes were consistently reflected across the consultation and scrutiny process and directly informed the subsequent refinement of Option A, particularly the development of a locality-based commissioning approach for GP Out of Hours services.

8.4.4 Consultation responses demonstrated broad support for improving consistency of UTC provision, increasing access to diagnostics and reducing pressure on Emergency Departments. Respondents also emphasised the importance of maintaining local access to services, retaining face-to-face care and ensuring GP Out of Hours services remain accessible within communities. These themes directly informed the refinement of Option A.

8.4.5 Against this context, the assessment considered whether the evidence supported a change to the original appraisal of the two service models. The evidence did not identify a sufficient basis to change the original assessment, but it did identify opportunities to strengthen the recommended model, particularly in relation to locality-based GP OOH provision.

8.4.6 The two options represent different approaches to the balance between hospital-based urgent care, community UTC provision and locality-based urgent care.

8.4.7 Option A retains four co-located UTCs at Queen Elizabeth Hospital, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital, alongside three community UTCs at Solihull Hospital, Erdington Stockland Green and South Birmingham. It therefore retains a broader network of established community UTC provision while strengthening the role of co-located UTCs as part of the hospital front-door model.

8.4.8 Option B retains Solihull UTC but moves away from the retention of the existing network of community UTCs, with community urgent care instead provided through a more locality-based model, particularly outside core GP hours. This provides the potential for a more locally distributed urgent care offer but represents a greater change to existing service arrangements.

8.4.9 The subsequent refinement of both options is important when considering the evidence. In particular, the GP Out of Hours (GP OOH) element of the model has evolved following consultation and scrutiny. The proposed approach under Option A is now to procure GP OOH through three locality-based lots: South/Central, North/West and Solihull/East - under a common overarching specification, with locality-specific arrangements for face-to-face and home visiting provision. This provides an opportunity to strengthen the locality responsiveness of Option A while retaining a consistent system-wide clinical framework.

8.4.10 The options review identified that stakeholders frequently valued elements of both models. Consultation and scrutiny feedback demonstrated strong support for care remaining close to communities, including accessible walk-in and face-to-face provision, while also identifying the importance of a more locally responsive GP OOH offer. This supported the subsequent development of Option A as amended, rather than treating the two original options as entirely discrete models.

8.4.11 The assessment therefore indicates that Option A provides a stronger balance between retaining familiar community provision and strengthening the hospital-based urgent care model, while the enhanced locality approach to GP OOH provides an opportunity to address

some of the local responsiveness associated with Option B.

8.5 Comparative Assessment – Activity, Capacity and System flows

8.5.1 Consultation and scrutiny feedback highlighted the importance of understanding how changes to community urgent care provision would affect patient flows across general practice, UTCs, GP OOH and emergency departments. In particular, concerns were raised about the potential for changes in community provision to result in additional journeys, delays or increased pressure on hospital services.

8.5.2 The subsequent activity and capacity analysis was therefore considered alongside the consultation evidence to test whether the proposed models could accommodate anticipated demand and whether assumptions regarding the movement of activity between services were sufficiently robust. The activity and capacity assessment considers whether each option can accommodate projected urgent care demand while supporting the wider objective of improving patient flow through the urgent and emergency care system. The comparative findings of the activity and capacity assessment for Options A and B are summarised in Table below, which informed the post-consultation options appraisal and consideration of whether the original assessments remained appropriate.

Comparatively, the 2 models are:

SITE	PCBC – Option A	PCBC – Option B
Summerfield	95% of Community Activity delivered at Midlands Met Hospital and 5% of activity delivered at Quen Elizabeth hospital	65% of activity continues to be delivered at Summerfield, and 35% of activity is delivered at Midlands Met Hospital
Warren Farm (North Birmingham UTC)	100% of activity is delivered at Good Hope Hospital.	100% of activity is delivered at Good Hope Hospital
Washwood Heath	100% of activity is delivered at Heartlands Hospital	No change - All activity continues to be provided at Washwood Health
Katie Road	No Change	Activity split between two community locations, Moseley Hall Hub and West Heath Surgery.
Erdington	No Change	No change
Solihull	Retain and improve existing UTC	Retain and improve existing UTC

8.5.3 Current UTC activity has reduced over recent years, with total activity falling from approximately 5,000 attendances per week in 2023/24 to approximately 3,500 per week in 2025/26. This reduction has been particularly evident in face-to-face activity. The future model therefore needs to consider not only existing activity but also the extent to which activity may move between UTCs, general practice, GP OOH, community services and emergency departments.

8.5.4 The financial and activity modelling estimated a broadly similar overall UTC requirement under the two options, but the distribution of activity differs. Under the Option B model, a greater proportion of daytime community urgent care activity is expected to be managed through general practice or other primary care arrangements. Where activity cannot be absorbed within general practice or other primary care services, there is a risk that patients may instead present to UTCs or emergency departments. This creates an important dependency for Option B.

8.5.5 Option A provides greater continuity with existing UTC activity patterns and retains three community UTC sites alongside the four co-located UTCs. This provides additional capacity and access points for lower-acuity urgent care, while the co-located UTCs are designed to support streaming and diversion from emergency departments and manage more complex urgent presentations.

8.5.6 The additional analysis provided greater understanding of the potential differences between the options but did not provide sufficient evidence to alter the original appraisal.

8.6 Comparative Assessment – Estates

8.6.1 **Es**tates considerations were assessed alongside consultation and scrutiny feedback regarding the importance of familiar local services and the practical accessibility of service locations. Feedback highlighted concerns about travel, parking and the potential impact of changes to established community service locations. These considerations were assessed alongside the estates evidence on premises, room capacity, operating hours and the infrastructure required to deliver the proposed models.

8.6.2 Estates is a relevant consideration in comparing Option A and Option B, particularly in relation to the number and configuration of community urgent care sites retained and the premises required to support the associated service models. Both options make use of the planned co-located UTC developments at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital. These developments are intended to provide suitable clinical accommodation aligned with national UTC principles and standards and support the integration of urgent care with emergency department pathways.

8.6.3 Option A retains three community UTCs at Solihull Hospital, Erdington/Stockland Green and South Birmingham/Selly Oak, alongside the four co-located UTCs. The estates assessment identifies no additional estates-related capital costs for community UTCs under Option A, Option A Plus or Option B. Activity modelling undertaken for the consultation options indicates that three consultation/examination rooms would provide sufficient capacity for the modelled UTC activity, with additional capacity available at all sites should further rooms be required. The assessment therefore supports the use of existing community estate to accommodate the proposed UTC provision.

8.6.4 Option B involves a greater change to the existing community UTC configuration, with Solihull retained as a community UTC and five other existing community UTCs no longer operating in that role. Its revised community urgent care model requires consideration of the premises and operating arrangements needed to deliver urgent care outside core GP hours. The estates assessment identifies no additional estates-related capital costs for community UTCs under Option B; however, operational costs associated with extended opening hours may arise.

8.6.5 For the recommended Option A Plus model, the estates assessment indicates that additional operational costs may be required to support extended opening hours. The scale of these costs will depend on the final GP OOH delivery model and associated use of premises. The

assessment expects these costs to fall within the range of the Option A and Option B assumptions, with Option B representing the upper-bound cost, assessed as affordable. The precise operational cost for Option A Plus will therefore need to reflect the final GP OOH specification and delivery arrangements.

8.6.6 Planning permissions and lease arrangements are identified as potential constraints where community UTC opening hours are extended beyond those typically permitted. Changes to planning permission or contractual arrangements may therefore be required. The estates assessment identifies out-of-hours noise as the main potential planning consideration but assesses the risk of objection as low, based on the modelled level of site movements. These requirements will need to be addressed where applicable as part of implementation planning.

8.6.7 The estates evidence supports the deliverability of the recommended Option A Plus model through the use of existing community premises and the planned capital developments at the four co-located UTC sites. The principal estates distinction between the options relates to the configuration of community services and the operational implications of extended opening hours, rather than identified additional community UTC capital costs. The final GP OOH delivery model will determine the additional estates operational costs associated with Option A Plus. These costs, together with any necessary planning or contractual changes, will be addressed through final service specification development and implementation planning.

8.6.8 states considerations were assessed alongside feedback regarding the importance of familiar local services and the practical accessibility of service locations. Consultation and scrutiny identified particular concerns regarding travel, parking and the potential impact of moving services from established community locations to hospital-based settings.

8.6.9 These considerations are particularly relevant when comparing Option A, which retains three community UTCs, with Option B, which involves a more substantial change to the existing community UTC estate.

8.7 Comparative assessment – Finance

8.7.1 Financial affordability was identified through HASC/HOSC engagement as an important consideration alongside clinical quality, access and equality. However, consultation feedback has not been treated as a financial appraisal in itself. The financial assessment has therefore considered the comparative costs and financial assumptions associated with each option alongside the wider evidence base.

8.7.2 The financial modelling undertaken for the options estimated an indicative annual cost of approximately £24.7 million for Option A and £24.1 million for Option B, a difference of approximately £0.6 million.

8.7.3 The relatively small difference between the estimated costs should not be interpreted as demonstrating that Option B is inherently more affordable. The financial position is sensitive to a number of assumptions, including UTC activity, tariff arrangements, the distribution of activity between community and co-located services, workforce requirements and the final configuration of GP OOH and locality urgent care.

8.7.4 The modelling also identified a significant relationship between the future UTC model and activity currently undertaken within emergency departments. The potential commissioner funding reduction associated with activity currently delivered in EDs under the co-located UTC model is estimated at approximately £16.5 million. This should not be regarded as a cash-releasing saving. The future model is intended to support a more effective use of UEC resources, with any released capacity or funding considered within the wider UEC system.

8.7.5 Although commissioner modelling identifies a significant theoretical reduction in tariff expenditure associated with transferring UTC-appropriate activity from Emergency Departments, these resources are expected to be redeployed within the wider urgent and emergency care system and therefore do not represent cash-releasing savings that materially differentiate the options.

8.7.6 On the evidence currently available, both options are considered financially deliverable within the broader system context. The relatively small difference in the indicative costs of the two options was not considered sufficient, in isolation, to determine the preferred option.

8.8 Comparative assessment – Access and Health Inequalities

8.8.1 Access and health inequalities were among the strongest themes arising from both the public consultation and subsequent HASC/HOSC scrutiny engagement. Feedback highlighted the importance of care being available close to home, including accessible face-to-face and walk-in provision, while also identifying barriers associated with transport, parking, digital access, disability, language and socioeconomic circumstances. Scrutiny feedback reinforced the need for these factors to be explicitly considered in determining the preferred option.

8.8.2 The consultation evidence did not demonstrate that one option was consistently superior across all dimensions of access. Option B offered potential advantages through a more locally distributed urgent care model, while Option A retained a greater number of established community UTC locations. Subsequent travel-time modelling demonstrated that both options improve aggregate geographical access compared with the current position, with Option B showing the greatest modelled improvement across a number of measures, particularly public transport and walking. Further detail on the travel-time analysis is provided in Chapter 11.6. The evidence was therefore considered in the round, rather than relying on a single measure of travel time or consultation preference.

8.8.3 Access and health inequalities are central considerations in the appraisal because the configuration of urgent care services can have different impacts depending on deprivation, disability, transport availability, digital access and other barriers to care. The assessment therefore considered quantitative travel analysis alongside the qualitative evidence from consultation, locality engagement and scrutiny.

8.8.4 The original appraisal scored Option B more highly against health outcomes and reducing inequality of access, reflecting its potential to provide a more locally distributed urgent care offer. The subsequent evidence review did not identify a sufficient basis to change this assessment. However, it identified both benefits and risks associated with the different access models and provided further evidence to inform the design of the recommended approach.

8.8.5 These findings are consistent with the conclusions of the Quality and Equality Impact Assessment, which identified both potential adverse impacts and opportunities for mitigation across protected characteristic groups, deprived communities, people experiencing digital exclusion and those with transport-related barriers to access. The recommended model incorporates a range of mitigating actions which are described in Chapter 11 and the supporting QEIA.

8.8.6 Travel-time modelling across 4,309 Output Areas demonstrated that both options improve aggregate access compared with the current position. Option B produced the greatest overall improvement in modelled travel times, particularly for public transport and walking. However, quantitative travel modelling does not capture all practical barriers to accessing care. It was therefore considered alongside the lived experience and concerns reported through consultation and scrutiny, including parking, public transport availability, mobility, caring

responsibilities, digital exclusion and the ability to access face-to-face care.

8.8.7 Consultation feedback highlighted the importance of retaining familiar, local, walk-in and face-to-face services. Concerns about longer journeys and increased reliance on hospital sites were particularly relevant for older people, disabled people, carers, people on lower incomes and those without access to a car. These issues were also reflected in HASC/HOSC feedback, which emphasised the need for the final decision to consider travel, public transport, parking, equality, health inequalities, digital exclusion and continued access to face-to-face services. Concerns were particularly prominent where existing community services could be perceived as closing or relocating.

8.8.8 Option A retains three community UTCs and therefore provides greater continuity of established community-based urgent care provision. This was considered an important mitigating factor in relation to the access concerns identified through consultation. However, retention of community UTCs alone does not address all access barriers, and the effectiveness of the wider locality model will depend on ensuring that services remain responsive to communities experiencing the greatest barriers to access.

8.8.9 The proposed locality-based GP OOH model provides an opportunity to strengthen this aspect of Option A in response to the evidence gathered through consultation and scrutiny. Procuring GP OOH through three locality-based lots, while retaining a common overarching clinical and service specification, provides scope for the future service to respond more directly to local circumstances and to determine appropriate face-to-face and home visiting arrangements within each locality.

8.8.10 The Programme Board concluded that the competing access benefits identified through the quantitative and qualitative evidence did not provide sufficient justification to change the original appraisal score. However, the evidence did influence the development of the recommended model. In particular, the concerns regarding local accessibility and responsiveness contributed to the refinement of Option A to include a strengthened locality-based approach to GP OOH provision.

8.9 Comparative assessment – Workforce

8.9.1 Workforce sustainability was considered both as an established appraisal criterion and in light of consultation and stakeholder feedback regarding the ability of the future model to provide safe, consistent and sustainable services.

8.9.2 The consultation did not provide evidence that would justify changing the original workforce assessment. The subsequent review therefore focused on whether the underlying workforce assumptions remained valid in light of the revised service configurations.

8.9.3 Workforce sustainability was identified as a key consideration for both options. The future model requires sufficient numbers of appropriately skilled staff across co-located UTCs, community UTCs, GP OOH and enhanced locality urgent care.

8.9.4 Option A provides greater continuity with existing workforce arrangements and allows development of a consistent workforce model across four co-located and three community UTCs. The co-located model also provides opportunities for broader multidisciplinary working, clinical supervision, development of advanced practice roles and closer links with hospital clinical pathways.

8.9.5 Option B provides potential benefits through a more distributed locality model but would require greater workforce redesign and potentially new workforce arrangements across

multiple community locations and services. The revised model also introduces uncertainty regarding the workforce required to provide community urgent care outside core GP hours and the extent to which general practice would absorb daytime activity.

8.9.6 The Programme Board's original assessment scored Option A more highly for workforce sustainability and staff development. The subsequent review did not identify evidence sufficient to change this assessment.

8.9.7 Workforce will nevertheless remain a critical dependency for implementation, particularly in relation to 24-hour or extended-hours co-located UTC requirements, GP OOH locality arrangements and the development of enhanced locality urgent care.

8.10 Comparative assessment – Deliverability

8.10.1 Deliverability was a recurring theme within the consultation and subsequent scrutiny engagement, including questions regarding affordability, workforce, estates, procurement and the practical implementation of the proposed models.

8.10.2 The options were therefore assessed not only on the intended service benefits but also on the extent to which the proposed configuration could realistically be implemented within the available financial, workforce and estate constraints. Deliverability encompasses the practical ability to implement the model within the available financial, workforce, estates and procurement constraints.

8.10.3 Option A requires procurement and mobilisation activity across UTC and GP OOH arrangements, together with the development of the enhanced locality urgent care offer. There are therefore material dependencies relating to market interest, procurement timescales, workforce transfer arrangements and provider capacity.

8.10.4 However, the proposed GP OOH approach provides an opportunity to strengthen Option A by moving from a single system-wide procurement approach to three locality-based lots. This responds directly to consultation and scrutiny themes around local access while retaining a common clinical specification.

8.10.5 Option B would involve more substantial changes to the existing community UTC estate and service footprint and would require the development and mobilisation of a different community urgent care model. This creates additional implementation dependencies, including workforce, estates, activity transfer and service navigation.

8.10.6 The options review concluded that both options were deliverable, but that Option A offered a more established platform from which to implement the future model. The remaining deliverability risks are therefore primarily associated with the detail and sequencing of implementation rather than the fundamental feasibility of the model and are further detailed alongside mitigations in chapter 12.

8.11 Weighted Options Appraisal

8.11.1 The formal weighted options appraisal was undertaken as part of the PCBC using the established five desirable criteria:

- Health outcomes and reducing inequity of access
- Improving workforce sustainability and enhancing staff training
- Patient and public clarity on where to get the right care at the right time

- Deliverability
- Impact on access, including access by car and public transport, and sustainability of access

The criteria and their respective weightings were established prior to consultation and were not changed as part of the post-consultation review. This ensured that the assessment remained consistent with the methodology used to develop and compare the shortlisted options.

The original weighted appraisal resulted in the following scores:

Criterion	Option A weighted score	Option B weighted score
Impact on health outcomes	106	133
Improving workforce sustainability	88	70
Patient and public clarity on where to get the right care at the right time	90	90
Deliverability	83	50
Impact on access	67	67
Total weighted score	**434**	**410**

On the basis of the formal weighted appraisal, Option A achieved the higher overall score.

8.11.2 Following consultation and HASC/HOSC engagement, the Programme Board considered whether the additional evidence materially changed the rationale underpinning the original scores. The post-consultation review did not constitute a new options appraisal or a re-scoring exercise. Instead, the established criteria were retained and the original assessment was tested against consultation findings, scrutiny feedback and subsequent evidence.

The review specifically considered whether the additional evidence reinforced, qualified or challenged the original assessment and whether there was sufficient material evidence to justify any change to the scores or supporting rationale.

8.11.3 The review identified evidence that both reinforced and qualified aspects of the original appraisal. In particular, consultation and scrutiny feedback highlighted the importance of:

- maintaining access to familiar community-based services;
- ensuring continued access to face-to-face and walk-in care;
- minimising barriers associated with travel, public transport and parking;
- considering the impact of service changes on people experiencing health inequalities, disability or digital exclusion;
- providing clear routes into urgent care and clarity regarding the relationship between UTCs, NHS 111, GP services and emergency departments; and

- ensuring that GP out-of-hours provision is sufficiently local and responsive to local circumstances.

These considerations were incorporated into the further assessment of the options and, where appropriate, into the refinement of the proposed service model. HASC/HOSC feedback specifically emphasised the need for the DMBC to demonstrate how consultation and scrutiny feedback had influenced the appraisal, mitigation and service design.

8.11.4 The additional evidence did not, however, provide sufficient material justification to alter the original weighted scores. The Programme Board therefore concluded that the original scores remained an appropriate representation of the relative performance of Options A and B against the agreed criteria.

Accordingly, the weighted appraisal remains:

- Option A – 434
- Option B – 410

The weighted appraisal is therefore an important component of the recommendation, but it is not considered in isolation. The final recommendation also takes account of the consultation findings, HASC/HOSC feedback, clinical evidence, activity and capacity modelling, financial implications, workforce, estates, access and deliverability.

8.11.5 The post-consultation review also considered whether feedback supported the development of a materially different hybrid Option C. The Programme Board concluded that the elements of the hybrid approach that offered potential benefits could be incorporated into Option A without creating a fundamentally different service model requiring a separate weighted appraisal.

In particular, the GP out-of-hours model within Option A was refined to provide a more localised approach through locality-based commissioning arrangements. This allowed relevant elements of the feedback associated with Option B to be incorporated into the preferred model while retaining the core configuration of Option A.

The outcome of the post-consultation appraisal is therefore not a new weighted score for Option A as amended. Rather, the formal score of 434 remains the score for Option A, with the recommended model subsequently refined in response to the evidence and feedback received.

8.12 Overall Comparative Assessment

8.12.1 The post-consultation review did not identify a clear overall preference between Options A and B. While consultation and scrutiny feedback highlighted strong support for accessible, familiar community services and more localised GP OOH provision, it also identified benefits and concerns associated with both options. HASC/HOSC feedback emphasised the need for a transparent, evidence-led decision that considers access, health inequalities, travel, face-to-face provision and deliverability.

8.12.2 These findings were considered alongside the established options appraisal criteria and the wider clinical, activity, financial, workforce, estates and access evidence. The review concluded that the additional evidence qualified aspects of the original assessment but did not provide sufficient material evidence to change the weighted scores, with Option A remaining higher scoring at 434 compared with 410 for Option B.

8.12.3 Consultation and scrutiny feedback did, however, influence the design of the recommended model. In particular, the GP OOH element of Option A has been refined to provide a more localised commissioning approach through three locality-based lots, while retaining a common overarching clinical and service specification.

8.12.4 Overall, Option A as amended provides the strongest balance of the established appraisal criteria and the wider evidence considered through the post-consultation review. The recommendation therefore reflects both the formal weighted appraisal and the refinements made in response to consultation and scrutiny, rather than the weighted score alone.

8.13 Recommended option – Option A as amended

8.13.1 In reaching this recommendation, the Programme Board and commissioners considered the consultation findings, HASC/HOSC feedback, Quality and Equality Impact Assessment, travel and access analysis, clinical evidence, financial modelling, workforce implications and deliverability assessment. Based on the totality of the evidence presented within this DMBC, the recommended option is Option A as amended. Option A achieved the highest score in the formal weighted options appraisal, with a score of 434 compared with 410 for Option B. Following consultation, HASC/HOSC engagement and the subsequent independent and commissioner-led options review, the Programme Board concluded that there was not sufficient material evidence to change the original comparative assessment.

8.13.2 The recommended model retains the core features of Option A, including:

- four co-located UTCs at Queen Elizabeth Hospital, Birmingham Heartlands Hospital, Good Hope Hospital and the Midland Metropolitan University Hospital;
- three community UTCs serving Solihull, Erdington and South Birmingham/Selly Oak;
- Specialist locality based urgent care service provided within Washwood Heath Primary Care Centre & Summerfield Primary Care Centre (East & West of Birmingham)
- a clear distinction between the role of co-located UTCs and community UTCs within the wider urgent and emergency care pathway; and
- a reconfigured GP out-of-hours service forming part of the wider locality-based urgent care model.

8.13.3 The principal amendment to Option A following consultation and scrutiny is the proposed locality-based approach to GP out-of-hours commissioning. Rather than relying on a single ICB-wide procurement arrangement, GP OOH provision will be developed through three locality-based lots:

- South/Central
- North/West
- Solihull/East

The lots will operate to a common overarching clinical and service specification while allowing the face-to-face and home visiting offer to reflect local circumstances and population need.

This amendment responds directly to feedback that GP OOH provision should be sufficiently local and accessible, while retaining the benefits of a consistent clinical and service framework across Birmingham and Solihull. The Programme Board specifically agreed that the original Option A specification was sufficiently broad to accommodate this refinement.

8.13.4 The consideration of a hybrid approach following consultation does not result in a separately recommended Option C. Instead, the review concluded that the relevant beneficial elements of the hybrid proposal could be incorporated within Option A.

This approach provides a means of responding to consultation and scrutiny feedback without moving away from the core configuration that performed most strongly through the formal options appraisal. It also avoids introducing a new option after consultation that had not been subject to the same formal appraisal methodology as Options A and B.

8.13.5 The recommendation has therefore been informed by three complementary elements:

- First, the formal weighted appraisal, which identified Option A as the higher-scoring option against the established criteria.
- Second, the post-consultation evidence review, which tested whether consultation findings, HASC/HOSC feedback and updated evidence materially changed the original assessment.
- Third, the refinement of the preferred model, through which specific concerns raised by patients, the public and scrutiny bodies have been incorporated into the proposed service design, most notably through the locality-based approach to GP OOH provision.

8.13.6 Whilst both options are considered viable and deliver the strategic objectives of the review, Option A provides the most balanced solution across the assessment criteria. It retains greater continuity of community UTC provision, performs more strongly against workforce sustainability and deliverability criteria, maintains alignment with the nationally funded co-located UTC programme, and addresses key consultation themes through amendments to the GP Out of Hours model. Taken together, the evidence demonstrates that Option A best balances quality, access, sustainability, deliverability and patient feedback and is therefore recommended for implementation.

8.13.7 The Programme Board's independent options review subsequently supported its recommendation that the ICB Board adopt Option A as amended as the preferred future configuration of urgent care and GP OOH services.

8.13.8 **Recommendation to the ICB Board**

The ICB Board is asked to:

- Approve Option A as amended as the preferred future configuration of Urgent Treatment Centres and GP out-of-hours services across Birmingham and Solihull, subject to interdependencies and risks outlined in this DMBC.
- This recommendation reflects the formal options appraisal, the evidence available following consultation, the views and concerns raised through HASC/HOSC engagement, and the subsequent refinement of the preferred model. It represents the best overall balance of clinical quality, access, health inequalities, workforce sustainability, affordability and deliverability, while incorporating appropriate refinements in response to the issues raised through consultation and scrutiny.

9 DELIVERABILITY ASSESSMENT

9.1 Capacity, Demand and Activity Modelling

Purpose & Approach

9.1.1 Capacity and demand modelling has been undertaken to assess how existing urgent care activity may be redistributed under the shortlisted options and to inform the capacity requirements of the proposed future model.

9.1.2 The activity team used 2025/26 UTC Service Information Reporting (SITREP) and Emergency Care Data Set (ECDS) data, including UTC-amenable ED attendances, GP-streamed activity at acute hospital sites and activity from Solihull's minor injuries service. Existing activity was mapped to anticipated destinations under each option, with projected annual activity and indicative daily requirements calculated for each site.

9.1.3 The modelling informs the assessment of service deliverability and should be considered alongside the clinical model, workforce, estates, financial and access assessments. Activity projections do not, in isolation, demonstrate that the required workforce, facilities, diagnostics or operating arrangements are in place.

9.1.4 Activity modelling for the preferred option was undertaken using UTC and hospital ED data for UTC-amenable attendances (UTCAA). The baseline data by locality and site is shown in the table.

BSol UTC and ED activity by locality

BSOL UTC ACTIVITY BASELINE				Annual baseline activity 2025/26		
Locality	Location	Total ED	% UTC AA	Attends	Phone	*Total*
North	Erdington UTC			10,819	10,510	*21,329*
North	Warren Farm UTC			4,622	6,288	*10,910*
North	**Good Hope Hospital ED**	**113,052**	**42%**	**41,420**		***41,420***
North	Good Hope Hospital GP streaming			12,248		*12,248*
East	Washwood Heath UTC			7,409	20,505	*27,914*
East	**Heartlands Hospital ED**	**165,039**	**36%**	**46,253**		***46,253***
East	Heartlands Hospital GP streaming			14,865		*14,865*
South	Katie Road UTC			24,195	38,769	*62,964*
South / Central	**Queen Elizabeth Hospital ED**	**136,552**	**26%**	**32,164**		***32,164***
South / Central	Queen Elizabeth Hospital GP streaming			18,144		*18,144*
West	Summerfield UTC			27,918	8,340	*36,258*
West	**Mid Met Hospital ED**	**84,671**	**33%**	**23,545**		***23,545***
West	Mid Met Hospital GP streaming			2,090		*2,090*
Solihull	Solihull UTC			11,656	10,705	*22,361*
Solihull	Solihull Minor Injuries			17,421		*17,421*
TOTAL		**499,314**	**35%**	**294,769**	**95,117**	***389,886***

Source: UTC SITREP Data and ECDS (ED) dataset

9.2 Demand modelling

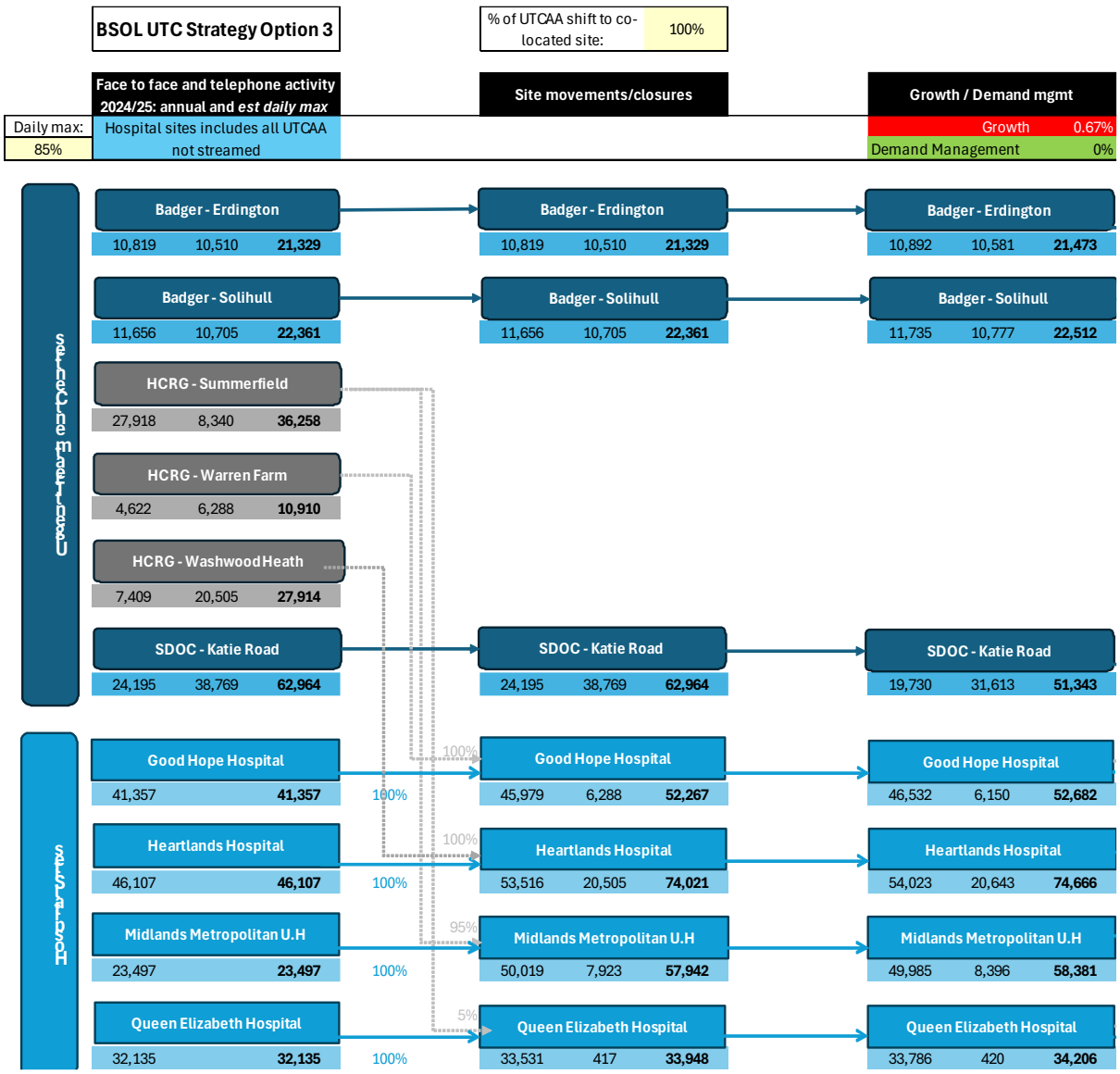
9.2.1 Using the baseline UTC and UTCAA data above, existing activity has been mapped to its expected new destination for each option. The diagrams below shows the patient flows, including net 0.7% growth. The activity paper presents indicative daily activity figures for capacity-planning purposes. These figures are used to inform initial site-level planning and should be interpreted alongside anticipated demand, workforce, operating hours and available clinical capacity. The figures are indicative and do not, in isolation, confirm the capacity required to meet demand on 95% of days.

9.3 Option A demand modelling

9.3.1 The patient flows in this model are:

- Erdington UTC is retained at its existing activity level;
- Warren Farm UTC closes with its activity going to the Good Hope UTC;
- UTC-amenable and GP-streamed activity at Good Hope ED goes to Good Hope UTC
- Washwood Heath UTC closes with its activity going to the new Heartlands Hospital UTC;
- UTC-amenable and GP-streamed activity at Heartlands Hospital ED goes the new Heartlands Hospital UTC;
- The Katie Road UTC is retained at its existing activity level;
- UTC-amenable and GP streamed activity at Queen Elizabeth Hospital ED goes the new QE UTC;
- Summerfield UTC closes with 95% of its activity going to the new Mid Met UTC and 5% going to the new Queen Elizabeth UTC;
- UTC-amenable and GP streamed activity at Mid Met Hospital ED goes the new Mid Met UTC;
- Solihull UTC retains its existing minor illness activity and takes on UTC amenable activity from Solihull minor injuries unit;

Option A demand modelling



9.4 Option A capacity modelling

9.4.1 The table below sets out the resulting annual activity by site together with the daily estimated capacity requirement. The baseline plus growth is shown on the left for comparative purposes.

Baseline	BSOL UTC STRATEGY OPTION 3		Including net growth				*Estimated daily capacity*		
+ GROWTH	Locality	Location	Attends	Phone	0.7% Total		Attends	Phone	Total
21,329	North	Erdington UTC	10,892	10,581	**21,473**		30	29	59
10,910	North	Warren Farm UTC	UTC closes / Develop Locality Hub				UTC closes / Develop Local Hub		
41,420	North	Good Hope Hospital ED	0	0	**0**		0	0	0
12,248	North	Good Hope Hospital GP streaming	0	0	**0**		0	0	0
0	North	Good Hope Hospital UTC	46,532	6,150	**52,682**		127	17	144
27,914	East	Washwood Heath UTC	UTC closes / Develop Locality Hub				UTC closes / Develop Local Hub		
46,253	East	Heartlands Hospital ED	0	0	**0**		0	0	0
14,865	East	Heartlands Hospital GP streaming	0	0	**0**		0	0	0
0	East	Heartlands Hospital UTC	54,023	20,643	**74,666**		148	57	205
62,964	South	Katie Road UTC	19,730	31,613	**51,343**		54	87	141
32,164	South / Central	Queen Elizabeth Hospital ED	0	0	**0**		0	0	0
18,144	South / Central	Queen Elizabeth Hospital GP streaming	0	0	**0**		0	0	0
0	South / Central	Queen Elizabeth Hospital UTC	33,786	420	**34,206**		93	1	94
36,258	West	Summerfield UTC	UTC closes / Develop Locality Hub				UTC closes / Develop Local Hub		
0	West	Summerfield UTC							
23,545	West	Mid Met Hospital ED	0	0	**0**		0	0	0
2,090	West	Mid Met Hospital GP streaming	0	0	**0**		0	0	0
0	West	Mid Met Hospital UTC	49,985	8,396	**58,381**		137	23	160
22,361	Solihull	Solihull UTC	11,735	10,777	**22,512**		32	30	62
17,421	Solihull	Solihull Minor Injuries	23,767	0	**23,767**		65	0	65
389,886	**TOTAL**		**250,450**	**88,580**	**339,030**		**686**	**243**	**929**

9.5 Relationship to the Recommended Option A Plus Model

9.5.1 The final recommendation is Option A Plus. It comprises four co-located UTCs at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital; three community UTCs at Solihull, Erdington and South Birmingham/Selly Oak; enhanced locality urgent care arrangements at Summerfield and Washwood Heath; and GP Out of Hours (OOH) services commissioned through three locality lots: South/Central, North/West and Solihull/East.

9.5.2 The activity model reflects the earlier Option A configuration and does not fully quantify all elements of Option A Plus. In particular, it models closure and redistribution of activity from Summerfield and Washwood Heath rather than quantifying the activity, capacity and service requirements of the enhanced locality arrangements now proposed at those sites. The figures should therefore be interpreted as indicative modelling of the earlier configuration, rather than a complete site-level capacity forecast for Option A Plus.

9.5.3 This limitation does not prevent the modelling from informing the strategic decision, but it should be recognised when interpreting the projected site volumes. The final service specifications and implementation plans will need to confirm capacity, opening hours, workforce, diagnostics, clinical pathways and operating arrangements for the recommended configuration.

9.6 Financial modelling

Introduction

9.6.1 This chapter provides a description of the financial impact of the proposed UTC changes. It begins with a description of the overall level of funding presently committed in support of UTC activity. In doing so the description of funding incorporates the sums presently spent by Bsol through its Commissioning Budgets with community service providers and also recognises the value of UTC amenable activity that is presently being paid for within the urgent and emergency care element of the ICB's contracts with local NHS providers.

9.6.2 Option A has been modelled to reflect delivering a revised UTC service model which incorporate the establishment of co-located UTC's on the Good Hope, Heartlands and Queen Elizabeth and Midland Metropolitan hospital sites and a level of ongoing community UTC service provision. The chapter describes the financial benefit associated for this option.

9.6.3 Sensitivity analysis is introduced to test the financial impact on the current financial modelling as a result of the application of different levels of Tariff for UTC activity; within this exercise the option is tested to establish the level of increase to Tariff that would be needed to achieve a break-even point.

9.6.4 The chapter concludes by discussing System affordability and the essential work that needs to be concluded upon to ensure that the new UTC service model once implemented is able to achieve financial balance at a System level.

The Overall level of funding committed in support of UTC activity and GP Out of Hours Services

9.6.5 The financial case has been constructed based upon a combination of UTC activity delivered to the Bsol population, this comprises activity:

- Presently delivered within a community setting across the Bsol geographical area; and
- Within the Emergency Departments of the four Hospital sites, Queen Elizabeth, Heartlands, Good Hope and Midlands Metropolitan. This activity described as UTC amenable activity (UTCAA) has been identified by adopting a coding methodology developed by UHB Foundation Trust.

9.6.6 The UTCAA activity appears as a subset of the total Emergency Department activity. The baseline for activity included in 2025/26 outturn and an estimate of growth (based on ONS population) for 2 years, to bring activity in line with the likely year of mobilisation for the key service changes.

9.6.7 The activity and finance baseline for the case also includes GP Out of Hours (OOHs) activity which is presently delivered under 5 legacy contracts for the BSOL population (2 current providers).

Total Activity and Finance Baseline – UTC activity

Location	Type	Site	Activity start point	Activity Growth	Tariff orginal £	Quantum start Point £000's
North	F2F	Erdington	10,819	73	£ 52.63	£ 573
Solihull	F2F	Solihull	11,656	78	£ 48.23	£ 566
Solihull	F2F	Solihull MIU	23,608	159	£ 118.89	£ 2,826
West	F2F	Summerfield	27,918	188	£ 66.53	£ 1,870
North	F2F	Warren Farm	4,622	31	£ 55.55	£ 258
East	F2F	Washwood Heath	7,409	50	£ 42.39	£ 316
South	F2F	South Birmingham (Katie Roac	19,598	132	£ 43.48	£ 858
North	F2F	GOOD HOPE HOSPITAL	41,420	279	£ 186.28	£ 7,768
East	F2F	HEARTLANDS HOSPITAL	46,253	311	£ 191.86	£ 8,934
West	F2F	MIDLAND METROPOLITAN UN	23,545	159	£ 220.05	£ 5,216
South	F2F	QUEEN ELIZABETH HOSPITAL	32,164	217	£ 212.47	£ 6,880
North	Non F2F	Erdington	10,510	71	£ 52.63	£ 557
Solihull	Non F2F	Solihull	10,705	72	£ 48.23	£ 520
West	Non F2F	Summerfield	8,340	56	£ 66.53	£ 559
North	Non F2F	Warren Farm	6,288	42	£ 55.55	£ 352
East	Non F2F	Washwood Heath	20,505	138	£ 42.39	£ 875
South	Non F2F	South Birmingham (Katie Roac	31,402	211	£ 43.48	£ 1,375
		Grand total	**336,762**	**2,267**		**£40,301**

9.6.8 As can be seen from the table above the total volume of activity amounts to 336,762 attendances and the financial value associated with this activity amounts to £40.301 million (on 26/27 price base).

Impact of the proposed change of the delivery of UTC activity

9.6.9 The proposed UTC service model, incorporates two changes:

- A revised community service of as set out for Option A and
- The establishment of co located UTC facilities at the hospital sites.

9.6.10 In establishing the new service model, a series of assumptions have been made in respect of the delivery of the previously community-based UTC activity. These assumptions are summarised in the table below.

	DMBC – Option A
Summerfield	95% of current community activity delivered at Midlands Met Hospital and 5% of activity delivered at Quen Elizabeth hospital
Warren Farm	100% of current community activity is delivered at Good Hope Hospital.
Washwood Heath	100% of current community activity is delivered at Heartlands Hospital
Katie Road	No change to current activity flows
Erdington	No change to current activity flows

9.6.11 In addition, the following volumes of UTC amendable activity are also assumed to be transferred from main emergency departments into the co-located UTCs.

Hospital site	**Volume of UTCAA within ED, transferred to co- located UTCs**
Good Hope Hospital	41,420
Heartlands Hospital	46,253
Midlands Metropolitan Hospital	23,545
Queen Elizabeth Hospital	32,164
Total	**143,382**

The financial cost of the proposed UTC solution

9.6.12 In calculating the financial cost of delivering the proposed UTC solution, a number of assumptions have been made:

- Tariffs for activity delivered within a co located UTC Tariffs at the 4 co-located UTC's have been modelled in the base case at £85 per physical (rather than telephone) attendance to

reflect the new setting and model of care – this has been used as an initial reasonable estimate for a potential market rate for activity between lower acuity ED attendances and the current rates for community UTC but including access to relevant diagnostics.

- Tariffs for activity delivered in community UTC - modelled using existing tariff rates as in the community,
- Non face to face activity (i.e. telephone consultations) - is modelled using existing tariff rates as in the community.

The proposed UTC Solution – Option A

9.6.13 The table describes the cost to the Commissioner of Option A:

Location	Site	Activity start point	Growth	Activity After	Activity Change	Quantum start Point	*of which is growth:*	Quantum After £000's	Price Change UTCAA	Community to co-located	Net Impact of Change
North	Erdington	21,329	144	21,473	0	£1,130	*£8*	£1,130	£0	£0	£0
Solihull	Solihull	22,361	151	22,512	0	£1,086	*£7*	£1,086	£0	£0	£0
Solihull	Solihull MIU	23,608	159	23,767	0	£2,826	*£19*	£2,826	£0	£0	£0
West	Summerfield	36,258	244	0	(36,502)	£2,428	*£16*	£0	£0	(£2,428)	(£2,428)
North	Warren Farm	10,910	73	0	(10,983)	£610	*£4*	£0	£0	(£610)	(£610)
East	Washwood Heath	27,914	188	0	(28,102)	£1,191	*£8*	£0	£0	(£1,191)	(£1,191)
South	South Birmingham (Katie Road	51,000	343	51,343	0	£2,232	*£15*	£2,232	£0	£0	£0
North	GOOD HOPE HOSPITAL	41,420	279	52,682	10,983	£7,768	*£52*	£4,292	(£4,223)	£747	(£3,476)
East	HEARTLANDS HOSPITAL	46,253	311	74,666	28,102	£8,934	*£60*	£5,467	(£4,976)	£1,509	(£3,467)
West	MIDLAND METROPOLITAN UN	23,545	159	58,381	34,677	£5,216	*£35*	£4,815	(£3,201)	£2,800	(£401)
South	QUEEN ELIZABETH HOSPITAL	32,164	217	34,206	1,825	£6,880	*£46*	£2,900	(£4,128)	£147	(£3,980)
	Grand total	**336,762**	**2,267**	**339,029**	**0**	**£40,301**	***£270***	**£24,747**	**(£16,528)**	**£974**	**(£15,554)**

9.6.14 Appendix 11 - Annex 1 – Provides a more detailed description of the cost of Option A, splitting activity between face to face and non-face to face activity.

9.6.15 As can be seen the immediate effect of establishing a co located facility for all work presently delivered within the four hospital sites, and as a consequence paying at a reduced UTC Tariff, is to reduce the level of funding paid to the four hospital sites by £16.528 million. The effect of switching activity from the community into the co located hospital sites is to increase the Income of the hospital sites by £5.204 million and the level of funding paid to the Community providers reduces by £4.230million, this difference (net £974,000) being attributed to the higher Tariff rate paid for co located UTC activity. The resulting level of reduced spending to Commissioners in respect of UTC activity is then £15.554 million.

Sensitivity Analysis

9.6.16 A range of scenarios have been considered, summarised in the tables below:

Tariff Change tested	Impact assessment
1. BSOL Non Contracted Activity (NCA) average tariff values charged per invoiced activity - Price increase to £110.88 for UTCAA and all Comm UTC and non-F2F at £41.62	Revised opportunity estimated at £11.6m for option A
2. 26/27 Type 3 A&E tariff and highest NCA Community tariffs - Price increase to £119.76 for UTCAA and all Comm UTC and non F2F at £54.72	Revised opportunity estimated at £8.2m option A.
3. Price increase to £100 for UTCAA, with Com UTC and non F2F unchanged	Revised output estimated at a £12.8m benefit for option A
4. Price increase to £169.27 for UTCAA, with Com UTC and non F2F unchanged - testing max tariff price before no net benefit under Option A	£169.27 is calculated as the UTCAA tariff that would result in a breakeven financial case under option A. This represents a 99% increase against base case.
5. Increasing all prices by 10%	Shows a 10% change to tariffs results in a 16% deduction in the benefit for option A but still delivers a positive saving.

9.6.17 The tables below show the impact of a variety of changes to the tariffs from the sensitivity analysis:

	Baseline		BSOL Non Contracted Activity (NCA) average tariff values charged per invoiced activity - Price increase to £110.88 for UTCAA and all Comm UTC and non-F2F at £41.62			26/27 Type 3 A&E tariff and highest NCA Community tariffs - Price increase to £119.76 for UTCAA and all Comm UTC and non F2F at £54.72			Price increase to £100 for UTCAA, with Com UTC and non F2F unchanged		
Option impact	Baseline @ £85 for UTCAA 27/28 £000's	Change Vs BAU	Total Cost £000's	New Benefit vs Base	Change to benefit £000's	Total Cost £000's	New Benefit vs Base	Change to benefit £000's	Total Cost £000's	New Benefit vs Base	Change £000's
BAU	40,301		40,301			40,301			40,301		
Option A	24,747	15,554	28,730	11,571	(3,983)	32,081	8,220	(7,334)	27,516	12,786	(2,768)

	Baseline		Price increase to £169.27 for UTCAA, with Com UTC and non F2F unchanged - testing max tariff price before no net benefit under Option A			Increasing all BAU prices by 10%		
Option impact	Baseline @ £85 for UTCAA 27/28 £000's	Change Vs BAU	Total Cost £000's	New Benefit vs Base	Change £000's	Total Cost £000's	New Benefit vs Base	Change £000's
BAU	40,301		40,301			40,301		
Option A	24,747	15,554	40,301	0	(15,554)	27,222	13,079	(2,475)

9.6.18 The sensitivity analysis shows that significant increases to the tariffs above reasonable levels would need to be made in order for the new model in Option A to become non-cost effective.

GP Out of hours Activity and Finance Baseline

9.6.19 The table below shows the current cost and activity levels split by telephone, home visits and face to face attendances:

	Total £000's	Telephone Calls	Home Visits	Face to Face Visits	Total Activity
Total	**£ 8,855**	**89,010**	**1,651**	**12,313**	**102,974**

9.6.20 The table below sets out one way in which the activity could be grouped and delivered under potential locality arrangements as described in Option A

Area Split	Telephone Calls	Home Visits	Face to Face Visits	Total Activity
North and West	20,465	356	2,950	**23,771**
South and Solihull	40,306	833	5,460	**46,599**
East and Central	28,239	461	3,903	**32,603**
Total	**89,010**	**1,651**	**12,313**	**102,974**

9.6.21 Note: The activity split has been based on relative list size shares for GP practices in each area apportioning the activity as the current OOH activity is not reported by GP practice.

9.6.22 No specific financial savings have been modelled from current OOHs budgets, but the potential market engagement would inform the approach to optimise value for money for this element of the service.

Capital

9.6.23 No specific capital requirements have been identified for the delivery of Option A relating to community facilities (where services are continuing in existing locations). The capital requirements for delivery of the co-located UTC facilities have been addressed through separate business cases via the relevant NHS providers with NHSE

The System Financial affordability position

9.6.24 The above analysis has considered the financial impact associated with the proposed future UTC service and the recommended Option A for delivering this new service by considering the cost of paying for UTC activity as a commissioner. In doing so, it has been possible to identify a significant level of cost saving amounting to circa £15 – 16 million. This level of cost saving is predominantly associated with the decision to establish co located UTC facilities on each of the four hospital sites and enable activity to be redirected from the existing Emergency Care Departments to the hospital based UTCs which attracts a much lower level of tariff than the standard tariff for Emergency Care activity in A&E which is presently being used to calculate

the overall contract payment for the delivery of Emergency Care available at each hospital site.

9.6.25 Calculation of the Bsol system financial impact requires an understanding of how the hospital-based service offering in respect of Emergency Care will change following the establishment of the co located UTC's, in particular the operational and workforce impact arising from the redirection of activity into this new facility from the existing Accident and Emergency Departments and there is a need to ensure that the resources are available to support the ongoing delivery of hospital based accident and emergency activity will appropriately reflect the level of funding required to deliver such care on each hospital site. The calculation of financial impact will also require an understanding of impact upon attributable functions such as Same Day Emergency Care, Acute Medical and Surgical admission Units and streaming facilities that presently exist within the hospital sites

9.6.26 This work is also linked with the broader developments for neighbourhoods and anchor (lead) provider contracts. This work will be used to support the calculation of the contract values for Urgent and Emergency Care within the contracts for the 2027/28 financial year and thereafter.

9.6.27 Work has commenced with provider colleagues to progress the agreement of the contract changes required to vary current contracts for the co-located UTCs, which will include financial terms and conditions including the activity and associated tariffs, starting from the activity and financial modelling under Option A, and in the context of the wider UEC pathways as noted above.

9.6.28 In making the decision to progress Options A to redesign the manner within which UTC activity is delivered, Birmingham and Solihull ICB will ensure that the resources available to support the ongoing delivery of hospital based Accident and Emergency care activity will appropriately reflect the level of funding required to deliver such care on each hospital site. There is no expectation that the change will compromise financially and/or operationally the delivery of hospital-based Accident and Emergency Care.

9.6.29 In practice, this is expected to be achieved through the repurposing of the identified commissioner savings into a combination of:

A. appropriate tariffs for hospital-based Accident and Emergency activity;) to meet ongoing costs and a higher-acuity patient mix (acknowledging also any potential stranded costs and any revised workforce models)

B. supporting the delivery of wider urgent and emergency care pathway improvements.

9.6.30 The realignment of resources made possible through Option A ensures that both Urgent Treatment Care Activity and Care provided within Hospital Based Accident and Emergency Care facilities remain affordable and sustainable over the longer term.

9.6 Implications for procurement and implementation

9.6.31 The activity and financial modelling provides an evidence base for progressing the recommended model into detailed procurement and implementation planning. It identifies anticipated changes in activity distribution and provides indicative financial estimates against stated assumptions. It does not, by itself, establish that final site-level capacity, workforce, estate, diagnostic or operating arrangements have been agreed.

9.6.32 Following approval of the recommended model, the activity estimates will inform service specifications and provider mobilisation plans. This will include confirmation of site-specific

capacity, operating hours, workforce establishment, clinical pathways, diagnostic provision, patient flow and arrangements for managing variation in demand. The four acute Trusts will need to confirm the operational requirements and dependencies for their co-located UTCs. The community UTC and enhanced locality urgent care requirements will also need to be reflected in the final implementation plans.

9.6.33 For GP OOH, the baseline will inform development of the three locality-based lots. The service specification and procurement process will establish the required balance of telephone assessment, face-to-face appointments and home visits, together with performance, workforce and reporting arrangements.

9.6.34 The programme should monitor actual activity, service utilisation, patient flows, waiting times, reattendance and financial performance following implementation, comparing observed outcomes with the planning assumptions. Any significant variation should be reviewed through established programme and contract governance, with corrective action taken where required.

9.8 Conclusion

9.6.35 The capacity, demand and financial assessments provide a supporting evidence base for the recommended urgent care model. The activity analysis illustrates how activity could be redistributed between co-located UTCs, community UTCs and other services under the modelled Option A configuration. The financial analysis identifies an indicative reduction in commissioner expenditure under the stated assumptions and highlights the importance of agreeing tariffs and understanding the impact on acute provider costs and the wider urgent and emergency care system. The GP OOH baseline provides a starting point for developing the proposed locality-based commissioning model.

9.6.36 The modelling has limitations and does not constitute a complete site-level capacity assessment of Option A Plus. In particular, the enhanced locality urgent care arrangements at Summerfield and Washwood Heath are not fully quantified in the supplied activity model, and the growth and daily capacity assumptions require clarification. The financial estimates are also dependent on assumptions that will need to be tested through provider engagement and procurement.

9.6.37 These limitations do not prevent the modelling from informing the strategic decision, provided they are transparent and are not overstated as confirmed forecasts. The recommended model should progress to detailed procurement and implementation planning, with providers and commissioners confirming the final capacity, workforce, estates, clinical, tariff and operational arrangements. Actual activity, access, quality and financial performance should then be monitored against the planning baseline through established governance.

9.6.38 Overall, the modelling supports progression to the next stage of planning, subject to the assumptions and limitations described in this chapter and the appropriate confirmation of deliverability through procurement, provider engagement and mobilisation.

10 ENABLERS

10.1 Workforce Overview

10.1.1 The workforce model is a critical component of the proposed future urgent treatment centre (UTC) configuration. The recommended Option A as amended comprises four co-located UTCs at Queen Elizabeth Hospital Birmingham (QEH), Birmingham Heartlands Hospital (BHH), Good

Hope Hospital (GHH) and Midland Metropolitan University Hospital (MMUH), supported by three community UTCs in Solihull, Erdington and South Birmingham/Selly Oak.

10.1.2 The workforce model is designed to provide a sustainable multidisciplinary workforce capable of delivering the national UTC specification, including assessment and treatment of minor illness and minor injury, appropriate streaming from Emergency Departments (EDs), access to diagnostics, safe discharge and onward referral, and senior clinical decision-making.

10.1.3 The model will also be supported by a locality-based GP Out of Hours (OOH) service, procured through three locality lots:

- South/Central;
- North/West; and
- Solihull/East.

10.1.4 The workforce requirements for each UTC will be determined through demand and capacity analysis and refined through procurement and mobilisation. Providers will be required to demonstrate that they can recruit, retain and sustain the workforce necessary to deliver the agreed service specification safely and consistently.

10.1.5 The future workforce model comprises an appropriately balanced multidisciplinary team (MDT), including, as required:

- GPs and/or Emergency Medicine doctors;
- Advanced Clinical Practitioners (ACPs);
- Advanced Nurse Practitioners (ANPs);
- Health Care Assistants (HCAs);
- reception and administrative staff;
- access to diagnostic support;
- clinical leadership and supervision; and
- appropriate mental health and wider multidisciplinary support.

10.1.6 The workforce model will support senior clinical decision-making, minor illness and minor injury pathways, diagnostics, safe discharge and follow-up, and multidisciplinary working. The current workforce provides a foundation for transition, but the future model will require targeted recruitment, workforce development and changes to skill mix at some sites.

10.2 Multidisciplinary working and workforce resilience

10.2.1 A key principle of the future model is to move towards a more consistent MDT across UTCs, with standardised competencies and clearer clinical leadership. The proposed model provides opportunities for shared roles and rotational posts across UTCs, GP OOH and NHS 111, where appropriate, supporting workforce flexibility and resilience.

10.2.2 Co-location is expected to provide additional opportunities for access to senior clinical expertise, diagnostics and wider acute hospital support. Community UTCs will continue to provide locally accessible urgent care for lower-acuity presentations, supported by an appropriately skilled workforce and clear escalation pathways. The future workforce approach will therefore combine:

- greater workforce resilience through co-location and MDT working;
- local community provision through the three community UTCs;
- strengthened senior clinical decision-making;
- standardised training and competencies;
- rotational and developmental opportunities; and
- locality-based GP OOH provision.

10.3 Workforce Considerations from Consultation

10.3.1 Workforce sustainability and staff development formed part of the original options appraisal. The criterion considered the ability of each option to support a sustainable, resilient and highly skilled UTC workforce, including recruitment, retention, professional development, clinical supervision, staff wellbeing and opportunities for career progression. Option A scored more highly than Option B against this criterion.

10.3.2 Following consultation, HASC/HOSC feedback and the subsequent options review, the original workforce assessment was revisited. The established appraisal methodology was retained rather than undertaking a new weighted scoring exercise. The purpose of the review was to establish whether subsequent evidence reinforced, qualified or challenged the original assessment.

10.3.3 The review did not identify evidence sufficient to change the original workforce assessment. However, consultation and staff feedback identified a number of specific workforce considerations which have informed the design of the recommended model and the requirements for implementation.

10.3.4 Key themes included:

- the availability of sufficient clinical workforce to meet demand;
- appropriate multidisciplinary skill mix;
- access to GP, ACP and ANP expertise;
- recruitment and retention;
- clinical leadership and supervision;
- training and professional development;
- staff wellbeing and sustainable working patterns; and
- the workforce implications of extended or 24-hour opening.

10.3.5 Public consultation feedback specifically identified the importance of sufficient staffing and an appropriate clinical skill mix, including access to GPs alongside ANPs, to ensure that services can safely manage the range of presentations expected within UTCs. Workforce shortages were also identified as a wider concern in relation to the sustainability and future-proofing of urgent care services.

10.3.6 Staff engagement similarly identified the benefits of co-location, MDT working, clearer triage and senior clinical oversight, alongside the importance of training, development and sustainable rotas. Staff also highlighted the workforce implications associated with any move towards 24-hour provision, including overnight staffing, contractual arrangements, fatigue,

supervision and maintaining an appropriate skill mix.

10.3.7 The consultation therefore did not result in a new workforce score. Instead, it strengthened the evidence base for the workforce requirements and implementation conditions that need to be met for the benefits of the recommended model to be realised.

10.4 Workforce Implications of the Recommended Model

Four co-located UTCs

10.4.1 The four co-located UTCs will operate alongside the acute hospital EDs and will provide an enhanced multidisciplinary urgent care environment, with access to senior clinical decision-making, diagnostics and specialist pathways.

10.4.2 The workforce model will be designed around demand and the agreed clinical model rather than applying an identical establishment to every site. The required staffing complement will therefore be determined through detailed demand, activity and capacity analysis.

10.4.3 The core workforce is expected to include GPs and/or Emergency Medicine doctors, ACPs, ANPs, UTC nurses, HCAs and administrative staff. The clinical workforce will support assessment and treatment of minor illness and injury, triage and streaming, prescribing, diagnostics, observation, safe discharge and onward referral.

10.4.4 Radiology for co-located UTCs will be provided through the host acute hospital, providing access to diagnostic capability that may not be available consistently within standalone community settings.

- Co-location therefore provides workforce benefits through:
- access to a wider range of clinical expertise;
- improved access to diagnostics;
- opportunities for cross-cover and flexible deployment;
- closer links with ED and acute clinical services;
- opportunities for multidisciplinary working and supervision; and
- broader training and professional development opportunities.

10.4.5 These benefits are consistent with consultation feedback, where respondents recognised the potential benefits of co-location in relation to diagnostics, escalation and access to specialist expertise, while also expressing support for retaining community-based provision.

Three community UTCs

10.4.6 The three community UTCs in Solihull, Erdington and South Birmingham/Selly Oak will provide locally accessible urgent care for lower-acuity presentations and will operate to the national UTC specification.

10.4.7 The workforce model will provide the clinical skills necessary to assess and treat minor illness and injury safely, with access to appropriate diagnostics and clear escalation and referral arrangements.

10.4.8 The retention of three community UTCs within Option A as amended provides a distributed workforce model rather than concentrating all UTC activity within acute hospital sites. This

responds to the consultation evidence which identified the importance of maintaining community-based urgent care and avoiding over-centralisation, while also recognising the benefits of co-location where access to diagnostics and acute clinical support adds value.

10.4.9 The workforce requirements at each community UTC will be proportionate to activity and opening hours, with establishment and skill mix determined through demand and capacity modelling and confirmed as part of procurement and mobilisation.

Senior clinical decision-making

10.4.10 A key requirement of the future UTC model is access to appropriate senior clinical decision-making throughout agreed operating hours. The future workforce model will therefore include access to appropriately qualified GPs, Emergency Medicine doctors and/or senior ACPs, supported by ANPs and UTC nursing staff.

10.4.11 The workforce plan recognises that the availability of advanced practitioners and clinicians with specific minor injury and musculoskeletal skills is a potential constraint. Targeted recruitment, upskilling and structured development pathways will therefore be required.

10.4.12 Importantly, 24-hour operation has not been assumed as an established workforce requirement across all UTCs. Opening hours remain subject to final assessment of demand, the clinical model, workforce availability and affordability but expected to deliver a minimum of 12 hours a day.

10.4.13 Where extended or 24-hour opening is agreed, the provider will need to demonstrate a safe and sustainable workforce model, including appropriate overnight staffing, supervision, skill mix, rest arrangements and clinical leadership.

10.4.14 This reflects staff feedback which identified significant workforce implications associated with 24-hour working, including redesigned rotas, overnight staffing, fatigue and maintaining appropriate clinical supervision and skill mix.

Training, development and workforce sustainability

10.4.15 The future model provides an opportunity to strengthen training and career development across the urgent care workforce. The proposed approach includes:

- standardised clinical competencies across UTCs;
- structured induction and mandatory training;
- enhanced training in minor illness, minor injury, musculoskeletal assessment, wound care, triage and streaming;
- training in near-patient testing and digital systems;
- supervised clinical practice and simulation;
- rotational opportunities across urgent care pathways where appropriate;
- leadership development and succession planning; and
- defined career development pathways for nurses, HCAs and advanced practitioners.

10.4.16 This responds directly to staff feedback identifying the importance of structured training, development opportunities and progression in supporting recruitment and retention. The future model also provides opportunities to broaden clinical experience through rotational

working.

10.4.17 The model will also need to retain sufficient clinical exposure across UTC and ED settings. Staff highlighted the potential risk that removing all minor injury activity from EDs could reduce training opportunities, reinforcing the importance of maintaining appropriate clinical exposure and educational pathways during implementation.

GP Out of Hours workforce

10.4.18 The recommended model includes the further development of GP OOH provision through three locality-based lots: South/Central, North/West and Solihull/East. The service will operate to a common overarching clinical and service specification, while allowing locality-specific arrangements for face-to-face appointments and home visiting.

10.4.19 Consultation feedback identified a clear interest in localised OOH provision, particularly in relation to face-to-face care and home visits. However, the consultation report also identified concerns about whether sufficient doctors would be available to provide home visits and whether the proposed service could meet demand in practice. The OOH comments were identified through free-text analysis and therefore should be treated as indicative rather than representative of all respondents.

10.4.20 The decision to develop three locality lots therefore provides a refinement to the recommended model which responds to these concerns while retaining a consistent clinical and governance framework. The GP OOH workforce model will need to ensure:

- sufficient GP and wider clinical capacity;
- appropriate clinical assessment and triage;
- access to face-to-face appointments;
- home visiting where clinically appropriate;
- safe management of demand across the three joint localities;
- appropriate supervision and clinical governance; and
- resilience during periods of increased demand.

10.5 Workforce risks and mitigation

10.5.1 A number of workforce risks have been identified, including the potential for workforce displacement and uncertainty associated with TUPE/COSOP processes, recruitment challenges for overnight and weekend shifts, and the risk of staff disengagement if communication is inadequate. There is also a risk of leadership gaps if a structured clinical leadership pipeline is not developed.

10.5.2 Mitigation strategies will include early and transparent TUPE/COSOP briefings, a clear workforce communications and engagement strategy, standardised competency and training frameworks, and the development of formal career pathways and leadership programmes. These measures will provide staff with reassurance, help maintain morale, and support a smooth transition to the new model.

Risk	Mitigation
Recruitment and retention	Workforce requirements will be clearly defined through procurement, with providers required to submit detailed workforce plans demonstrating how they will recruit, retain and sustain the required workforce. Recruitment will be phased in line with mobilisation, with targeted action for identified shortage roles and a focus on retention, career development and succession planning. Workforce capacity and vacancy levels will be monitored through contract management.
Skill Mix	The service specification will set out the required multidisciplinary skill mix and competencies for each UTC, based on activity, demand and the clinical model. This will include appropriate access to GPs, ACPs, ANPs, nurses and support staff, with standardised competencies, clinical supervision and opportunities for upskilling and rotational working to support resilience and flexibility.
GP OOHs Availability	The proposed three-locality commissioning model will enable workforce requirements to be considered at a local level while maintaining a consistent overarching clinical specification. Providers will be required to demonstrate sufficient GP and wider clinical capacity to deliver virtual assessment, face-to-face appointments and home visiting where clinically appropriate, with workforce capacity and service performance monitored throughout the contract.
Workforce transition	A structured workforce transition plan will be developed with existing providers and staff representatives, taking account of TUPE/COSOP requirements where applicable. Early engagement, clear communication and phased mobilisation will support continuity of staffing and minimise disruption to services. Workforce risks associated with changes to roles, establishments and working arrangements will be identified and managed throughout mobilisation.
Training and development	A system-wide competency framework will underpin the future workforce model, supported by structured induction, mandatory and role-specific training, clinical supervision and ongoing competency assessment. Training will focus on the skills required for the future UTC model, including minor illness and injury, triage and streaming, diagnostics, digital systems and advanced practice. Career development and rotational opportunities will also support recruitment and retention.
24/7 sustainability	All UTCs will operate for a minimum of 12 hours per day, in line with the national UTC service specification. For co-located UTCs, the final operating hours will be determined through local demand and capacity modelling, taking account of activity, patient need, workforce availability, clinical requirements and the wider acute pathway. The final operating hours will be locally agreed as part of implementation with the relevant acute provider, ensuring that the agreed model is clinically safe, operationally deliverable and appropriately aligned with demand. Where modelling demonstrates a requirement for extended operating hours,

Risk	Mitigation
	this will be considered as part of the local implementation arrangements. 24-hour operation is therefore not assumed across all co-located UTCs, but may be considered where supported by the demand and capacity assessment and agreed locally. For the community UTCs, a minimum of 12 hours per day will apply, with specific operating hours established through implementation and provider arrangements.

Overall workforce risk management

10.5.3 Workforce availability remains a material implementation consideration for the recommended model, particularly in relation to advanced practice, senior clinical decision-making and any extended opening hours. The mitigations above are therefore intended to ensure that workforce requirements are established before implementation rather than being addressed retrospectively. Workforce planning will be undertaken alongside demand and activity modelling, procurement and mobilisation, with providers required to demonstrate that the required workforce, skill mix, competencies and clinical leadership arrangements can be secured and sustained. This will provide assurance that the recommended service configuration is supported by a safe and deliverable workforce model.

Workforce Implementation

10.5.4 Delivery of the preferred workforce model will be secured through the procurement and mobilisation of the future UTC/HP OOHs services. The workforce requirements set out in this DMBC will be translated into clear contractual requirements within the service specification, including the required multidisciplinary skill mix, clinical leadership, senior clinical decision-making, competencies, training, supervision and workforce resilience. The procurement process will provide an opportunity for bidders to demonstrate how they will recruit, retain and sustain the workforce required to deliver the service safely and in accordance with the specification.

10.5.5 Workforce requirements will form an explicit part of the procurement evaluation and due diligence process. Evaluation questions will test not only whether providers can describe an appropriate workforce model, but whether they can demonstrate that it is credible, deliverable and sustainable within the proposed service configuration and operating hours. This will include consideration of workforce availability, recruitment and retention, skill mix, clinical leadership, training and competency, workforce resilience and the management of workforce risks.

10.5.6 Examples of areas that will be tested through procurement evaluation and provider assurance include:

Areas for Assurance	Example evaluation / assurance question
Workforce Model and Skill Mix	How will the bidder ensure that the proposed workforce establishment and skill mix are sufficient to deliver all elements of the UTC/GP OOHs specification, including minor illness, minor injury, triage, streaming, diagnostics and safe discharge?

Areas for Assurance	Example evaluation / assurance question
Senior clinical decision-making	How will the provider ensure appropriate access to senior clinical decision-making throughout the agreed operating hours, including arrangements for escalation and clinical oversight?
Recruitment and retention	How will the provider recruit and retain the required workforce, particularly in known shortage areas, and what evidence demonstrates that the proposed staffing model is achievable?
Workforce Resilience	How will the provider maintain safe staffing during periods of sickness, vacancies, annual leave and increased demand, and minimise reliance on temporary or agency staffing?
Advanced Practice	How will the provider ensure sufficient GP, ACP and ANP capacity and maintain the competencies required to manage the anticipated patient cohort?
Training and Competency	How will the provider ensure that all staff have the required competencies, induction, mandatory training, clinical supervision and ongoing professional development to deliver the UTC/GP OOHs specification?
Clinical leadership	What arrangements will be put in place for clinical leadership, supervision, governance and succession planning, and how will continuity be maintained where key clinical leadership posts are vacant?
Opening hours	How will the provider ensure that the workforce model is aligned to the agreed operating hours at each site and can flex appropriately to changes in demand?
Mobilisation	How will the provider mobilise the required workforce prior to service commencement, including recruitment, onboarding, training, competency assessment and any TUPE/COSOP requirements?
Workforce transition	How will the provider manage workforce transition and engagement with existing staff, including TUPE/COSOP where applicable, while maintaining continuity and safety of service?
Workforce sustainability	How will the provider demonstrate that the proposed workforce model is sustainable over the life of the contract, including arrangements for career development, succession planning and retention?

10.5.7 The ICB will also require successful providers to provide a detailed workforce mobilisation plan prior to service commencement, demonstrating that the workforce required by the specification is in place, appropriately trained and competent. Workforce performance and sustainability will subsequently be monitored through contract management, including staffing levels, vacancies, turnover, use of temporary staffing, training compliance and delivery of the required clinical skill mix.

10.6 Workforce Conclusion

10.6.1 The preferred workforce model provides a clear framework for delivering a safe, sustainable and effective UTC service across Birmingham and Solihull. It establishes the required multidisciplinary skill mix, clinical leadership, senior clinical decision-making, training and competency requirements across the future service configuration.

10.6.2 For the co-located UTCs, workforce implementation has also considered how the existing workforce capacity within the acute provider can be aligned with the new model. This may include the redeployment or realignment of staff currently delivering minor injury and related urgent care functions into the co-located UTC workforce, where this supports the agreed clinical model and operating arrangements. This approach provides an opportunity to maximise existing skills and workforce capacity, minimise unnecessary duplication and support the integration of UTC services with the wider acute pathway.

10.6.3 Workforce delivery will be secured through the procurement process, with the requirements set out in the service specification and tested through provider evaluation. Bidders will be required to demonstrate how they will utilise existing workforce capacity appropriately alongside any additional recruitment required, and how they will ensure the required workforce, skill mix and clinical leadership are available throughout the agreed operating hours.

10.6.4 Following contract award, these requirements will be carried forward through mobilisation and ongoing contract management, including appropriate workforce transition arrangements where required. This will provide assurance that the workforce model is both deliverable and sustainable, while making effective use of existing workforce capacity across the system.

10.7 Estates and sustainability

Estate Strategy and Approach

10.7.1 The future UTC model will make use of existing NHS estate where practicable, alongside the development of co-located UTC facilities at the four acute hospital sites. This approach is consistent with the BSol ICB Infrastructure Strategy, which supports the development of community-based services, integrated neighbourhood working and more efficient use of the NHS estate. This infrastructure strategy was supported by the NHS England ADEPT modelling and benchmarking work that was undertaken in October 2024 (Appendix 12– ADEPT BSoL Estates Report) The Locality Infrastructure Group (LIG), comprising system providers and NHS landowners, has supported the identification of potential premises and locality infrastructure to enable the proposed service configuration.

10.7.2 The estates assessment undertaken for the earlier consultation options provides a basis for assessing the deliverability of the recommended Option A Plus model. However, the final configuration differs from the original Option A and therefore requires confirmation of the premises, operating requirements and associated costs for the enhanced locality urgent care arrangements at Summerfield and Washwood Heath, together with the three community UTCs and locality-based GP OOH service.

Community UTC Estate, Capacity and Operating Hours

10.7.3 The estates assessment has considered the capacity and suitability of existing community premises to accommodate the proposed UTC services. Activity modelling undertaken for Options A and B confirms that three consultation/examination rooms would provide sufficient

capacity for the modelled UTC activity, with additional capacity available at all sites should further rooms be required.

10.7.4 No estates-related capital costs have been identified for community UTCs under Options A, A Plus or B. The proposed approach therefore makes use of existing estate, including underused or void accommodation, to support delivery of the community UTC model.

10.7.5 The estates assessment identifies planning permissions and lease arrangements as the principal constraints on extending community UTC opening hours beyond those typically permitted, which are around 12 hours per day. Delivery of longer operating hours may require changes to planning permission and contractual arrangements. The main potential planning consideration identified is noise associated with out-of-hours activity. The activity assessment indicates that car park movements would be fewer than four visits per hour per site until 22:00 and fewer than one visit per hour per site after 22:00. On this basis, the estates assessment considers the risk of objection associated with noise to be low.

Washwood Heath

The centre has c103 parking spaces split between visitors and staff, with 8 accessible spaces with no charge for visitor parking. Access from the accessible parking spaces is accessible to a wheelchair user unaided. It is accessed by a range of bus services. The closest rail station is a 12-minute walk to Adderley Park which can then access New Street station.

Refer to AccessAble report at: https://www.accessable.co.uk/community-health-partnerships/access-guides/washwood-heath-primary-care-and-wellbeing-centre

The estate is maintained to Estate code Condition B and is a Core asset.

Summerfield

Summerfield has c71 parking spaces split between visitors and staff, with 5 accessible spaces with no charge for visitor parking. Public transport options are available by bus:

- Local bus routes serving the centre/area include 11A&C, 66, 66A, 80, 81, 82, 87 and 88. The 88 stops outside City Hospital's main entrance (around 8 minutes' walk to the Primary Care Centre).
- From Birmingham city centre / New Street area, services linking towards City Hospital/Winson Green include 80, 81, 82, 87, 66, 11 and 88.
- Nearest mainline station: Birmingham New Street. From New Street, continue by bus (e.g., 80/81/82/87/66/11/88) or taxi to the centre.

Refer to AccessAble report at: https://www.accessable.co.uk/community-health-partnerships/access-guides/summerfield-primary-care-centre

- The estate is maintained to Estatecode Condition B and is a Core asset.

Stockland Green

The centre has c156 parking spaces split between visitors and staff, with 4 accessible spaces and no charge for visitor parking.
Public transport options are available by bus from Birmingham city centre: take bus 65 (Birmingham–Aston–Short Heath–Perry Common) and alight near Reservoir Road / Northcroft Hospital, then walk to the centre. Other services operating through Stockland Green include 11A and 11C (Outer Circle).
Refer to AccessAble report at: https://www.accessable.co.uk/community-health-partnerships/access-guides/stockland-green-primary-care-centre

- The estate is maintained to Estatecode Condition B and is a Core asset.

Katie Road

There is free on-site car parking and adjacent Pay and Display, several accessible spaces.

Several local and regional bus routes serve the Selly Oak and Bristol Road corridor, making the centre accessible by bus. Key routes that run near the area include 61 and 63, and express services such as X21 and X22; these routes are operated by local bus companies and serve the University, Selly Oak and city centre corridors. Route timetables and stop locations are published by the operators and local journey planners.

The nearest rail link is Selly Oak station, which is on the West Midlands Railway network and provides regular services to and from Birmingham New Street and other regional stations. From the station the centre is within a short walk along local streets and Bristol Road.

North Birmingham UTC (located at Good Hope Hospital)

North Birmingham UTC is located on Rectory Road in Sutton Coldfield (postcode B75 7RR) and is well connected to the strategic road network, with access from Junction 9 of the M42, Junctions 5 and 6 of the M6 (northbound exit only at J5), the M6 Toll, and nearby A-roads including the A38, A453 and A5127. *The site sits within Birmingham's Clean Air Zone, and some vehicles may be subject to charges when travelling by car.*

The hospital provides approximately 288 visitor parking spaces within designated visitor car parks, with parking capacity subject to pressure at peak times; all on-site car parks are accredited under the Park Mark® Safer Parking Scheme. A simplified free-parking process is in place for Blue Badge holders, using barrier verification rather than on-site validation.

The site is well served by public transport, with multiple bus routes stopping directly outside the hospital on Rectory Road and Sutton Coldfield town centre accessible by bus or train. You can take bus services: 71, 76, 78A, 110, X4 & X14.

Sutton Coldfield railway station is approximately a 15-minute walk from the hospital and offers frequent services to Birmingham New Street, step-free access, and over 300 parking spaces

Solihull Hospital

Solihull Hospital is located on Lode Lane in the town centre of Solihull (postcode B91 2JL), with good access from Junction 5 of the M42, the A41 Warwick Road and the A34 Stratford Road.

The hospital provides 222 parking spaces for visitors, with parking located in the designated Main Visitor Car Park outside the north entrance; however, capacity can be constrained at peak times. All car parks on the site are accredited under the Park Mark® Safer Parking Scheme, with blue badge parking available in the main visitor car park and at designated points around the hospital perimeter.

Parking charges apply, with free parking for blue badge holders and short-stay parking available for up to 30 minutes free of charge.

The site is well served by public transport, with multiple bus routes stopping directly outside the hospital on Lode Lane and Solihull town centre is also served by regular bus routes and is a 10-minute walk from the hospital.

The closest railway station to Solihull Hospital is Solihull, which is around 15 minutes' walk from the hospital. Solihull railway station is served by West Midlands Railway and Chiltern Railways. There are trains approximately every 10 – 20 minutes from Birmingham Moor Street and Birmingham Snow Hill to Solihull. The station has almost 300 car parking spaces and step-free access for those with limited mobility.

Locality	Hub Location	UTC Option A+
East	Washwood Heath	Heartlands Hospital
West	Summerfield	Midland Metropolitan University Hospital
North	Stockland Green	Good Hope Hospital
South & Central	West Heath Hospital (South Locality)	Katie Road and Queen Elizabeth
	Moseley Hall (Central Locality)	
Solihull	Solihull Hospital	Solihull Hospital

Estates operational costs under Option A Plus

10.7.6 The estates assessment identifies that additional estates operational costs may be required to support the extended operating hours associated with full UTC provision under Option A. Under Option A Plus, additional estates costs will depend on the final model for delivery of GP OOH services and the associated opening hours and use of premises. The assessment indicates that these costs are expected to fall within the range of the Option A and Option B assumptions, with Option B representing the upper-bound cost, which is assessed as affordable.

10.7.7 The precise operational cost associated with Option A Plus will therefore need to reflect the

final GP OOH specification and delivery arrangements. This should be considered alongside the financial assessment of the recommended model, distinguishing estates operational costs from the wider service costs

Co-located UTCs Capital Developments

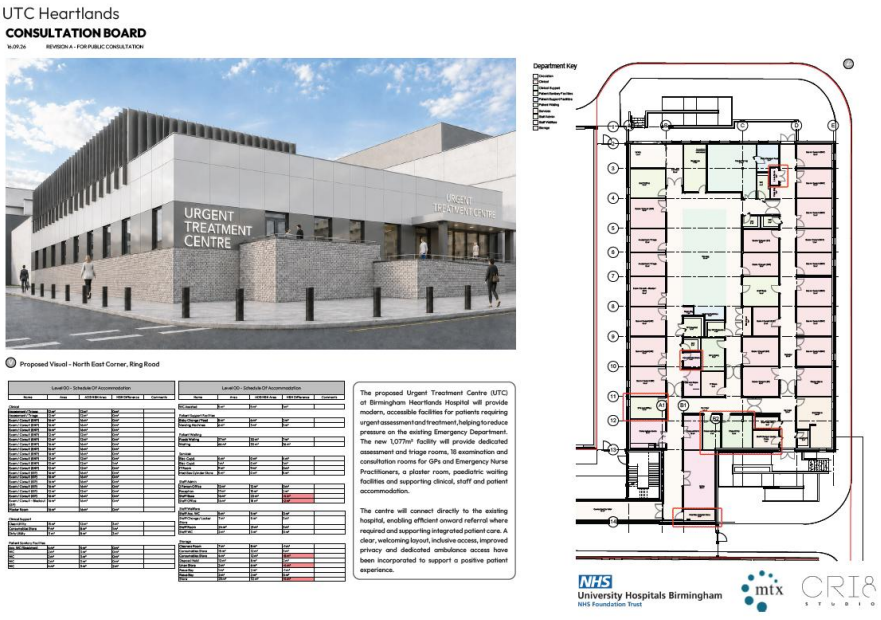
10.7.8 The co-located UTC developments at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital are funded through the national Return to Constitutional Standards Programme. The developments provide the estate infrastructure required to accommodate the proposed UTC services alongside emergency department provision.

10.7.9 The estates assessment identifies the accommodation and development approach at each hospital site. These schemes support the intended co-located model, including appropriate clinical space and interfaces with emergency department pathways. Site-level implementation planning will need to align the estates developments with the agreed service specifications, operational arrangements and mobilisation requirements.

Heartlands Hospital

10.7.10 Phase 1 – Provide a new modular Urgent Care Centre to decant the existing Clinic 9 service from Block 41 at the Heartlands Hospital site. The new unit will deliver both adult and paediatric care, featuring eight clinical rooms, a waiting area, and supporting clinical spaces. It will be located adjacent to the existing ED Minors unit.

10.7.11 Phase 2 – Partially demolish Block 41, formerly used for outpatients and therapies, to enable installation of a new modular Urgent Treatment Centre. This facility will serve as the new front door for patients, supporting their pathways and alleviating pressure on the Emergency Department. The new UTC will be a fully compliant space for both adult and paediatric patients, providing twenty clinical rooms—including examination, consultation, treatment, assessment, and plaster rooms—along with all associated staff and clinical support areas.



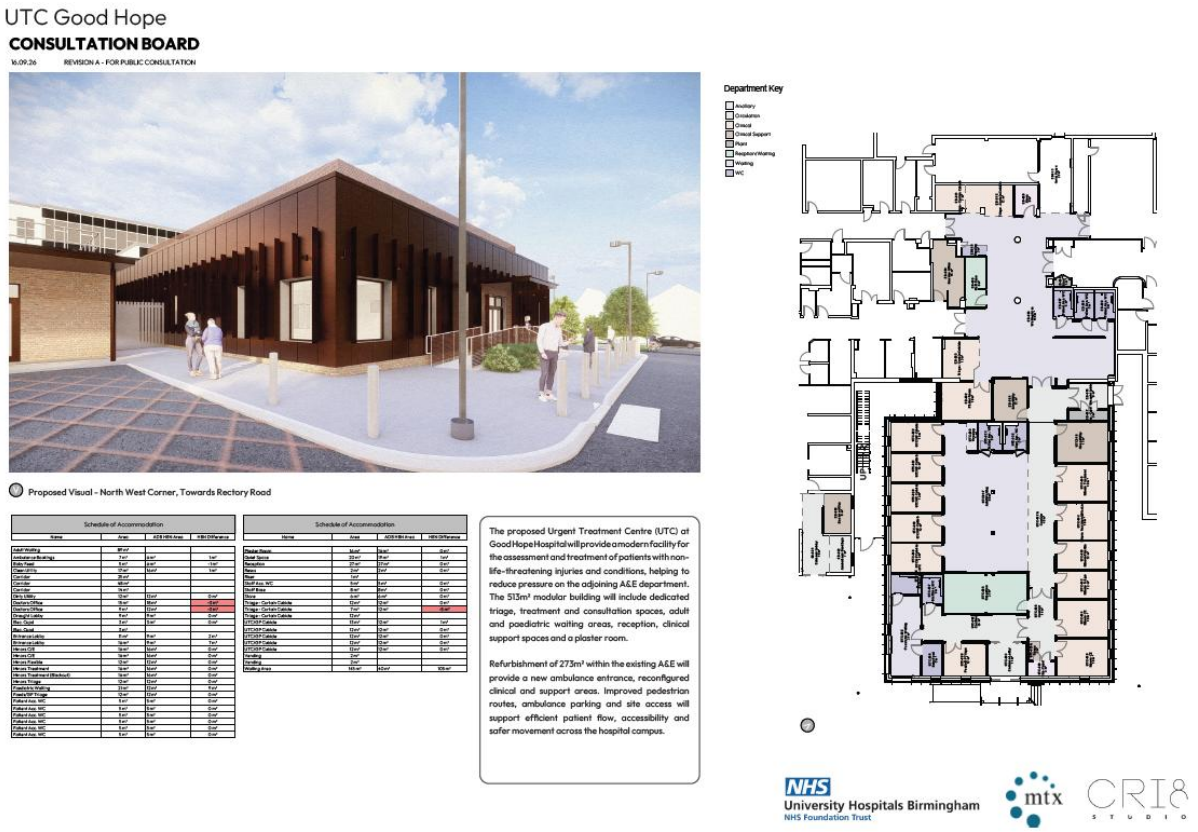
Queen Elizabeth Hospital

10.7.12 Phase 1 – Remodel and refurbish the existing north and south suite areas on the first floor of the Acute Building at Queen Elizabeth Hospital to create a new Urgent Treatment Centre. This will enhance the efficiency of patient pathways within the Emergency Department, enabling patients requiring urgent care to receive immediate consultation. The overall aim is to reduce Emergency Department waiting times and streamline the delivery of urgent care.

10.7.13 Phase 2 – Undertake a full back-to-brick refurbishment of the currently vacant East Lower Ground A and B wards to accommodate the Therapies teams currently based in the PFI Building. The scheme also includes relocating the Therapies gym by moving Manual Handling and Theatres Training. These works will free up space in the PFI Building adjacent to the Emergency Department, allowing for the provision of the new Urgent Treatment Centre.

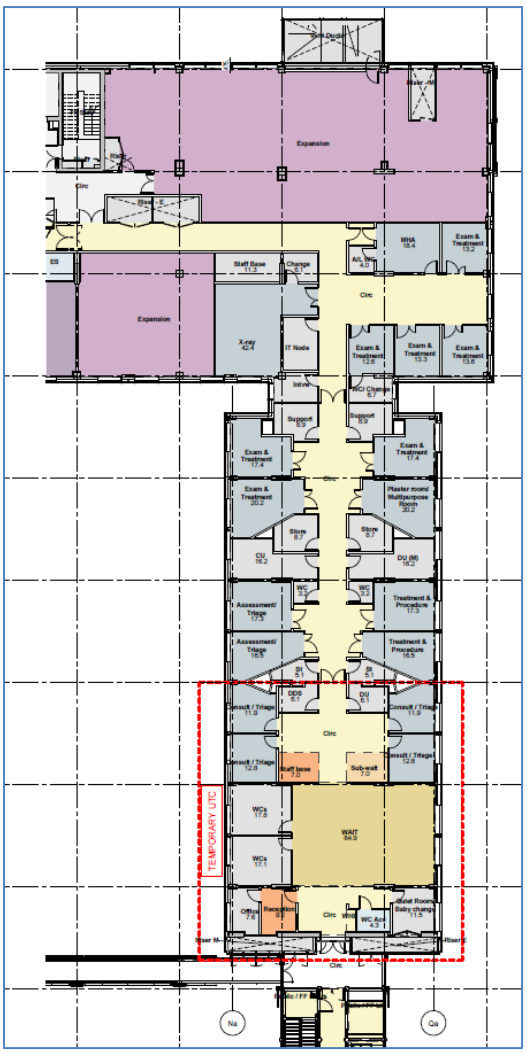
Good Hope Hospital

10.7.14 External alterations will be made to the main hospital entrance, site vehicular access, and current ambulance parking to enable installation of a new Urgent Treatment Centre at the front of the site, adjacent to Paediatric ED and ED Majors. This new unit will become the primary entrance for the hospital, providing streamlined patient pathways to other key services. The UTC will include new clinical spaces for treatment, examination, and consultation, along with all associated clinical and support areas. The modular building will replace the existing main entrance, which will be reconfigured to create a new waiting area.



Midland Metropolitan University Hospital (MMUH)

10.7.15 The UTC service will be provided from the main MMUH by converting existing space to accommodate key consult / examination space.



Functional Capacity Requirements

10.7.16 From an estate perspective if the facilities are to function correctly it is imperative that or all UTC facilities have sufficient functional content to accommodate the anticipated patient flows in a way that respects patient and family's privacy and dignity as well as providing good access.

10.7.17 The key functional activity related capacity are consult exam rooms and carparking spaces. Both measures are driven by hourly increase in activity.

10.7.18 The activity data analysis shows that for option A+ there are activity movements that lead to either increase or decrease on the affected sites. Where the activity decreases then no further consult / exam room or car parking analysis has been undertaken. Those that increase in activity are summarised below.

Option A+

Site	Increase in daily activity	Potential increase in parking hourly activity	Capacity of car park
Heartlands Hospital	38	2	Refer to site travel plans
Mid Met	90	4	
Queen Elizabeth	11	1	
Good Hope	7	1	

Conclusions

10.7.19 The estates assessment supports the deliverability of the recommended Option A Plus model through the use of existing community estate and the development of co-located UTC facilities at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital. Activity modelling confirms that three consultation/examination rooms would provide sufficient capacity for the community UTC requirements under the assumptions assessed, with additional capacity available at all sites if required. No estates-related capital costs have been identified for community UTCs under Options A, A Plus or B.

10.7.20 The principal estates consideration for Option A Plus relates to the additional operational costs associated with extended opening hours, which will depend on the final GP Out of Hours delivery model. The estates assessment indicates that these costs are expected to fall within the range of the Option A and Option B assumptions, with Option B representing the upper-bound cost, assessed as affordable. Planning permissions and lease arrangements may also require amendment where extended operating hours are proposed. The assessment identifies the associated risk of noise-related objections as low, based on the modelled level of out-of-hours site movements.

10.7.21 The co-located UTC developments are funded through the national Return to Constitutional Standards Programme and provide the infrastructure to support the proposed acute-site model. The estates requirements will continue to be aligned with the agreed service specifications, operating arrangements and implementation plans. Overall, the estates assessment provides a basis for progressing Option A Plus, with the precise additional operational costs and any necessary planning or contractual changes to be addressed through the final GP OOH delivery arrangements and implementation planning.

10.8 Digital

10.8.1 The Birmingham and Solihull Integrated Care System Digital, Data and Technology Strategy (Nov 2022) [Appendix 13 – BsoL Digital Strategy Full Version 2022] sets out a five-year roadmap for transforming health and social care through digital innovation. Its ambition is to create a citizen-centred, digitally enabled system that improves population health, reduces inequalities, increases productivity and delivers simpler, faster and better care.

10.8.2 The proposed redesign of Urgent Treatment Centre provision supports this strategy and the wider NHS ambition to move services from analogue to digital. It will enable a more integrated

urgent and emergency care model, with improved coordination between UTCs, NHS 111, primary care and community urgent-care services.

10.8.3 Under the future model, commissioned UTC providers will be required to demonstrate the capability set out by NHS England UTC principles and Standards.

10.8.4 These principles and standards, will support clinically prioritised scheduling and triage through the NHS App and telephone services, improve information sharing across care pathways and provide patients with more accessible and coordinated care. The anticipated benefits include reduced waiting times, improved urgent and emergency care responsiveness, a better patient experience and increased efficiency.

10.8.5 Following the recent options review, Option A has been confirmed as the preferred approach for the UTC review. To provide assurance that each provider complies with the required national IT, digital and operational standards, all providers will be required to complete the UTC Self-Assessment Tool.

10.8.6 The tool contains two assessment tabs: one covering the IT and digital standards and the other covering the national operational standards. It assigns a RAG status to each requirement and provides visibility of the provider's current level of compliance. Where a requirement is not fully met, the provider must record:

- the interim arrangements currently in place;
- the actions being taken to achieve compliance; and
- the target date for fully meeting the requirement.

10.8.7 An example of the UTC Self-Assessment Tool is shown below. The full tool is available at in the following Appendix 14 - BSoL Digital and Enhanced Self-Assessment Tool.

UTC IT & Digital Standards and Enhanced Assurance — Self-Assessment

Digital Standard Theme	Item	Self-Assessment Requirement	Are you fully compliant with this standard?	If the standard isn't met, what interim solution do you have in place to meet this requirement?	What steps are being taken to implement the requirement full	Target date to fully meet the standard	RAG Status
Electronic Record Access							
Electronic Record Access	25.1	Staff can access an up-to-date electronic patient care record throughout UTC opening hours.	Not In Place				Red
Electronic Record Access	25.2	The available record includes relevant patient flags and crisis data.	Partially Implemented				Amber
Electronic Record Access	25.3	Record access follows an operational and auditable consent/best-interests process, including emergencies where the patient lacks capacity.	Fully Met & Evidenced				Green
Electronic Record Access	26.1	The patient's registered GP receives the clinical outcome of every UTC encounter through a post event message (PEM).	Please Select				
Post-Event Communication							
Post-event Communication	26.2	The local electronic patient care record is updated promptly after the UTC encounter.	Please Select				
Post-event Communication	26.3	Where clinically appropriate and known, a child's episode is communicated to the health visitor or school nurse within 2 working days.	Please Select				
		Capacity and waiting-time data is available to ICS partners for					

10.8.8 Once completed, each assessment will be reviewed by the relevant ICB Commissioning Managers. The agreed actions, owners and target dates will form the implementation plan for each UTC site and provide evidence to support formal sign-off that the provider is operating in line with the national UTC standards.

10.8.9 For the two current UTCs that will be ceasing (Summerfield UTC & Washwood Heath UTC) consideration is needed for decommissioning the sites if their IT kit has been provided by the ICB and the decommission of any network links that are currently in place.

11 QUALITY, EQUALITY, ACCESS AND HEALTH INEQUALITIES

11.1 Introduction

11.1.1 Quality, equality, access and health inequalities have been considered throughout the review and have informed the development, appraisal and refinement of the recommended service model.

11.1.2 The ICB's Quality and Equality Impact Assessment (QEIA) process has been used to assess the potential impact of the proposed changes on patient safety, clinical effectiveness, patient experience and wider system sustainability, alongside equality considerations, health inequalities and access to services.

11.1.3 The assessment considers the Public Sector Equality Duty, protected characteristics, inclusion health groups, deprivation and the Core20PLUS5 framework. It also considers practical barriers to accessing urgent care, including travel time, public transport, car ownership, parking, digital exclusion, communication needs, service familiarity and the availability of face-to-face care.

11.1.4 Three QEIAs were undertaken during the development of the review: a full QEIA of the proposed clinical model, and separate QEIAs for Options A and B to support the Pre-Consultation Business Case and public consultation. Following consultation, the QEIA has been refreshed to reflect the recommended model and the wider evidence available to support the DMBC.

11.1.5 The refreshed QEIA assessment which can be found in Appendix 15 – UTC & GP OOHs Review QEIA_OptionAPlus has been considered the consultation findings, targeted engagement with under-represented communities, scrutiny feedback, updated travel and locality analysis, clinical evidence, workforce and estates considerations and implementation planning. Particular consideration has been given to issues consistently raised through consultation, including travel and transport barriers, digital exclusion, language and communication needs, accessibility of hospital-based sites, reliance on NHS 111 and the potential impact on disadvantaged and vulnerable populations.

11.1.6 The assessment identified significant quality benefits associated with a more consistent UTC model, improved integration with urgent and emergency care pathways, strengthened clinical governance and retention of community-based urgent care provision. Consultation respondents recognised many of these potential benefits, particularly improved access to diagnostics, specialist advice and more consistent service standards. However, consultation and locality analysis also identified potential equality, access and health inequality risks associated with travel, transport, digital exclusion, communication barriers, service familiarity and the accessibility of hospital-based sites. These risks and the associated mitigating actions are discussed throughout this chapter.

11.2 Quality impact of the recommended model

11.2.1 The recommended model is expected to provide a number of quality benefits.

Standardisation of urgent care

11.2.2 The model will provide a clearer and more consistent UTC offer across Birmingham and Solihull, with UTCs operating in accordance with the relevant NHS England UTC Principles and Standards.

11.2.3 The four co-located UTCs will provide the full UTC offer alongside the acute hospital front door, supporting access to diagnostics, specialist advice and escalation pathways where required. The three community UTCs will provide locally accessible urgent care for lower-acuity presentations, with appropriate diagnostics and links to wider community services.

11.2.4 This differentiation provides a clearer relationship between community and acute urgent care and supports patients being managed in the most appropriate setting.

Integration with the acute pathway

11.2.5 Co-location of UTCs with the acute hospital sites provides an opportunity for more integrated clinical streaming and decision-making at the urgent and emergency care front door.

11.2.6 The QEIA previously identified potential benefits from improved access to on-site diagnostics, clinical expertise, streaming and step-up/step-down pathways. These benefits remain relevant to the recommended model.

11.2.7 The model is also intended to support improved flow through the urgent and emergency care system by ensuring that patients who can appropriately be treated within UTC services are identified and managed through the appropriate pathway.

Community access

11.2.8 The recommended model retains three community UTCs in Solihull, Erdington and South Birmingham, rather than moving all UTC provision to hospital sites.

11.2.9 This provides an important balance between the benefits of acute co-location and the need to retain accessible community-based urgent care.

11.2.10 In addition, locality urgent care provision at Summerfield and Washwood Heath provides an opportunity to retain and develop additional in-hours community urgent care capacity, with the precise future configuration subject to further development through wider ICS transformation work.

11.2.11 Consultation participants generally supported the principle of improving consistency and standardisation across urgent care services. Respondents particularly recognised the benefits of co-located UTCs providing access to diagnostics, specialist support and integrated clinical pathways. However, consultation responses emphasised that these quality improvements should not be achieved at the expense of maintaining convenient and accessible community-based urgent care services.

Workforce and clinical governance

11.2.12 A standardised service model provides opportunities for greater consistency in clinical governance, workforce competencies, supervision, training and clinical leadership.

11.2.13 The recommended model will require an appropriate multidisciplinary workforce and clear clinical governance arrangements across co-located and community UTCs. Workforce requirements will be translated into procurement, mobilisation and contractual requirements rather than relying solely on workforce assumptions made during the options appraisal.

11.3 Potential adverse impacts on quality

11.3.1 The refreshed QEIA recognises that the recommended model also creates potential adverse impacts which require active management.

11.3.2 These include:

- changes to the location of some urgent care services;
- potential loss of familiarity with existing services;
- increased travel requirements for some individuals and communities;
- public transport accessibility challenges for some localities;
- parking availability and affordability at hospital-based sites;
- concerns regarding navigation of NHS 111 and urgent care pathways;
- digital exclusion impacting on awareness and access;
- communication barriers affecting people whose first language is not English;
- accessibility challenges for some disabled people, including those with mobility, sensory or neurodevelopmental needs;
- potential confusion while patients and professionals adjust to the new service configuration.

11.3.3 the possibility that proximity of a UTC to an ED could result in increased precautionary escalation for some patients.

11.3.4 The previous QEIA identified these risks, including the potential for changes in service location to affect vulnerable groups and people who rely on public transport.

11.3.5 The DMBC therefore does not assume that an overall improvement in population-level access means that every individual will experience an improvement. The impact on specific communities and population groups is considered alongside the aggregate modelling.

11.3.6 Public consultation demonstrated that these concerns were most commonly expressed by older people, disabled people, carers, people with long-term conditions, people experiencing socio-economic disadvantage, people without access to private transport and individuals requiring accessible communication support. The implementation programme will therefore incorporate targeted mitigations and monitoring arrangements to ensure any emerging impacts are identified and addressed.

11.4 Equality and health inequalities

11.4.1 Health inequalities are particularly relevant to the UTC review because deprivation, transport availability, digital access, long-term health conditions and other factors can affect an individual's ability to access urgent care.

11.4.2 The previous QEIA identified substantial variation in deprivation across Birmingham and Solihull, with the greatest concentration of deprivation in East and West Birmingham and significantly lower levels in Solihull.

11.4.3 Locality analysis demonstrated significant variation in deprivation, health outcomes, transport access and population characteristics across Birmingham and Solihull. The highest levels of deprivation are concentrated within East and West Birmingham, where residents are more likely to experience barriers associated with lower car ownership, poorer health outcomes, greater prevalence of long-term conditions and increased reliance on public transport.

11.4.4 People living in more deprived communities may experience a combination of barriers, including:

- lower car ownership;
- greater reliance on public transport;
- digital exclusion;
- language and communication barriers;
- higher levels of long-term conditions and multimorbidity;
- financial constraints associated with travel and parking;
- reduced flexibility to travel further for care.

11.4.5 The recommended model therefore needs to be assessed not only on the basis of overall population access, but also according to its potential differential impact on people who experience these barriers.

11.4.6 The ICB considers that retaining community UTC provision, maintaining walk-in and face-to-face access, strengthening GP Out of Hours services, developing locality-based urgent care provision and implementing targeted communication and engagement programmes are critical components of the mitigation strategy.

11.4.7 The assessment found that these impacts are partially mitigated through the retention of three community UTCs, maintenance of walk-in and face-to-face access, multiple routes into urgent care, locality-based urgent care provision, accessible and multilingual communications, adherence to the Accessible Information Standard, targeted engagement with affected communities and ongoing monitoring of utilisation, patient experience and equality impacts.

11.4.8 The assessment found that these impacts are partially mitigated through retention of three community UTCs, maintenance of walk-in and face-to-face access, multiple routes into urgent care, locality-based urgent care provision, targeted communications and ongoing monitoring of service utilisation and patient experience.

11.4.9 Whilst residual risks remain, the QEIA did not identify evidence that the recommended model would result in unlawful discrimination, systematic worsening of health inequalities or disproportionate adverse impacts that could not reasonably be mitigated through implementation and ongoing service monitoring.

11.4.10 The ICB considers that the requirements of the Public Sector Equality Duty have been met through the completion and ongoing refresh of the QEIA, targeted engagement with affected groups, consideration of differential impacts and the identification of mitigating actions.

11.5 Impact on population groups

11.5.1 The QEIA and engagement undertaken during the programme identified particular relevance for:

- older people, including people living with dementia;
- people with disabilities;
- people with learning disabilities and autism;
- people with long-term conditions and mental health needs;

- unpaid carers;
- people without access to a car;
- people experiencing digital exclusion;
- people with communication or language needs;
- people on lower incomes;
- people experiencing homelessness or insecure accommodation;
- refugees, asylum seekers and vulnerable migrants;
- Black, Asian and minority ethnic communities and faith groups.

11.5.2 The engagement programme specifically included targeted work with these groups alongside wider public consultation activity. The public consultation generated over 1,600 responses and included dedicated engagement with under-represented communities, community organisations, staff, patients and stakeholders. This supplemented extensive pre-consultation engagement and provided additional insight into the potential impacts on protected groups and inclusion health populations.

11.5.3 Engagement and consultation consistently highlighted concerns relating to travel and transport, service familiarity, communication needs, language barriers, digital exclusion, accessibility of hospital-based sites and understanding of NHS 111 pathways. Respondents also emphasised the importance of retaining face-to-face and walk-in access, particularly for older people, disabled people, carers, vulnerable communities and individuals experiencing deprivation. These findings have directly informed the mitigating actions identified within the QEIA and the implementation requirements associated with the recommended model.

11.5.4 The QEIA concluded that the most significant potential differential impacts relate to age, disability, ethnicity and socio-economic disadvantage. Whilst these impacts are considered capable of mitigation, implementation will require continued focus on accessibility, communication, transport considerations and equality monitoring to ensure that the benefits of the recommended model are realised fairly across all communities and population groups.

11.6 Travel and accessibility

11.6.1 Travel-time modelling has been undertaken across 4,309 Output Areas, considering access by car, public transport and walking. The analysis used TRACC modelling and considered weekday and Saturday travel periods. Detailed analysis can be found in Appendix 16 – UTC Travel Time Analysis Summary_OptionAPlus

11.6.2 At a system level, the modelling indicates improved aggregate access under the recommended Option A configuration:

Access measure	Current	Option A
Car – within 15 mins	71%	81%
Public transport – within 30 mins	62%	70%
Walking – within 30 mins	22%	24%

11.6.3 However, these results should not be interpreted as demonstrating that access improves for every resident.

11.6.4 Ward-level analysis identified a number of areas where travel times may increase. No ward experiences an increase of more than 15 minutes by car or public transport, while increases of more than five minutes are concentrated in a smaller number of wards, predominantly around the edges of the geography.

11.6.5 The ICB has therefore considered the quantitative modelling alongside:

- deprivation;
- car ownership;
- public transport reliance;
- disability and mobility;
- digital exclusion;
- consultation feedback;
- lived experience;
- parking;
- service familiarity; and
- the availability of face-to-face care.

11.6.6 This is important because travel-time modelling cannot fully capture the practical experience of making a journey, including affordability, interchange, mobility, caring responsibilities, confidence navigating unfamiliar locations or the availability of transport at the time care is required.

11.6.7 Travel and accessibility analysis was reviewed alongside deprivation data and population characteristics to consider the potential impact on communities experiencing the greatest health inequalities. Although some localised increases in travel times were identified, the analysis did not demonstrate a consistent or systematic pattern whereby the most deprived communities experienced disproportionately adverse impacts compared with the wider population. Retention of community UTC provision, together with continued development of locality-based urgent care services, was considered an important mitigation in reducing the

potential risk of widening inequalities.

11.6.8 Recognising that quantitative travel-time analysis does not capture all practical barriers to access, the ICB has commenced work with Transport for West Midlands (TfWM), supported by neighbourhood and locality leads, to explore practical measures to improve public transport access to future UTC and locality urgent care provision.

11.6.9 This work is considering both immediate and longer-term measures, including travel support for patients attending NHS appointments, improved public transport information and journey planning, and opportunities to better understand the interaction between healthcare locations and the existing bus and Metro network.

11.6.10 TfWM has identified potential mechanisms to support patients travelling to NHS appointments, including bus travel codes providing a period of free travel followed by discounted travel. Consideration is also being given to the use of QR codes and web-based access to travel support, alongside the ability to evaluate uptake, journeys undertaken and the experience of people receiving the support.

11.6.11 TfWM has also offered support to develop local maps showing relevant bus and Metro routes and healthcare locations. An initial test site is being explored, with the intention that learning from this work can inform the approach to other localities where appropriate.

11.6.12 Longer-term discussions are also being progressed with TfWM's planning teams to consider the relationship between proposed healthcare locations and the existing bus network, including opportunities to explore bus routes and potential routing changes where appropriate. This work will continue alongside locality planning and the development of the future urgent care model.

11.6.13 The ICB will continue to work with TfWM, neighbourhood and locality leads to identify and develop practical transport mitigations as implementation progresses. The effectiveness of these measures will be considered through appropriate monitoring and evaluation, including patient uptake and feedback where available.

11.7 Public transport, parking and affordability

11.7.1 Public transport access is particularly important for communities with lower car ownership and for people who may be unable to drive. The ICB recognises that physical accessibility is influenced not only by journey time, but also by the availability and frequency of public transport, the affordability of travel, ease of journey planning and the ability of individuals to navigate unfamiliar locations.

11.7.2 The travel analysis demonstrates an overall improvement in population-level public transport access under Option A, but also identifies localised increases in travel times. These impacts require ongoing consideration alongside the availability and frequency of services at the times patients require urgent care.

11.7.3 The ICB is working with TfWM and locality partners to explore practical measures to support patients where public transport presents a barrier. These include potential free or discounted bus travel for NHS appointments, improved information about available bus and Metro routes, journey-planning support and longer-term consideration of the relationship between healthcare locations and the public transport network.

11.7.4 TfWM's travel support programme also provides an opportunity to evaluate the impact of travel incentives on patients. Subject to implementation arrangements, information can be

collected on the number of people accessing support, journeys undertaken and patient experience. This will help inform understanding of whether travel support is effective in addressing affordability barriers and whether similar interventions should be considered more widely.

11.7.5 Transport information and journey planning will also form part of the wider communications and patient navigation approach. This will be particularly important where patients are required to attend a different location from the service they currently use and for people who may have limited confidence navigating public transport.

11.7.6 Parking is also an important component of physical accessibility. Increased activity at co-located UTCs is expected to create additional parking demand at some acute sites. The impact of parking charges and availability has been considered because attendance at a UTC would not normally qualify for reimbursement under the NHS Healthcare Travel Costs Scheme.

11.7.7 Site-specific parking requirements will therefore need to be considered as part of implementation, including the availability and capacity of parking, accessibility for people with mobility difficulties and the potential affordability impact of parking charges. This will need to be considered alongside the relevant acute provider's existing travel and transport arrangements.

11.7.8 Taken together, the travel analysis and emerging work with TfWM provide a framework for addressing both the physical and practical barriers to accessing urgent care. The ICB recognises that transport mitigation will need to remain an active area of implementation rather than being considered complete at the point of DMBC approval.

11.8 Maintaining inclusive access

11.8.1 The recommended model will retain multiple routes into urgent care. These include:

- walk-in and face-to-face access through UTCs;
- NHS 111;
- GP Out of Hours;
- primary care routes;
- Pharmacy First and other community pathways;
- appropriate referral routes from health and care professionals.

11.8.2 This is particularly important because a greater reliance on digital or telephone access could disproportionately affect people experiencing digital exclusion, communication difficulties, cognitive impairment or other barriers.

11.8.3 Communications will therefore need to be accessible and available in appropriate formats, including translated and Easy Read information where required.

11.8.4 The consultation specifically identified the need for simple and clear explanations of “if X, go to Y”, alongside clearer information about what UTCs provide and the role of NHS 111. These findings will inform implementation communications, signage and patient navigation.

11.9 Impact of consultation and scrutiny

11.9.1 The public consultation and subsequent scrutiny engagement have provided important

evidence about how the proposed changes may be experienced by patients and communities.

11.9.2 The feedback did not identify a single universal view of the future model. Instead, there were different priorities across communities, including:

- maintaining familiar local services;
- retaining face-to-face and walk-in care;
- improving diagnostics;
- reducing reliance on digital access;
- maintaining accessible services for people without cars;
- improving communication and navigation;
- retaining local responsiveness within GP OOH;
- addressing parking and transport concerns.

11.9.3 These findings informed the refinement of the recommended model and were considered alongside the clinical, operational, workforce, financial and accessibility evidence.

11.9.4 In particular, concerns relating to local accessibility, transport, service familiarity, face-to-face access and local responsiveness influenced the development of the final proposal. The recommended model retains three community UTCs and incorporates a locality-based approach to GP Out of Hours commissioning through three geographical lots, reflecting issues raised through consultation and scrutiny whilst retaining the clinical and operational benefits of the wider UTC model.

11.9.5 The independent options review subsequently concluded that Option A should remain the recommended option, with further development of the GP Out of Hours specification recommended in response to consultation and scrutiny feedback.

11.10 Role of Quality, Equality, Access and Health Inequalities in Decision-Making

11.10.1 The quality, equality, access and health inequalities evidence formed an integral part of the options appraisal and decision-making process. This evidence was considered alongside clinical effectiveness, patient outcomes, workforce sustainability, deliverability, affordability, consultation findings and wider system objectives.

11.10.2 The assessment identified a number of potential risks relating to travel, accessibility, service familiarity, public transport, parking, digital exclusion and the potential differential impact on some communities and protected groups. These risks have been considered throughout the development of the recommended model and have informed the mitigations and implementation requirements described within this chapter.

11.10.3 Having considered the evidence in the round, the ICB concluded that no quality, equality, access or health inequality impacts were identified that, either individually or collectively, outweigh the anticipated benefits of the recommended model. The residual risks identified are considered capable of mitigation through procurement, mobilisation, service design, communications, ongoing engagement and post-implementation monitoring.

11.10.4 The ICB recognises that some equality and access impacts cannot be fully mitigated before implementation and will need to be monitored once the new model is operational. Monitoring should include, where practicable:

- UTC activity by locality and deprivation;
- ED attendance and referral patterns;
- NHS 111 dispositions;
- face-to-face versus other access routes;
- waiting times and clinical outcomes;
- patient experience;
- complaints and concerns relating to access;
- equality-related incidents or themes;
- travel and transport issues;
- parking utilisation and affordability;
- impacts on vulnerable and inclusion health groups;
- service utilisation by locality.

11.10.5 Where monitoring identifies an unexpected adverse impact, the ICB and providers should consider whether additional mitigation or service/pathway changes are required.

11.10.6 The QEIA will therefore remain a live assessment throughout mobilisation and implementation, rather than being treated as a one-off approval document.

11.11 Mitigation of identified impacts

Potential Impact	Mitigation
Increased travel for some communities	Retention of three community UTCs, locality urgent care capacity and ongoing travel impact monitoring
Public transport barriers	Active partnership with TfWM and neighbourhood/locality leads to identify practical improvements in connectivity and access
Travel affordability	Active partnership with TfWM and neighbourhood/locality leads to identify practical improvements in connectivity and access as well as ensuring acute Providers have appropriate travel and transport arrangements in place.
Difficulty navigating new locations	Development of accessible public transport maps, journey-planning information and clear patient communications
Limited public transport connectivity	Engagement with TfWM planning teams to understand the interaction between healthcare locations and the bus network and explore longer-term opportunities
Digital exclusion	Ensure transport information and travel support can also be accessed through non-digital routes, including support from clinical staff, volunteers and community organisations
Lack of evidence about effectiveness of mitigation	TfWM evaluation of travel-support uptake, journeys and service-user experience

Potential Impact	Mitigation
Language and communication barriers	Accessible, translated and culturally appropriate information
Confusion about where to access care	Clear patient navigation and "right care/right place" communications
Impact on deprived communities	Impact on deprived communities
Impact on vulnerable/inclusion health groups	Continued VCFSE engagement and targeted monitoring
Potential increased ED use	Monitoring of UTC/ED activity, streaming and referral pathways

11.12 Conclusion

11.12.1 The refreshed QEIA identifies significant potential quality benefits from the recommended model, particularly through greater consistency of UTC provision, improved integration with acute pathways, access to diagnostics and clinical expertise, strengthened governance and retention of community-based urgent care.

11.12.2 The assessment also recognises residual risks relating to changes in service location, travel, public transport, parking, service familiarity, digital exclusion and the potential differential impact on deprived and vulnerable communities.

11.12.3 Travel modelling indicates that the recommended Option A configuration improves aggregate access compared with the current model, including access by car, public transport and walking. However, the ICB recognises that aggregate improvements do not necessarily translate into improved access for every individual or community. The travel evidence has therefore been considered alongside the QEIA, consultation findings, scrutiny feedback and lived experience.

11.12.4 The ICB recognises that equitable access cannot be assessed through travel-time modelling alone. The emerging partnership with Transport for West Midlands, neighbourhood and locality leads provides an opportunity to develop practical measures addressing transport affordability, journey planning, public transport information and longer-term connectivity. These measures will be developed alongside implementation and evaluated to determine their effectiveness in supporting access to the future urgent care model.

11.12.5 Overall, the assessment does not identify evidence that the recommended model would systematically worsen access or health inequalities across Birmingham and Solihull. Whilst some individuals and communities may experience adverse impacts, these have been considered through the QEIA process and are considered capable of mitigation through the measures described within this chapter and ongoing implementation monitoring.

11.12.6 However, the ICB considers that the residual risks identified must be actively managed through service design, procurement, mobilisation, communications, transport and parking planning, continued engagement and post-implementation monitoring. The QEIA will be refreshed as implementation progresses to ensure that identified mitigations remain appropriate and that any emerging differential impacts are identified and addressed.

12 BENEFITS, RISKS & ASSURANCE

12.1 Benefits Overview

12.1.1 The recommended model is intended to deliver benefits across quality and patient safety, access and patient experience, urgent and emergency care flow, health inequalities, workforce sustainability and wider system sustainability.

12.1.2 The benefits identified in the previous business case remain relevant and have been refined following the development of the preferred model, the public consultation, Health and Adult Social Care Scrutiny (HASC), Health Overview and Scrutiny Committee (HOSC) feedback, further clinical review and development of the service specifications.

12.1.3 The previous business case identified benefits including improved quality and consistency, improved access and timeliness, more effective use of resources, improved patient-centred care, enhanced collaboration and better integration of UTCs within the wider urgent and emergency care system. Benefits were proposed to be realised through the service development plan and contractual performance arrangements.

12.1.4 The recommended model provides four co-located UTCs alongside Emergency Departments, three community UTCs and a locality-based GP Out of Hours (OOH) model. The co-located UTC service specification is intended to provide a consistent, clinically governed front-door urgent care model, with access to diagnostics, senior clinical decision-making, streaming from the Emergency Department and links to wider urgent and emergency pathways. The specification also requires UTCs to provide both booked and walk-in access and to support appropriate referrals from NHS 111 and 999.

12.1.5 The benefits framework will therefore consider both:

- service-level benefits, which can be directly measured through UTC and GP OOH contractual performance; and
- system-level benefits, where the intended outcome is influenced by multiple services and cannot reasonably be attributed to the UTC or GP OOH service alone.

12.1.6 This distinction is important when assessing the impact of the redesign. For example, changes in Emergency Department activity, ambulance conveyance, Same Day Emergency Care (SDEC) or admission rates may be influenced by wider changes in demand, primary care, NHS 111, ambulance services and other urgent care pathways.

12.2 Benefits framework

12.2.1 Benefits realisation will be embedded within procurement, mobilisation, contract management and ongoing programme governance. The benefits framework will establish:

- the intended benefit;
- the baseline position;
- the relevant measure or indicator;
- the data source;
- the frequency of reporting;

- the responsible service or system owner;
- the expected direction of travel;
- any agreed thresholds or triggers for further investigation; and
- the action to be taken where benefits are not being realised or unintended consequences emerge.

12.2.2 The final benefits framework will be developed jointly by Business Intelligence, clinical, commissioning, finance and implementation leads and will be aligned with the final UTC and GP OOH service specifications.

12.2.3 The approach will combine existing national and contractual measures with locally developed measures identified through the Clinical Review Group work and subsequent consultation and scrutiny.

12.2.4 Several elements of the benefits framework have been strengthened in response to issues raised through the public consultation and subsequent HASC/HOSC scrutiny process. In particular, respondents and stakeholders highlighted the importance of monitoring access to services, travel and transport impacts, availability of face-to-face care, patient experience, health inequalities and the practical accessibility of urgent care pathways. The proposed benefits measures have therefore been designed not only to assess service performance and clinical outcomes, but also to provide assurance that the implementation of the preferred model does not result in unintended adverse impacts for patients, communities or population groups and that identified mitigations remain effective over time.

12.2.5 The previous business case proposed that benefits would be realised through the service development plan, with key performance indicators incorporated into service specifications and monitored through NHS Standard Contract arrangements. This approach is retained but strengthened through the broader system-level measures described below.

12.3 UTC benefits and measures

12.3.1 The UTC service specification provides the contractual foundation for benefits realisation. It sets out requirements for clinical performance, access, patient safety, diagnostics, workforce, patient experience, data quality and system integration.

12.3.2 The specification requires providers to submit monthly performance information and includes both national requirements and locally defined measures. These include ECDS compliance, data quality, integrated reporting across UTC, ED, NHS 111 and community services, monthly KPI reporting and participation in clinical audit and quality review. The core UTC benefits and measures are proposed as follows:

Benefit Area	Intended Benefit	Measure/Indicator
Quality and safety	Safe, clinically effective urgent care delivered consistently across UTCs	Patient safety incidents; escalation/deterioration events; clinical audit; safeguarding compliance; clinical supervision
Timely care	Patients receive timely assessment, treatment and clinical decision-making	Initial assessment within 15 minutes; clinical decision within 90 minutes; 4-hour performance

Benefit Area	Intended Benefit	Measure/Indicator
Patient flow	Appropriate patients are streamed to UTC rather than requiring ED assessment	ED-to-UTC streaming rates; UTC activity; appropriate ED minor-acuity activity
Right place, first time	Patients receive assessment and treatment in the most appropriate setting	Discharge rates; onward referral; avoidable repeat contacts; completed episodes of care
Diagnostics	Appropriate diagnostic investigation is available without unnecessary ED attendance or onward referral	Diagnostic utilisation; access to X-ray/blood testing; referral and turnaround times
Alternative pathways	Increased use of community and primary care alternatives where clinically appropriate	Pharmacy First/community pathway referrals; signposting and redirection
NHS 111 integration	Timely and appropriate management of NHS 111 referrals	NHS 111 referral volumes; telephone clinical assessment times; booked appointment timeliness; disposition accuracy
Patient experience	Patients understand and have confidence in the service	Friends and Family Test; patient feedback; complaints and concerns
Access and equity	Services are accessible to different population groups	Activity, access and patient experience analysed by locality and, where data permits, deprivation and relevant demographic characteristics
Workforce sustainability	Safe staffing and sustainable clinical workforce	Safe staffing compliance; vacancy rates; agency/bank use; mandatory training; clinical supervision
Data quality	Reliable information supports service management and benefits evaluation	ECDS compliance; data completeness; linked system reporting

12.3.3 The service specification already identifies monthly contractual measures including the 4-hour standard, time to treatment, initial assessment, appointment allocation, walk-in waiting time, NHS 111 appointment timeliness, NHS 111 telephone assessment, re-attendance, left without being seen, senior clinical decision-maker availability, clinical supervision and patient safety indicators.

12.3.4 It also includes measures relating to reduction in ED minor attendances, NHS 111 disposition accuracy, streaming effectiveness, community pathway use, avoidable multiple contacts, patient experience, complaints, safe staffing, training, vacancies and agency/bank use.

12.3.5 These measures provide a strong basis for the contractual benefits framework and should be supplemented by the wider system measures described below.

12.4 GP Out of Hours benefits and measures

12.4.1 The recommended model responds to consultation and scrutiny feedback by refining GP OOH commissioning into three locality-based lots:

- South/Central;
- North/West; and
- Solihull/East.

12.4.2 Each lot will operate to a common overarching clinical and service specification, while allowing locality-specific arrangements for face-to-face assessment and home visiting. Access will remain through NHS 111, with a predominantly virtual model supplemented by face-to-face assessment and home visits where clinically required.

12.4.3 This refinement directly responds to consultation and HASC feedback concerning the importance of local access, face-to-face care and home visiting, and concerns that a single system-wide service could be perceived as insufficiently local.

12.4.4 The consultation feedback on GP OOH should, however, be interpreted carefully. Respondents were not directly asked to rate the GP OOH element of Option A; the consultation analysis identified relevant free-text comments. These comments were broadly supportive of improved OOH provision but highlighted the importance of face-to-face appointments, home visits, sufficient GP availability and accessible local provision.

The GP OOH benefits framework should therefore include:

Benefit Area	Intended Benefit	Measure/Indicator
Access to urgent GP care	Patients receive an appropriate response outside core GP hours	NHS 111 referrals; response times; access by locality
Local access	OOH provision is responsive to locality needs	Activity and access by locality/lot
Face-to-face care	Patients requiring physical assessment can access appropriate face-to-face care	Number/proportion of F2F contacts; waiting times; location
Home visiting	Patients unable to attend a service can receive appropriate home-based assessment	Number/proportion of home visits; response times; locality distribution
Virtual care	Appropriate patients receive timely remote assessment where clinically suitable	Virtual contacts; clinical outcomes; onward referrals
Integration with NHS 111	Patients are directed to the appropriate service first time	NHS 111 dispositions; referral appropriateness; completed episodes
Patient experience	Patients experience accessible, responsive and coordinated OOH care	Friends and Family Test/local feedback; complaints; qualitative feedback

Benefit Area	Intended Benefit	Measure/Indicator
Continuity and information sharing	Relevant information is available to support safe care across daytime and OOH services	Record sharing; post-event information; repeat contacts
Equity of access	OOH provision meets the needs of people experiencing mobility, digital, transport or other access barriers	Activity and experience by locality/deprivation and, where available, relevant equality characteristics
Workforce sustainability	Sufficient clinical capacity is maintained across the three joint localities	Staffing, vacancies, agency use, capacity and resilience

12.4.5 The detailed thresholds and contractual KPIs will be established through the GP OOH procurement specification rather than pre-emptively set out in the DMBC.

12.5 System-wide outcome measures

12.5.1 The Clinical Review Group identified that the success of the redesign should not be assessed solely through individual UTC KPIs. A broader urgent care system dashboard is therefore proposed to understand whether the redesign is contributing to the intended shift in activity and improvement in patient pathways. The proposed measures include:

- Demonstrating left shift of activity
- West Midlands Ambulance Service calls and conveyance – monitoring changes in ambulance calls and conveyance where urgent care alternatives may influence demand.
- Use of NHS 111 OOH – monitoring changes in use and disposition.
- Pharmacy First utilisation – monitoring appropriate redirection to community pharmacy.
- Same-day urgent care activity – developing a system-level view of urgent care contacts across NHS 111, UTCs, ED and SDEC, alongside relevant admission activity.
- UTC activity – monitoring hospital and community UTC activity separately.
- ED activity – monitoring changes in Emergency Department attendance, including activity potentially suitable for UTC management.

12.5.2 These measures should be reviewed at Birmingham and Solihull system level and, where data quality and sample sizes permit, at locality level.

12.5.3 Importantly, these are system outcome measures rather than direct UTC KPIs. Changes in these measures cannot automatically be attributed to the UTC redesign because urgent care demand is influenced by multiple services and wider factors.

12.6 Right place, first time and patient experience

12.6.1 A specific area of benefits realisation will be whether the future model enables patients to receive care in the right place first time and reduces unnecessary multiple contacts. Subject to data quality and development, proposed measures include:

- diagnostic coding mapped to place of care;
- patients having three or more urgent care contacts within a 72-hour period;

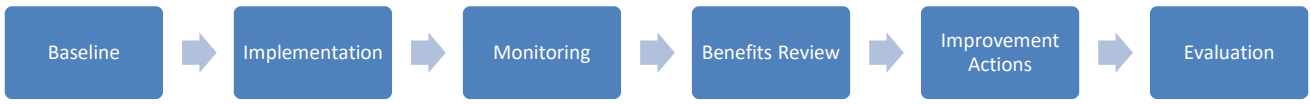
- completed episodes of care without unnecessary referral between services;
- repeat or unplanned attendance within an agreed period;
- NHS 111 disposition accuracy;
- UTC streaming effectiveness;
- onward referral and escalation rates; and
- patient experience of navigation between services.

12.6.2 The existing UTC specification already requires monitoring of re-attendance and avoidable multiple contacts and requires providers to participate in linked reporting across UTC, ED, NHS 111 and community services.

12.6.3 Further development of patient-level linkage will be required before some of these measures can be considered sufficiently robust for formal benefits attribution.

12.7 Benefits realisation and monitoring arrangements

12.7.1 Benefits realisation will commence before implementation rather than being undertaken solely after services go live. The implementation plan will establish baseline measures before each relevant service change. Benefits realisation performance will be reviewed through provider contract meetings, the programme governance structure and the relevant ICB committees. Significant concerns, unintended consequences or failure to realise anticipated benefits will be escalated through established governance arrangements. The proposed approach is:



12.7.2 During mobilisation, the ICB will establish:

- baseline performance for agreed measures;
- data sources and definitions;
- reporting responsibilities;
- reporting frequency;
- data quality requirements;
- agreed thresholds or triggers;
- escalation arrangements; and
- arrangements for qualitative patient and staff feedback.

12.7.3 The UTC service specification already requires contemporaneous quantitative and qualitative data, including patient experience, and requires locally collected information to support

continuous quality improvement. It also requires UTC capacity and waiting-time data to be made available to ICS partners for system-wide capacity management.

12.7.4 Monthly contractual reporting will therefore provide the core operational monitoring mechanism, with a broader system dashboard providing the strategic view of benefits.

12.7.5 Where performance falls below agreed standards, the relevant provider contract and governance arrangements will be used to establish the cause, identify corrective action and monitor improvement.

12.8 Monitoring access, equality and health inequalities

12.8.1 The benefits framework will include monitoring of whether the redesigned model is delivering equitable access in practice. This is particularly important because travel-time modelling demonstrates an improvement at an aggregate level but does not capture all practical barriers to access. Monitoring should therefore include, where data permits:

- activity by locality;
- activity by deprivation;
- use of community versus co-located UTCs;
- NHS 111 and other referral routes;
- face-to-face versus virtual contacts;
- home visits within GP OOH;
- patient experience;
- complaints and concerns;
- travel and transport issues;
- digital exclusion;
- language and communication needs; and
- relevant protected characteristic and Health Inclusion Group information.

12.8.2 This approach directly responds to scrutiny feedback that implementation monitoring should cover access, patient flows, equality impacts and patient experience, with clear arrangements for corrective action where unintended consequences emerge. Scrutiny bodies also requested continued communication and updates during implementation.

12.9 Consultation and scrutiny feedback informing implementation

12.9.1 The consultation and subsequent HASC/HOSC engagement have informed not only the recommended configuration but also the proposed approach to implementation and evaluation. The consultation identified particular importance attached to:

- maintaining accessible face-to-face care;
- retaining walk-in access;
- ensuring sufficient clinical workforce;
- access to diagnostics;

- home visiting through OOH;
- transport and parking;
- avoiding excessive reliance on digital routes; and
- clear communication about how patients should access services.

12.9.2 The scrutiny feedback subsequently reinforced the need for the DMBC to demonstrate how feedback would influence mitigation, implementation and post-implementation monitoring, rather than simply recording the feedback as part of the consultation process. Scrutiny specifically identified the need for:

- clear benefits and outcome measures;
- baseline data and reporting arrangements;
- monitoring of patient flows;
- monitoring of access and equality impacts;
- agreed triggers for corrective action where unintended consequences emerge; and
- continued reporting to scrutiny bodies on implementation.

12.9.3 The ICB will therefore maintain engagement with relevant scrutiny bodies, providers, staff, patients and the public during implementation and benefits evaluation. The final recommendation is based on the totality of clinical, operational, financial, equality, access, consultation and scrutiny evidence.

12.10 Post-implementation evaluation

12.10.1 A formal post-implementation evaluation will be undertaken to determine whether the intended benefits have been achieved and whether any unintended consequences have emerged. This is consistent with the recommendation of the West Midlands Clinical Senate that formal post-implementation evaluation should be undertaken to assess delivery against objectives and inform future service improvement. The previous pre-consultation business case also identified post-implementation evaluation as an important part of benefits realisation. The evaluation should consider:

- Clinical quality and safety
- Patient experience
- Access and health inequalities
- UTC and GP OOH activity
- ED streaming and activity
- NHS 111 and ambulance pathways
- Same-day urgent care and admission activity
- Use of diagnostics
- Repeat contacts and completed episodes of care
- Workforce sustainability
- Financial performance

- Patient and staff qualitative experience

12.10.2 The timing of formal evaluation should reflect the implementation timetable and allow sufficient time for services to stabilise before conclusions are drawn. Early post-implementation monitoring will nevertheless be required to identify immediate safety, access or capacity concerns.

12.11 Benefits Realisation Conclusion

12.11.1 The recommended model provides a clear basis for benefits realisation through a combination of contractual service KPIs, system-wide outcome measures, patient experience measures and formal post-implementation evaluation.

12.11.2 The UTC service specification already provides a comprehensive set of monthly contractual measures covering clinical timeliness, patient safety, streaming, NHS 111 integration, patient experience, workforce and data quality.

12.11.3 The GP OOH service will have a corresponding benefits and performance framework developed through procurement, reflecting its locality-based model and the importance of virtual, face-to-face and home-visit provision.

12.11.4 The broader system dashboard will allow the ICB to assess whether the redesign is contributing to the intended left shift of urgent care activity, improved patient flow, right-place-first-time care, reduced inappropriate ED use, improved access and reduced inequalities. However, system-level measures will be interpreted cautiously because these outcomes are influenced by the wider urgent and emergency care system and cannot necessarily be attributed to UTCs or GP OOH services alone.

12.11.5 The ICB will establish baseline measures before implementation, monitor performance through contractual and programme governance arrangements, and undertake formal post-implementation evaluation. This will provide a mechanism not only to demonstrate whether expected benefits have been realised, but also to identify and respond to unintended consequences.

12.11.6 This approach responds directly to the feedback from HASC/HOSC that the DMBC should demonstrate clear benefits, measurable outcomes, baseline data, ongoing monitoring and defined mechanisms for corrective action, with continued engagement with scrutiny bodies following implementation.

12.12 Risks and Mitigations

12.12.1 A detailed risk register has been established for the programme and included as Appendix 17 – BSoL UTC GP OOHs Risk Register_v7. While a number of unmitigated risks result in a red rating for likelihood and impact, there are no red risks remain after mitigation. Mitigating actions are monitored as part of programme governance arrangements.

12.12.2 Implementation of the recommended UTC model presents a range of risks relating to estates and capital works, financial affordability, procurement, workforce sustainability, clinical pathways, digital infrastructure and service transition. These risks have been recorded within the programme risk register, which identifies the potential consequences, responsible risk owners, existing controls and further actions required.

12.12.3 The principal risks relate to the timely delivery of the required estate developments, availability of capital and revenue funding, procurement capacity and timescales, workforce

availability, and the establishment of the clinical and digital arrangements needed to support the future model. There are also site-specific dependencies relating to planning permissions and agreement with landlords for any extended operating hours.

12.12.4 Estates and capital works: Delays to the design, approvals or delivery of the co-located UTC capital developments could affect site readiness and the timing of anticipated benefits. The co-located UTC developments are funded through the national Return to Constitutional Standards Programme. For community UTCs, the estates assessment identifies no additional estates-related capital costs under Options A, A Plus or B, and activity modelling indicates that three consultation/examination rooms would provide sufficient capacity, with additional capacity available at all sites if required.

12.12.5 The principal estates-related dependency for Option A Plus is the potential for additional operational costs associated with extended opening hours, which will depend on the final GP Out of Hours (OOH) delivery model. Planning permissions and lease arrangements may also need to be amended where extended operating hours are proposed. Although the estates assessment considers the risk of noise-related planning objections to be low based on the modelled level of out-of-hours site movements, the relevant planning and contractual requirements will need to be addressed where applicable.

12.12.6 Mitigations include continued engagement with the relevant estates partners and acute Trusts, alignment of site-level implementation plans with the agreed service specifications and operating hours, and confirmation of any necessary planning or landlord approvals. The financial implications of extended opening hours will be confirmed alongside the final GP OOH delivery arrangements to ensure that any additional estates operational costs are reflected in the affordability assessment.

12.12.7 Delays to construction, design, approvals or capital funding could affect the readiness of co-located UTCs and delay the anticipated delivery of benefits. Community UTC and locality service accommodation must also be suitable, affordable and capable of supporting the required service model. Mitigations include ongoing engagement with relevant estates partners, early identification of site requirements, development of activity and capacity modelling, and confirmation of funding, planning and landlord approvals. Site-specific implementation plans will be developed with the relevant acute Trusts.

12.12.8 **Financial affordability:** The availability of sufficient revenue and capital funding remains a key implementation dependency. Changes in service configuration and activity may also affect the financial flows between commissioners and providers, potentially impacting the ability to agree contracts that deliver the required service specification.

12.12.9 **Procurement and contractual arrangements:** Delays to procurement or contract award, insufficient specialist procurement resource, or challenge to the procurement process could affect delivery against the proposed implementation timeline. Mitigations include early engagement with procurement and legal specialists, development of clear service specifications and evaluation criteria, alignment of procurement activity with existing contract timelines, and consideration of transitional arrangements where required. The final procurement strategy, including whether separate procurement approaches are required for Community UTC and GP Out of Hours services, will need to be confirmed.

12.12.10 **Workforce sustainability:** There is a risk that sufficient staff with the required skills and experience will not be available to deliver the future UTC, locality urgent care and GP Out of Hours arrangements. This could compromise service delivery and patient experience. Mitigations include workforce planning, provider engagement, recruitment and retention plans, appropriate multidisciplinary skill mix, training and competency requirements, and

structured mobilisation and transition arrangements. Existing services will continue to be monitored during the review and implementation period to identify and address emerging concerns.

12.12.11 **Clinical pathways and digital infrastructure:** Inconsistent adoption of agreed clinical pathways, variation in the treatment of minor injuries, or delays in implementing the digital systems required for direct referrals, information sharing and access to clinical records could affect patient safety and the intended benefits of the model. Mitigations include standardised clinical protocols, training, clinical governance and audit, alongside dedicated digital leadership, identification of system dependencies and incorporation of digital requirements into service specifications and implementation plans.

12.12.12 **Communications, engagement and statutory requirements:** Effective communication and continued engagement with stakeholders will be important to support implementation, maintain confidence and ensure that consultation feedback and scrutiny recommendations are reflected in the final arrangements.

12.12.13 **Planning permissions and landlord agreements:** The risk register identifies potential barriers to securing the necessary planning approvals and contractual agreements for extended operating hours. Further work is required to obtain the relevant formal approvals and agreements for the preferred model before operating arrangements can be confirmed.

Risk monitoring and assurance

12.12.14 The programme risk register as shown in figure 12.12.14 will remain the principal record of implementation risks, controls, actions and ownership. Risk owners will be responsible for progressing agreed mitigations and providing updates through the established programme governance arrangements. Risks requiring escalation will be considered by the UTC Programme Board, with material issues affecting affordability, deliverability, patient safety or the proposed implementation timeline escalated through the appropriate ICB governance routes.

Figure 12.12.14

BSol ICB Changing UTC & GP Out of Hours provision: decision-making business case

Title	Description: Risk - Cause - Consequence	Target Score	Impact	Likelihood	Unmitigated Risk Score	Controls in place
Capital Works	There is a risk the start of operation of co-located UTCs will be delayed as a result of design and works delays including Building Safety Act and other approvals, leading to delays in delivery of patient benefits including improvement of ED performance.	3	4	4	16	Engagement with Black Country and SWBH to understand joint timelines
Estates	There is a risk that estate solutions for community UTCs, locality services and out of hours services are not fit for purpose, including having insufficient capacity, onerous operating hours restrictions, not supporting co-location with supporting services (such as CDCs) and insufficient car parking, etc. This will result in sub-optimal service location and delivery.	3	3	3	9	Early development of activity and capacity model
Capital Works and Funding	There is a risk that community UTCs, locality hubs and out of hours provision require estates works that are unaffordable resulting in estate that is sub-optimal or not fit for purpose.	3	4	2	8	Early agreement to project parameters
Capital Funding	There is a risk that insufficient capital funding is made available in a timely manner as a result of the funding applications being outside the national allocations prioritisation system, resulting in inability to proceed with capital developments for co-located UTCs	3	4	1	4	
Revenue funding	There is a risk that insufficent revenue funding is available as a result of the local system resource limitations and the constraint imposed that the project deliver within current resources, leading to inability or delay in implementing desired service options.	3	4	4	16	UEC transformation programme, working with external partners KPMG, to support overall resource asseessment and transfer
Procurement	There is a risk of delay in implementation of the agreed UTC strategy as existing contracts run their course, resulting in delays to delivery of patient benefits.	2	3	4	12	A detailed implementation plan will align procurement and contract timelines, supported by early engagement with providers and commissioners. Contractual milestones and dependencies will be monitored through programme governance, with transitional arrangements considered where necessary to maintain momentum and minimise delays to delivery of patient benefits.
Procurement	There is a risk of delay in implementation of the agreed UTC strategy as a result of provider challenge to the procurement or contract award process, resulting in delays to delivery of patient benefits.	2	3	3	9	Procurement activity will be undertaken in accordance with applicable procurement legislation, NHS guidance and organisational governance requirements, supported by specialist procurement and legal advisors throughout the process. Clear service specifications, transparent evaluation criteria and robust governance arrangements will be established to ensure procurement decisions are objective, evidence-based and auditable. Market engagement activities will be undertaken to ensure providers understand the proposed model and procurement requirements prior to tender. Legal and procurement assurance will be
Workforce	There is a risk that current services will not continue to be viable as a result of uncertainty created during the strategic review process, resulting in current services being compromised leading to poorer patient experiences and outcomes.	2	3	3	9	Maintain clear and regular communication with providers, staff and stakeholders regarding review progress and anticipated timelines. Continue robust contract management and performance oversight of existing services, with regular monitoring of workforce, activity, quality and patient experience indicators. Escalate emerging concerns through programme governance arrangements and provide targeted support where required to ensure current services remain safe, effective and sustainable throughout the review and implementation period.
Workforce	There is a risk that workforce will not be available to support the new arrangements in UTCs, locality hubs and in out of hours services as a result of contracting or recruitment difficulties, leading to service delivery being compromised with associated patient disbenefit.	3	3	3	9	Workforce risks will be mitigated through comprehensive workforce planning, provider engagement, market testing and mobilisation arrangements. Service specifications will support multidisciplinary models of care and flexible deployment of staff across urgent care settings where appropriate. Recruitment and retention plans will be developed by appointed providers, supported by transition planning with incumbent services where required. Workforce capacity and vacancy metrics will be monitored through programme governance arrangements, with contingency actions implemented where workforce availability risks delivery of the preferred model. A phased implementation approach will support safe transition and minimise the risk of disruption to patient care.
Clinical	There is a risk that clinical staff will adopt desired clinical pathways including failing to direct away from UTC to self care, Pharmacy First or GP at walk-in triage point.	3	3	4	12	A comprehensive clinical engagement and change management programme will be implemented to support adoption of agreed urgent care pathways across the system. Standardised clinical protocols, triage criteria and referral guidance will be developed and agreed through clinical governance processes, supported by training and education for all relevant staff groups. Performance measures will be established to monitor pathway compliance, including rates of onward referral, self-care advice, Pharmacy First utilisation and GP redirection from UTC triage points. Routine audit, feedback mechanisms and clinical oversight will be used to identify variation in practice and implement corrective actions where required. Clinical champions will be identified within provider organisations to promote consistent application of pathways and support behavioural change.
Clinical	There is a risk that minor injuries are not appropriately or consistently treated by UTCs as a result of lack of consistent wound care offer across BSOL, resulting in activity continuing to be treated in acute EDs.	2	2	2	4	A consistent wound care and minor injury model will be developed and incorporated into service specifications. Workforce competencies, training requirements and clinical standards will be established, with activity and outcome monitoring used to support the transfer of appropriate activity from EDs to UTCs. Wound Care pathway to continued to be developed under Anchor organisations
Clinical/Digital	There is a risk in delay of implementation of IT systems to support direct booking, access to GP records and electronic prescribing, as a result of IT procurement delays, leading to patient care being compromised.	3	3	3	9	A dedicated digital workstream and specialist digital lead will oversee implementation of required systems. Digital requirements have been mapped through a Digital and IT Requirements Operational Spreadsheet, with supplier engagement, milestone tracking and escalation processes in place to manage procurement and implementation risks. Contingency arrangements will be used where required to maintain safe service delivery during transition.
Financial arrangements	There is a risk that the contracts cannot be agreed with providers as a result of the failing to deliver the assumed financial flows associated with changes in service provision and location, resulting in commissioned services not being to the specification required, with associated impact on patient safety and experience.	3	4	4	16	UEC transformation programme, working with external partners KPMG, to support overall resource asseessment and transfer
Procurement	There is a risk to the delivery of the Urgent Treatment Centre (UTC) redesign programme due to a lack of procurement specialist involvement. This may result in non-compliance with procurement regulations and increase the likelihood of legal challenge. Additionally, if the UTC redesign outcome is that a full procurement of the UTC and GP OOH pathways are necessary, there is a current lack of procurement process knowledge and necessary resource to ensure a smooth tender process of this size.	2	4	4	16	Procurement, legal and commissioning specialists will be engaged throughout the programme to ensure compliance with procurement regulations and governance requirements. If a full procurement is required, a dedicated procurement workstream and specialist resource will be established to develop and deliver the procurement strategy, service specifications, market engagement, evaluation process and mobilisation arrangements. Risks relating to procurement capacity, capability and timescales will be monitored through programme governance, with additional specialist support secured where necessary to minimise the risk of legal challenge, delays or unsuccessful procurement outcomes.
Digital	A key risk for the UTC Redesign Programme is the lack of a dedicated digital specialist lead to oversee the new UTC standards and principles to align them with existing or required clinical systems and interoperability. UTCs operate across primary, community and secondary care, all of which use different clinical systems that do not naturally work together. Without specialist input, there is a risk that digital requirements are under-specified and that the redesigned model do not fully account for the requirements of delivery in practice. This is particularly significant for direct referrals and access to diagnostics, which often sit within acute systems and require clear digital pathways to function effectively. There is also a risk that the digital impacts of decommissioning or reconfiguring existing UTC provision are not fully understood, leading to disrupted referral routes or information gaps during transition, including cost implications of contracts currently in term. Specialist digital leadership is therefore needed to articulate these risks clearly within the business case and to provide assurance that the new UTC model can operate safely and consistently across the system.	3	4	2	8	A dedicated digital lead has been allocated to the programme and a Digital and IT Requirements Operational Spreadsheet has been developed to identify system dependencies, interoperability requirements, referral pathways, diagnostic access requirements and contractual implications. Digital requirements will be incorporated into service specifications, procurement documentation and implementation plans. Ongoing engagement with providers and system partners, supported by programme governance arrangements, will ensure digital impacts are identified and managed throughout mobilisation, reducing the risk of disruption to clinical pathways, information sharing and service delivery.
Estates	There is a risk that variation to planning permissions for extended hours of operation cannot be secured	3	3	2	6	Facilities currently have planning permission for out of normal working hours (proposals extend these hours potentially to 24hr operation).
Estates	There is a risk that extended hours of operation cannot be contractually agreed with landlord	3	3	2	6	By negotiation extended hours of use will need to be agreed with landlord. Increased use with reduced operation many lead to increased cost of use and service. Initial discussion identifies these are NHS facilities and the changes are in line with NHS strategy, no barriers identified
Clinical	There is a risk that in the change to anchor organisations there is a delay in the articulation of the specialist locality urgent care offer covering the west and east of the City.	4	4	3	12	Clear delivery responsibility for the East and West locality urgent care offers will be assigned to the relevant anchor organisations, supported by defined milestones, programme governance and regular progress monitoring. Clinical and operational stakeholders will be engaged in service design, with escalation routes in place to address delays. A phased implementation approach will enable delivery of the core UTC model whilst locality-specific service development continues.
Digital	There is a risk that the transition of the GPIT provider contract to BCHC will not include ongoing IT support arrangements for Urgent Treatment Centres (UTCs) and other non-GP services co-located within affected sites. This may result in a lack of IT infrastructure support, system access, connectivity, clinical system functionality, and operational resilience for UTC services at sites such as South Birmingham UTC (Katie Road) and Erdington UTC (Stockland Green PCC). The risk could also adversely impact the decommissioning, mobilisation, or transition of UTC services as part of the UTC Strategic Review Programme, leading to service disruption, delays, additional costs, or patient safety concerns.	6	4	4	16	Engage with relevant digital and commissioning stakeholders to confirm future GPIT support arrangements for UTC services, assess IT requirements and dependencies across affected sites, agree ownership and funding responsibilities, and ensure digital infrastructure requirements are incorporated into transition plans. Escalate unresolved issues through appropriate governance routes and develop contingency arrangements to maintain service continuity during any transition period.

12.12.15 The risk register will be reviewed and updated as the procurement strategy, site-specific implementation requirements and mobilisation plans are developed. This will include confirming current risk scores, residual risk following mitigation, action owners and target dates, and the evidence required to demonstrate that controls are effective.

12.13 Assurance Undertaken

12.13.1 The programme has been subject to a range of clinical, strategic, consultation, equality and options appraisal assurance.

West Midlands Clinical Senate

12.13.2 The West Midlands Clinical Senate undertook an independent clinical review of the UTC programme in April 2025, with its final report published in July 2025. The review supported the need for change and emphasised the importance of bringing UTC provision up to national standards, strengthening co-located UTC provision and ensuring that implementation was sufficiently developed to deliver safe and effective services.

12.13.3 The Clinical Senate also recommended formal post-implementation evaluation to assess whether the intended objectives were achieved and to inform future service improvement.

NHS England Assurance

12.13.4 The programme has been subject to NHS England service change assurance throughout its development, including the Stage 1 Strategic Sense Check on 20 November 2024 and Stage 2 Assurance, completed on 20 January 2026. The assurance process considered the relevant service change requirements, including public and patient engagement, patient choice, clinical evidence, commissioner support and capacity implications.

12.13.5 Following Stage 2, the Pre-consultation business case was amended to reflect NHS England feedback, with subsequent consultation and scrutiny informing the development of the DMBC and recommended model.

12.13.6 The programme subsequently progressed through public consultation and scrutiny, with the findings informing the Decision-making business case and recommended option for implementation.

Independent Options Review

12.13.7 Following consultation and scrutiny, Olovus undertook an independent options review workshop on 24 September 2026. The review considered Options A and B and the potential for a hybrid approach, including consultation and HASC/HOSC feedback.

12.13.8 The existing weighted appraisal scores were reviewed rather than undertaking a new options appraisal. A Commissioner only Programme Board subsequently confirmed that the original scores remained appropriate. The independent review supported progression of Option A, subject to further development of the GP OOH specification.

Consultation and Scrutiny Assurance

12.13.9 The public consultation was independently analysed and provided evidence on patient and public views, including access, quality, travel, face-to-face care, NHS 111, GP OOH and local service provision.

12.13.10 The ICB has subsequently considered HASC/HOSC feedback as part of the post-

consultation options review and has incorporated relevant findings into the preferred model, including the locality-based GP OOH procurement approach.

Quality, Equality and Access Assurance

12.13.11 The decision has also been informed by the QEIA, travel-time analysis, health inequalities analysis, activity and capacity modelling, workforce analysis, estates assessment and financial modelling. The combined assurance approach is intended to ensure that the final decision is not based on a single evidence source and that clinical quality, patient safety, access, equality, affordability and deliverability are considered together.

Decision-Making Assurance

12.13.12 The Decision-Making Business Case has been developed in accordance with NHS England service change requirements and has considered the outputs of the consultation, scrutiny review, options appraisal, equality assessment, clinical review, finance, workforce, estates and activity modelling. The recommendation reflects the totality of evidence rather than any individual evidence source.

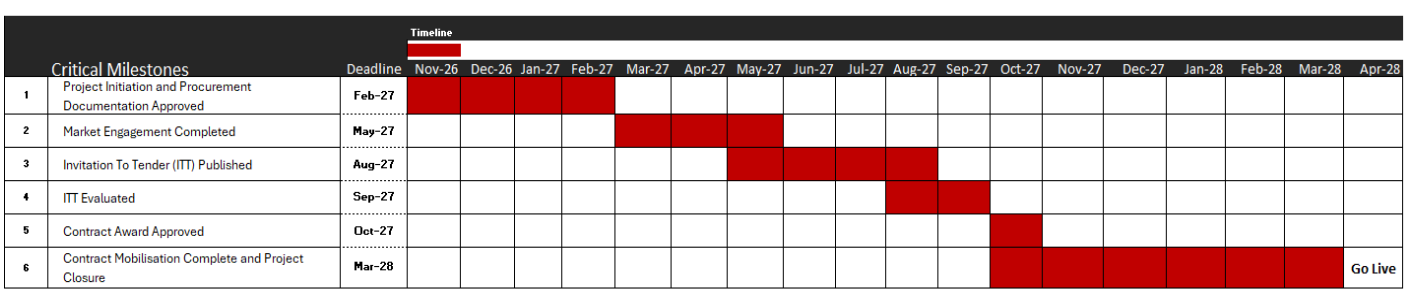
13 IMPLEMENTATION PLANNING

13.1 Implementation approach and timeline

13.1.1 The implementation approach sets out an indicative procurement, mobilisation and transition programme to support delivery of the recommended model. Subject to approval of the preferred option, detailed implementation planning will be undertaken with key ICB stakeholders, procurement and contracting colleagues, providers and other relevant partners to confirm the procurement approach, final scope, dependencies and associated timescales.

13.1.2 The current programme anticipates procurement activity commencing in November 2026, with procurement documentation developed and approved by February 2027, market engagement completed by May 2027, and publication of the Invitation to Tender (ITT) anticipated by August 2027. Evaluation of bids is expected to conclude by September 2027, with contract award anticipated in October 2027. This would be followed by mobilisation and transition ahead of a proposed go-live date of 1 April 2028.

13.1.3 **Figure 13.1 UTC and GP Out of Hours Procurement – Critical Milestones**



13.1.4 The timeline shown above is indicative and is intended to provide a planning baseline for the programme. It will be subject to review and validation with the procurement lead and key stakeholders once the preferred option has been approved and the procurement strategy is confirmed.

13.1.5 The current programme assumes a period of approximately six months for mobilisation following contract award. The mobilisation period will provide time for successful providers

to establish the required workforce, estates, digital and information arrangements, clinical pathways, governance, operational processes and other requirements necessary to ensure safe and effective service commencement.

13.2 Procurement considerations

13.2.1 The current implementation plan presents the UTC and GP Out of Hours procurement activity as a single programme for planning purposes. However, separate procurement approaches may ultimately be required for Community UTC services and GP Out of Hours services, subject to the final procurement strategy and specialist procurement advice.

13.2.2 The final procurement route, lotting arrangements, contractual requirements and associated timescales will therefore be confirmed through the procurement planning process. The programme timeline will be updated accordingly once the procurement approach and procurement lead arrangements are established.

13.2.3 Some activities within the detailed implementation plan Appendix 18 – UTC & GP OOHs Review Implementation GANT Chart are currently included as placeholders and will require further confirmation. These include, for example, potential TUPE requirements, contract extensions and associated approval processes. The need for, sequencing of and responsibility for these activities will be established as part of detailed implementation planning**.**

13.3 Contracting approach and procurement routes

13.3.1 The contracting approach for the recommended model will differ between co-located UTCs, Community UTCs and GP Out of Hours (OOH) services, reflecting the existing contractual arrangements and the proposed future service configuration.

Co-located UTCs

13.3.2 For the four proposed co-located Urgent Treatment Centres (UTCs) at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital, the Integrated Care Board (ICB) intends to commission services through the relevant host acute Trust.

13.3.3 The commissioning approach for each co-located UTC will be considered individually, taking account of the existing contractual arrangements in place with each acute Trust and the requirements of the Provider Selection Regime.

13.3.4 Subject to the outcome of the decision-making process and the necessary approvals, the ICB intends to establish the co-located UTC services through amendments to, or other appropriate arrangements within, the existing contractual framework with the relevant acute Trusts where this is considered the most appropriate commissioning route.

13.3.5 Any contractual changes required to establish the co-located UTC services will be undertaken in accordance with applicable regulatory and governance requirements, including transparency requirements where relevant.

13.3.6 The ICB will only progress commissioning arrangements following formal approval of the recommended service model, having given conscientious consideration to consultation feedback, equality impacts, and all relevant evidence.

Community UTCs and GP Out of Hours services

13.3.7 The Integrated Care Board (ICB) intends to commission Community Urgent Treatment Centre (UTC) and GP Out of Hours (OOH) services through a competitive procurement process.

13.3.8 The Community UTC procurement will reflect the recommended service configuration of three community UTCs located in Solihull, Erdington and South Birmingham/Selly Oak.

13.3.9 The GP OOH procurement will reflect the proposed delivery model across three localities: South/Central, North/West and Solihull/East. The service specification will include requirements relating to integration with NHS 111, remote clinical assessment, face-to-face appointments and home visiting services where clinically required.

13.3.10 The procurement approach, service specifications, contract arrangements and lot structure will be developed through the ICB's governance and procurement processes following approval of the recommended service model.

13.3.11 Any procurement exercise will be undertaken in accordance with the applicable regulatory, governance and transparency requirements.

13.3.12 The ICB will only commence procurement activity following formal approval of the recommended service model, having given conscientious consideration to consultation feedback, equality impacts and all relevant evidence.

13.4 Current Contractual Arrangements and Transition Approach

13.4.1 Current contractual arrangements for Urgent Treatment Centre (UTC) services across Birmingham and Solihull are in place until 31 March 2028. As such, the recommended approach does not require the routine early termination of existing UTC contracts. This reduces implementation risk, avoids disruption to patients and staff, and provides sufficient time to undertake the necessary procurement, mobilisation and service development activities.

13.4.2 Subject to approval of this DMBC, the ICB intends to commence procurement activities for the future UTC and associated urgent care services. The procurement process will be undertaken over the period leading up to the expiry of existing contracts, enabling new contractual arrangements to be in place and operational from 1 April 2028.

13.4.3 Whilst the current planning assumption is that existing UTC contracts will remain in place until their scheduled expiry on 31 March 2028, it is recognised that the phased implementation of the Option A Plus model, including the development of new co-located UTCs at acute hospital sites, may require consideration of alternative transition arrangements for some existing community UTC services. At this stage, the precise phasing, timing and operational implications of the new co-located facilities have not been finalised and may vary by site.

13.4.4 As a result, there remains the potential for notice to be served on certain existing contracts ahead of their contractual end date where this may be necessary to support earlier implementation of the approved model. Any such decisions will be considered on a site-by-site basis, taking account of service readiness, workforce implications, patient access, contractual requirements and the need to maintain continuity of care. The overarching principle will be to ensure a safe and managed transition that minimises disruption to patients and staff while supporting delivery of the future urgent care model.

13.4.5 The proposed implementation approach will allow for detailed service design, specification development, market engagement, workforce planning, estates considerations and mobilisation activities to be completed prior to service commencement. It will also provide

time to work collaboratively with providers, stakeholders and local communities to develop the enhanced locality urgent care arrangements proposed within the Option A Plus model.

13.4.6 Whilst the intention is for the new service model to become fully operational from 1st April 2028, it is recognised that implementation may require a phased approach. This may include phased mobilisation of individual service elements, transitional arrangements for specific sites, or the staged introduction of locality-based urgent care services where this is necessary to ensure safe implementation and service continuity. Any such phasing arrangements will be developed during the implementation planning phase and will be subject to appropriate governance, clinical assurance and stakeholder engagement.

13.4.7 Should implementation requirements change during the mobilisation period, the ICB will retain the ability to explore contractual variations, extensions or other transitional mechanisms where permissible and necessary to maintain continuity of service and ensure delivery of the approved model.

13.5 Mobilisation and transition

13.5.1 Following contract award, a structured mobilisation and transition programme will be required to ensure that the recommended model can be implemented safely and effectively while maintaining continuity of urgent care services.

13.5.2 Mobilisation will include, as appropriate, workforce planning and recruitment, workforce transition arrangements, estates and facilities, digital and information requirements, clinical pathways, diagnostics, equipment, governance, communications and engagement, operational policies and procedures, training and competency, data and performance reporting, and contractual readiness.

13.5.3 Particular attention will be required to ensure that any changes to existing services are appropriately managed and that transition arrangements are agreed with affected providers and staff. Where applicable, TUPE/COSOP considerations will be assessed and incorporated into the mobilisation plan.

13.5.4 Implementation will also need to take account of dependencies across the wider urgent and emergency care system, including NHS 111, Emergency Departments, primary care, GP Out of Hours, community services, diagnostics and other relevant pathways.

13.6 Implementation planning and assurance

13.6.1 Detailed implementation planning will continue following approval of the recommended model. The programme will maintain oversight of key milestones, dependencies, risks and decisions through the established programme governance arrangements.

13.6.2 A detailed implementation plan and Gantt chart, setting out the activities and dependencies across the procurement, mobilisation and transition phases, is provided at Appendix 18 - UTC & GP OOHs Review Implementation GANT Chart – Detailed Implementation Plan and Gantt Chart. This provides the supporting detail to the critical milestones presented in Figure 13.1.

13.6.3 The detailed plan should be regarded as indicative at this stage rather than a fixed delivery commitment. It will be refined as the procurement strategy is confirmed, including confirmation of the final scope, procurement route, lotting arrangements, mobilisation requirements and dependencies. Any material changes to the proposed timeline will be managed through the programme's established governance and approval arrangements.

13.7 Urgent Treatment Centre Designation

13.7.1 All proposed Urgent Treatment Centres (UTCs) will be required to complete the formal NHS England UTC designation process prior to being recognised and operating as a designated UTC. This process provides assurance that services meet the nationally mandated UTC standards, ensuring consistency in access, clinical quality, patient experience, workforce arrangements, digital interoperability and integration with wider urgent and emergency care pathways.

13.7.2 The designation process involves a detailed self-assessment and assurance review against the national specifications, with evidence submitted and formally reviewed by NHS England.

13.7.3 Where services are unable to immediately demonstrate full compliance with all elements of the specification, NHS England may agree temporary exemptions where robust mitigating actions are in place. In such circumstances, clear improvement plans and timescales are agreed, with providers expected to achieve full compliance within the designated timeframe.

13.7.4 This approach enables safe implementation of services whilst maintaining a clear trajectory towards full national compliance. The ICB will continue to work closely with providers and NHS England throughout implementation to support designation and ensure all UTCs meet the required standards as quickly as practicable.

13.8 Co-located UTC Implementation

Birmingham Heartlands Hospital	The co-located UTC at Birmingham Heartlands Hospital remains a key component of the proposed urgent care model. Progression of the scheme is currently subject to ongoing review following the identification of additional estates-related considerations, including asbestos and ground condition issues, which require further assessment and resolution before final construction and mobilisation schedules can be confirmed. Subject to completion of these enabling works and approval of the revised programme, the current expectation is for the service to become operational during Summer 2027. University Hospitals Birmingham, the ICB and wider programme partners will continue to review delivery milestones and associated risks to ensure that the implementation timetable remains achievable and aligned to the wider urgent care transformation programme. Work will continue throughout the mobilisation period to ensure the service is developed in line with national UTC standards, including workforce planning, pathway development, digital integration and preparation for formal NHS England UTC designation.
Good Hope Hospital	The co-located UTC at Good Hope Hospital is currently anticipated to become operational during July/August 2027. The implementation timeline has been influenced by planning and estates-related considerations, which have resulted in revisions to the original delivery programme. Notwithstanding these challenges, work continues between University Hospitals Birmingham and the ICB to progress the development of the service model, workforce requirements, clinical pathways and supporting infrastructure required to support delivery of the future UTC service. The planned timeframe provides an opportunity to

	ensure that the service is fully integrated within the wider urgent and emergency care system and aligns with national UTC standards from commencement. In advance of go-live, the service will be subject to the formal NHS England UTC designation process, with any outstanding compliance requirements addressed through agreed mobilisation plans and mitigating actions where appropriate. The ICB will continue to work closely with partners to maintain delivery momentum and support implementation within the revised timescales.
Queen Elizabeth Hospital	University Hospitals Birmingham planned to commence the new co-located urgent care service at Queen Elizabeth Hospital from 12 October 2026. Work has been undertaken with the ICB to assess the service against the national Urgent Treatment Centre standards and progress the formal NHS England UTC designation process. The provider has identified a number of areas requiring further development, principally relating to digital integration and functionality to support bookable appointments and national booking/referral standards. Adastra implementation is currently anticipated for August 2027, with temporary exemptions and mitigating arrangements proposed in the interim. The ICB continues to work with UHB to support earlier implementation. Other temporary arrangements have also been identified in relation to paediatric minor injury pathways and diagnostic requesting which are anticipated to be resolved by November 2026.
Midland Metropolitan Hospital	The Trust's co-located UTC is planned to open April 2027, subject to approval of the recommended model and completion of mobilisation. The proposed milestones are completion and handover of the build in February 2027, followed by service go-live in April 2027, allowing time for equipping, commissioning and operational readiness checks. Key dependencies include the ICB confirming the commissioning model and contractual arrangements; reconciliation of discrepancies between ICB and Trust activity data; and agreement of a sustainable financial model. The ICB & Trust are working together to develop a transitional plan that enables the service to mobilise without creating any adverse financial impact, while also removing unnecessary costs from the system. The workforce model, including staffing levels, skill mix and recruitment requirements, must therefore be agreed alongside the service scope, anticipated activity and funding. These dependencies will need to be resolved before the Trust can commit to the final operating model and go-live timetable, with estates completion, digital readiness and clinical pathway arrangements sequenced accordingly.

13.9 Implementation conclusion

13.9.1 The proposed implementation programme provides a planning framework for procurement, contract award, mobilisation and transition, with a proposed go-live date of 1 April 2028. The timetable provides sufficient structure to support planning while recognising that the final procurement approach and detailed implementation requirements are yet to be confirmed.

13.9.2 Successful delivery will depend on early confirmation of the procurement strategy, effective stakeholder and provider engagement, robust mobilisation planning and management of workforce, estates, digital, clinical and contractual dependencies. The detailed implementation plan will therefore be maintained and updated as the programme progresses, with appropriate governance oversight of any changes to milestones, dependencies or delivery assumptions.

14 DECISION, APPROVALS AND NEXT STEPS

14.1 Decision Requested

14.1.1 The ICB is asked to consider and approve the recommendations set out within this Decision-Making Business Case (DMBC) for the future configuration of Urgent Treatment Centres (UTCs) and GP Out of Hours (OOH) services across Birmingham and Solihull.

14.1.2 Specifically, the ICB is asked to approve Option A as amended (Option A Plus) as the preferred model for future urgent care provision, comprising:

- four co-located UTCs at Queen Elizabeth Hospital Birmingham, Birmingham Heartlands Hospital, Good Hope Hospital and Midland Metropolitan University Hospital;
- three community UTCs at Solihull, Erdington and South Birmingham/Selly Oak;
- enhanced locality urgent care arrangements at Summerfield and Washwood Heath;
- recommissioning of GP OOH services through three locality-based lots covering South/Central, North/West and Solihull/East; and
- further development of the detailed service, workforce, estates, procurement and mobilisation arrangements required to implement the model.

14.1.3 Approval is sought to progress to the next stages of procurement, detailed implementation planning and mobilisation, subject to the necessary governance, assurance and statutory requirements.

14.2 Governance and approval process

14.2.1 The DMBC will be considered through the ICB's established governance arrangements prior to the formal decision being sought from the ICB Public Board.

14.2.2 The UTC Programme Board reviewed the DMBC on 15 October 2026 and provided final commentary and recommendations to support its progression through the governance process.

14.2.3 Following consideration by the UTC Programme Board, the DMBC was submitted to the Executive Board on 26 October 2026. The Executive Board, as the ICB's internal executive management forum, reviewed the proposal and considered recommendations prior to its progression to the ICB Board. Any comments or actions arising from this review were

incorporated into the final documentation, where appropriate.

14.2.4 The final DMBC will be presented to the ICB Board in public on 9 November 2026, for formal consideration and decision on the recommended urgent care model.

14.2.5 The DMBC will also be shared with NHS England in advance of the final Board decision, to support ongoing assurance and provide visibility of the final recommendation and supporting evidence.

14.3 Actions following approval

14.3.1 Subject to approval by the ICB Public Board, the programme will progress to detailed implementation and procurement planning. This will include confirmation of the procurement strategy and route, development of procurement documentation and specifications, engagement with affected providers and staff, and further development of the mobilisation and transition arrangements.

14.3.2 For the co-located UTCs, detailed implementation planning will continue with the relevant acute Trusts to confirm site-specific requirements, dependencies, workforce, estates, clinical pathways, digital arrangements and operational readiness.

14.3.3 For GP OOH, further work will be undertaken to finalise the service specification and locality-based procurement arrangements, incorporating the feedback received through consultation and scrutiny.

14.3.4 The programme will continue to maintain appropriate governance and oversight of the implementation, including monitoring of key risks, dependencies, benefits, workforce requirements, financial implications, access and equality considerations.

14.4 Next steps

14.4.1 Following the Board decision, the key next steps will be to:

1. confirm the approved service model and communicate the decision to relevant stakeholders;
2. finalise the procurement strategy and route for UTC and GP OOH services;
3. develop and issue procurement documentation and commence the procurement process;
4. undertake detailed mobilisation and transition planning with successful providers and relevant acute Trusts;
5. develop site-specific implementation plans for the co-located UTCs;
6. continue work on workforce, estates, digital, clinical pathways and operational readiness;
7. establish the detailed benefits realisation and monitoring arrangements; and
8. maintain programme governance and assurance through implementation to proposed service commencement.

14.4.2 The current implementation plan anticipates contract award during 2027, followed by mobilisation towards a proposed go-live date of 1 April 2028. These timescales remain indicative and will be subject to confirmation through the procurement and mobilisation planning processes.

14.5 Conclusion

14.5.1 The DMBC brings together the evidence, consultation findings, scrutiny feedback, financial analysis, equality and access considerations and subsequent options review undertaken to determine the preferred future model for urgent care across Birmingham and Solihull.

14.5.2 The proposed decision is to approve Option A as amended (Option A Plus) and authorise progression to the next stage of procurement, implementation and mobilisation, subject to the appropriate governance and assurance requirements. The decision will enable the programme to move from strategic planning into detailed implementation, while retaining appropriate flexibility to refine the final service configuration and delivery arrangements as procurement and mobilisation work progresses.

15 APPENDICES

15.1 Appendix 1: CLEAN_BSOL UTC Strategy PCBC V1.6 FINAL_260125

15.2 Appendix 2: Supplementary Locality Analysis

15.3 Appendix 3: Consultation Plan

15.4 Appendix 4: Healthwatch Report

15.5 Appendix 5: Public Consultation Document

15.6 Appendix 6: Summary Public Consultation Document

15.7 Appendix 7: Easy Read Consultation Document

15.8 Appendix 8: Consultation Survey

15.9 Appendix 9: Independent Consultation Report

15.10 Appendix 10: HOSC Feedback

15.11 Appendix 11: Annex 1 Detailed Financial Modelling Option A

15.12 Appendix 12: ADEPT BSoL Estates Report

15.13 Appendix 13: BSoL Digital Strategy Full Version_29.09.2022

15.14 Appendix 14: BSoL Digital & Enhanced Self-Assessment Tool_Oct26

15.15 Appendix 15: UTC & GP OOHs Review QEIA_Option APlus_Oct26

15.16 Appendix 16: UTC Travel Time Analysis Summary_v4_Option APlus

15.17 Appendix 17: BSoL UTC & GP OOOHs Risk Register_v7_Oct2026

15.18 Appendix 18: UTC & GP OOHs Review Implementation GANT Chart

NHS Birmingham and Solihull

NHS Black Country

	Agenda No: 11	
Report to:	Joint ICB Cluster Board	
Date of Meeting:	28 September 2026 and 26 October 2026	
Title:	Population Health and Strategy Committee Assurance and Escalation Report	
Presenting Officer:	Committee Chair	
Author(s) of Report:	Peter McKenzie, Associate Director Governance and Corporate Affairs	
To be Withheld from Publication?	**Yes** ☐ **No** ☐	No. The report contains a high-level reference to a confidential item only.
Relevant Conflicts of Interest	September: no additional interests beyond the register were declared. October: existing indirect and loyalty interests relating to the confidential item were declared and managed; the relevant representatives withdrew before the Committee reached its conclusion. (September minutes, item 2; October draft minutes, item 2)	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☐	Digital Transformation	☐	Treatment to Prevention	☐

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☐
Tackle inequalities in outcomes, experience and access	☐
Enhance productivity and value for money	☐
Support the broader social and economic development of Birmingham, Black Country and Solihull	☐

ICB Focus					
Delivery	☐	**Transformation**	☐	**Transition**	☐

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☐	**Accelerating Neighbourhood Health at Pace**	☐	**Digital First**	☐
Financial Discipline/ 2% productivity	☐	**Advancing Health Equity**	☐	**Integrated Partnership**	☐

Purpose:
To provide the Joint ICB Cluster Board with assurance and escalation from the Population Health and Strategy Committee meetings held on 28 September and 26 October 2026, including material risks, actions, positive assurance and decisions.



Return to Agenda

Decision Required			
Approval	☐	**Accept as Assurance**	☒
Endorsement	☐	**Noted**	☐

Report Journey:

Population Health and Strategy Committee meetings on 28 September 2026 and 26 October 2026.

Executive Summary

The Committee has reviewed a broad programme of strategic commissioning, Place development and population health work across its September and October meetings. A key focus in September was the Neighbourhood Health Readiness Assessment, which brought together NHS England feedback, the completed self-assessment across the six Places and the resulting 6- and 18-month improvement action plan. The assessment identified relative strengths in urgent, rehabilitation and reablement capacity and Better Care Fund pooled funding, alongside lower maturity in integrated neighbourhood teams and devolved budgets, organisational ownership, governance and operational arrangements, and the consistent measurement of outcomes. The assessment is appended to this report and the committee will be receiving an initial update in January 2027 and subsequent reporting by exception.

The September meeting also approved the seven common Place principles, associated decision boundaries, the initial common scope and Terms of Reference, and a phased approach to implementing the Place governance and commissioning model. The Committee supported the development of a single cluster-wide primary care approach aligned with the Clinical Strategy and neighbourhood health. It also agreed not to progress immediately to Stage 2 of the Mental Health, Learning Disability and Autism Anchor process, instead approving a 12-month shadow arrangement supported by joint governance, defined improvement measures and continuing oversight before readiness is reassessed.

In October, the Committee considered the ten-year counterfactual demand methodology and baseline modelling and discussed further work that is required to ensure the modelling accurately describes the residual demand and financial implications. The Committee also reviewed an initial analysis of urgent and emergency care utilisation. This provided a useful starting point but requires further interpretation, neighbourhood-level analysis, patient insight, Place ownership and a clear programme of action before fuller assurance could be taken. A report providing consolidated feedback from the quality improvement events held by the Quality and Safety Committee highlighting common themes across the services reviewed including demand exceeding capacity, inequalities, variable intelligence, prevention and co-production. Improvement foundations were developing, although measurable benefits remained at an early stage.

Appendices

Appendix 1: Summary of Neighbourhood Health Readiness Assessment considered by the Committee on 28 September 2026



Return to Agenda

Recommendations & Next Steps
The Board is asked to accept the report for assurance.

Assessments:					
Impact Assessments Completed		**Yes**	**No**	**N/A**	**Details (N= explain why: Y= approval date)**
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	Demand growth, variation in urgent and emergency care, early-stage quality improvement programmes and MHLDA readiness create continuing quality and access risks. Controls include established governance, improvement plans, contract oversight and further utilisation review work
Legal and Regulatory	Place decision rights remain advisory unless authority is expressly delegated or pooled. MHLDA progression remains subject to readiness, governance and future decision processes.
Finance	The unmitigated demand model indicates material long-term cost pressure and a funding gap under the stated assumptions; values are indicative and are not contractual baselines. Place resource attribution and any delegation require validated baselines and financial guardrails.
People	Delivery depends on leadership and analytical capacity, partner engagement, primary care capability and provider readiness.
Reputational	Incomplete mitigation plans, inconsistent Place maturity or unclear future data capability could reduce confidence in the ICB cluster as a strategic commissioner.

Return to Agenda

Population Health and Strategy Committee Assurance and Escalation Report
9 November 2026

1. Introduction

This report summarises the assurance, matters for escalation, actions and formal decisions arising from the Population Health and Strategy Committee meetings held on 28 September and 26 October 2026

2. ALERT- Matters of Concern or Escalation, and gaps in Assurance (Minimal or Partial Assurance)

Item	Key Risks, Issues and Opportunities	Recommendations and Agreed Actions
System demand modelling and mitigated demand position	The ten-year counterfactual methodology and baseline modelling provided useful assurance, but the Committee did not accept full assurance on the mitigated system position. Provider work had focused mainly on existing two-to-three-year programmes, and further work was required to understand residual demand, financial implications and deliverability. Committee members highlighted that it was vital that this included robust engagement with Place, primary care, local authority and voluntary-sector stakeholders to ensure that the understanding of mitigated demand was consistent and robust as a basis for future planning.	Review provider submissions due on 30 October 2026 and consider review through ICB, Place and neighbourhood lenses with wider system engagement
MHLDA transitional anchor arrangements	Whilst both providers met the Stage 1 gateway requirements the Committee agreed not to progress immediately to Stage 2 and approved a 12-month shadow arrangement with joint governance, improvement measures and established oversight	Monitor through contract and executive oversight; establish joint governance and improvement framework; reassess readiness before the future award decision.
Analytics continuity and future data capability	The Committee recognised an increased risk whilst national hosting and data-service arrangements remained unresolved and uncertainty was beginning to impede population health development and deployment.	Revise the risk wording and continue escalation through the risk register and Board Assurance Framework.

3. ADVISE- Actions Commissioned and Progressed (For Information)

Item	Assurance Obtained	Further Recommendations and Agreed Actions
Urgent and emergency care utilisation	The initial review identified material variation and possible pathway issues, but did not establish causation. The Committee considered the work a useful starting point but required stronger operational interpretation, neighbourhood-level analysis, patient insight, Place ownership and a clear programme of action before fuller assurance could be taken.	Agree phased next steps and timescales, including support to Place Committees; document learning from positive as well as adverse variation.
Work Programme	The Committee reviewed the developing work programme and supported the intention to establish a structured programme of Commissioning Utilisation Reviews to inform strategic commissioning priorities, identify	Align the programme's scope, sequencing and timing with the national planning timetable and the work of Place and other committees.

Item	Assurance Obtained	Further Recommendations and Agreed Actions
	opportunities for improvement and ensure findings translate into clear, Place-owned actions.	

4. ASSURE- Positive assurances to provide (Reasonable or Significant Assurance)

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
Place governance	The Committee approved seven common Place principles, advisory and delegated decision boundaries, endorsed the initial scope and common Terms of Reference, and supported phased implementation. The model retains ICB statutory commissioning, payer, financial and assurance accountability unless authority is expressly delegated or pooled.	Reasonable assurance	Complete membership, chairing, secretariat, BCF schedules, resource-attribution methodology and Place baselines through the agreed phased workplan.
Neighbourhood health readiness and Place development	The September assessment reported overall neighbourhood plan readiness as EARLY STAGE, with strengths in urgent/reablement arrangements and Better Care Fund plans, but lower maturity in devolved budgets, organisational ownership, governance and consistent outcomes. The Committee noted the assessment and requested future reporting against milestones and measurable outcomes. The assessment summary is appended.	Reasonable assurance on progress – recognising that gaps remain.	Progress the 6- and 18-month action plan, confirm outstanding leads and return an initial update in January 2027, with further reporting by exception to provide assurance on delivery.
Primary care transformation strategy	The Committee supported development of a single cluster-wide primary care approach aligned to the Clinical Strategy and neighbourhood health. Work included a leadership workshop, engagement with pharmacy, optometry and dentistry, and a three-year phased framework.	Reasonable assurance on development of the strategy	Bring the refreshed cluster-wide strategy and recommendations back by the end of Quarter 3.
Quality improvement and cross-committee learning	The October consolidated report identified common themes across mental health, learning disability and autism, maternity/neonatal and children and young people services: demand exceeding capacity, inequalities, variable	Reasonable assurance	Cross-reference the Quality and Safety Committee improvement plan with relevant Population Health and Strategy Committee work and 2027/28 planning.

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
	intelligence, prevention, co-production and stronger strategic commissioning. Improvement foundations were developing, but measurable benefits remained at an early stage.		

5. ASSURE- Decisions made

Item	Summary of Key Discussion	Decision
Place governance and commissioning model	The Committee discussed the proposed Place governance and commissioning model, including common principles, the distinction between advisory and delegated decision-making, the initial scope and common Terms of Reference, and a phased implementation approach. It noted that statutory commissioning, payer, financial and assurance accountability would remain with the ICB unless authority was expressly delegated or pooled, with further work required on membership, chairing, secretariat arrangements, Better Care Fund schedules, resource attribution and Place baselines.	Approved the seven Place principles and decision boundaries; endorsed the common Place scope and Terms of Reference; supported phased implementation.
MHLDA transitional anchor arrangements	The Committee reviewed the MHLDA Anchor readiness assessment and noted that both providers had met the Stage 1 gateway requirements and demonstrated foundational capability. However, continuing readiness gaps remained, so the Committee agreed not to progress immediately to Stage 2 and supported a 12-month shadow arrangement, underpinned by joint governance, improvement measures and established contract and executive oversight. Readiness will be reassessed before any future award decision; correspondence implementing the agreed approach was noted in October.	Approved the Stage 1 gateway outcome, pause, 12-month shadow model and improvement plan.

6. Recommendations

The Board is asked to accept the report for assurance.

NHS Birmingham, Black Country and Solihull cluster

Return to Agenda

4 September 2026

Cluster at a glance

Cluster CEO	David Melbourne (former Birmingham and Solihull CEO)	**Local authorities**	6 metropolitan districts, no two-tier LGR; West Midlands Combined Authority
Cluster Chair	Danielle Oum	**Providers**	Acute: UHB, Sandwell and West Birmingham, Birmingham Women's and Children's, Royal Orthopaedic, Royal Wolverhampton, Walsall Healthcare, Dudley Group. Community / MH: BCHC, BSMHFT, Black Country Healthcare FT. Ambulance: WMAS.
Registered population	Total population 2,953,702 Black Country 1,329,540, Birmingham/Solihull 1,624,162	**Trusts in regulatory action**	4 CQC Requires improvement: Birmingham Women's and Children's, Walsall Healthcare, Dudley Group, BSMHFT. UHB and Sandwell and West Birmingham hold no current trust-level rating.
Places	6, co-terminous with 6 HWBs (Birmingham, Solihull, Dudley, Sandwell, Walsall, Wolverhampton)	**Financial position**	Cluster efficiency requirement 220 million pounds for 2026/27
Footprint	Urban and densely populated; significant deprivation and high ethnic diversity	**NNHIP / NHC Wave 1**	3 of 6 Places wave-1 NNHIP: Walsall, East Birmingham, Solihull. NHC Wave 1 status

Places

Place	Birmingham Place score - 39	Solihull Place score - 46	Dudley Place score - 55	Sandwell Place score - 50	Walsall Place score - 55	Wolverhampton Place score - 49
Population	1,385,446	238,716	340,703	374,645	309,665	304,527
HWB sign-off	29 January 2026	Not evidenced	30 June 2026	25 September 2025	19 March 2026	11 March 2026
NbH footprints / PCNs	36 neighbourhoods, 6 localities, 33 PCNs, 184 surgeries; 18 INTs = 50% coverage (visit notes)	1 of 2 Place Committees; 1 of 6 Locality Delivery Partnerships; c. 5 to 6 of the 36 BSol neighbourhoods. Same model as Birmingham	6 neighbourhoods = Dudley PCNs; 6 INTs; 11 weekly MDTs	6 town teams (Oldbury, Rowley Regis, Smethwick, Tipton, Wednesbury, West Bromwich)	7 neighbourhoods on PCN footprints, 4 localities; named clinical directors and weekly MDTs	4 INT footprints aligned to Family Hubs
HLE (M / F)	58.3 / 58.0	63.2 / 62.9	59.6 / 58.9	52.7 / 51.3	56.2 / 55.0	59.2 / 58.6
GP federation	Not evidenced; GP Collaboration Programme is the at-scale vehicle	Not evidenced; same GP Collaboration Programme as Birmingham	Dudley PCC (chair Dr Richard Evans); Black Country PCC	Sandwell PCC (chair Dr Tehmina Rahman); Black Country PCC	Walsall PCC (chair Mas Asghar); Black Country PCC	Wolverhampton PCC (chair Sarah Southall) ; Black Country PCC
MNP / integrator	UHB named as anchor (self-report)	UHB anchor, same model as Birmingham	DGFT working towards Anchor status. Lead Provider designate as agreed by Health and Care Partnership Board	Trust as Host (self-report)	WHT hosts partnership governance; anchor model DRAFT	Anchor governance marked illustrative and DRAFT

Examples of practice surfaced across Places (all system self-report; not independently verified and no rating rests on them)

- Sandwell: Harvest View, purpose-built integrated health and social care facility, 80 beds supporting step-up and step-down, enabling closure of 27 community hospital beds. Source: Sandwell deck.
- Dudley: Community Bridge hospital-to-home social prescribing, live April 2026 from Russells Hall; Q1 reports 73 referrals received, 30 volunteers trained, 10 VCS partners; jointly funded through BCF, Dudley NHS Trust Charity and Volunteer for Health Funding. Source: Dudley deck.
- Wolverhampton: Home First programme under a named board SRO covering intermediate care pathways, care coordination and urgent community response, tech-enabled care and frailty, alongside 24/7 Neighbourhood Mental Health Centres. The visit notes assess North Black Country intermediate care and single point of access as well developed and effective.
- Walsall: Feel Good Friday frailty clinic, 17 October 2025 to 10 July 2026, 124 completed appointments and 108 people supported; Home First cut Pathway 1 length of stay by 7 days with recurrent BCF staffing investment secured. Source: Walsall Together deck.
- Birmingham: The East Birmingham Children and Young People (CYP) Integrated Neighbourhood Team is designed to provide coordinated, multidisciplinary support for children and families whose needs span health, education, social care and wider community services.

Five Goals (RAG vs England, May 2026 national metric refresh; scores at Annex B)

	G1 Health outcomes	G2 GP access and experience	G3 Planned care	G4 Urgent and emergency	G5 Patient and staff
QHL	1.73	1.0	3.11	2.0	2.25
QUA	2.27	1.0	2.89	2.38	1.0

Enablers (Section 4)

Enabler	Key point (Section 4)
Data	EARLY STAGE. No cluster shared care record and no accountable CCIO. Wolverhampton PRADA in 60% of practices; Walsall DSAs progressing with DPIAs in place.
Workforce	EARLY STAGE. QUA staff experience Red (G5 1.0; all four measures 38-39/42) and confirmed Bottom at -1.77 in the July 2026 Explorer. No stabilisation plan in any source read.
Finance	EARLY STAGE. 55 million pounds recurrent left shift for the Birmingham and Black Country programme (visit notes); 220 million pound efficiency requirement 2026/27. ICB remains fundholder; delegation to Place is intent, not instrument.
Digital	EARLY STAGE. Ambient voice technology deployed at Dudley; outpatient PID commits AVT to 30 specialties at each of RWT and WHT, plus Patient Portal and NHS App.
Estates	EARLY STAGE. Hodge Hill Community Health Centre in development with services relocating from Heartlands. Harvest View operating in Sandwell. NHC opening-hours commitment (A8) not evidenced.

Framework readiness: Stage 1 (foundations) and Stage 2 (NbH Plan)

Ref	Stage 1 minimum requirement (2026/27)	Self-assessed Scoring
S1	Reduce non-elective admissions via UCR and reablement	7
S2	Reduce variation; improve access to general practice	5
S3	Neighbourhood footprints agreed	6
S4	INTs for high-priority cohorts; devolved budgets	3
S5	Neighbourhood approach to elective pathways	4
S6	18-week community waits; eliminate 52-week waits	5
S7	BCF pooled funding plans confirmed with LAs	7
S8	Primary / secondary care interface (RTC)	4
S9	Organisational ownership of planned deliverables	3
S10	Data-sharing arrangements for patient identification	5
TOTAL	(Average of total score for each place)	**49**

Stage 2 NbH Plan: seven-step readiness. Overall EARLY STAGE.		
1	National objectives via the three reform agendas	IN PROGRESS
2	Support wider local goals and public service reform	IN PROGRESS
3	Local objectives informed by JSNA	EARLY STAGE
4	Confirm final geographies	IN PROGRESS
5	Confirm organisational delivery responsibilities	EARLY STAGE
6	Confirm governance and operational arrangements	EARLY STAGE
7	Align other initiatives (Family Hubs, Pride in Place, MH hubs, employment)	IN PROGRESS

Ratings as carried in pack v9.5.0 at 2 August 2026. NOT EVIDENCED is neutral: absence of evidence, not verified poor performance. Evidence basis for each rating is at companion note v2.0; open gaps and their owners at gap document v1.0.

Forward look for next 6 months

1. Develop Strategic Commissioning approach and contract frameworks for Neighbourhood Health:
   a. With a focus on a PCN DES variation for proactive, rising risk and reactive elements of neighbourhood care,
   b. Commissioning plan for the four models of planned care for place and cluster
2. Develop your place provider partnerships and anchor organisation, tailored to each place, with local governance and accountability agreed through your locally developed framework
3. Finalise place-based Neighbourhood Health plans, considering your own local PHM data on need, learning from NNHIP and your place self-assessments and linked to BCF investment plans
4. Create a reporting framework for each facet of NbH, with baselines at place, system and cluster, to support your strategic commissioning plan
5. Develop your estates strategy (based on your clinical strategy), establishing Neighbourhood Health Centres as planned
6. Develop your digital and data capability, with shared care record expanded and data sharing and reporting arrangements strengthened to support evaluation
7. Create a mechanism to support the spread of learning, best practice and evidence, including public board reporting

Forward look for 18 months

1. Formalise 24/7 ICC with a direct line for patients / carers of the adult complex care cohort
2. Co-location of teams (beyond general practice and PCNs) in the majority of neighbourhoods
3. Proactive care has been commissioned for at least 1 percent of the registered population
4. Healthy neighbourhood: local authority and VCFSE supporting individuals with targeted interventions that focus on wider determinants, e.g. youth workers, housing support, employment support
5. Delegated budget is in place to commission healthy neighbourhood partnership work addressing wider determinants
6. All four models of elective care commissioned for pressured specialties
7. A plan agreed naming which pathways are to be redesigned in line with the NHSE community waiting times action lists and checklist (March 2026) and demonstrated progress on at least two pathways to give reassurance the plan is deliverable.
8. There is a documented interface priority plan aligned to the GIRFT checklist and red tape challenge, covering medicines management, digital and estates, with the full primary care family in scope (general practice, community pharmacy, dental, optometry). All provider organisations, including community and mental health, have completed the self-assessment and use the data to target improvement
9. Delegated authority exercised by Anchor, with decisions minuted and delivery reporting established
10. 99% sign up to DSA and data sharing as appropriate in place across all health and care partners

NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

			Agenda No: 12
Report to:	ICB Cluster Board		
Date of Meeting:	9th November 2026		
Title:	Quality & Safety Committee Escalation and Assurance Report		
Presenting Officer:	Sally Roberts, Chief Clinical Quality Officer/Chief Nursing Officer, Birmingham, Black Country & Solihull ICB Cluster		
Author(s) of Report:	Sarah Quinton, Deputy Chief Nursing Officer, Birmingham, Black Country & Solihull ICB Cluster		
To be Withheld from Publication	**Yes** ☒ **No** ☐	**(If Yes) Rationale: Sensitive**	
Relevant Conflicts of Interest	None		

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☒	Digital Transformation	☒	Treatment to Prevention	☒

Related ICB Aims	**Check box**
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☐

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☒

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☐	**Digital First**	☐
Financial Discipline/ 2% productivity	☐	**Advancing Health Equity**	☒	**Integrated Partnership**	☐

Purpose:

This report highlights the evidence and assurance related to the management and monitoring of the Quality, Safety and Statutory duties of the Birmingham, Black Country and Solihull systems, considered by the joint Quality and Safety Committee at the September and October 2026 improvement event/meeting.

Decision Required			
Approval	☐	**Accept as Assurance**	☒
Endorsement	☐	**Noted**	☐



Return to Agenda

Report Journey:

This report presents the key quality, safety and statutory assurance matters considered by the Quality and Safety Committee and its supporting sub-governance groups during September and October 2026. The Committee has reviewed, challenged and assessed the evidence presented and provides its assurance opinion and recommendations to the Board.

Executive Summary

The Quality and Safety Committee reviewed quality, patient safety, safeguarding, SEND, Continuing Healthcare (CHC) and quality improvement activity across Birmingham, Black Country and Solihull during September and October 2026.

The Committee considered evidence from provider quality reports, regulatory intelligence, quality surveillance activity, statutory assurance reports and commissioned improvement programmes. The Committee is satisfied that effective governance, escalation and oversight arrangements are operating across the system to identify, manage and monitor quality and safety risks.

The Committee's overall assessment is that Good Assurance can be taken in relation to governance, safeguarding, SEND and statutory oversight arrangements. Moderate Assurance is available in relation to diagnostics recovery and elective care services, where improvement plans are established and governance arrangements are robust. Partial Assurance remains in relation to maternity services, urgent and emergency care flow, and neurodevelopmental pathways, where significant improvement activity is underway but evidence of sustained improvement and/or comprehensive assurance regarding outcomes remains incomplete.

The Committee also reviewed the outputs from system-wide Quality Improvement Events undertaken during 2026 across Mental Health, Learning Disability and Autism; Maternity and Neonatal Services; Children and Young People; and Urgent and Emergency Care. These events have informed a programme of commissioning, assurance and partnership actions designed to improve outcomes, reduce inequalities and strengthen system-wide quality governance.

No unmanaged quality or patient safety risks were identified during the reporting period. However, continued Board oversight is recommended in relation to maternity services, urgent and emergency care, neurodevelopmental pathways and diagnostics recovery, recognising the requirement for sustained improvement and continuing assurance.

There have been no changes to overall CQC ratings for acute or mental health providers during the reporting period and no provider is currently rated Inadequate overall.

Quality & Safety Assurance Dashboard

Area	Assurance Rating	Key Issue	Committee Position	Next Step
Governance & Oversight	Good	Governance and escalation arrangements operating effectively	Good assurance that oversight and escalation arrangements remain effective	Continue routine oversight
Diagnostics (UHB Endoscopy)	Moderate	Recovery trajectory remains behind plan	Robust governance with no reported harm; sustainability	Continue enhanced

			of recovery requires ongoing monitoring	assurance arrangements
Maternity Services (MMUH)	Partial	Ongoing enhanced national oversight	Improvement evident; assurance remains dependent on sustained delivery and outcomes	Continue monitoring improvement milestones
Elective Care (RWT Ophthalmology)	Moderate	Glaucoma follow-up backlog	Recovery plan progressing and community pathways under development	Continue delivery oversight
Urgent & Emergency Care	Partial	Ambulance handover delays and patient flow pressures	Improvement activity established; assurance constrained by gaps in provider-level intelligence	Enhanced quality assurance and system improvement actions
Safeguarding	Good	Statutory safeguarding responsibilities	Controls and governance arrangements operating effectively	Routine monitoring
SEND	Good	Delivery of SEND reforms	Progress evident and governance arrangements functioning effectively	Ongoing monitoring
Neurodevelopmental Pathways	Partial	Extended waiting times and demand pressures	Improvement programme underway but outcomes not yet realised	Continued MDT oversight

Overall Assurance Opinion

Good Assurance
The Committee is assured that governance, safeguarding, SEND and statutory quality oversight arrangements are operating effectively, with appropriate controls, reporting mechanisms and escalation processes in place.

Moderate Assurance
The Committee has moderate assurance regarding diagnostics recovery and elective care services. Governance arrangements are robust and recovery plans are being delivered; however, continued monitoring is required to demonstrate sustainable improvement.

Partial Assurance
The Committee has partial assurance regarding:

- Maternity and Neonatal Services
- Urgent and Emergency Care Flow
- Neurodevelopmental Pathways

Return to Agenda

Whilst improvement programmes and oversight arrangements are in place, assurance remains constrained by ongoing external oversight requirements, the need for sustained improvement over time or insufficient evidence of impact on patient outcomes, safety and experience.

Appendices

None

Recommendations & Next Steps

1. **Take assurance** from the Quality and Safety Committee that effective governance, escalation and oversight arrangements are operating across Birmingham, Black Country and Solihull to identify, manage and monitor quality and safety risks.
2. **Note that Partial Assurance remains** in relation to maternity services, urgent and emergency care flow and neurodevelopmental pathways, and support the continuation of enhanced oversight and improvement arrangements.
3. **Endorse the system-wide improvement priorities** arising from the 2026 Quality Improvement Events, particularly those relating to Urgent and Emergency Care, and support delivery through commissioning, assurance and partnership mechanisms.
4. **Agree that future Board reporting should focus on exception reporting** for areas rated Partial Assurance or below, including progress against agreed milestones and evidence of impact on patient safety, quality, experience and outcomes.

Assessments:

Impact Assessments Completed		**Yes**	**No**	**N/A**	**Details (N= explain why: Y= approval date)**
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	Noted within the report
Legal and Regulatory	Noted within the report
Finance	Noted within the report
People	Noted within the report
Reputational	Noted within the report



Return to Agenda

Quality & Safety Committee Escalation and Assurance Report
9th November 2026

1.0 Introduction

This report highlights the evidence and assurance relating to the management, oversight and monitoring of quality, patient safety and statutory responsibilities across Birmingham, Black Country and Solihull, considered by the Quality and Safety Committee during September and October 2026.

The report draws attention to areas where assurance remains partial, where quality risks require continued oversight, and where the Committee considers further Board attention appropriate.

2.0 ALERT- Matters of Concern or Escalation, and gaps in Assurance (Partial or Limited Assurance)

Item	Key Risks, Issues and Opportunities	Recommendations and Agreed Actions
Midland Metropolitan University Hospital (MMUH) remains subject to enhanced maternity oversight following the CQC Section 29A Warning Notice, ongoing NHS England support arrangements and inclusion within the national maternity review programme.	Significant progress has been demonstrated across culture, leadership, workforce engagement, anti-racism initiatives, governance and community engagement.	**Partial Assurance** The Committee is encouraged by progress made and notes that MOSS status has moved to Reset following a sustained period without escalation. However, assurance remains partial whilst national oversight arrangements remain in place and further evidence is required to demonstrate sustainable improvements in patient outcomes, safety and organisational culture. **Agreed Actions** • Continue monitoring through local, regional and national oversight arrangements. • Monitor implementation of NHS England diagnostic review recommendations. • Seek assurance regarding delivery of workforce actions arising from the Birthrate Plus review. • Continue oversight of quality, safety and culture indicators.
Mental Health and Learning Disability & Autism	An unannounced CQC inspection of CAMHS services at Birmingham and Solihull Mental Health NHS Foundation Trust identified concerns relating to waiting times and access to services. The Trust has submitted a response and improvement plan.	**Limited Assurance** The Committee requires further evidence regarding delivery of improvements and any impact on children and young people waiting for services. **Agreed Actions** • Maintain enhanced oversight of CAMHS recovery arrangements. • Seek assurance regarding harm reviews and risk mitigation.

		• Ensure recommendations from regulatory reviews are incorporated into a single improvement plan. • Continue system-wide oversight involving safeguarding, education, acute and community partners.
Primary Care	Following CQC action relating to a GP practice in Birmingham, caretaker arrangements remain in place while quality concerns continue to be addressed.	**Moderate Assurance** Quality and contracting teams are working jointly to oversee improvement actions. There is no evidence of unmanaged patient safety risk; however, continued monitoring remains necessary. **Agreed Actions** • Maintain weekly ICB oversight meetings. • Continue monitoring quality, access and patient safety indicators. • Escalate any emerging concerns through Quality surveillance arrangements.
Diagnostics Recovery - UHB Endoscopy	Patients continue to experience waits for endoscopy procedures that exceed national expectations, with recovery trajectories remaining behind plan.	**Moderate Assurance** The Committee is assured that governance and oversight arrangements are robust, that risks are actively managed and that no new patient harms have been reported. However, continued recovery remains necessary before higher assurance can be achieved. **Agreed Actions** • Continue NHS England Enhanced Assurance arrangements. • Monitor delivery across digital, booking, operational and clinical pathway workstreams. • Continue reporting of patient safety indicators and waiting list risk.
Urgent and Emergency Care	Ambulance handover delays continue across several providers within the Cluster, resulting in prolonged waits for some patients accessing urgent and emergency care services. UHB remains subject to NHS England Tier 1 oversight arrangements, with increasing pressures also observed in other parts of the system.	**Partial Assurance** Partial assurance can be taken from performance monitoring, escalation reporting and current governance arrangements. However, assurance remains limited by insufficient access to provider-level harm reviews, incident investigations, patient experience intelligence and real-time qualitative evidence. This limits the Committee's ability to fully assess the impact of prolonged handover delays on patient safety, patient experience and clinical outcomes.

		Agreed Actions • Undertake quality assurance visits during November 2026. • Strengthen provider reporting of harms, incidents and patient experience relating to delays. • Monitor delivery of actions arising from the September 2026 UEC Improvement Event. • Continue oversight through provider quality review arrangements.
Children's Services	Children's Services in Dudley remain subject to a significant improvement programme following the Ofsted Inadequate judgement.	Governance and partnership arrangements are established, and improvement actions are underway. However, assurance remains dependent upon evidence of sustainable improvement in quality, safeguarding and outcomes for children and young people. **Agreed Actions** • Continue multi-agency oversight arrangements. • Receive regular progress reports against improvement milestones. • Monitor outcome measures and impact on vulnerable children and families.

3.0 ADVISE - Actions Commissioned and Progressed (For Information)

Item	Assurance Obtained	Further Recommendations and Agreed Actions
University Hospitals Birmingham - Heartlands Maternity Services	**Good Assurance** Improvement actions continue to be delivered through PIOAG arrangements. Full compliance with PMRT requirements has been maintained and improvement trajectories remain positive.	**Actions** • Continue routine monitoring through PIOAG. • Review oversight arrangements following the November assessment.
Royal Wolverhampton NHS Trust Ophthalmology Recovery	**Moderate Assurance** No new patient harms have been identified. Community glaucoma pathway development continues and is expected to improve long-term sustainability.	**Actions** • Continue oversight through the Glaucoma Task and Finish Group. • Seek assurance regarding delivery of community pathway arrangements.
Neurodevelopmental Pathways	**Partial Assurance** Significant demand and waiting time pressures remain. Improvement initiatives, additional investment and pathway redesign are underway but measurable impact has yet to be realised.	**Actions** • Continue MDT oversight. • Monitor recovery trajectories and patient experience. • Strengthen contractual accountability arrangements.



4.0 ASSURE- Positive assurances to provide (Good or Significant Assurance)

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
Pharmacy, Optometry and Dental Complaints Transfer	The transfer of the complaints function back to ICB arrangements was completed safely and effectively. No operational, governance or patient safety concerns have been identified.	**Good Assurance**	**Actions** • Undertake a six-month post-implementation review.
SEND Reform Programme	All local partnerships have secured national approval of their SEND Reform Plans and governance arrangements remain effective. Whilst system pressures remain, the Committee is assured that oversight structures are functioning appropriately and delivery plans are progressing.	**Good Assurance**	**Actions** • Continue monitoring implementation and outcomes. • Receive further updates following publication of inspection outcomes.

5.0 ASSURE - Decisions made

Item	Summary of Key Discussion	Decision
Improvement Events (2026)	The Committee accepted the reports from the 2026 Quality Improvement Events.	The Committee agreed ongoing monitoring of delivery against priorities.
Urgent and Emergency Care Improvement Event	The Committee endorsed the outputs from the Urgent and Emergency Care Improvement Event.	The Committee requested routine updates regarding implementation and impact.
Special Educational Needs (SEND) Reform Plans	The Committee received assurance regarding progress against SEND Reform Plans.	The Committee agreed ongoing monitoring.
Learning from Deaths Terms of Reference	The Committee discussed the Learning from Deaths Terms of Reference.	The Committee requested amendments to the Learning from Deaths Terms of Reference prior to approval.
Weight Management Policy Programme	The Committee confirmed its role in the governance and oversight of Clinical Policies, including the Weight Management Policy Programme.	

6.0 Improvement Event Feedback

6.1 Urgent and Emergency Care Improvement Event

The Committee received a summary of the Urgent and Emergency Care Improvement Event held on 16 September 2026.

The event identified opportunities to strengthen:

- Contractual accountability and commissioning levers.
- Quality governance and assurance arrangements.
- Outcomes-focused performance measures.
- Use of patient experience and population health intelligence.
- Alignment of existing improvement programmes.
- Prevention and demand management initiatives.

A set of priorities were identified and will be discussed and agreed at a forthcoming Q&S Committee meeting.

7.0 Recommendations

The ICB Board is asked to:

- **Take assurance** from the Quality and Safety Committee that effective governance, escalation and oversight arrangements are operating across Birmingham, Black Country and Solihull to identify, manage and monitor quality and safety risks.
- **Note that Partial Assurance remains** in relation to maternity services, urgent and emergency care flow, and neurodevelopmental pathways, and support the continuation of enhanced oversight and improvement arrangements.
- **Endorse the system-wide improvement priorities** arising from the 2026 Quality Improvement Events, particularly those relating to Urgent and Emergency Care, and support delivery through commissioning, assurance and partnership mechanisms.
- **Agree that future Board reporting should focus on exception reporting** for areas rated Partial Assurance or below, including progress against agreed milestones and evidence of impact on patient safety, quality, experience and outcomes.

8.0 Overall Committee Conclusion

The Quality and Safety Committee is satisfied that robust governance, surveillance and escalation arrangements are operating across Birmingham, Black Country and Solihull. The Committee recommends that the Board takes **Good Overall Assurance** from the effectiveness of these arrangements whilst recognising that **targeted oversight remains necessary in those areas where assurance is currently assessed as Partial**.

Sally Roberts
Chief Nurse/Clinical and Quality Officer, Birmingham, Black Country and Solihull (Cluster) ICB
October 2026



NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

Agenda No: 13		
Report to:	Joint ICB Cluster Board	
Date of Meeting:	9 November 2026	
Title:	Resource Allocation and Payer Committee Assurance Report	
Presenting Officer:	Paul Taylor, Committee Chair	
Author(s) of Report:	Heather Moorhouse, Deputy Chief Finance Officer Peter McKenzie, Associate Director of Governance and Corporate Affairs Martin Stevens, Associate Director of Performance	
To be Withheld from Publication?	**Yes** ☐ **No** ☒	(If Yes) Rationale:
Relevant Conflicts of Interest	N/a	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☐	Digital Transformation	☐	Treatment to Prevention	☐

Related ICB Aims	Check box
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☒

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☐

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☒	**Digital First**	☐
Financial Discipline/ 2% productivity	☒	**Advancing Health Equity**	☒	**Integrated Partnership**	☒

Purpose:

To provide the Board with assurance from the Resource Allocation and Payer Committee meeting held on 23 October 2026, including matters requiring attention, assurance received and decisions made. The integrated commissioning assurance report and the month 6 finance summary report are appended to this report.

Decision Required			
Approval	☐	**Accept as Assurance**	☒
Endorsement	☐	**Noted**	☐



Return to Agenda

Report Journey:

The Committee met on 23 October 2026. This was a joint assurance and development session

Executive Summary

The Committee met on 23 October 2026 for a joint assurance and development session. In the assurance session the Committee reviewed the integrated commissioning assurance report, the Month 6 financial position and quarterly update on contracts and procurements for both ICBs, considered two time-limited investment proposals and reviewed the committee risk register. The development session was focused on work supporting the development of the financial elements of Anchor arrangements. The integrated commissioning assurance report is appended and forms part of the evidence supporting this report.

In relation to commissioning performance, the Committee accepted an overall judgement of qualified assurance. Diagnostics and community services were assessed as limited assurance; urgent and emergency care, elective care, cancer and mental health were assessed as partial assurance. The Committee noted that the principal performance challenges remained patient flow and long waits in urgent and emergency care, elective waiting-list growth, diagnostic recovery and variation, and waiting times within community services. The interdependencies between these areas were also recognised, particularly the impact of diagnostic capacity on elective and cancer recovery and of community capacity and timely discharge on urgent and emergency care. There were areas of positive progress, including reductions in the longest elective waits, improving cancer pathways, ambulance responsiveness and primary care activity and access. The Committee noted that recovery remained dependent on sustained improvement against agreed trajectories, clearer milestones, analysis of provider variation and stronger evidence of the impact of recovery actions.

The Committee also supported development of a broader approach to patient-experience intelligence, drawing together national information, local engagement, General Practice collaborative intelligence and VCFSE insight so that patient experience is more clearly reflected through strategy, commissioning and assurance.

On finance, the Month 6 report showed both ICBs continuing to forecast break-even for the year, with favourable year-to-date positions (Black Country underspend of £942,000 YTD and BSOL underspend £483,000 YTD). Full delivery of efficiency plans still forecast by both ICBs. The Committee noted that the Birmingham and Solihull position has slightly more risks to manage but that neither has room for complacency in taking all appropriate actions to ensure both ICBs meet their statutory financial targets for the year. Assurance was supported by retained performance contingencies, prior-year flexibility and active operational and contractual mitigations. Capital plans, running-cost controls, mental health investment standards and Better Payment Practice Code requirements remained on track. Residual risks remained in acute activity, Continuing Healthcare reviews, prescribing volatility, Right to Choose (in acute and ADHD/ASD services) and efficiency delivery, and continued active management and monthly oversight were required.

On contracting, all lead in-system NHS contracts for 2026/27 had been signed, with a small number of associate and out-of-area agreements still outstanding and subject to escalation. Preparation for 2027/28 has commenced through common cluster templates, streamlined lead-and-associate arrangements, review of contract terms and support for development of anchor contracts. The Committee received assurance on Provider Selection Regime annual reporting for 2025/26 and noted revised procedures and training and a single cluster approach to compliance would be developed. It was also noted that the team were consolidating the approach for contracting for Continuing Healthcare across the ICBs, and progressive standardisation of Contract Quality Review



Meetings and multidisciplinary contract-management processes. Activity-management arrangements were operating for elective and ADHD/ASD Right to Choose providers. Procurement capacity and accreditation arrangements remained areas requiring continued management.

The Committee considered business cases that had been reviewed by the Strategic Investment Committee that required its approval. This included investment in the BSOL Community Frailty Programme to 31 March 2027 which would be subject to formal evaluation before any recurrent commitment and in the enhancement of the BSOL Single Point of Access winter model, which was agreed in principle, subject to some further review of the proposed operating model. The Committee delegated determination of this and confirmation of the funding to the Chief Executive and Chief Finance Officer.

The development session focused on discussion on the scope of physical health anchor contract arrangements (and the associated draft financial envelopes) and the relative merits of using GP registered and resident populations to determine allocations and development of initial financial envelopes (based on current expenditure) and for in-year reporting. It was noted that further work was being undertaken on paediatric activity (within the scope of current UEC contracts in scope). The Committee concluded the work should progress on the basis of GP registrant populations noting this had best fit with the current basis for ICB allocations and how services are contracted and so would be used in Anchor and Place reporting, noting that this would not be completely in line with local authority commitments (based on residents) and may need some specific considerations for any reporting arrangements at neighbourhood level.

Appendices

Appendix 1: Integrated commissioning assurance report considered by the Committee on 23 October 2026.
Appendix 2: Month 6 Finance Summary Report

Recommendations & Next Steps

The Board is asked to note the Committee's report for assurance

Assessments:

Impact Assessments Completed		Yes	No	N/A	Details (N= explain why: Y= approval date)
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	Performance pressures in urgent care, elective care, diagnostics, cancer, mental health and community services may affect timely access, outcomes and patient experience; recovery actions remain under oversight.



Return to Agenda

Implications	Impact
Legal and Regulatory	Provider Selection Regime compliance, procurement capacity, accreditation challenges and continued operation of two statutory ICBs require ongoing governance and oversight.
Finance	The Committee received assurance that both ICBs continued to forecast break-even, subject to active management of identified risks.
People	Workforce constraints affect diagnostics, mental health, community services, procurement and corporate functions; the merger-delay risk reflects continued dual-running requirements.
Reputational	Failure to demonstrate sustained improvement, robust evaluation or compliant contracting could affect confidence among patients, partners, regulators and the public.



Return to Agenda

Resource Allocation and Payer Committee Assurance Report
9 November 2026

1. Introduction

This report summarises assurance, risks, actions and decisions from the 23 October 2026 meeting of the Resource Allocation and Payer Committee. This meeting was a joint assurance and development session The integrated commissioning assurance report is appended and forms part of the evidence supporting the performance assurance entries below.

2. ALERT- Matters of Concern or Escalation, and gaps in Assurance (Minimal or Partial Assurance)

Item	Key Risks, Issues and Opportunities	Recommendations and Agreed Actions
Commissioning performance	Overall qualified assurance was accepted. Diagnostics and community services were assessed as limited assurance. Urgent and emergency care, elective care, cancer and mental health were assessed as partial assurance. Recovery remains dependent on sustained improvement, clearer trajectories, provider variation analysis and stronger links between metrics and recovery plans.	Continue reporting against trajectories, milestones, provider variation and recovery actions. Return a clarified system-wide diagnostics demand and capacity position to a future meeting.
Merger-delay capacity risk	A new risk was endorsed at score 12 for the operational and capacity consequences of continuing to operate two statutory ICBs, including dual finance, workforce and governance requirements, due to the delay of the 2 ICBs merging	Review controls and resource decisions to reduce the risk over time; retain oversight through future risk-reporting cycles.

3. ADVISE- Actions Commissioned and Progressed (For Information)

Item	Assurance Obtained	Further Recommendations and Agreed Actions
Patient experience	During the discussion on performance reporting, the Committee supported exploring how a broader approach combining national data, local engagement, General Practice collaborative intelligence and VCFSE insight, with patient experience would help to provide a clearer thread through strategy, commissioning and assurance.	The performance team to work with the Involvement Team to consider how patient-experience intelligence could be integrated into wider performance reporting.
Anchor arrangements	The Committee discussed the scope of anchor contract arrangements as part of the development session. This included debating the management of paediatric activity within anchor contract arrangements, using GP-registered populations or resident populations to determine anchor allocations and reporting arrangements.	Further analysis to be undertaken on paediatric activity within UEC scope. Committee view was that the work should progress on the basis of GP registrant populations. Further discussions on reporting arrangements at a sub-neighbourhood level.

ASSURE- Positive assurances to provide (Reasonable or Significant Assurance)

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
Financial position	The Month 6 report showed both ICBs continuing to forecast break-even. with favourable year-to-date positions (Black Country underspend of £942,000 YTD and BSOL underspend £483,000 YTD). Full delivery of efficiency plans still forecast for both ICBs. The Committee noted that the Birmingham and Solihull position has slightly more risks to manage but that neither has room for complacency in taking all appropriate actions to ensure both ICBs meet their statutory financial targets for the year. Assurance was supported by retained performance contingencies, prior-year flexibility and active operational and contractual mitigations. Capital plans, running-cost controls, mental health investment standards and Better Payment Practice Code requirements remained on track. Residual risks remained in acute activity, Continuing Healthcare reviews, prescribing volatility, Right to Choose (in acute and ADHD/ASD services) and efficiency delivery, and continued active management and monthly oversight were required	Reasonable assurance	Maintain monthly oversight of the break-even forecast, efficiency delivery and mitigations. Continue focused management of acute variable activity, Continuing Healthcare packages, prescribing price concessions and uptake of Right to Choose activity. Escalate deterioration where mitigations or review trajectories are not delivered.
Contracting progress	All lead in-system NHS contracts for 2026/27 had been signed, with only a small number of associate and out-of-area agreements outstanding. Preparation for 2027/28 has commenced through development of common cluster templates, streamlined lead-and-associate arrangements, review of contract terms and support for development of anchor contracts. Assurance was also received on Provider Selection Regime annual summaries for 2025/26, the development of single cluster procedures and training, Continuing Healthcare contracting, and progressive standardisation of	Reasonable assurance	Continue escalation of outstanding agreements and complete preparation for the 2027/28 contracting round. Embed common Provider Selection Regime procedures, procurement policies and contract-management frameworks

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
	Contract Quality Review Meetings and multidisciplinary contract-management processes. Activity-management arrangements were operating for elective and Right to Choose providers. Procurement capacity remained constrained following the cessation of CSU support, but interim resilience was being developed through transferred staff, temporary capacity and a proposed virtual function with neighbouring ICBs. The ADHD and autism accreditation process remained paused pending simplification and pathway redesign, while patients retained access through commissioned provision and Right to Choose.		

4. ASSURE- Decisions made

Item	Summary of Key Discussion	Decision
Strategic Investment Committee	**BSOL Community Frailty Programme** The strategic direction was supported recognising the need for formal evaluation of delivery, assumptions, service use and outcomes before any recurrent commitment. **BSOL Single Point of Access winter model** Agreed in principle, subject to review of the revised operating model with delegation Chief Executive and Chief Finance Officer to confirm funding when completed.	Approved for the period to 31 March 2027 Approved in principle
Risk Update	The Committee reviewed the risk report	Approved reduction of the oversight-framework risk to 8, endorsed the opening of a new merger-delay risk at 12 and closed the superseded finance-capacity risk (as covered by the merger delay risk).
Anchor decisions	Considered the basis for opening anchor arrangements (GP registered or resident basis) for use for target allocations and also for the basis of initial contract envelopes based on current expenditure	Agreed that GP-registered populations should be used for opening target anchor allocations and also for the basis of initial contract envelopes based on current expenditure

5. Recommendations

The Board is asked to note the report for assurance.

Return to Agenda

NHS Birmingham and Solihull

NHS Black Country

Bsol and BC ICS Commissioning oversight report.

Integrated Performance Report.



NHS Black Country ICB

NHS Birmingham and Solihull ICB

Month 7 2026/27

Report Info & Methodology

Return to Agenda

Title:	NHS Oversight Framework
Report Id:	Project 001
Period:	Data up to Sep26
Data Source(s):	National Data sources
Publication date:	October 2026
Lead Analyst(s):	Nathan Atkins
Contact Email	bbcs.phianalytics.nhs.net
Last Updated:	12/10/2026

Criteria/Methodology:

Some of the metrics will take further 'work up' as they are intended to be age-standardised, based on rates per population or based on undefined sources/methodologies.

NHSE were due to publish their first suite of ICB-responsible metrics relating to the Oversight Framework towards the end of September 2026, but this is yet to be released

Data Quality of the metrics in this report is subject to ongoing development and review and will improve over time as more of the metric data is made available nationally

Report Purpose/Description:

- This report is for Finance and Performance Committee
- Reflects all available metrics within the NHS Oversight Framework for 2026/27, prioritising those that are ICB-responsible
- Is reported at Provider/Site or ICB/Cluster level

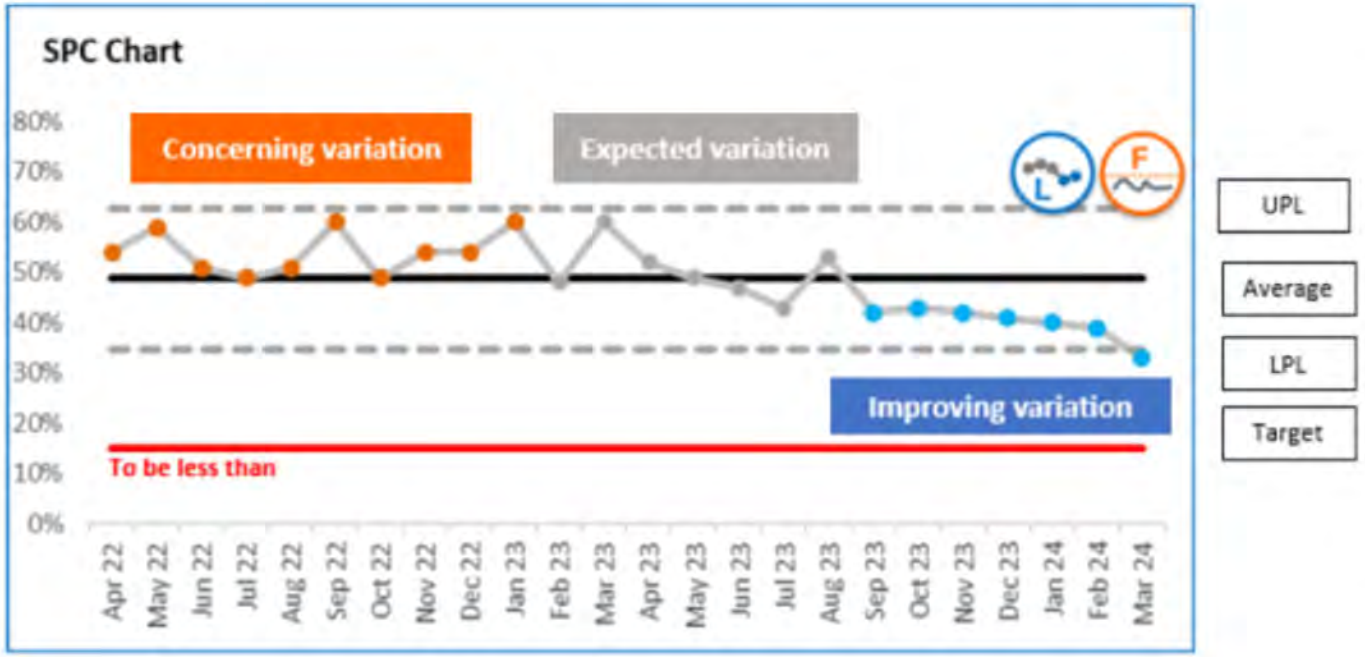
A statistical process control (SPC) chart is a useful tool to help distinguish between signals (which should be reacted to) and noise (which should not, as it is occurring randomly).

The following colour convention identifies important patterns evident within the SPC charts in this report:

Orange - there is a concerning pattern of data which needs to be investigated and improvement actions implemented

Blue - there is a pattern of improvement which should be learnt from

Grey - the pattern of variation is to be expected. The key question to be asked is whether the level of variation is acceptable



The dotted lines on SPC charts (upper and lower process limits) describe the range of variation that can be expected.

Process limits are very helpful in understanding whether a target or standard (the **red** line) can be achieved always, never (as in this example) or sometimes.

SPC charts therefore describe not only the type of variation in data, but also provide an indication of the likelihood of achieving target.

Summary icons have been developed to provide an at-a-glance view. These are described on the following page.

Interpreting summary icons

Return to Agenda

These icons provide a summary view of the important messages from SPC charts:

	Variation/Performance Icons		
Icon	***Technical Description***	***What does this mean?***	***What should we do?***
	Common cause variation, NO SIGNIFICANT CHANGE.	This system or process is **currently not changing significantly**. It shows the level of natural variation you can expect from the process or system itself.	**Consider if the level/range of variation is acceptable**. If the process limits are far apart you may want to change something to reduce the variation in performance.
	Special cause variation of an CONCERNING nature where the measure is significantly HIGHER.	**Something's going on!** Your aim is to have low numbers but you have some high numbers – something one-off, or a continued trend or shift of high numbers.	**Investigate** to find out what is happening/ happened. Is it a one off event that you can explain? Or do you need to change something?
	Special cause variation of an CONCERNING nature where the measure is significantly LOWER.	**Something's going on!** Your aim is to have high numbers but you have some low numbers - something one-off, or a continued trend or shift of low numbers.	
	Special cause variation of an IMPROVING nature where the measure is significantly HIGHER.	**Something good is happening!** Your aim is high numbers and you have some - either something one-off, or a continued trend or shift of low numbers. Well done!	Find out what is happening/ happened. **Celebrate** the improvement or success. Is there **learning** that can be shared to other areas?
	Special cause variation of an IMPROVING nature where the measure is significantly LOWER.	**Something good is happening!** Your aim is low numbers and you have some - either something one-off, or a continued trend or shift of low numbers. Well done!	

	Assurance Icons		
Icon	***Technical Description***	***What does this mean?***	***What should we do?***
	This process will not consistently HIT OR MISS the target as the target lies between the process limits.	The process limits on SPC charts indicate the normal range of numbers you can expect of your system or process. If a target lies **within** those limits then we know that the target may or may not be achieved. The closer the target line lies to the mean line the more likely it is that the target will be achieved or missed at random.	Consider whether this is acceptable and if not, you will need to change something in the system or process.
	This process is not capable and will consistently FAIL to meet the target.	The process limits on SPC charts indicate the normal range of numbers you can expect of your system or process. If a target lies **outside of those limits in the wrong direction** then you know that the target cannot be achieved.	**You need to change something in the system or process if you want to meet the target.** The natural variation in the data is telling you that you will not meet the target unless something changes.
	This process is capable and will consistently PASS the target if nothing changes.	The process limits on SPC charts indicate the normal range of numbers you can expect of your system or process. If a target lies **outside of those limits in the right direction** then you know that the target can consistently be achieved.	**Celebrate the achievement**. Understand whether this is by design (!) and consider whether the target is still appropriate; should be stretched, or whether resource can be directed elsewhere without risking the ongoing achievement of this target.

Interpreting SPC alerts

Return to Agenda

The ***4 alerts*** used to denote special cause variation in the SPC charts and summary-level tables are as follows:

Positive SPC alerts:

1. Single Point outside the control limit in the improving direction

- A single data point falls beyond the upper or lower control limit (3 sigma) and represents better performance.

2. Shift

- **7 or more consecutive points** on the improving side of the mean

3. Trend

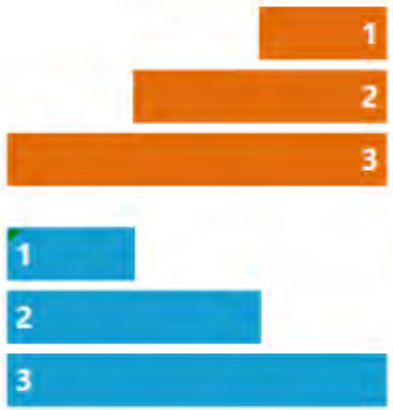
- **7 or more consecutive points** continuously improving (all rising or all falling depending on whether higher or lower is better).

4. Two out of three points beyond the 2-sigma limit

- Two of three consecutive points fall beyond the 2-sigma limit on the improving side of the mean.
- Often an early indication of a genuine improvement before a full shift develops.

Negative alerts would be identical trends but going in the direction of worsening performance for that indicator.

Rather than just showing one icon reflecting whether a process is significantly worsening or improving, these SPC alerts identified above are quantified and the number of alerts currently relating to the latest reported data are reflected in the Summary tables on this report as follows:



1 Negative SPC alert - **one** of the SPC rules is flagging an alert which means that performance has worsened

2 Negative SPC alerts - **two** of the SPC rules are flagging an alert which means that performance has worsened over time

3 Negative SPC alerts - **three** of the SPC rules are flagging an alert which means that performance has consistently worsened over time

1 Positive SPC alert - **one** of the SPC rules is flagging an alert which means that performance has improved

2 Positive SPC alerts - **two** of the SPC rules are flagging an alert which means that performance has improved over time

3 Positive SPC alerts - **three** of the SPC rules are flagging an alert which means that performance has consistently improved over time

Planning Indicators *(page 1 of 3)*

Return to Agenda



Ref.	Indicator	Month	Apr-26		May-26		Jun-26		Jul-26		Aug-26		Sep-26		Latest month SPC alerts	
		ICB	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Negative	Positive
16.01	Consultant-led first outpatient attendances (Spec acute)	BC ICB	41,781	42,168	41,229	41,838	47,730	46,173	46,231	48,171	40,018	41,110		44,335		
		BSOL ICB	55,372	61,010	53,957	59,541	60,771	68,105	59,258	70,250	52,579	65,190		67,846		
		Cluster	97,153	103,178	95,186	101,379	108,501	114,278	105,489	118,421	92,597	106,300		112,181		
16.02	Consultant-led follow-up outpatient attendances (Spec acute)	BC ICB	66,799	65,926	62,940	65,961	73,398	71,136	72,817	74,469	57,609	60,850		72,942		
		BSOL ICB	82,792	85,253	78,135	83,128	90,594	94,879	90,241	97,752	74,723	90,628		94,285		
		Cluster	149,591	151,179	141,075	149,089	163,992	166,015	163,058	172,221	132,332	151,478		167,227		
16.03	Total number of specific acute elective day case spells in the period	BC ICB	14,725	14,835	14,510	15,471	15,834	15,737	16,468	16,699	14,299	14,912		15,007		
		BSOL ICB	15,237	13,671	15,033	13,387	16,144	15,321	16,278	15,796	14,834	14,665		15,244		
		Cluster	29,962	28,506	29,543	28,858	31,978	31,058	32,746	32,495	29,133	29,577		30,251		
16.04	Total number of specific acute elective ordinary spells in the period	BC ICB	1,788	2,011	2,076	2,096	2,102	2,136	2,173	2,265	1,960	2,022		2,033		
		BSOL ICB	2,433	2,344	2,483	2,416	2,585	2,697	2,627	2,759	2,406	2,586		2,691		
		Cluster	4,221	4,355	4,559	4,512	4,687	4,833	4,800	5,024	4,366	4,608		4,724		
16.05	RTT completed admitted pathways	BC ICB	6,109	7,139	6,222	6,363	6,834	7,562	7,145	6,908	6,014	7,059		6,684		
		BSOL ICB	6,171	5,291	5,981	5,646	6,268	6,124	6,174	6,157	5,653	5,677		5,855		
		Cluster	12,280	12,430	12,203	12,009	13,102	13,686	13,319	13,065	11,667	12,736		12,539		
16.06	RTT completed non-admitted pathways	BC ICB	28,343	33,353	28,009	29,726	32,744	35,333	31,500	32,278	27,707	32,981		31,229		
		BSOL ICB	31,582	32,466	28,773	32,974	34,069	33,676	33,613	35,173	29,059	32,100		35,212		
		Cluster	59,925	65,819	56,782	62,700	66,813	69,009	65,113	67,451	56,766	65,081		66,441		
16.07	Diagnostic Tests	BC ICB	53,489	51,685	52,888	50,484	55,805	53,412	56,575	56,221	54,335	52,302		52,082		1
		BSOL ICB	61,245	63,826	62,920	64,316	65,921	64,318	67,215	64,475	63,053	63,984		64,475		2
		Cluster	114,734	115,511	115,808	114,800	121,726	117,730	123,790	120,696	117,388	116,286		116,557		2
16.08	Appointments in General Practice	BC ICB	714,373	670,537	677,555	637,011	793,942	737,592	790,193	770,573	662,825	670,063		737,070		
		BSOL ICB	843,959	792,257	787,282	751,011	931,292	869,227	921,754	908,604	783,612	790,091		869,100		
		Cluster	1,558,332	1,462,794	1,464,837	1,388,022	1,725,234	1,606,819	1,711,947	1,679,177	1,446,437	1,460,154		1,606,170		
16.09	Total Waiting List	BC ICB	167,927	165,228	167,673	161,929	169,425	158,629	169,139	155,329	172,245	152,029		148,729		3
		BSOL ICB	157,042	153,579	158,886	152,159	156,881	150,738	158,885	149,318	161,173	147,897		146,476		3
		Cluster	324,969	318,807	326,559	314,088	326,306	309,367	328,024	304,647	333,418	299,926		295,205		3
16.1	Percentage Outpatient firsts within 18 Weeks	BC ICB	69.41%	67.65%	71.71%	67.78%	71.24%	68.21%	71.44%	68.61%	69.50%	69.07%	68.65%	69.49%		3
		BSOL ICB	66.78%	63.38%	68.03%	63.80%	68.18%	64.26%	67.58%	64.73%	65.53%	65.14%	65.32%	65.58%		3
		Cluster	68.12%	65.37%	69.86%	65.67%	69.70%	66.13%	69.51%	66.57%	67.50%	67.01%	66.98%	67.45%		3

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This page summarises how many Negative and Positive SPC alerts there are in the latest month's data for each indicator in this report at **ICB level**. If there are no coloured bars within an indicator line, there are no current SPC alerts.

Planning Indicators *(page 2 of 3)*

Return to Agenda

Ref.	Indicator	Month	Apr-26		May-26		Jun-26		Jul-26		Aug-26		Sep-26		Latest month SPC alerts	
		ICB	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Negative	Positive
16.11	Reliance on mental health inpatient care for adults with LD	BC ICB					20.00%	22.00%						20.00%		
		BSOL ICB					40.00%	32.00%						31.00%		
		Cluster					60.00%	54.00%						51.00%		
16.12	Reliance on mental health inpatient care for autistic adults	BC ICB					15.00%	12.00%						11.00%		
		BSOL ICB					40.00%	30.00%						28.00%	1	
		Cluster					55.00%	42.00%						39.00%		
16.13	Reliance on mental health inpatient care for U18s	BC ICB						4.00%						4.00%		
		BSOL ICB					10.00%	6.00%						6.00%		
		Cluster					10.00%	10.00%						10.00%	1	
16.16	MHST Coverage	BC ICB	4,155	3,910	4,210	4,040	4,310	4,175	4,360	4,310	4,500	4,440		4,575		3
		BSOL ICB	6,560	4,355	6,790	4,415	7,015	4,615	6,820	4,815	6,785	4,815		5,015		3
		Cluster	10,715	8,265	11,000	8,455	11,325	8,790	11,180	9,125	11,285	9,255		9,590		3
16.17	% Type 1 in 4hrs	BC ICB	68.67%	67.55%	67.18%	68.31%	66.48%	69.18%	65.87%	69.74%	63.97%	69.63%	65.59%	69.32%		1
		BSOL ICB	63.04%	61.40%	63.13%	60.95%	60.76%	61.90%	63.92%	62.96%	61.81%	63.06%	59.42%	62.73%		
		Cluster	65.99%	64.74%	65.21%	64.83%	63.74%	65.97%	64.96%	66.94%	62.96%	66.93%	62.72%	66.51%		1
16.19	Number of patients waiting over 52 weeks for care	BC ICB	1,619	1,345	1,603	1,224	1,160	1,114	1,064	1,013	1,151	922		839		3
		BSOL ICB	653	737	556	632	536	528	559	423	534	320		215		3
		Cluster	2,272	2,082	2,159	1,856	1,696	1,642	1,623	1,436	1,685	1,242		1,054		3
16.2	Cancer 28 day waits (faster diagnosis standard)	BC ICB	79.55%	80.89%	82.18%	80.90%	81.21%	80.91%	82.04%	81.01%	79.63%	81.00%		81.00%		1
		BSOL ICB	82.11%	80.26%	85.03%	80.28%	85.17%	80.27%	85.63%	79.44%	85.24%	80.32%		80.07%		3
		Cluster	80.86%	80.53%	83.73%	80.56%	83.21%	80.55%	83.89%	80.11%	82.45%	80.62%		80.48%		3
16.21	Cancer 62-day pathways. Total patients seen, and of which those seen within 62 days	BC ICB	72.18%	76.41%	72.41%	76.67%	71.45%	77.05%	70.19%	77.20%	71.82%	76.91%		78.39%		2
		BSOL ICB	65.13%	74.13%	65.93%	74.85%	70.87%	74.91%	73.01%	78.21%	72.06%	80.55%		77.69%		3
		Cluster	68.43%	75.08%	69.15%	75.61%	71.14%	75.82%	71.63%	77.79%	71.94%	79.10%		77.97%		3
16.22	Cancer 31 day performance	BC ICB	93.91%	93.00%	92.87%	94.02%	92.79%	94.07%	92.97%	94.06%	93.10%	94.02%		94.03%		1
		BSOL ICB	91.80%	93.07%	93.14%	92.93%	93.58%	98.90%	94.02%	95.95%	92.09%	95.88%		94.66%		
		Cluster	92.82%	93.04%	93.01%	93.41%	93.20%	96.82%	93.49%	95.12%	92.58%	95.10%		94.39%		1
16.23	Access to Children and Young People Mental Health Services	BC ICB	18,070	17,900	18,055	18,090	18,110	18,285	18,240	18,485	18,510	18,765		18,965		2
		BSOL ICB	22,050	24,834	21,805	24,834	21,215	24,834	20,790	24,834	20,725	24,834		24,834		3
		Cluster	40,120	42,734	39,860	42,924	39,325	43,119	39,030	43,319	39,235	43,599		43,799		3

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Planning Indicators *(page 3 of 3)*

Return to Agenda

Ref.	Indicator	Month	Apr-26		May-26		Jun-26		Jul-26		Aug-26		Sep-26		Latest month SPC alerts	
		ICB	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Actual	Plan	Negative	Positive
16.24	Diagnostic 6-week waits	BC ICB	23.55%	16.91%	22.42%	16.41%	21.54%	16.40%	20.38%	16.16%	21.13%	15.98%		15.65%		1
		BSOL ICB	39.02%	31.45%	38.73%	30.48%	37.26%	29.50%	36.43%	28.50%	37.98%	27.48%		26.43%	1	
		Cluster	31.67%	24.80%	31.12%	24.24%	29.96%	23.48%	28.89%	22.73%	30.09%	22.22%		21.45%		
16.25	A&E 4-hour performance	BC ICB	80.19%	79.23%	79.10%	79.42%	78.25%	80.03%	78.16%	80.21%	76.98%	80.52%	77.44%	79.95%		1
		BSOL ICB	71.70%	74.38%	72.72%	73.77%	70.55%	75.49%	72.60%	78.26%	71.17%	79.07%	69.95%	78.46%		3
		Cluster	76.59%	77.12%	76.31%	76.89%	74.93%	78.08%	75.81%	79.36%	74.55%	79.90%	74.28%	79.28%		2
16.26	Number of attendances in A&E over 12 hours	BC ICB	4,190	4,693	4,665	3,513	4,520	2,936	4,505	2,834	4,705	2,302	4,960	2,689		
		BSOL ICB	4,460	3,901	4,970	4,228	5,020	3,908	4,425	3,595	4,155	3,035	4,675	3,044		
		Cluster	8,650	8,594	9,635	7,741	9,540	6,844	8,930	6,429	8,860	5,337	9,635	5,733		
16.28	Patients discharged on discharge ready date	BC ICB	87.10%	84.95%	87.49%	85.36%	88.15%	86.80%	84.32%	86.25%	85.99%	86.97%		86.26%		
		BSOL ICB	88.76%	88.73%	89.22%	89.56%	88.89%	89.72%	89.25%	88.88%	89.68%	89.14%		89.30%	1	
		Cluster	87.95%	86.83%	88.38%	87.46%	88.53%	88.24%	87.09%	87.57%	88.06%	88.05%		87.80%	2	
16.29	Virtual ward capacity	BC ICB	86.34%	85.27%	87.94%	86.83%	100.90%	84.64%	84.43%	83.39%	73.16%	77.57%	60.42%	84.11%		
		BSOL ICB	334.71%	98.10%	270.59%	98.10%	90.00%	97.47%	89.41%	97.47%	85.88%	97.47%	220.59%	97.47%		1
		Cluster	168.48%	89.52%	148.82%	90.57%	97.22%	88.89%	86.11%	88.05%	77.41%	84.13%	114.23%	88.52%		
16.3	Percentage of Annual Health Checks delivered to patients aged 14+ on the GP Learning Disability Register	BC ICB					11.91%	12.27%						18.79%		
		BSOL ICB					14.05%	12.73%						21.22%		
		Cluster					13.22%	12.55%						20.27%		
16.31	Access to NHS talking therapies for anxiety and depression - reliable recovery	BC ICB	45.61%	43.02%	44.66%	45.52%	44.52%	45.48%	44.88%	47.48%	45.08%	50.00%		49.03%		
		BSOL ICB	48.39%	50.98%	47.77%	50.98%	48.67%	50.98%	50.00%	50.98%	49.35%	50.98%		50.98%		3
		Cluster	47.24%	47.63%	46.59%	48.57%	46.89%	48.62%	47.59%	49.33%	47.37%	50.53%		50.18%		1
16.32	Access to NHS talking therapies for anxiety and depression - reliable improvement	BC ICB	65.71%	63.99%	67.30%	63.99%	66.32%	66.97%	67.36%	66.00%	66.54%	68.00%		66.02%		
		BSOL ICB	69.03%	69.02%	69.45%	69.02%	70.13%	69.02%	69.09%	69.02%	68.57%	69.02%		69.02%		2
		Cluster	67.67%	66.90%	68.64%	66.80%	68.50%	68.14%	68.28%	67.59%	67.64%	68.55%		67.78%		1
16.33	Urgent dental appointments-E.D.28	BC ICB		7,708		7,708		7,708		7,708		7,708		7,708		
		BSOL ICB		7,754		7,754		7,754		7,754		7,754		7,755		1
		Cluster		15,462		15,462		15,462		15,462		15,462		15,463		1

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This page summarises how many Negative and Positive SPC alerts there are in the latest month's data for each indicator in this report at **ICB level**. If there are no coloured bars within an indicator line, there are no current SPC alerts.

Oversight metrics - ICB indicators *(page 1 of 5)*

Return to Agenda

ICB OvF



** Data sourced from FDP Performance Overview Dashboard (no Cluster position available)*

Domain	ICB ID	Metric	Frequency	ICB	Target	2023/24	2024/25	2025/26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Latest SPC alerts: Negative	Latest SPC alerts: Positive
1	8.01	Under 75s cancer mortality rate		BC			149.44									2
			Annual	BSOL			138.72									2
				Cluster												
	8.02	Under 75s cardiovascular disease mortality rate		BC			146.52									
			Annual	BSOL			124.87									
				Cluster												
	8.03	Cervical screening rate		BC	80%	**68.05%**										
			Annual	BSOL	80%	**64.17%**										
				Cluster	**80%**											
	8.04	Bowel screening rate		BC	60%	65.74%	65.70%									
			Annual	BSOL	60%	64.04%	64.10%									
				Cluster	**60%**											
	8.05	Breast screening rate		BC	70%	**65.92%**	**66.67%**									
			Annual	BSOL	70%	**62.32%**	**64.28%**									
				Cluster	**70%**											
	8.06	MMRV (measles, mumps, rubella and varicella) uptake rate		BC	95%	**82.10%**	**83.00%**									
			Annual	BSOL	95%	**74.50%**	**75.30%**									
				Cluster												
	8.07	Eligible people supported by obesity prevention programmes (rate per 100,000)	Monthly	BC		291.93	363.00	255.07								
				BSOL		187.71	217.22	296.73								
				Cluster												
	8.14	Digital weight management*	Quarterly	BC			363.00	255.07			151.86					
				BSOL			217.22	296.46			240.17					
				Cluster												
	8.15	Diabetes prevention programme*	Quarterly	BC			14.62	14.62			13.00					
				BSOL			13.50	13.50			15.46					
				Cluster												
	8.09	% of pregnant women who quit smoking		BC		3.00%	0.00%	0.00%							3	
			Monthly	BSOL		16.94%	11.47%	0.00%							3	
				Cluster		**16.26%**	**10.97%**	**0.00%**							3	
	8.10	Deprivation gap in early cancer diagnosis		BC		14.24	0.56	0.56	0.56	0.56						3
			Monthly	BSOL		0.57	0.57	0.55	0.55	0.56						3
				Cluster		**0.56**	**0.57**	**0.56**	**0.56**	**0.56**						3

Oversight metrics - ICB indicators *(page 2 of 5)*

Return to Agenda

ICB OvF

** Data sourced from FDP Performance Overview Dashboard (no Cluster position available)*

Domain	ICB ID	Metric	Frequency	ICB	Target	2023/24	2024/25	2025/26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Latest SPC alerts: Negative	Latest SPC alerts: Positive
	8.11	Deprivation and ethnicity gap in pre-term births*		BC			0.50	1.37			1.02					
			Quarterly	BSOL			2.53	1.55			2.27					
				Cluster												
	8.12	Deprivation gap in lipid management		BC			91.69%	93.67%							1	
			Quarterly	BSOL			98.02%	101.21%							2	
				Cluster												
	8.13	Deprivation gap in hypertension management		BC		89.77%	89.86%									
			Quarterly	BSOL		98.54%	100.53%									
				Cluster												
	9.01	% change in overall waiting list size versus previous year		BC			-6.67	-7.49	-5.56	-4.00	-3.11	-2.37	-0.15		2	
2A			Monthly	BSOL			-13.18	-9.70	-7.23	-7.28	-7.50	-3.86	-2.25		2	
				Cluster			**-9.97**	**-8.57**	**-6.37**	**-5.63**	**-5.27**	**-3.10**	**-1.17**		2	
	9.02	Level of growth in community care contacts		BC		-6.1%	4.3%	-4.4%	-3.3%	-8.2%	-11.0%	-8.5%			1	
			Monthly	BSOL		36.8%	-80.0%	382.5%	370.3%	320.9%	-5.1%	-6.5%				
				Cluster		**14.1%**	**-43.3%**	**72.4%**	**71.2%**		**-7.7%**					
	9.03	% of cancers diagnosed at stage 1 or 2		BC		33.96%	39.00%	39.54%	37.94%	38.10%	36.65%					1
			Monthly	BSOL		35.42%	38.91%	35.16%	34.38%	35.12%	29.78%					
				Cluster		**34.68%**	**38.96%**	**37.45%**	**36.25%**	**36.59%**	**33.42%**					2
	9.04	% of acute mental health admissions with no contact with community mental health services in the previous year		BC		12.50%	13.41%	9.46%	12.16%	12.68%	14.29%	12.70%				
			Monthly	BSOL		13.92%	5.95%	8.96%	10.77%	12.00%	12.00%	11.84%				
				Cluster		**13.17%**	**9.64%**	**9.22%**	**11.51%**	**12.33%**	**13.10%**	**12.23%**				
	9.05	Occupied bed days per 100,000 head of population		BC												
			Monthly	BSOL												
				Cluster												
	9.06	% of clinically urgent appointments that happen on the same day		BC		64.71%	66.51%	70.17%	72.72%	73.50%	74.40%	73.62%	73.36%			2
			Monthly	BSOL		71.02%	70.14%	74.11%	73.70%	74.39%	75.87%	76.71%	78.49%			3
				Cluster		**67.76%**	**68.35%**	**72.28%**	**73.26%**	**73.98%**	**75.20%**	**75.32%**	**76.18%**			2
	9.07	Units of dental activity delivered as a % of commissioned		BC												
			Monthly	BSOL												
				Cluster												
	10.14	Percentage of people with a learning disability or autistic people in a mental health hospital with the longest length of stay		BC					57.14%	57.14%	57.14%	57.14%	50.00%			
			Monthly	BSOL					56.25%	52.94%	52.94%	50.00%	52.94%			
				Cluster					**56.52%**	**57.14%**	**57.14%**	**57.14%**	**50.00%**			

Oversight metrics - ICB indicators *(page 3 of 5)*

Return to Agenda

ICB OvF





** Data sourced from FDP Performance Overview Dashboard (no Cluster position available)*

Domain	ICB ID	Metric	Frequency	ICB	Target	2023/24	2024/25	2025/26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Latest SPC alerts Negative	Latest SPC alerts Positive
3	11.03	% of patients rating their experience of GP access as easy	Monthly	BC			70.80%	62.70%	69.30%	68.00%	71.40%	71.20%	72.20%	73.40%		1
				BSOL			67.80%	66.20%	65.00%	64.50%	69.10%	67.10%	67.30%	65.90%		2
				Cluster			**69.18%**	**64.84%**	**66.67%**	**65.88%**	**69.91%**	**68.57%**	**69.11%**	**68.81%**		2
	11.04	% of patients with a preferred general practice professional able to see or speak with that professional	Monthly	BC				65.20%	66.70%	71.10%	69.50%	66.20%	61.70%	53.20%		
				BSOL				59.90%	69.80%	77.50%	80.50%	63.80%	66.10%	53.50%		1
				Cluster				**62.21%**	**68.61%**	**74.91%**	**76.60%**	**64.65%**	**64.37%**	**53.38%**		
	11.06	% of complaints open beyond 6 months	Quarterly	BC												
				BSOL												
				Cluster												
	11.07	12 month adult admission rate for people with a learning disability or autistic people	Quarterly	BC					0.0053%	0.0048%	0.0053%	0.0053%	0.0064%			
				BSOL					0.0048%	0.0053%	0.0053%	0.0053%	0.0053%			
				Cluster												
	11.09	% of ICB commissioned adult inappropriate out of area placement bed days as a proportion of all ICB commissioned bed days*	Monthly	BC			2.0%	3.0%	3.0%	2.0%	2.0%	1.0%				
				BSOL			4.0%	6.0%	6.0%	7.0%	2.0%	1.0%				
				Cluster												
	11.13	Average number of days between discharge ready and actual discharge date	Monthly	BC			0.54	0.36	0.31	0.30	0.31	0.28	0.30			3
				BSOL			0.53	0.34	0.30	0.32	0.34	0.29	0.28			1
				Cluster			**0.53**	**0.35**	**0.30**	**0.31**	**0.33**	**0.28**	**0.29**			3
4	12.03	% of NHS Talking Therapies patients completed a course of treatment and achieving reliable recovery	Monthly	BC		46.39%	47.24%	42.22%	45.90%	44.44%	44.52%	45.05%				
				BSOL		42.64%	46.18%	49.03%	48.69%	47.77%	48.67%	49.73%				3
				Cluster		**44.23%**	**46.61%**	**46.40%**	**47.53%**	**46.51%**	**46.89%**	**47.52%**				
	12.05	% of people with 3 or more emergency admissions in the last 90 days of life	Monthly	BC			3.35%	4.44%	3.83%	3.98%	3.51%					
				BSOL			2.82%	4.17%	3.58%	2.84%	2.95%					
				Cluster			**3.09%**	**4.31%**	**3.70%**	**3.41%**	**3.23%**					
	12.07	Over 65s bed days per 100,000 head of population	Quarterly	BC												
				BSOL												
				Cluster												
	12.10	% of patients who receive all 8 diabetes care processes	Annual	BC		39.49%	35.17%	36.41%								
				BSOL		47.35%	41.54%	40.03%								
				Cluster		**43.59%**	**38.53%**	**38.31%**								
	12.11	Number of patients with GP recorded cardiovascular disease whose cholesterol is managed to National Institute for Health and Care Excellence (NICE) guidance	Quarterly	BC			50.58%	54.61%								1
				BSOL			45.85%	49.81%								1
				Cluster			**48.27%**	**52.26%**								1

Oversight metrics - ICB indicators *(page 4 of 5)*

Return to Agenda

ICB OvF

** Data sourced from FDP Performance Overview Dashboard (no Cluster position available)*

Domain	ICB ID	Metric	Frequency	ICB	Target	2023/24	2024/25	2025/26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Latest SPC alerts: Negative	Latest SPC alerts: Positive
	12.12	Number of patients with GP recorded hypertension whose blood pressure is managed to NICE guidance	Quarterly	BC		70.69%	70.73%	74.45%								1
				BSOL		70.33%	68.71%	73.01%								1
				Cluster		**70.51%**	**69.72%**	**73.73%**								**1**
	12.13	% of patients on GP severe mental illness registers to receive a full physical health check in the last 12 months	Quarterly	BC			69.46%	69.10%			60.04%					
				BSOL			65.18%	61.99%			56.75%					
				Cluster			**66.80%**	**64.67%**			**57.99%**					
	12.14	% of patients over 14 years old on GP learning disability registers to receive a full physical health check and action plan in the last 12 months	Monthly	BC		9.15%	9.19%	9.77%	3.57%	3.74%	5.77%	7.07%				
				BSOL		7.83%	8.05%	7.38%	4.43%	4.28%	6.03%	6.59%				
				Cluster		**8.36%**	**8.50%**	**8.31%**	**4.09%**	**4.07%**	**5.93%**	**6.78%**				
5	13.03	Percentage of children aged 9 and under prescribed antibiotics	Monthly	BC		57.23%	55.13%	50.70%	50.58%	51.47%						3
				BSOL		55.09%	52.63%	49.10%	49.49%	50.41%						3
				Cluster		**56.06%**	**53.76%**	**49.82%**	**49.98%**	**50.89%**						**3**
	13.08	Number of neonatal deaths and stillbirths per 1,000 total births	Monthly	BC		11.99	3.46	1.15	2.53							1
				BSOL		4.42	8.80	2.93	2.83							
				Cluster		**8.12**	**6.35**	**2.12**	**2.69**							
6	14.01	Planned surplus or deficit	Annual	BC				£0								
				BSOL				£0								
				Cluster				**£0**								
	14.02	Variance to plan year-to-date	Monthly	BC									-£ 371			
				BSOL									-£ 721			
				Cluster									#####			
	14.04	Implied productivity growth	Quarterly	BC												
				BSOL												
				Cluster												
	14.05	% of clinical trials set up within 90 days	Quarterly	BC												
				BSOL												
				Cluster												

Oversight metrics - ICB indicators *(page 5 of 5)*

Return to Agenda

ICB OvF

** Data sourced from FDP Performance Overview Dashboard (no Cluster position available)*

Domain	ICB ID	Metric	Frequency	ICB	Target	2023/24	2024/25	2025/26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Latest SPC alerts: Negative	Latest SPC alerts: Positive
7	15.01	Sickness absence rate	Monthly	BC			2.79%	4.00%	3.95%	3.84%	2.99%					
				BSOL			4.46%	4.46%	4.81%	5.72%	5.49%					
				Cluster			**3.66%**	**4.24%**	**4.40%**	**4.84%**	**4.33%**				1	
	15.02	NHS staff survey engagement theme sub-score	Annual	BC			6.73	6.47							1	
				BSOL			6.76	6.75								
				Cluster												
	15.06	GP headcount per capita	Monthly	BC			119.52	119.77	122.96	122.47	122.52	122.36	124.87			3
				BSOL			125.40	128.22	129.97	130.12	130.17	129.90	135.89			3
				Cluster			**122.71**	**124.39**	**126.79**	**126.65**	**126.70**	**126.48**	**130.90**			3

Oversight metrics - Provider/ICS indicators

Return to Agenda

ICS OvF

Domain	Ref.	Indicator	ICS level: BC ICS	ICS level: BSOL ICS	ICS level: Total	Latest Date	SPC alerts (Provider alerts): BC ICS	SPC alerts (Provider alerts): BSOL ICS	SPC alerts (Provider alerts): Total
Access to Services	10.01	% of patients waiting up to 18 weeks for care	63.75%	67.99%	**65.44%**	Aug-26	10	10	20
	10.03	% of patients waiting over 52 weeks for care	0.72%	0.25%	**0.53%**	Aug-26	11	12	23
	10.04	% of patients waiting less than 18 weeks for community care	71.04%	60.88%	**64.27%**	Jul-26	6		7
	10.05	% of diagnostic referrals waiting over 6 weeks	18.66%	38.24%	**21.36%**	Aug-26			6
	10.06	% of cases meeting the 28 day faster diagnosis standard	79.64%	86.41%	**82.40%**	Aug-26	4	4	8
	10.07	% of cases treated within 62 days of referral	72.65%	71.47%	**72.06%**	Aug-26	3	3	6
	10.08	% of patients admitted, discharged or transferred within 4 hours	77.44%	69.95%	**74.28%**	Sep-26		3	
	10.09	% of patients spending over 12 hours in department	10.46%	25.13%	**14.61%**	Sep-26			
	10.15	% of cases treated within 31 days of referral	93.22%	91.87%	**92.46%**	Aug-26	5		7
	10.16	% of cases waiting over 52 weeks for community care	9.92%	22.03%	**17.99%**	Jul-26	7		10
Effectiveness of Care	12.08	30 day readmission rate band	9.33%	6.85%	**8.34%**	Jul-26	3		4
People	15.05	Healthcare worker flu vaccination rate	30.32%		**30.32%**	Feb-26			
Population Health, Prevention and Reducing	8.08	% of inpatients making a supported quit attempt with an in-house tobacco dependence treatment	21.55%	87.71%	**58.39%**	Aug-26	6 4		6 6
Access to Services	10.12	% of total open childrens and young people mental health related waits that are over 104 weeks		855	**855**	Jul-26			
Experience of Care	11.08	Number of patients cared for in corridors	2,049	553	**2,602**	Aug-26	2		2
Patient Safety	13.05	Rolling 12-month count of MRSA cases	10	8	**18**	Jul-26	7	4	11
	13.06	Rolling 12-month count of E. coli cases	319	512	**831**	Jul-26	7	5	7 7
	13.07	Rolling 12-month count of C. diff cases	238	377	**615**	Jul-26	11	4	4 11
Experience of Care	11.13	Average number of days between discharge ready and actual discharge date	0.31	0.25	**0.28**	Aug-26	7	2	9
Experience of Care	11.11	Mean length of stay for older adult acute discharges	0.99	1.06	**1.02**	May-26	4 9	3	4 12
Effectiveness of Care	12.01	Summary Hospital Mortality Indicator	98.99	92.15	**95.77**	May-26	4 9	3	4 12

This page summarises how many Negative and Positive SPC alerts there are in the latest month's data for each indicator in this report at **Provider level**. As there are up to 7 Providers measured across each metric, and up to 4 SPC alerts, there could be ***up to 28 alerts per metric***, and there may be a mix of positive and negative alerts across the Providers measured.

If there are no coloured bars within an indicator line, there are no current SPC alerts.

NHS
Birmingham and Solihull

Return to Agenda

NHS
Black Country

Urgent and Emergency Care.



10.08 (OF0013) - % of patients admitted, discharged or transferred within 4 hours

Return to Agenda

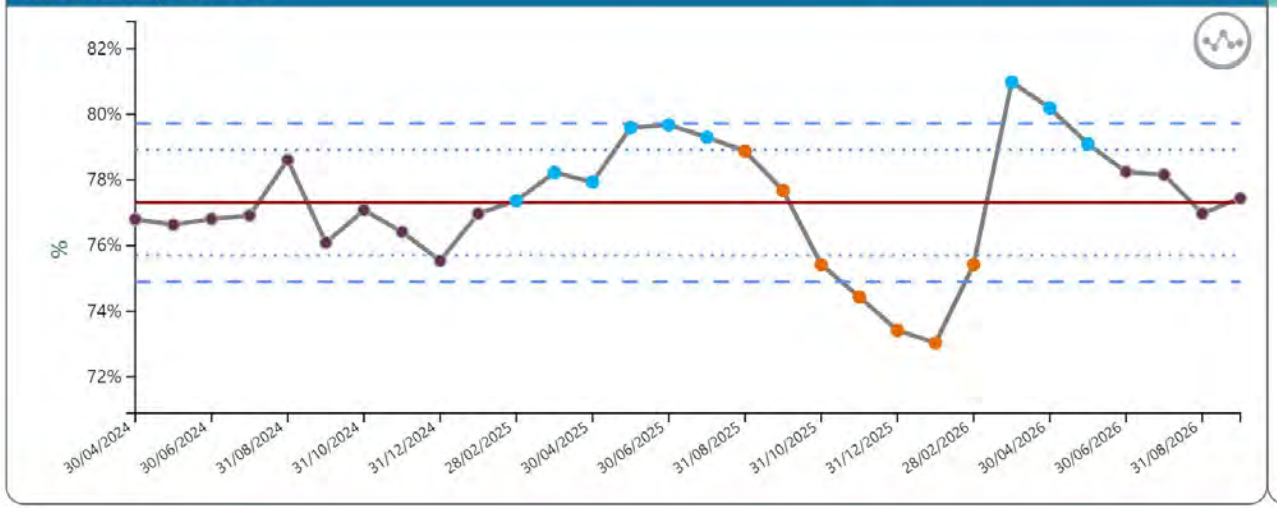
Domain: *Access to Services*

Update frequency: Monthly

Oversight Framework / Planning

Data up to: Sep-26

Black Country ICS - SPC



BSOL ICS - SPC



ICS level performance

ICS	Provider	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26
BC ICS	**Total**	**80.19%**	**79.10%**	**78.25%**	**78.16%**	**76.98%**	**77.44%**
	DGFT	82.17%	80.81%	77.27%	78.43%	76.65%	80.30%
	RWHT	80.96%	80.04%	80.46%	79.80%	79.56%	79.99%
	SWBH	74.99%	76.34%	73.33%	75.05%	74.14%	74.47%
	WHT	84.66%	80.13%	83.26%	80.09%	77.86%	75.30%
BSOL ICS	**Total**	**71.70%**	**72.72%**	**70.55%**	**72.60%**	**71.17%**	**69.95%**
	BWCH	84.21%	88.39%	83.71%	86.57%	93.71%	78.25%
	UHB	70.16%	70.82%	68.95%	71.02%	68.91%	68.95%

SPC Summary - Latest data point

ICS Grouping	Latest Date	Value	Variation
BC ICS	30/09/2026	77.44%	
BSOL ICS	30/09/2026	69.95%	

10.09 (OF0014) - % of patients spending over 12 hours in department

Return to Agenda

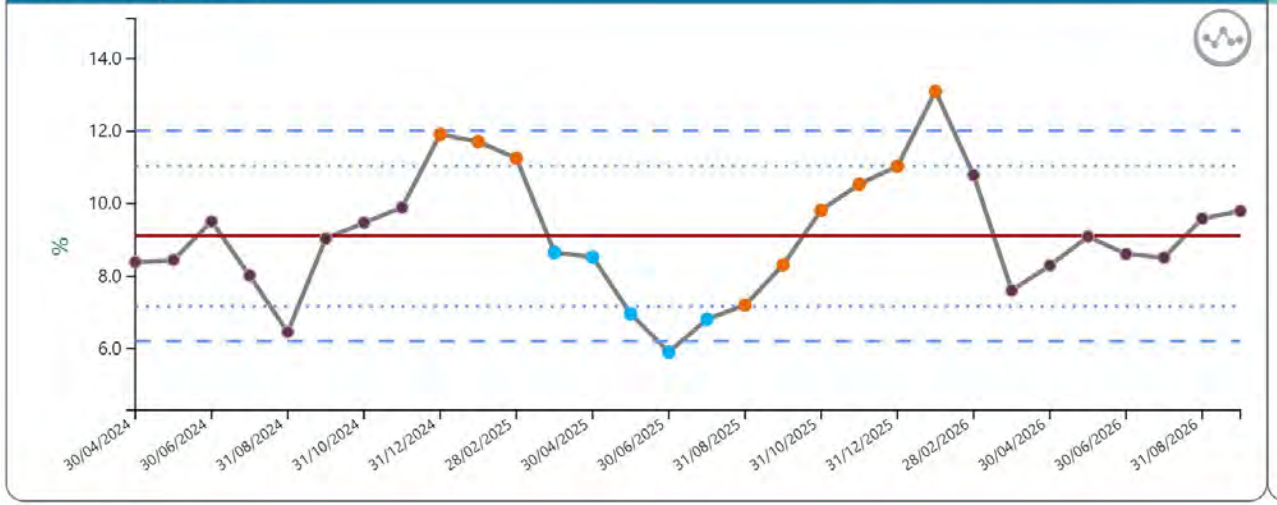
Domain: *Access to Services*

Update frequency: Monthly

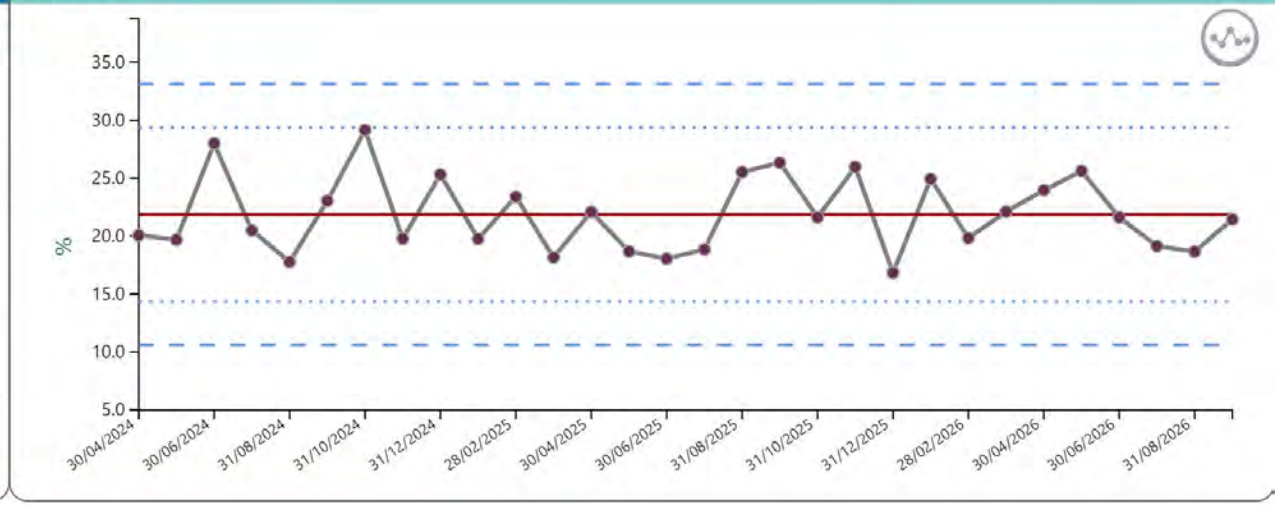
Oversight Framework / Planning

Data up to: Sep-26

Black Country ICS - SPC



BSOL ICS - SPC



ICS level performance

ICS	Provider	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26
BC ICS	**Total**	**8.72%**	**9.36%**	**9.02%**	**8.85%**	**9.85%**	**9.84%**
	DGFT	8.29%	10.43%	10.16%	7.57%	11.03%	11.69%
	RWHT	11.03%	11.62%	11.21%	11.32%	12.35%	10.68%
	SWBH	10.34%	8.98%	9.91%	9.43%	8.98%	8.88%
	WHT	3.52%	5.34%	3.18%	5.76%	5.99%	7.96%
BSOL ICS	**Total**	**31.97%**	**33.54%**	**30.21%**	**27.39%**	**27.92%**	**29.52%**
	BWCH	0.09%	0.08%	0.18%	0.42%	0.16%	0.76%
	UHB	47.81%	51.13%	43.05%	37.85%	37.15%	42.11%

SPC Summary - Latest data point

ICS Grouping	Latest Date	Value	Variation
BC ICS	30/09/2026	9.84%	
BSOL ICS	30/09/2026	29.52%	

10.1 (OF0017) - Mean response time to category 2 calls

Return to Agenda

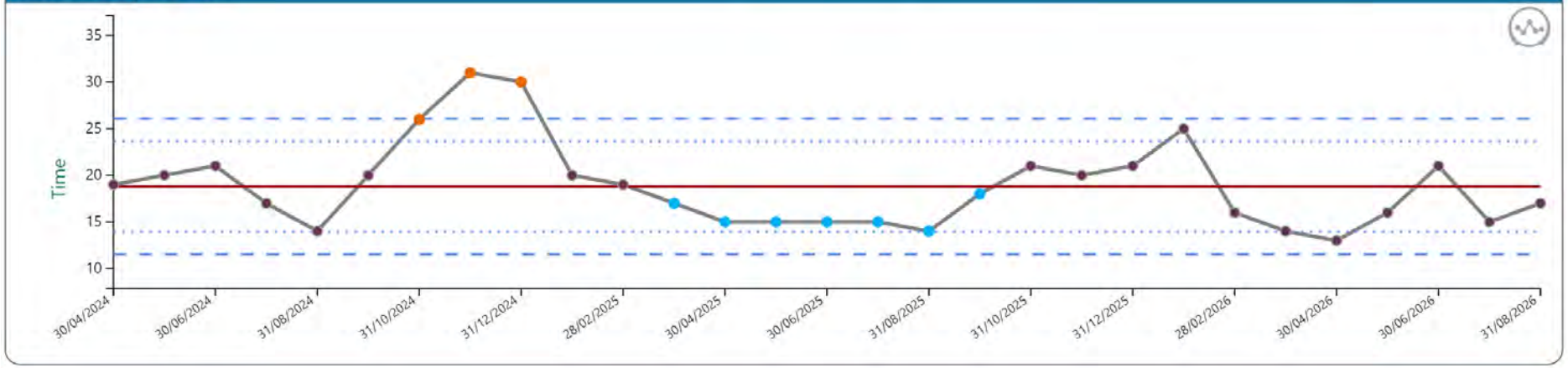
Domain: *Access to Services*

Update frequency: Annually

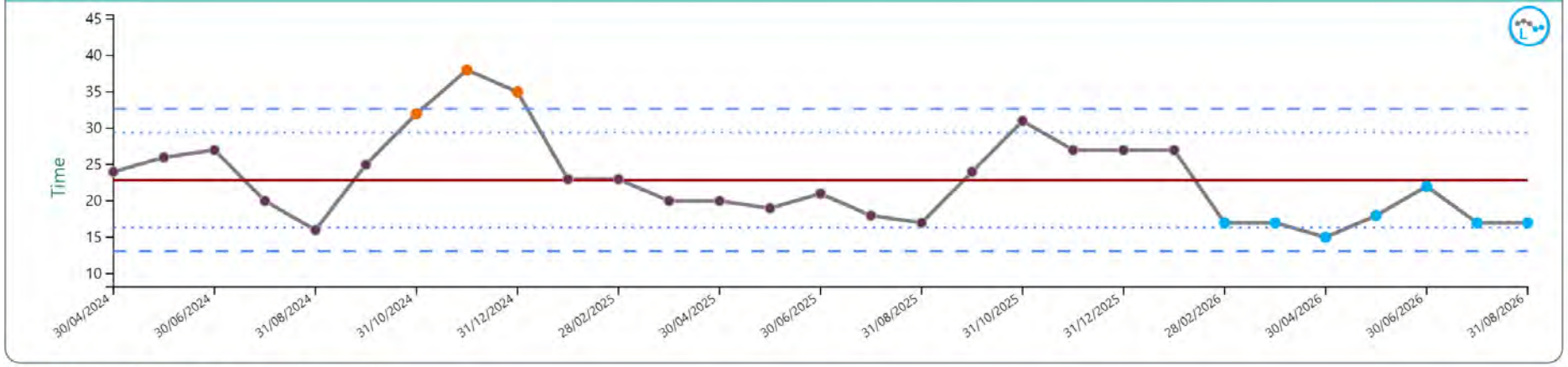
Oversight Framework / Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB	13.00	16.00	21.00	15.00	17.00
BSOL ICB	15.00	18.00	22.00	17.00	17.00

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	17.00	
BSOL ICB	31/08/2026	17.00	

16.28 (XX0028) - Patients discharged on discharge ready date

Return to Agenda

Report level: ICB

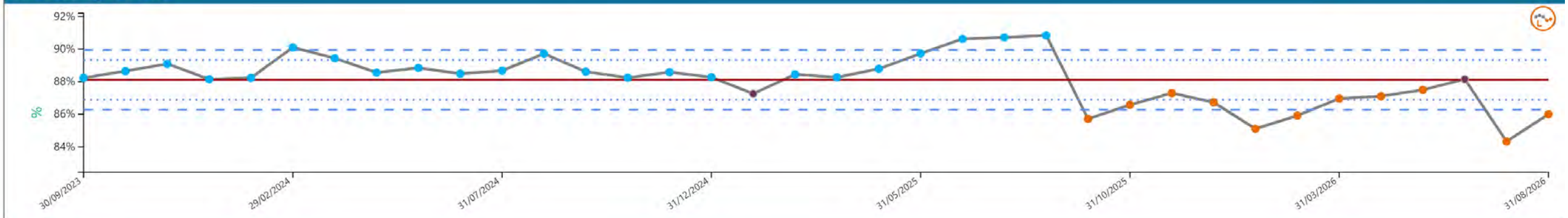
Domain: Other Planning Metrics

Update frequency: Monthly

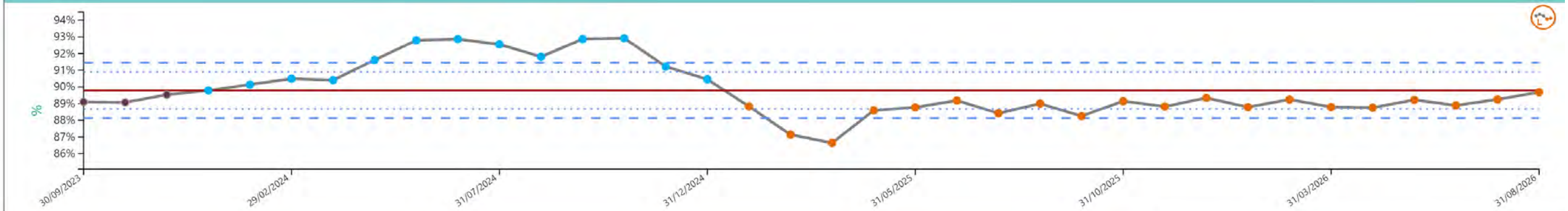
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance - based on Providers within each ICB

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	87.10%	87.49%	88.15%	84.32%	85.99%							
	Plan	84.95%	85.36%	86.80%	86.25%	86.97%	86.26%	85.52%	86.06%	85.35%	87.73%	84.88%	86.97%
BSOL ICB	Actual	88.76%	89.22%	88.89%	89.25%	89.68%							
	Plan	88.73%	89.56%	89.72%	88.88%	89.14%	89.30%	92.85%	91.10%	90.40%	88.68%	86.98%	86.51%
Cluster	Actual	87.95%	88.38%	88.53%	87.09%	88.06%							
	Plan	86.83%	87.46%	88.24%	87.57%	88.05%	87.80%	89.20%	88.61%	87.92%	88.22%	85.94%	86.73%

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	85.99%	86.97%		
BSOL ICB	31/08/2026	89.68%	89.14%		
Cluster	31/08/2026	88.06%	88.05%		

NHS
Birmingham and Solihull

NHS
Black Country

Elective Care.



10.01 (OF0023) - % of patients waiting up to 18 weeks for care

Return to Agenda

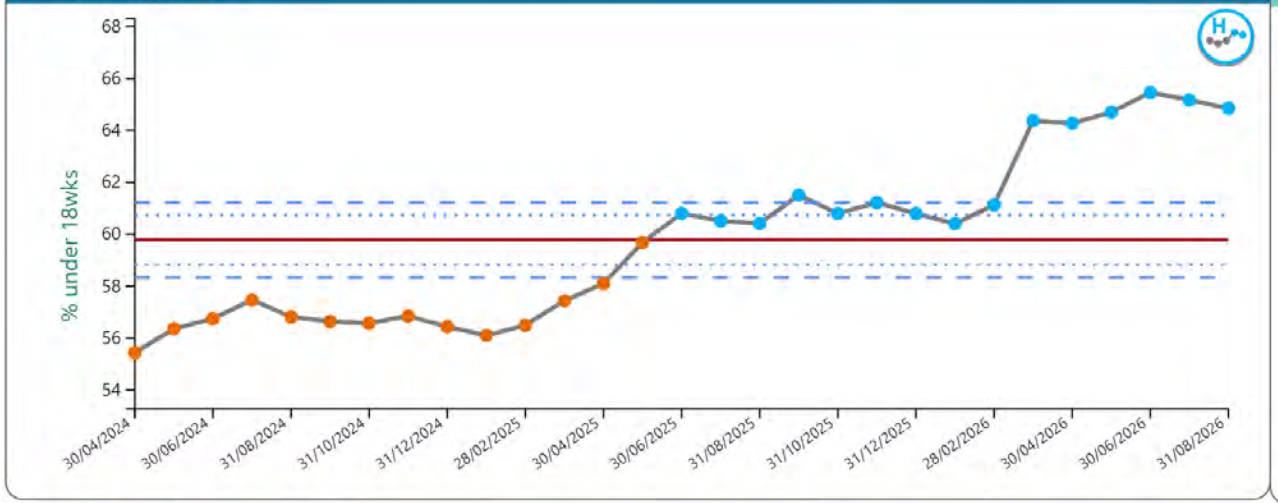
Domain: *Access to Services*

Update frequency: Monthly

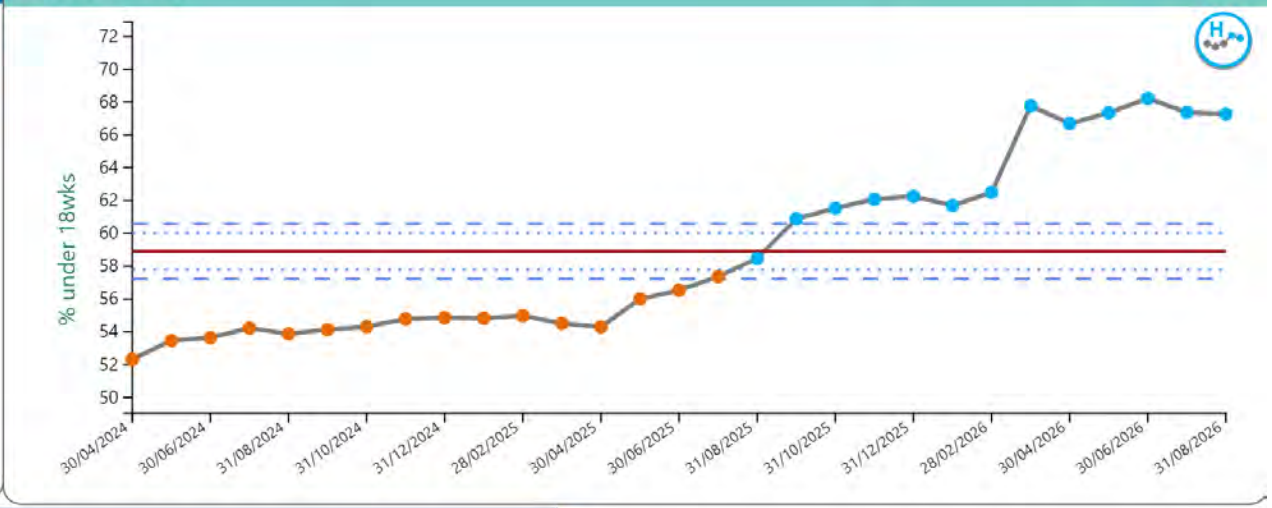
Oversight Framework / Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB		64.28%	64.70%	65.46%	65.17%	64.85%
BSOL ICB		66.69%	67.35%	68.23%	67.38%	67.29%
Cluster		65.44%	65.99%	66.79%	66.24%	66.02%
Provider						
BC ICS	**Total**	**63.48%**	**63.61%**	**64.29%**	**64.09%**	**63.75%**
	DGFT	65.09%	65.14%	65.02%	63.95%	63.06%
	RWHT	60.25%	60.42%	62.09%	63.89%	63.95%
	SWBH	62.26%	61.54%	61.75%	60.39%	60.14%
	WHT	72.19%	74.38%	74.80%	73.77%	72.83%
BSOL ICS	**Total**	**68.13%**	**68.94%**	**69.59%**	**68.51%**	**67.99%**
	BCHC	77.85%	78.72%	80.23%	81.93%	82.10%
	BWCH	62.10%	64.75%	65.94%	65.66%	65.34%
	ROH	65.71%	67.39%	68.18%	66.85%	65.78%
	UHB	68.38%	68.61%	68.93%	67.25%	66.57%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	64.85%	
BSOL ICB	31/08/2026	67.29%	
Cluster	31/08/2026	66.02%	

16.19 (XX0019) - Number of patients waiting over 52 weeks for care

Return to Agenda

Report level: ICB

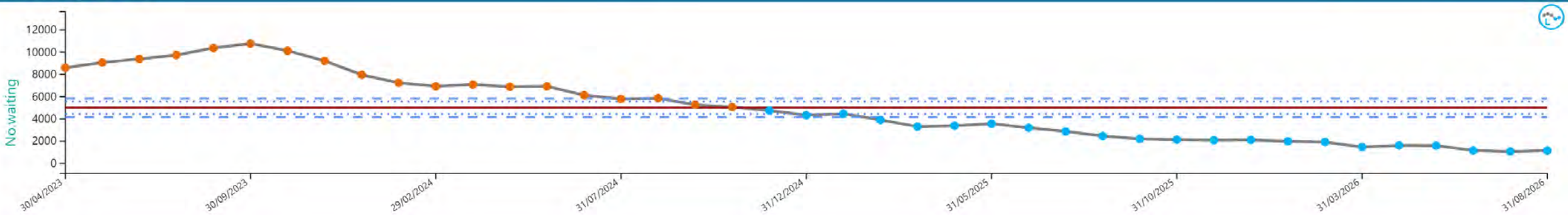
Domain: Access to Services

Update frequency: Annually

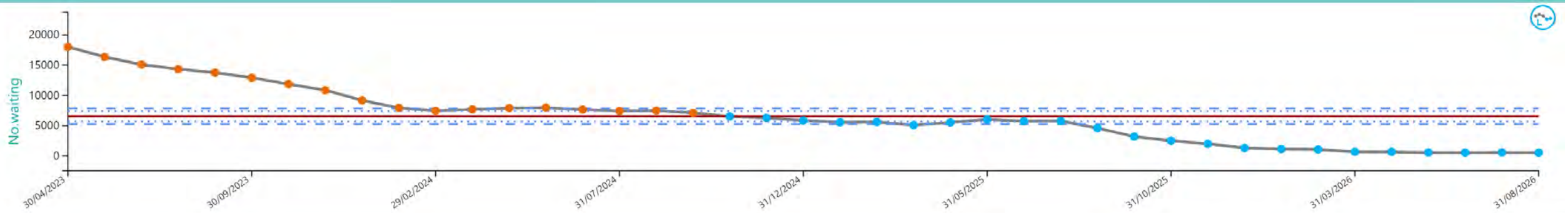
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	1,619	1,603	1,160	1,064	1,151							
	Plan	1,345	1,224	1,114	1,013	922	839	764	695	632	575	524	477
BSOL ICB	Actual	653	556	536	559	534							
	Plan	737	632	528	423	320	215	111	103	97	90	75	75
Cluster	Actual	2,272	2,159	1,696	1,623	1,685							
	Plan	2,082	1,856	1,642	1,436	1,242	1,054	875	798	729	665	599	552

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	1151	922	L	F
BSOL ICB	31/08/2026	534	320	L	F
Cluster	31/08/2026	1685	1242	L	F

10.03 (OF0003) - % of patients waiting over 52 weeks for care

Return to Agenda

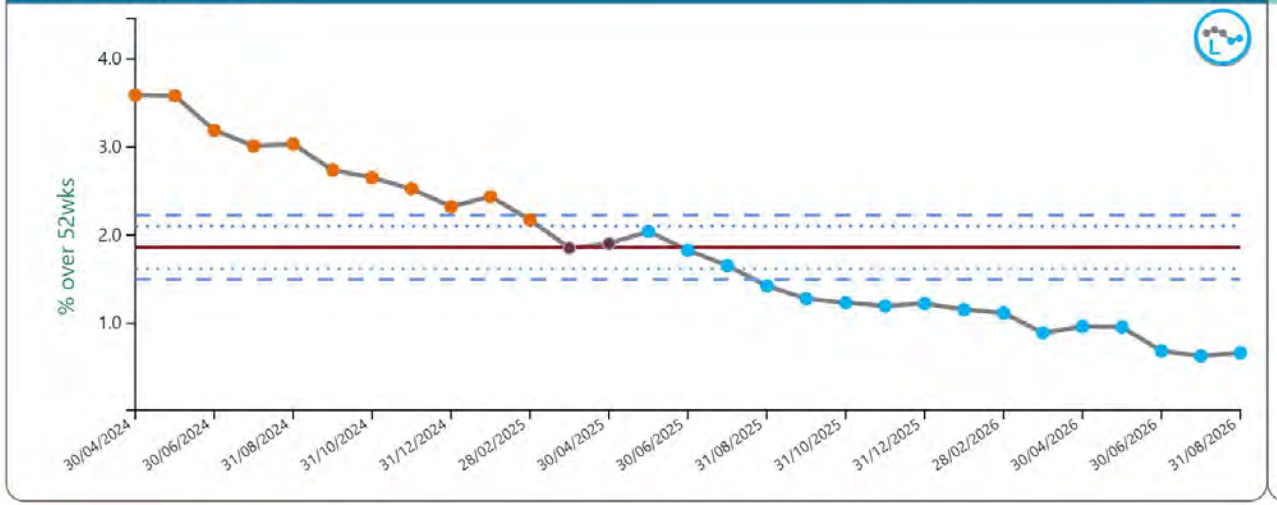
Domain: *Access to Services*

Update frequency: Monthly

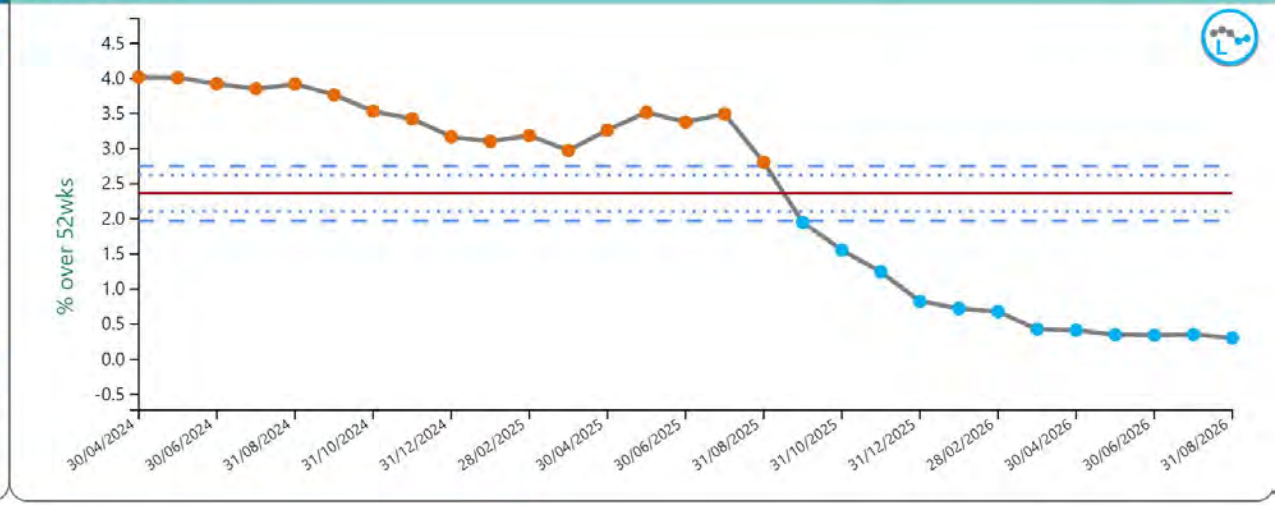
Oversight Framework

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping		Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB		0.96%	0.96%	0.68%	0.63%	0.66%
BSOL ICB		0.42%	0.35%	0.34%	0.35%	0.30%
Cluster		0.70%	0.66%	0.52%	0.49%	0.49%
Provider						
BC ICS	**Total**	**1.04%**	**1.03%**	**0.72%**	**0.67%**	**0.72%**
	DGFT	0.01%	0.02%	0.05%	0.04%	0.04%
	RWHT	2.58%	2.47%	1.69%	1.47%	1.49%
	SWBH	0.49%	0.56%	0.44%	0.55%	0.69%
	WHT	0.01%	0.01%	0.01%	0.05%	0.04%
BSOL ICS	**Total**	**0.39%**	**0.33%**	**0.30%**	**0.28%**	**0.25%**
	BCHC	0.01%	0.01%	0.00%	0.01%	0.00%
	BWCH	0.60%	0.50%	0.49%	0.51%	0.59%
	ROH	0.87%	0.82%	0.52%	0.50%	0.51%
	UHB	0.33%	0.26%	0.28%	0.24%	0.18%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	0.66%	
BSOL ICB	31/08/2026	0.30%	
Cluster	31/08/2026	0.49%	

16.09 (XX0009) - Total Waiting List

Return to Agenda

Report level: ICB

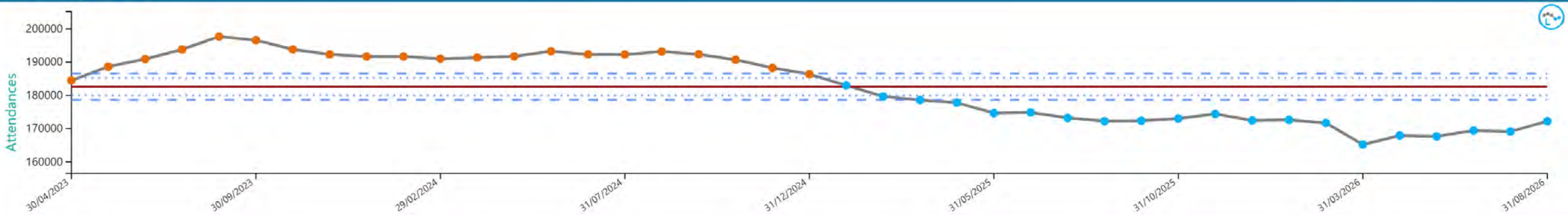
Domain: Other Planning Metrics

Update frequency: Monthly

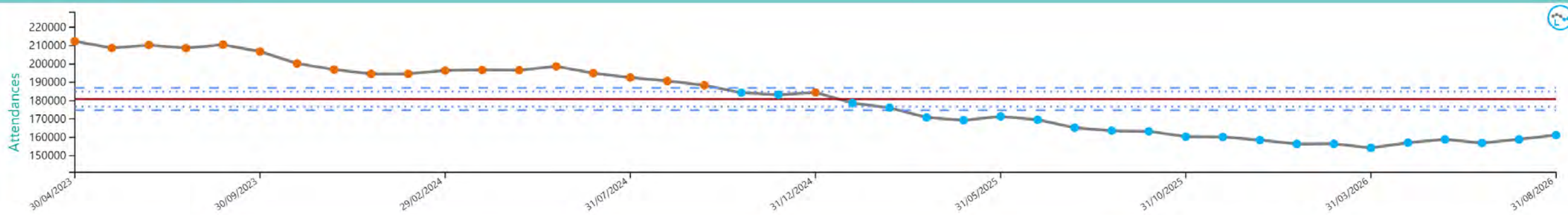
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	167,927	167,673	169,425	169,139	172,245							
	Plan	165,228	161,929	158,629	155,329	152,029	148,729	145,429	142,129	138,829	135,529	132,229	128,929
BSOL ICB	Actual	157,042	158,886	156,881	158,885	161,173							
	Plan	153,579	152,159	150,738	149,318	147,897	146,476	145,056	143,635	142,214	140,794	139,373	137,953
Cluster	Actual	324,969	326,559	326,306	328,024	333,418							
	Plan	318,807	314,088	309,367	304,647	299,926	295,205	290,485	285,764	281,043	276,323	271,602	266,882

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	172245	152029	L	F
BSOL ICB	31/08/2026	161173	147897	L	F
Cluster	31/08/2026	333418	299926	L	F

16.09 (XX0009) - Total Waiting List

Return to Agenda

Report level: **Provider**

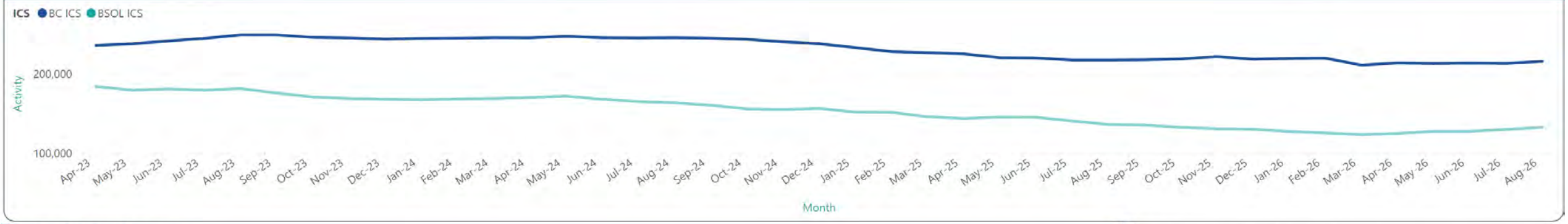
Domain: *Other Planning Metrics*

Update frequency: **Monthly**

Planning

Data up to: **Aug-26**

BBCS Provider Total - Run chart



Provider level performance

ICS	Provider		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
Total		**Actual**	**338,774**	**340,672**	**341,415**	**343,185**	**348,257**							
		Plan	**338,712**	**338,411**	**338,187**	**334,510**	**330,904**	**327,207**	**322,227**	**317,239**	**312,289**	**307,581**	**302,931**	**297,597**
BC ICS	**Total**	**Actual**	**213,820**	**213,170**	**213,766**	**213,326**	**215,629**							
		Plan	**210,888**	**208,776**	**206,663**	**204,366**	**202,070**	**199,763**	**197,074**	**194,384**	**191,645**	**188,677**	**185,708**	**182,152**
	DGFT	Actual	46,627	46,240	47,246	48,039	48,882							
		Plan	46,411	45,389	44,367	43,345	42,323	41,301	40,279	39,257	38,235	37,213	36,191	35,169
	RWHT	Actual	73,591	73,004	71,812	70,201	71,030							
		Plan	71,559	71,008	70,456	69,720	68,985	68,249	67,146	66,042	64,939	63,652	62,364	61,077
	SWBH	Actual	65,630	66,145	66,744	67,233	67,228							
		Plan	65,133	64,644	64,155	63,666	63,177	62,688	62,199	61,710	61,221	60,732	60,243	59,756
	WHT	Actual	27,972	27,781	27,964	27,853	28,489							
		Plan	27,785	27,735	27,685	27,635	27,585	27,525	27,450	27,375	27,250	27,080	26,910	26,150
BSOL ICS	**Total**	**Actual**	**124,954**	**127,502**	**127,649**	**129,859**	**132,628**							
		Plan	**127,824**	**129,635**	**131,524**	**130,144**	**128,834**	**127,444**	**125,153**	**122,855**	**120,644**	**118,904**	**117,223**	**115,445**
	BWCH	Actual	21,616	21,820	21,368	22,077	23,169							
		Plan	24,000	23,911	23,822	23,733	23,733	23,644	23,556	23,467	23,467	23,378	23,289	23,200
	ROH	Actual	12,610	12,929	13,205	13,221	13,403							
		Plan	13,096	12,971	12,856	12,808	12,741	12,682	12,621	12,554	12,484	12,407	12,328	12,253
	UHB	Actual	90,728	92,753	93,076	94,561	96,056							
		Plan	90,728	92,753	94,846	93,603	92,360	91,118	88,976	86,834	84,693	83,119	81,606	79,992

SPC Summary - Latest data point

Provider	Latest Date	Value	Plan	Variation	Assurance
BWCH	31/08/2026	23169	23733	L	?
DGFT	31/08/2026	48882	42323		F
ROH	31/08/2026	13403	12741	L	F
RWHT	31/08/2026	71030	68985	L	F
SWBH	31/08/2026	67228	63177	L	F
UHB	31/08/2026	96056	92360	L	F
WHT	31/08/2026	28489	27585	L	F

9.01 (OF0002) - % change in overall waiting list size versus previous year

Return to Agenda

ICB OvF

Domain: *Allocating Resources*

Update frequency: Monthly

Oversight Framework

Data up to: Aug-26

Performance chart



ICB / ICS / Provider performance

ICB	Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB	-5.56	-4.00	-3.11	-2.37	-0.15
BSOL ICB	-7.23	-7.28	-7.50	-3.86	-2.25
Cluster	-6.37	-5.63	-5.27	-3.10	-1.17

Provider		Apr-26	May-26	Jun-26	Jul-26	Aug-26
⊟ BC ICS	**Total**	**-3.32**	**-2.57**	**-1.79**	**-1.05**	**-0.90**
	DGFT	2.58	0.28	1.24	2.55	4.88
	RWHT	-14.44	-8.93	-9.96	-10.52	-9.16
	SWBH	0.62	0.52	1.84	5.37	0.48
	WHT	-2.05	-2.14	-0.26	-1.62	0.20
⊟ BSOL ICS	**Total**	**-10.13**	**-10.40**	**-8.73**	**-6.43**	**-9.79**
	BCHC	-9.06	-9.43	-1.33	-2.67	-6.94
	BWCH	-17.15	-21.35	-24.98	-15.55	-18.38
	ROH	-0.81	1.17	1.99	-0.90	-8.75
	UHB	-13.51	-12.01	-10.60	-6.60	-5.09

NHS Birmingham and Solihull

NHS Black Country

Diagnostics.



10.05 (OF0008) - % of diagnostic referrals waiting over 6 weeks

Return to Agenda

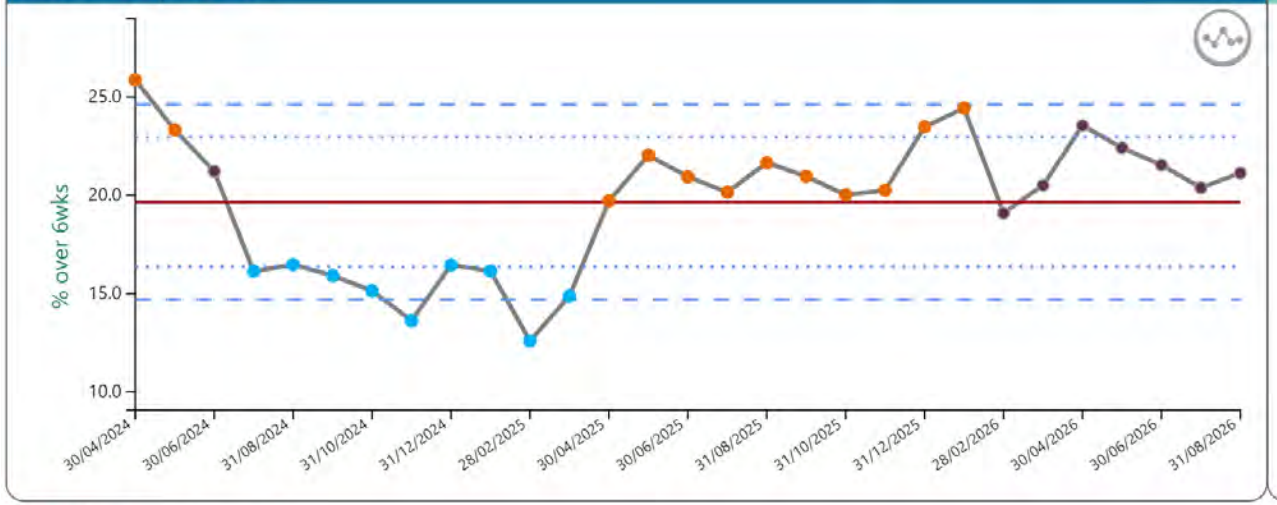
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Update frequency: Monthly

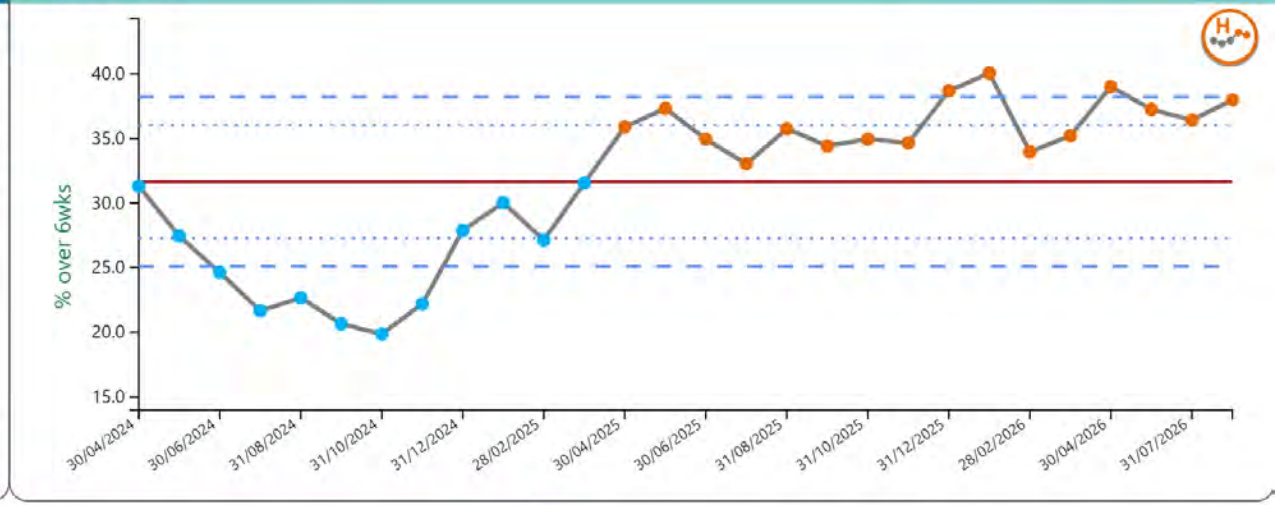
Oversight Framework / Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping		Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB		23.55%	22.41%	21.54%	20.38%	21.13%
BSOL ICB		39.02%		37.26%	36.43%	37.98%
Cluster		31.67%	22.41%	29.96%	28.89%	30.09%
Provider						
BC ICS	**Total**	**22.42%**	**19.94%**	**20.77%**	**19.79%**	**21.17%**
	DGFT	12.80%	12.31%	15.18%	18.10%	19.68%
	RWHT	4.88%	4.59%	4.79%	4.62%	4.65%
	SWBH	29.64%	29.27%	29.60%	29.22%	32.59%
	WHT	34.83%	33.09%	27.72%	21.13%	20.19%
BSOL ICS	**Total**	**41.79%**	**37.95%**	**38.99%**	**38.63%**	**39.50%**
	BWCH	36.04%	23.41%	26.68%	26.59%	24.01%
	ROH	0.32%	1.35%	0.30%	0.00%	0.11%
	UHB	42.83%	41.37%	40.67%	40.20%	41.50%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	21.13%	
BSOL ICB	31/08/2026	37.98%	
Cluster	31/08/2026	30.09%	

NHS Birmingham and Solihull

NHS Black Country

Cancer



10.06 (OF0010) - % of cases meeting the 28 day faster diagnosis standard

Return to Agenda

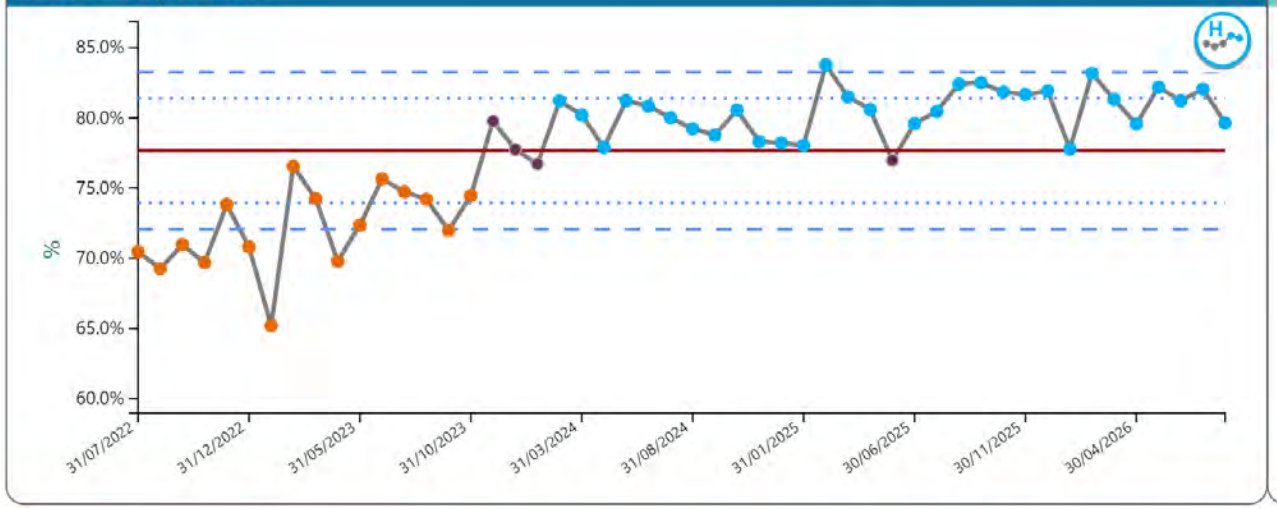
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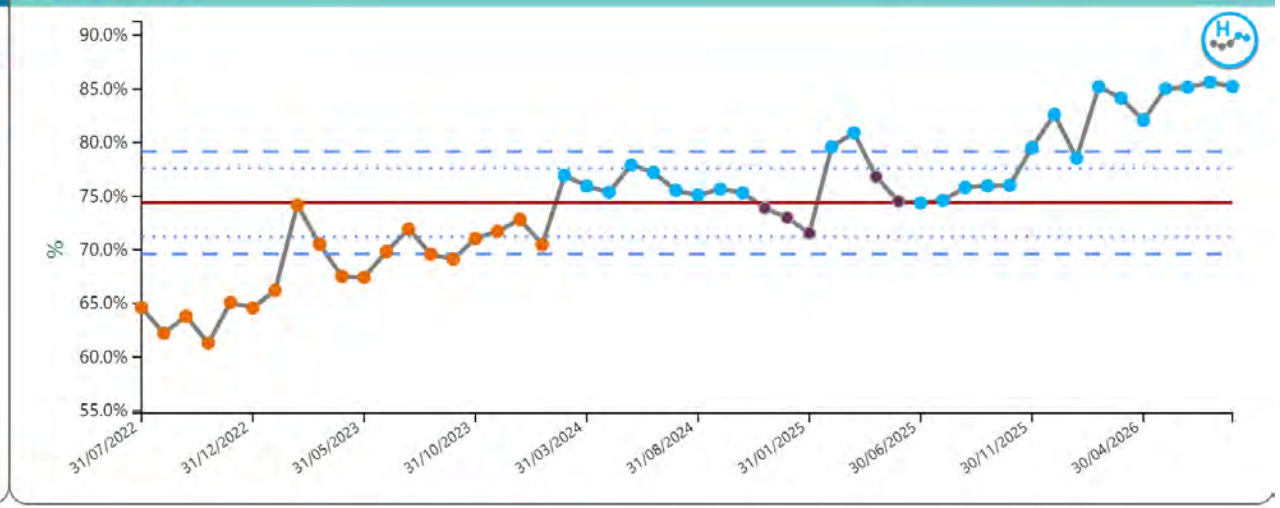
Oversight Framework / Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping		Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB		79.55%	82.18%	81.21%	82.04%	79.63%
BSOL ICB		82.11%	85.03%	85.17%	85.63%	85.24%
Cluster		80.86%	83.73%	83.21%	83.89%	82.45%
Provider						
BC ICS	**Total**	**78.94%**	**81.49%**	**81.05%**	**81.81%**	**79.63%**
	DGFT	85.87%	86.83%	83.36%	84.35%	78.05%
	RWHT	80.72%	81.60%	82.10%	80.00%	80.29%
	SWBH	63.96%	73.79%	74.38%	78.79%	75.96%
	WHT	86.14%	86.49%	88.60%	86.66%	87.04%
BSOL ICS	**Total**	**83.92%**	**86.42%**	**86.48%**	**86.24%**	**86.43%**
	BWCH	81.13%	82.46%	88.02%	83.33%	80.46%
	ROH	64.71%	63.49%	76.54%	70.37%	66.67%
	UHB	84.36%	86.96%	86.59%	86.75%	87.09%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	79.63%	
BSOL ICB	31/08/2026	85.24%	
Cluster	31/08/2026	82.45%	

10.15 (OFX001) - % of cases treated within 31 days of referral

Return to Agenda

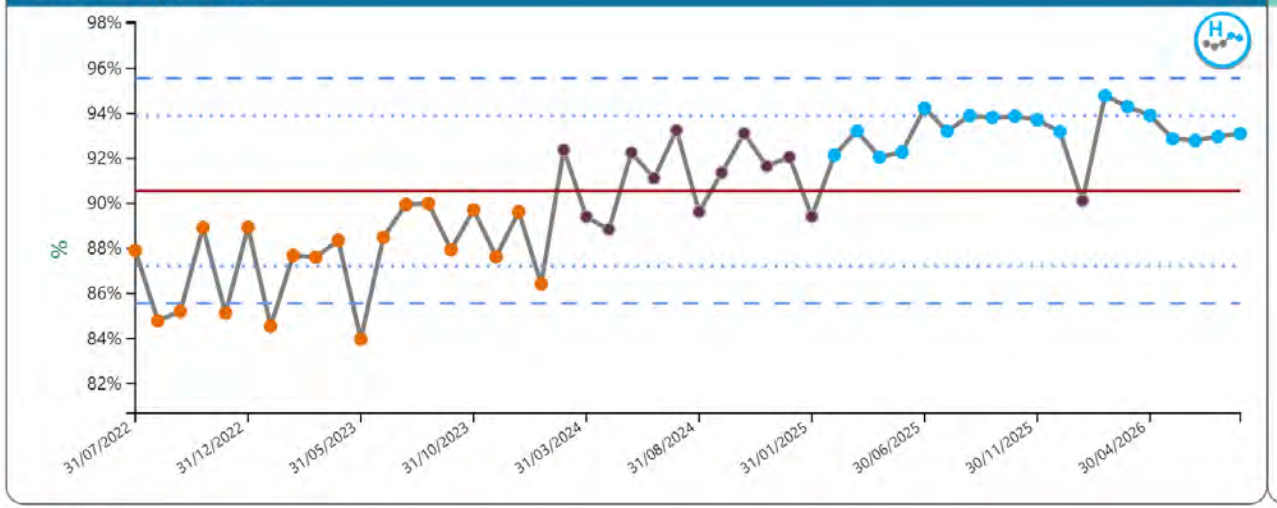
Domain: Access to Services

Update frequency: Monthly

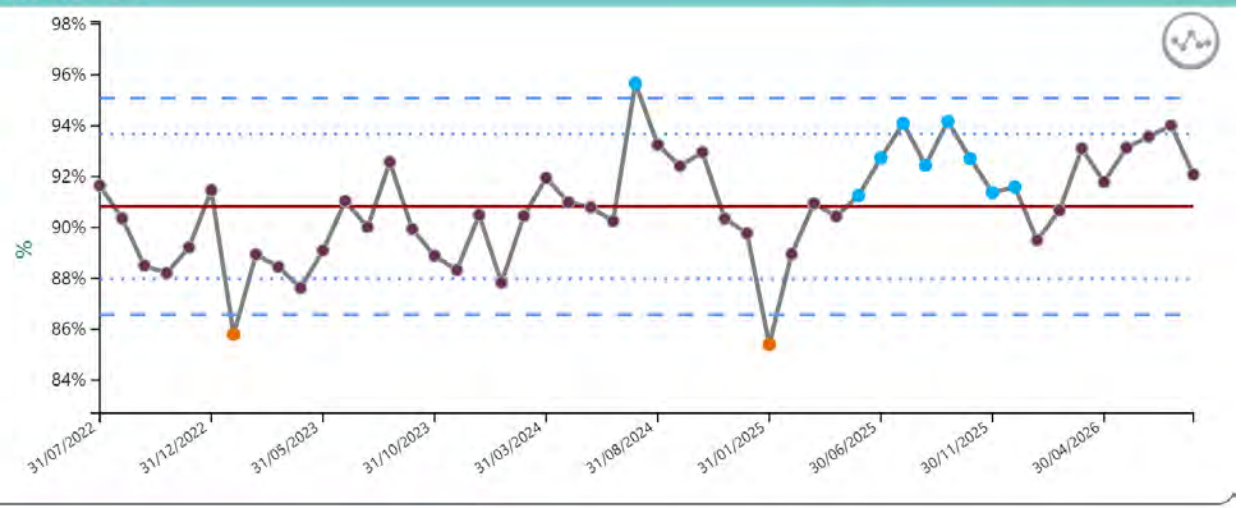
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping	Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB	93.91%	92.87%	92.79%	92.97%	93.10%
BSOL ICB	91.80%	93.14%	93.58%	94.02%	92.09%
Cluster	92.82%	93.01%	93.20%	93.49%	92.58%

Provider

⊟ BC ICS	**Total**	**93.75%**	**92.95%**	**94.44%**	**93.65%**	**93.22%**
	DGFT	94.03%	93.21%	96.44%	95.36%	94.21%
	RWHT	92.57%	91.80%	92.19%	91.85%	92.03%
	SWBH	95.74%	93.33%	97.27%	96.13%	93.94%
	WHT	98.63%	98.88%	98.10%	97.62%	97.53%
⊟ BSOL ICS	**Total**	**91.57%**	**92.70%**	**91.76%**	**92.35%**	**91.87%**
	BWCH	100.00%	100.00%	100.00%	100.00%	100.00%
	ROH	100.00%	100.00%	95.00%	100.00%	100.00%
	UHB	91.15%	92.37%	91.37%	91.85%	91.33%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	93.10%	H
BSOL ICB	31/08/2026	92.09%	
Cluster	31/08/2026	92.58%	H

10.07 (OF0011) - % of cases treated within 62 days of referral

Return to Agenda

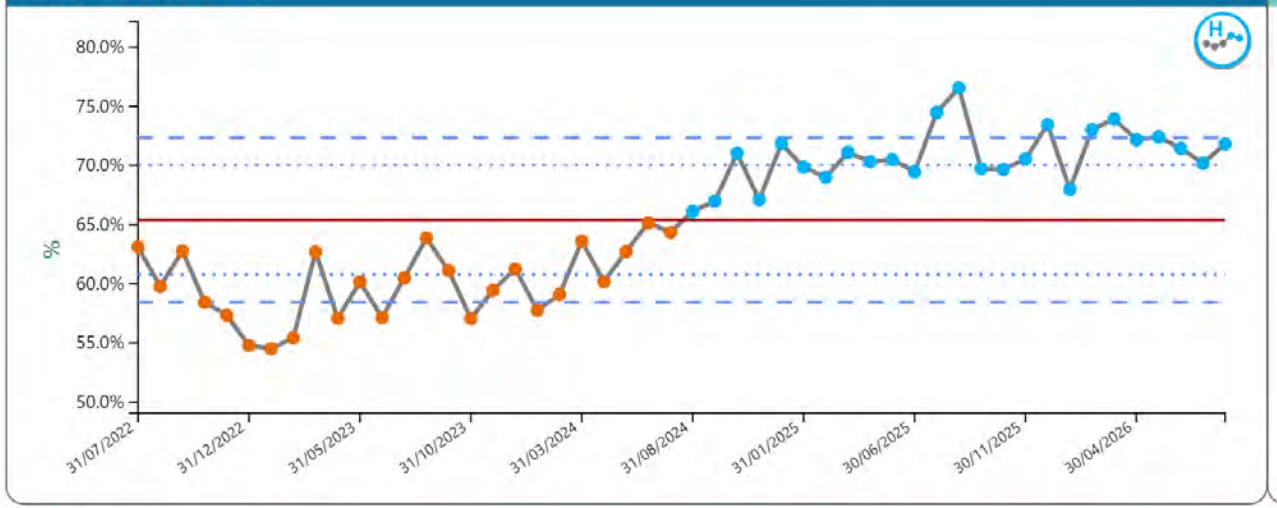
Domain: *Access to Services*

Update frequency: Monthly

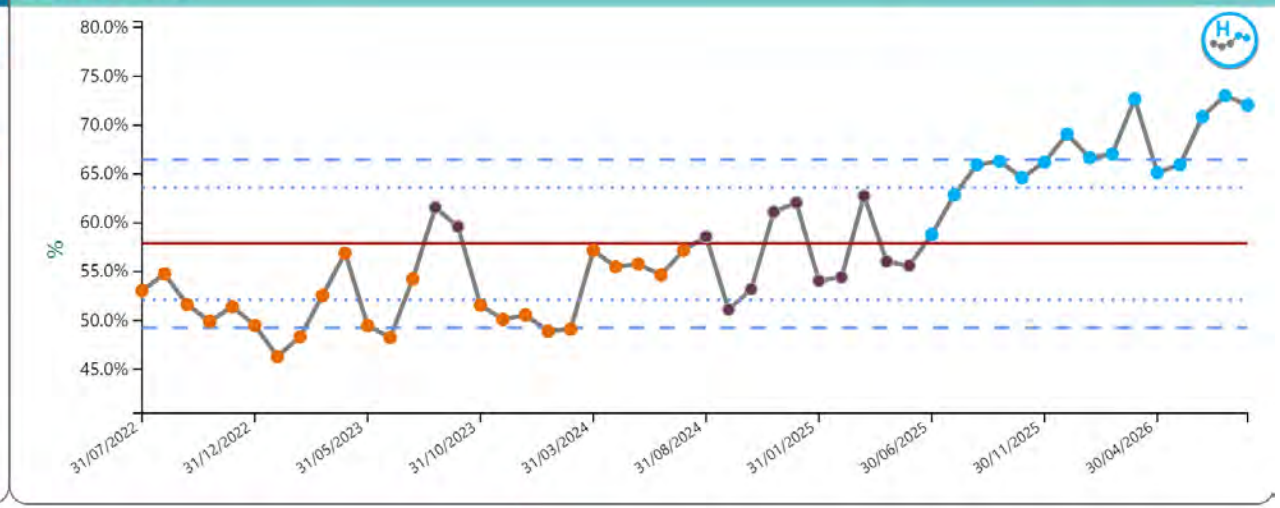
Oversight Framework / Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping		Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB		72.18%	72.41%	71.45%	70.19%	71.82%
BSOL ICB		65.13%	65.93%	70.87%	73.01%	72.06%
Cluster		68.43%	69.15%	71.14%	71.63%	71.94%
Provider						
⊟ BC ICS	**Total**	**72.10%**	**71.07%**	**72.94%**	**73.80%**	**72.65%**
	DGFT	75.43%	76.04%	76.23%	69.95%	73.37%
	RWHT	71.52%	74.06%	72.37%	72.65%	75.56%
	SWBH	70.20%	54.76%	66.28%	75.22%	64.68%
	WHT	68.85%	77.94%	80.22%	84.31%	77.04%
⊟ BSOL ICS	**Total**	**65.32%**	**67.52%**	**69.53%**	**72.35%**	**71.47%**
	BWCH	78.57%	71.43%	78.57%	100.00%	100.00%
	ROH	52.63%	36.84%	27.59%	67.74%	70.00%
	UHB	65.36%	68.00%	70.24%	71.97%	70.97%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	71.82%	H
BSOL ICB	31/08/2026	72.06%	H
Cluster	31/08/2026	71.94%	H

NHS Birmingham and Solihull

NHS Black Country

Mental Health



16.32 (XX0032) - Access to NHS talking therapies for anxiety and depression - reliable improvement

Report level: ICB

Return to Agenda

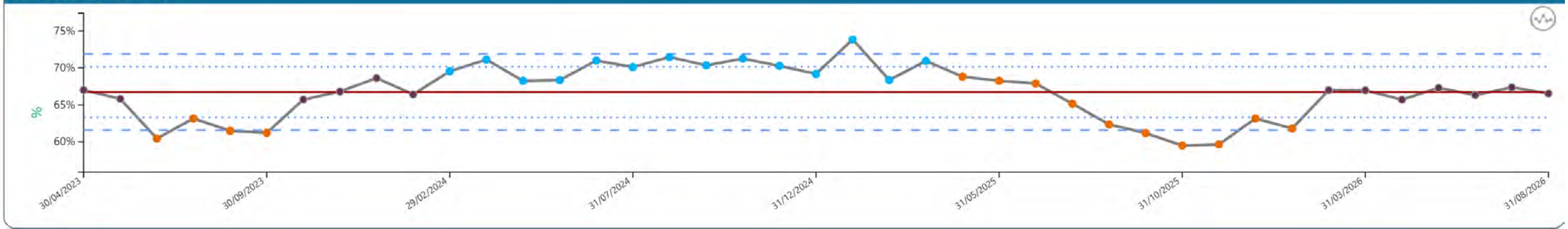
Domain: Other Planning Metrics

Update frequency: Monthly

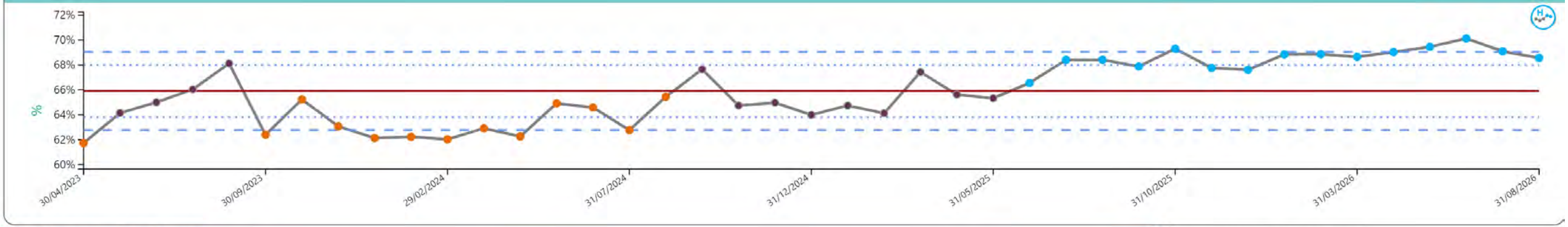
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	65.71%	67.30%	66.32%	67.36%	66.54%							
	Plan	63.99%	63.99%	66.97%	66.00%	68.00%	66.02%	66.99%	67.96%	66.95%	68.99%	66.96%	69.02%
BSOL ICB	Actual	69.03%	69.45%	70.13%	69.09%	68.57%							
	Plan	69.02%	69.02%	69.02%	69.02%	69.02%	69.02%	69.02%	69.02%	69.02%	69.02%	68.87%	68.87%
Cluster	Actual	67.67%	68.64%	68.50%	68.28%	67.64%							
	Plan	66.90%	66.80%	68.14%	67.59%	68.55%	67.78%	68.08%	68.54%	68.15%	69.01%	68.07%	68.93%

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	66.54%	68.00%		
BSOL ICB	31/08/2026	68.57%	69.02%	H	
Cluster	31/08/2026	67.64%	68.55%	H	

12.03 (OF0006) - % of NHS Talking Therapies patients completed a course of treatment and achieving reliable recovery

Return to Agenda

ICB OvF

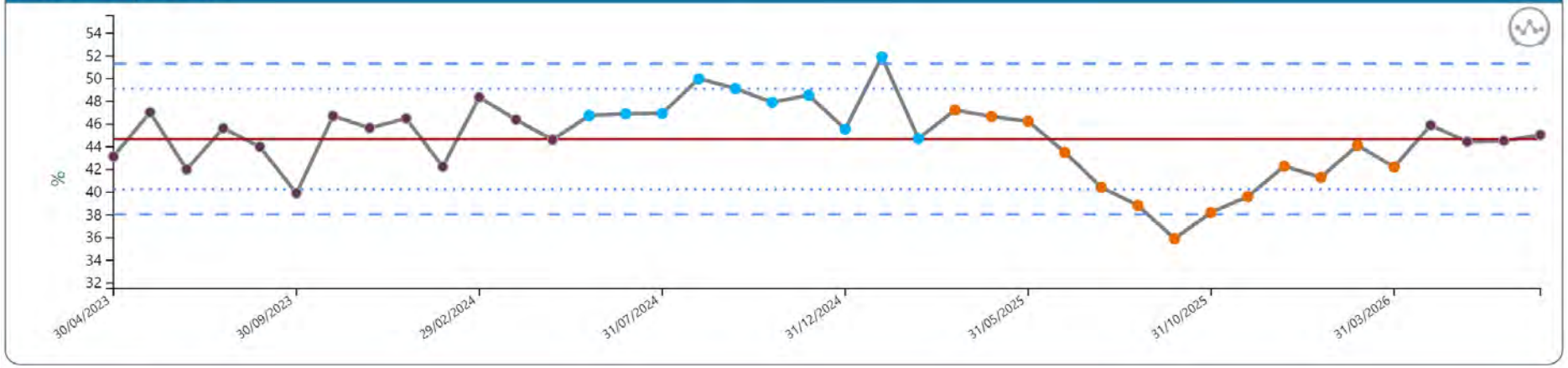
Domain: *Effectiveness of Care*

Update frequency: Monthly

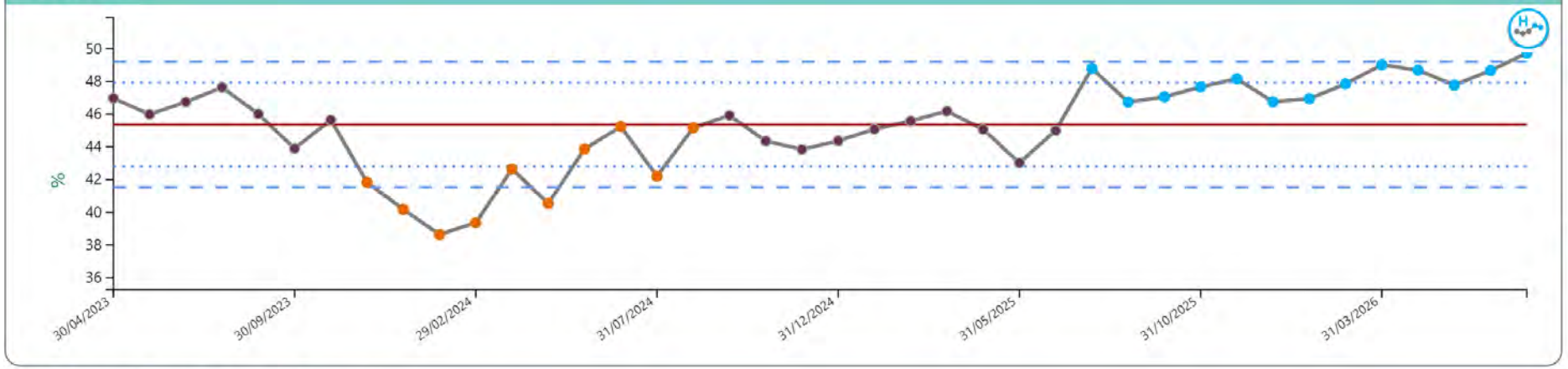
Oversight Framework

Data up to: Jul-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Apr-26	May-26	Jun-26	Jul-26
BC ICB	45.90	44.44	44.52	45.05
BSOL ICB	48.69	47.77	48.67	49.73
Cluster	47.53	46.51	46.89	47.52

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/07/2026	45.05	
BSOL ICB	31/07/2026	49.73	
Cluster	31/07/2026	47.52	

16.23 (XX0023) - Access to Children and Young People Mental Health Services

Return to Agenda

Report level: ICB

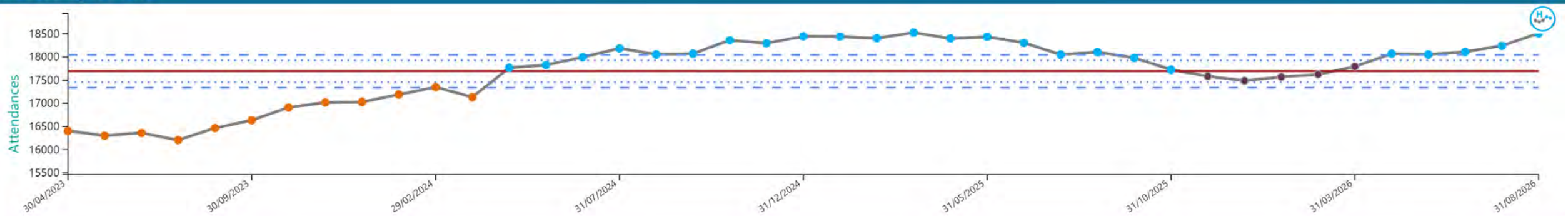
Domain: Other Planning Metrics

Update frequency: Monthly

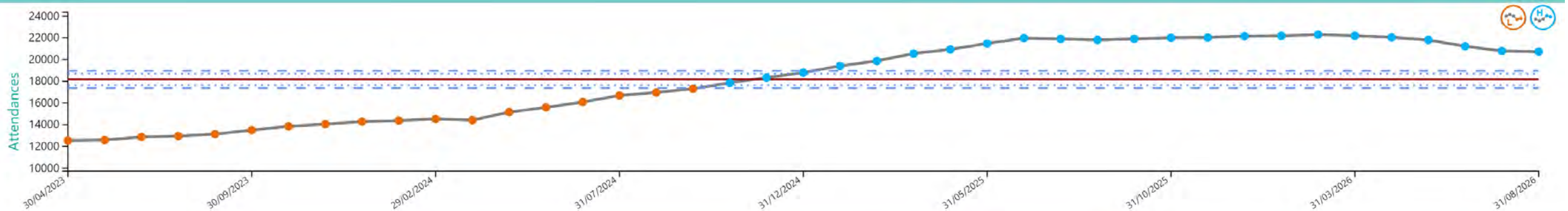
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	18,070	18,055	18,110	18,240	18,510							
	Plan	17,900	18,090	18,285	18,485	18,765	18,965	19,160	19,350	19,640	19,775	19,990	20,240
BSOL ICB	Actual	22,050	21,805	21,215	20,790	20,725							
	Plan	24,834	24,834	24,834	24,834	24,834	24,834	24,834	24,834	24,834	24,834	24,834	24,834
Cluster	Actual	40,120	39,860	39,325	39,030	39,235							
	Plan	42,734	42,924	43,119	43,319	43,599	43,799	43,994	44,184	44,474	44,609	44,824	45,074

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	18510	18765	H	F
BSOL ICB	31/08/2026	20725	24834	H	F
Cluster	31/08/2026	39235	43599	H	F

10.12 (OF0036) - % of total open childrens and young people mental health related waits that are over 104 weeks

Domain: *Access to Services*

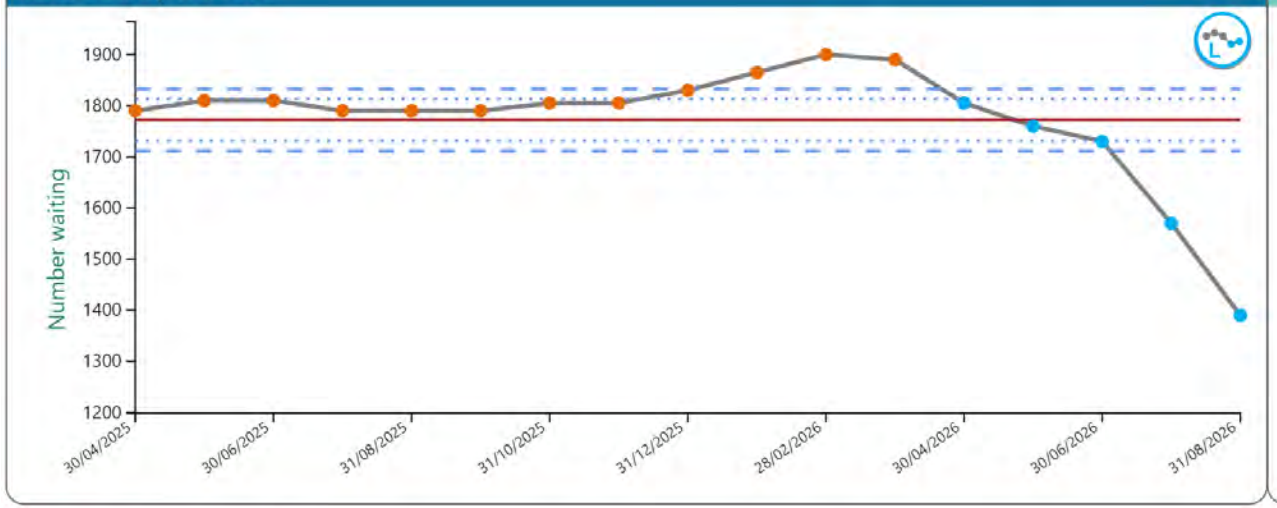
Update frequency: Monthly

Oversight Framework

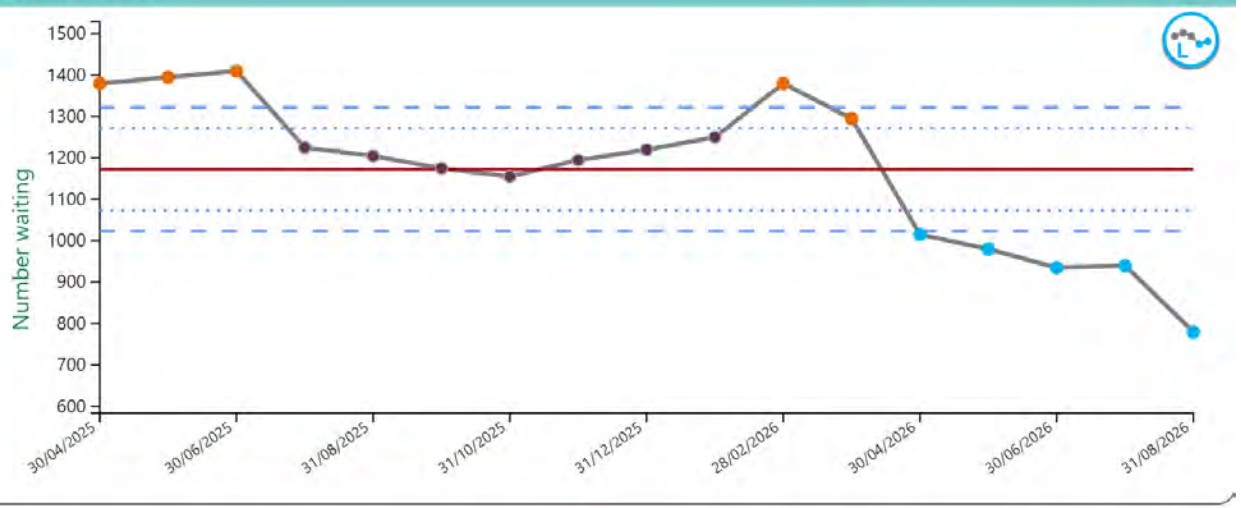
Return to Agenda

Latest Date: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping	Apr-25	May-25	Jun-25	Jul-25	Aug-25	Sep-25	Oct-25	Nov-25	Dec-25	Jan-26	Feb-26	Mar-26	Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB	1,790	1,810	1,810	1,790	1,790	1,790	1,805	1,805	1,830	1,865	1,900	1,890	1,805	1,760	1,730	1,570	1,390
BSOL ICB	1,380	1,395	1,410	1,225	1,205	1,175	1,155	1,195	1,220	1,250	1,380	1,295	1,015	980	935	940	780
Cluster	3,170	3,205	3,220	3,015	2,995	2,965	2,960	3,000	3,050	3,115	3,280	3,185	2,820	2,740	2,665	2,510	2,170

Provider

BSOL ICS	**Total**	**220**	**205**	**195**	**1,135**	**1,100**	**1,065**	**1,035**	**1,020**	**995**	**985**	**975**	**955**	**940**	**895**	**855**	**855**	**690**
	BSMHT	220	205	195	1,135	1,100	1,065	1,035	1,020	995	985	975	955	940	895	855	855	690

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	1390	
BSOL ICB	31/08/2026	780	
Cluster	31/08/2026	2170	

NHS Birmingham and Solihull

NHS Black Country

Community health wait lists



9.02 (OF0039) - Level of growth in community care contacts

Return to Agenda

ICB OvF

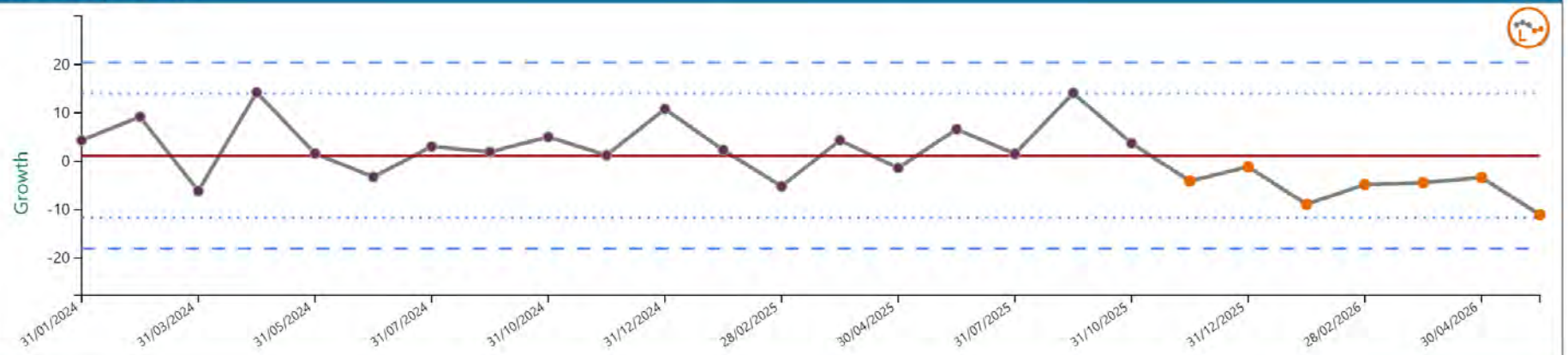
Domain
Allocating Resources

Update frequency
Monthly

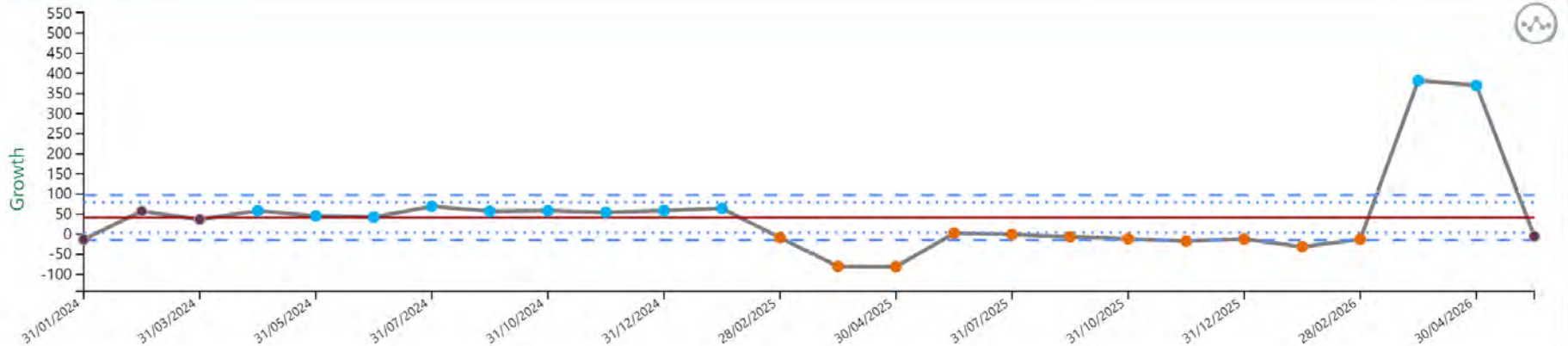
Oversight Framework

Data up to
Jun-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Apr-25	Jun-25	Jul-25	Sep-25	Oct-25	Nov-25	Dec-25	Jan-26	Feb-26	Mar-26	Apr-26	Jun-26
BC ICB	-1.36	6.63	1.54	14.09	3.72	-4.08	-1.15	-8.87	-4.80	-4.42	-3.35	-11.03
BSOL ICB	-80.56	2.77	-0.39	-6.33	-11.36	-16.96	-11.65	-31.08	-12.30	382.53	370.29	-5.05
Cluster	-45.61	4.46	0.47	3.01	-4.42	-11.11	-6.88	-21.11	-8.97	72.41	71.25	-7.73

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	30/06/2026	0.00	
BSOL ICB	30/06/2026	0.00	
Cluster	30/06/2026	0.00	

10.04 (OF0001) - % of patients waiting less than 18 weeks for community care

Return to Agenda

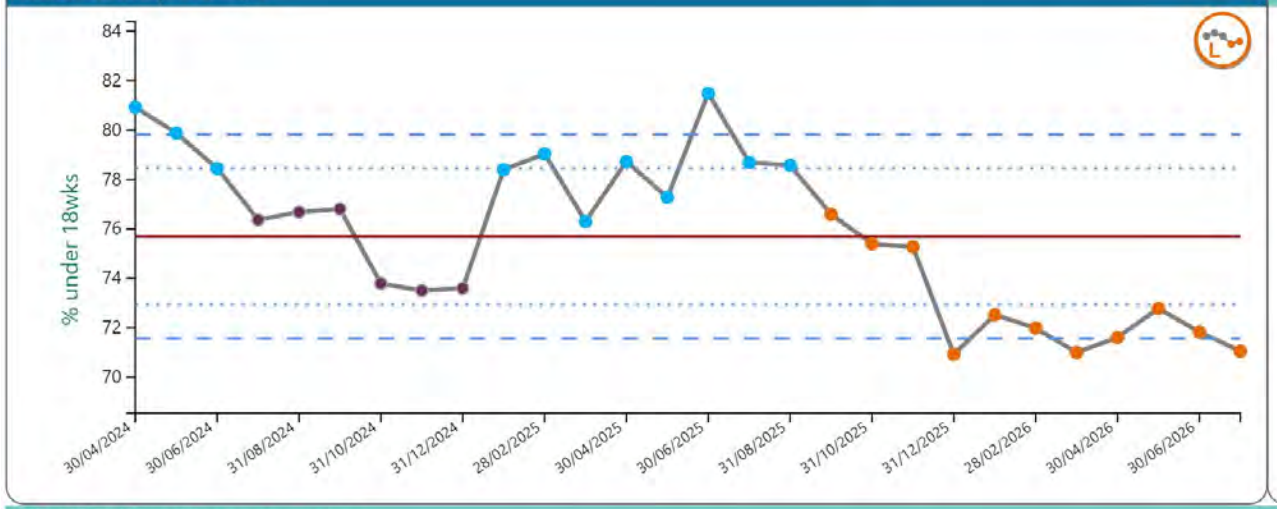
Domain: *Access to Services*

Update frequency: Monthly

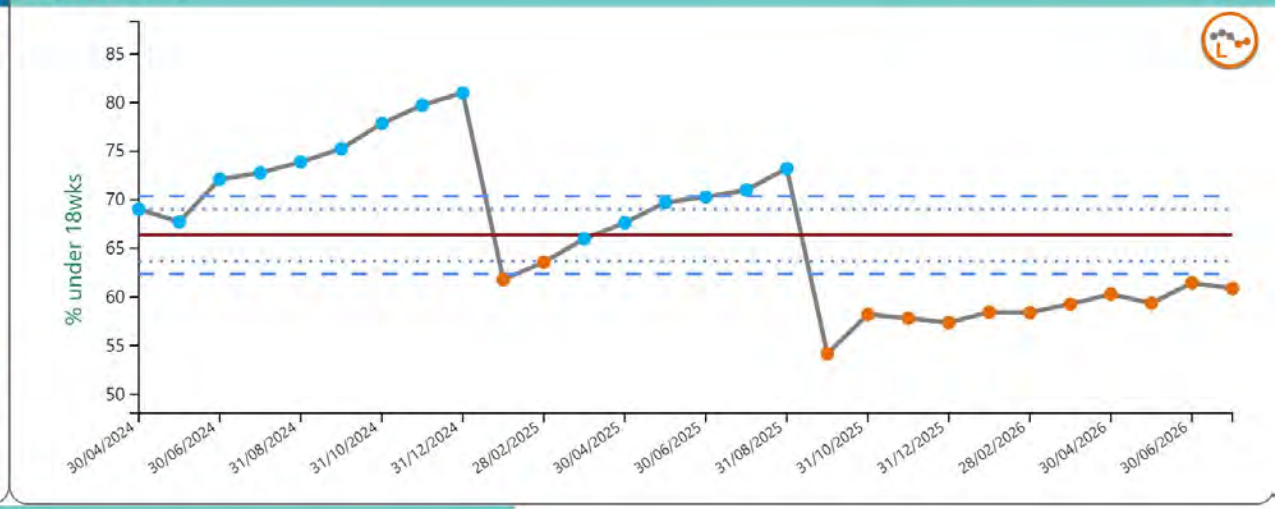
Oversight Framework

Data up to: Jul-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / ICS / Provider performance

ICB Grouping		Apr-26	May-26	Jun-26	Jul-26
BC ICB		71.61%	72.78%	71.83%	71.05%
BSOL ICB		60.30%	59.37%	61.45%	60.88%
Cluster		64.42%	64.22%	65.18%	64.39%
Provider					
BC ICS	**Total**	**71.53%**	**72.65%**	**71.76%**	**71.04%**
	DGFT	86.99%	87.66%	84.94%	85.16%
	RWHT	45.48%	48.20%	45.76%	44.40%
	SWBH	79.84%	82.76%	80.58%	82.03%
	WHT	64.35%	64.59%	65.85%	64.27%
BSOL ICS	**Total**	**60.30%**	**59.37%**	**61.45%**	**60.88%**
	BCHC	57.76%	56.90%	58.81%	58.38%
	UHB	90.12%	88.06%	91.74%	89.16%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/07/2026	71.05%	
BSOL ICB	31/07/2026	60.88%	
Cluster	31/07/2026	64.39%	

NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

Primary Care Performance Report

9.06 (OF0007) - % of clinically urgent appointments that happen on the same day

Return to Agenda

ICB OvF

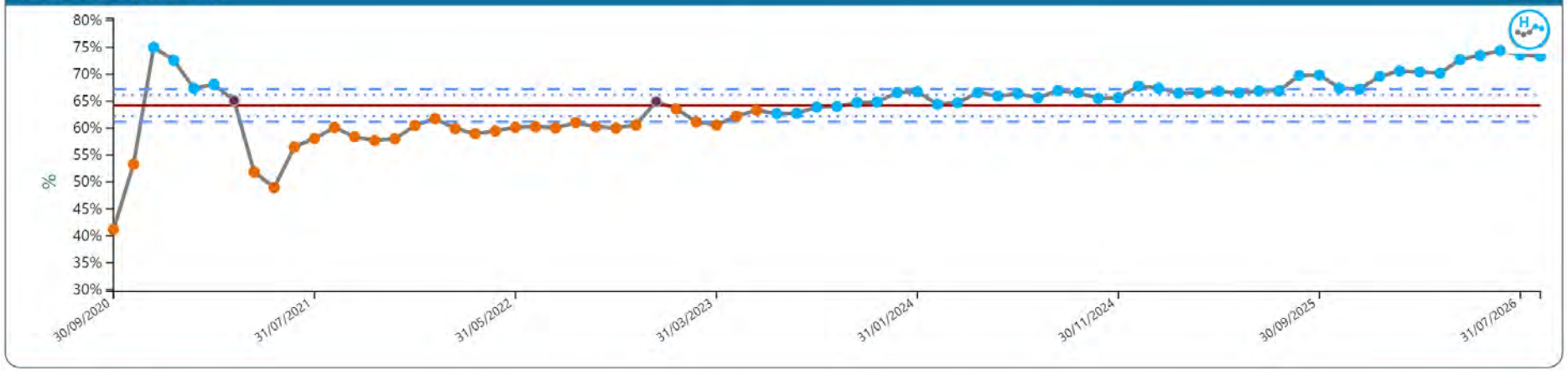
Domain
Allocating Resources

Update frequency
Monthly

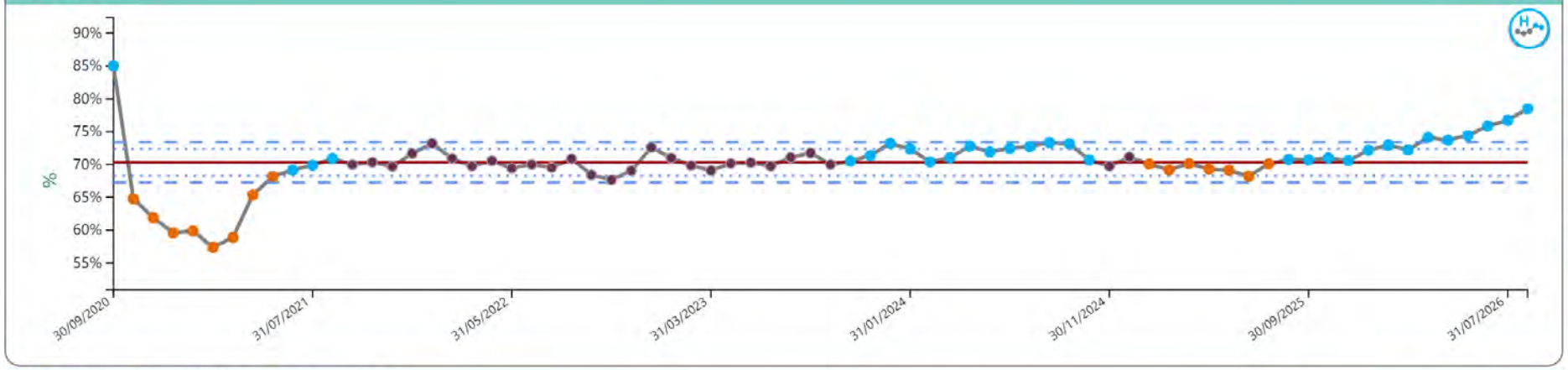
Oversight Framework

Data up to
Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Apr-25	May-25	Jun-25	Jul-25	Aug-25	Sep-25	Oct-25	Nov-25	Dec-25	Jan-26	Feb-26	Mar-26	Apr-26	May-26	Jun-26	Jul-26	Aug-26
BC ICB	66.86%	66.56%	66.92%	66.94%	69.81%	69.89%	67.42%	67.24%	69.61%	70.64%	70.46%	70.17%	72.72%	73.50%	74.40%	73.62%	73.36%
BSOL ICB	69.29%	69.12%	68.21%	70.08%	70.78%	70.70%	70.99%	70.58%	72.16%	72.93%	72.20%	74.11%	73.70%	74.39%	75.87%	76.71%	78.49%
Cluster	68.09%	67.87%	67.58%	68.56%	70.30%	70.31%	69.29%	69.01%	70.96%	71.86%	71.39%	72.28%	73.26%	73.98%	75.20%	75.32%	76.18%

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/08/2026	73.36%	H
BSOL ICB	31/08/2026	78.49%	H
Cluster	31/08/2026	76.18%	H

16.08 (XX0008) - Appointments in General Practice

Return to Agenda

Report level: ICB

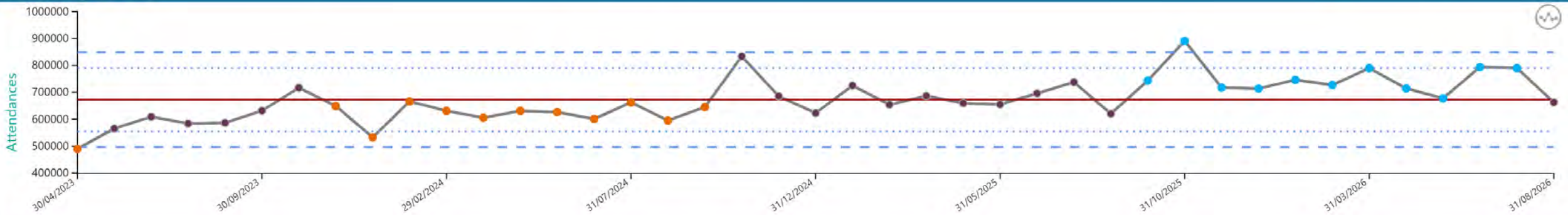
Domain: Other Planning Metrics

Update frequency: Monthly

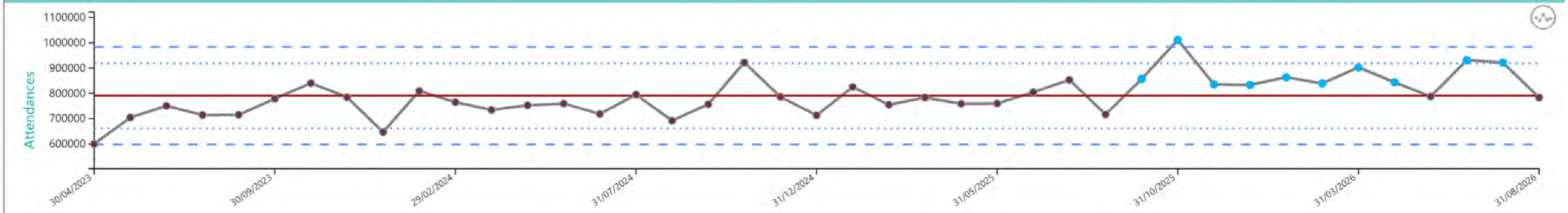
Planning

Data up to: Aug-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster level performance

ICB		Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27
BC ICB	Actual	714,373	677,555	793,942	790,193	662,825							
	Plan	670,537	637,011	737,592	770,573	670,063	737,070	737,592	704,064	704,064	670,537	670,063	703,566
BSOL ICB	Actual	843,959	787,282	931,292	921,754	783,612							
	Plan	792,257	751,011	869,227	908,604	790,091	869,100	872,291	830,255	829,668	790,119	790,091	829,595
Cluster	Actual	1,558,332	1,464,837	1,725,234	1,711,947	1,446,437							
	Plan	1,462,794	1,388,022	1,606,819	1,679,177	1,460,154	1,606,170	1,609,883	1,534,319	1,533,732	1,460,656	1,460,154	1,533,161

SPC Summary - Latest data point

ICB	Latest Date	Value	Plan	Variation	Assurance
BC ICB	31/08/2026	662825	670063		
BSOL ICB	31/08/2026	783612	790091		
Cluster	31/08/2026	1446437	1460154		

Indicator-level detail : 11.03 - % of patients rating their experience of GP access as easy

ICB OvF

Return to Agenda

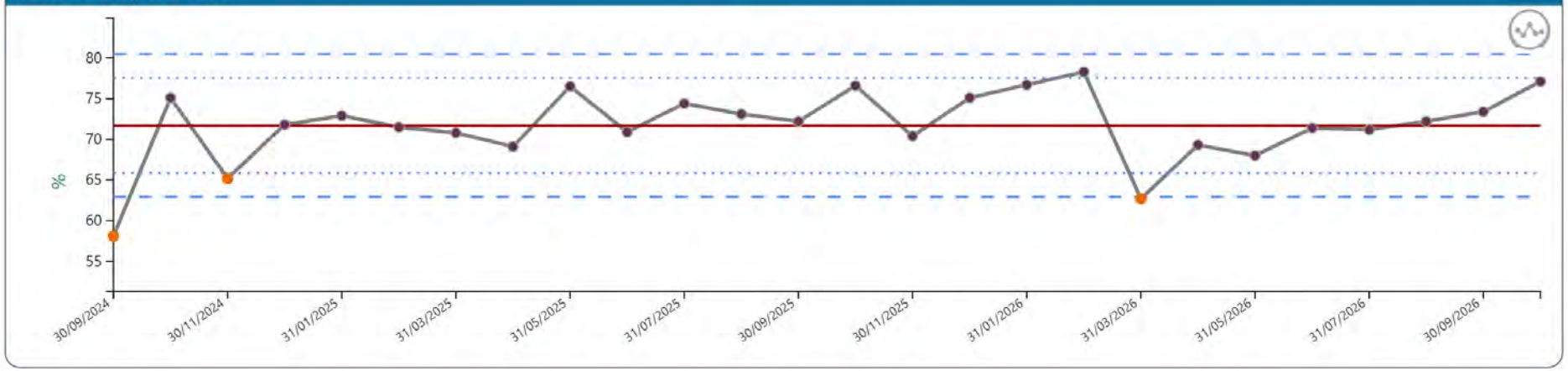
Domain
Access to Services

Update frequency
Monthly

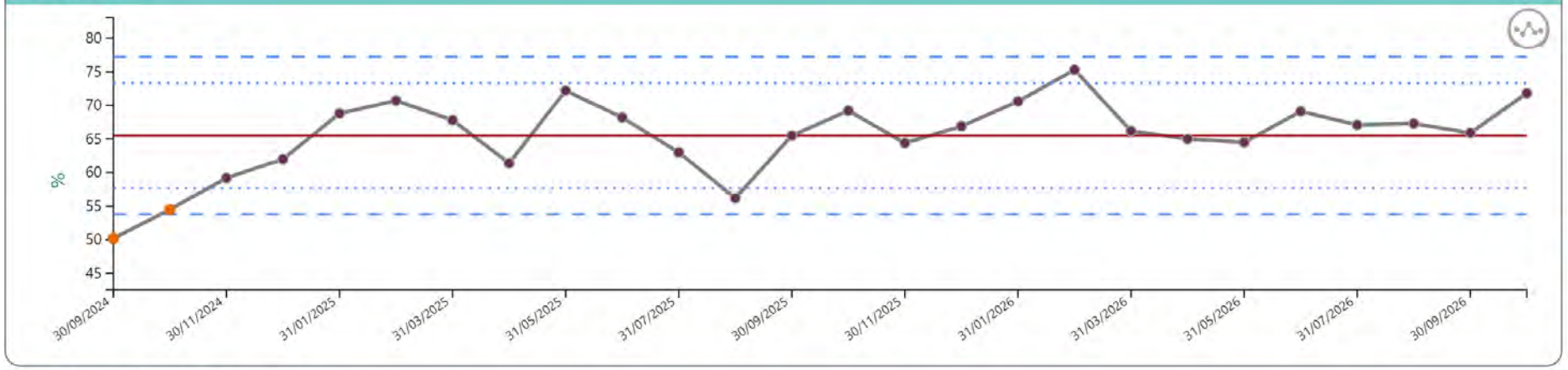
Oversight Framework

Data up to
Oct-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26
BC ICB	69.30	68.00	71.40	71.20	72.20	73.40	77.10
BSOL ICB	65.00	64.50	69.10	67.10	67.30	65.90	71.80
Cluster	66.67	65.88	69.91	68.57	69.11	68.81	73.78

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/10/2026	77.10	
BSOL ICB	31/10/2026	71.80	
Cluster	31/10/2026	73.78	

11.04 (OF0064) - % of patients with a preferred general practice professional able to see that professional

ICB OvF

Domain: *Experience of Care*

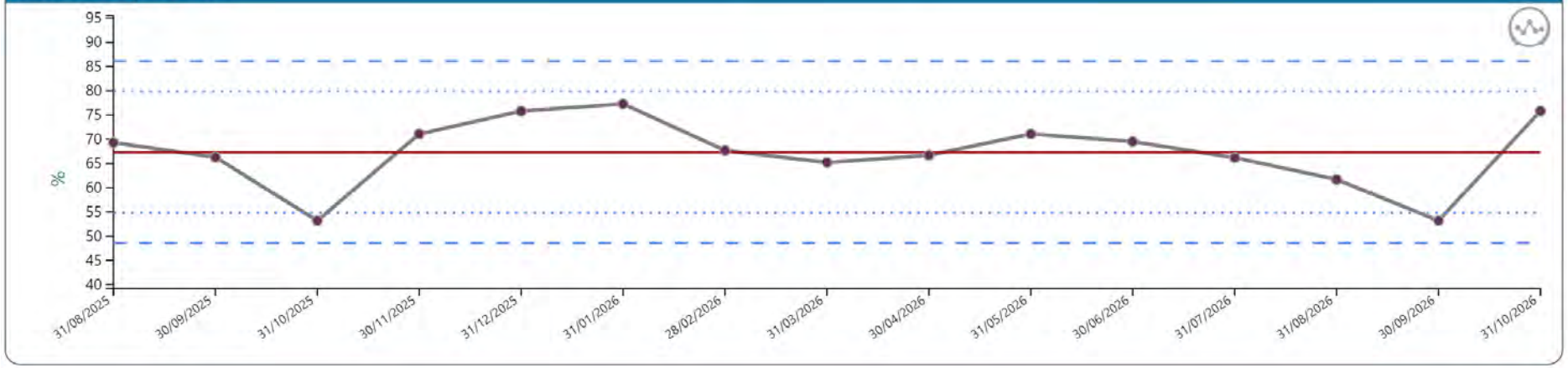
Update frequency: Monthly

Oversight Framework

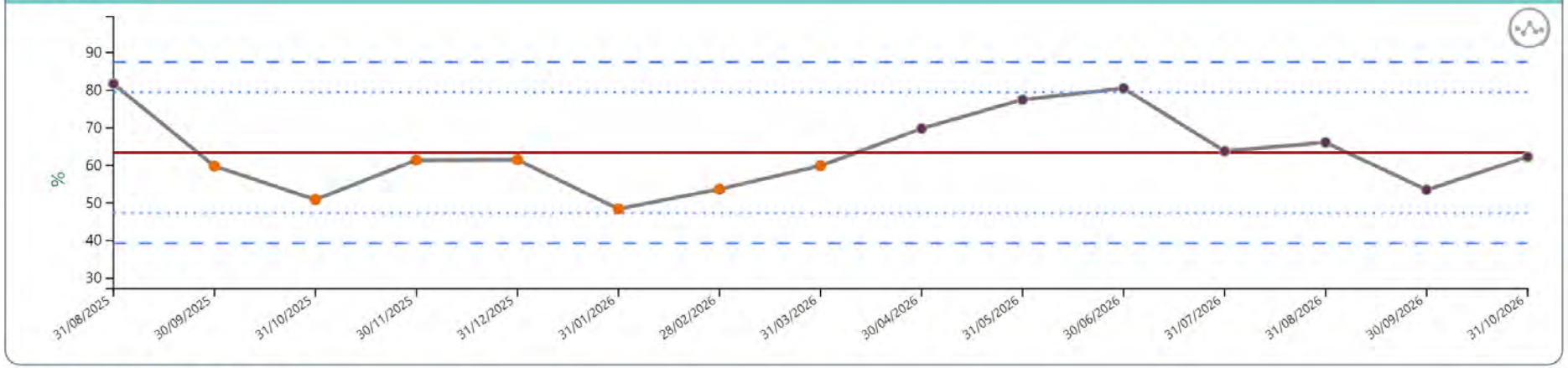
Return to Agenda

Data up to: Oct-26

Black Country ICB - SPC



BSOL ICB - SPC



ICB / Cluster / Provider level performance

ICB	Aug-25	Sep-25	Oct-25	Nov-25	Dec-25	Jan-26	Feb-26	Mar-26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26
BC ICB	69.30	66.30	53.20	71.10	75.80	77.30	67.70	65.20	66.70	71.10	69.50	66.20	61.70	53.20	75.80
BSOL ICB	81.70	59.80	50.90	61.40	61.50	48.40	53.70	59.90	69.80	77.50	80.50	63.80	66.10	53.50	62.30
Cluster	75.95	63.05	51.94	65.39	67.81	60.60	61.17	62.21	68.61	74.91	76.60	64.65	64.37	53.38	67.14

SPC Summary - Latest data point

ICB	Latest Date	Value	Variation
BC ICB	31/10/2026	75.80	
BSOL ICB	31/10/2026	62.30	
Cluster	31/10/2026	67.14	

NHS
Birmingham and Solihull

Return to Agenda

NHS
Black Country

Financial Performance 2026/27 Month 6

Paul Athey, Chief Financial Officer



Return to Agenda

NHS Birmingham and Solihull

Revenue Performance

NHS Black Country

Forecast position: Both ICBs at break-even

- As previously reported, for Black Country ICB, Spec Comm are currently showing an underspend relating to an unutilised reserve of £3.7m and we are showing assumed spend within the "Other" line (the Spec Comm line needs to balance to regional figures). All BSOL values were fully allocated within the original financial plan. Furthermore, the release of Spec Comm MH reserves to the ICB are shown as an underspend on the Spec Comm position (£1.6m for BC and £3.2m for BSOL), with the reinvestment of this resource showing as overspends within the MH line.
- For Black Country ICB, the forecast overspend within acute reflects the impact of patient not present and the net over-performance on NHS contracts both offset by the release of reserves, plus IS under-performance and release of some prior year flex. In CHC the forecast includes pressure relating to an out of area placement. For Prescribing, the £4.6m adverse pressure remains in relation to national price concessions and the impact of prescribing margins. These pressures are currently being mitigated through the release of non-recurrent flexibility.
- For BSOL ICB, the acute position also includes the impact of patient not present plus an assumed net underperformance on contracts, predominantly elective performance for some providers, where it is assumed current underperformance may not be pulled back entirely and some IS pressure. As in BCICB, CHC pressures continue, along with the pressure in prescribing of £4.3m. Release of reserves in Other are offsetting pressures.

Year to date position: BC £942k favourable, BSOL £483k favourable

- For Black Country the acute position includes underperformance on IS and NHS contracts, alongside pressures around patient not present and ambulance handover delays, offset by the release of prior year flex. As detailed above, the YTD surplus on delegated spec comm is offset by an offsetting variance in other.
- For the year-to-date position in BSOL, the acute position includes pressures relating to IS and Patient not present, offset by underspends on NHS performance and the release of prior year flex. The net underspend in acute, along with the release of flex, offsets pressures relating to ADHD/ASD, Continuing Healthcare (predominantly fast tracks) and prescribing pressures.

Revenue position as at Month 6	Black Country						BSOL					
	Annual Budget £'000	YTD Budget £'000	YTD Actual £'000	YTD Variance £'000	FOT Actual £'000	FOT Variance £'000	Annual Budget £'000	YTD Budget £'000	YTD Actual £'000	YTD Variance £'000	FOT Actual £'000	FOT Variance £'000
Allocation												
ICB Programme Allocation	2,930,574	1,465,287	1,465,287	0	2,930,574	0	3,378,835	1,689,418	1,689,418	0	3,378,835	0
Primary Medical Care Services incl POD	458,519	198,751	198,751	0	458,519	0	537,306	266,359	266,359	0	537,306	0
Running Costs	19,759	9,880	9,880	0	19,759	0	23,381	11,691	11,691	0	23,381	0
Delegated Specialised Commissioning	379,400	192,794	192,794	0	379,400	0	574,570	289,852	289,852	0	574,570	0
Total ICB Allocations	3,788,252	1,866,711	1,866,711	0	3,788,252	0	4,514,092	2,257,319	2,257,319	0	4,514,092	0
Expenditure												
Acute	1,732,008	862,304	861,730	573	1,730,999	1,009	1,920,086	960,043	957,171	2,872	1,918,152	1,934
Community Health	315,839	157,366	157,743	-377	315,728	111	357,321	178,661	178,493	167	357,005	317
Mental Health incl LD	375,815	188,354	188,478	-124	377,406	-1,591	558,866	279,433	280,982	-1,549	562,189	-3,323
Continuing Health Care	124,330	62,165	62,219	-54	126,020	-1,690	160,884	80,442	81,810	-1,368	164,531	-3,648
Primary Care (excluding Prescribing)	25,733	12,851	12,924	-73	25,798	-64	25,105	12,549	12,451	98	24,907	198
Prescribing	281,018	138,052	138,055	-3	285,569	-4,551	285,485	142,743	144,038	-1,296	289,876	-4,391
Other	75,831	17,147	18,377	-1,229	74,433	1,398	76,188	38,100	36,749	1,351	70,497	5,691
Primary Medical Care Services	298,810	149,402	149,401	0	298,810	0	362,412	181,203	180,761	442	362,412	0
Delegated Primary Care (POD)	159,709	78,285	77,944	342	159,706	3	174,894	85,155	85,388	-234	174,895	-1
Delegated Specialised Commissioning	379,400	190,905	189,017	1,888	374,025	5,375	569,470	287,302	287,302	0	566,246	3,224
Running Costs	19,759	9,880	9,880	0	19,759	0	23,381	11,691	11,691	0	23,381	0
Total ICB Expenditure	3,788,252	1,866,711	1,865,769	942	3,788,252	0	4,514,092	2,257,319	2,256,836	483	4,514,092	0
Surplus / (Deficit)	0	0	942	942	0	0	0	0	483	483	0	0

Return to Agenda

NHS Birmingham and Solihull

Efficiencies

NHS Black Country

Efficiency Position

- For 2026/27, the BC ICB efficiency plan is £97.182m and the BSOL ICB efficiency plan is £130.629m.
- Table 1 summarises the update for Month 6, with full delivery of efficiency still forecast for both Black Country ICB and BSOL ICB.

Table 1:Efficiency Position

Efficiencies	Black Country						BSOL					
	YTD Plan £'000	YTD Actual £'000	YTD Variance £'000	Annual Plan £'000	FOT £'000	FOT Variance £'000	YTD Plan £'000	YTD Actual £'000	YTD Variance £'000	Annual Plan £'000	FOT £'000	FOT Variance £'000
Recurrent	39,629	48,430	8,801	79,581	97,181	17,600	65,080	64,534	-546	130,629	130,629	0
Non-Recurrent	8,802	1	-8,801	17,601	1	-17,600	0	0	0	0	0	0
Total Efficiencies	**48,431**	**48,431**	**0**	**97,182**	**97,182**	**0**	**65,080**	**64,534**	**-546**	**130,629**	**130,629**	**0**

Efficiency Status

- **Delivery Status**: Since the initial plan submission, all Black Country efficiency plans have been fully developed. This includes the CHC and prescribing initiatives, which operational teams have reviewed to meet the savings requirements. As a result of this review, the risk rating appetite for these schemes has decreased from high to medium. In BSOL all schemes are now fully developed, except for £1.4m prescribing. This reflects the additional stretch efficiency target applied to MMO budgets. Further opportunities for savings continue to be explored.
- **Risk Status**: There remains £3.4m of high-risk schemes in BSOL at M6. £1.4m of prescribing, and £2.0m CHC Solihull small homes scheme. There is shortfall in delivery across several of the CHC schemes which are in part mitigated within overall CHC budgets with the balance of mitigation of circa £1.4m from ICB reserves.
- **Oversight** in-year will be managed through BAU monthly monitoring as part of financial management processes with escalations put in place if delivery goes off track.
- Further detail at a scheme level is provided on the next 2 slides

Table 2:Efficiency Status

Efficiency Status	Black Country			BSOL		
	Plan £'000	Forecast £'000	Variance £'000	Plan £'000	Forecast £'000	Variance £'000
Risk						
High Risk	19,808	0	-19,808	10,250	3,357	-6,893
Medium Risk	18,467	38,275	19,808	22,894	13,239	-9,655
Low Risk	58,907	58,907	0	97,485	114,033	16,548
Total Efficiencies	**97,182**	**97,182**	**0**	**130,629**	**130,629**	**0**
Delivery Status						
Fully Developed	64,890	97,182	32,292	97,338	129,260	31,922
Plans in Progress	12,151	0	-12,151	23,041	1,369	-21,672
Opportunity	20,141	0	-20,141	10,250	0	-10,250
Unidentified	0	0	0	0	0	0
Total Efficiencies	**97,182**	**97,182**	**0**	**130,629**	**130,629**	**0**



Return to Agenda

NHS Birmingham and Solihull

Key Risks and Mitigations

NHS Black Country

The net risk in month 6 remains at a risk of £3.1m for BSol ICB and £2.6m for BC ICB. The table below includes a £5m reduction to the mitigations reported in the month 6 IFR for BC ICB, to correct an error where a £2.4m net mitigated position was reported.

Description of risk

Efficiency risk – stratified risk based on an assumed 100% delivery for fully developed efficiency schemes which are in delivery, 75% for fully developed efficiency schemes which have yet to start, 50% where schemes have plans in progress, 25% for schemes which have been identified as an opportunity and 0% for unidentified.

Prescribing price and volume volatility –This risk now reflects the YTD price concession issues noted earlier in the report.

CHC volume and price volatility –Current risk reflects known issues with eg fast track volumes and risk that current interventions will not impact sufficiently to maintain expenditure within current forecasts

ADHD/ASD Right to Choose activity –Current forecast assumes ability to take some activity management plans to manage expected levels of activity; risk reflective of impact if this is not achieved.

Outcome of contract negotiations – noted above outstanding issues for BSOL contracts. If unsuccessful these will impact into ICB in-year contingency reserves

Pay awards/contract uplifts –The ICBs are assessing the overall impact of the CUF changes against allocations; no issues identified now for BSOL but Black Country work is just being concluded v allocations received.

Net Risk Category	Description	BSOL £m	BCICB £m	Total £m
Efficiency risk	Efficiency risk based on risk stratification	1.3	2.6	3.9
Efficiency risk	Further efficiency risk	0.3		0.3
Prescribing/CHC	Prescribing price and volume volatility	3.2	2.5	5.7
Prescribing/CHC	CHC volume and price volatility	6.1	1.5	7.6
Contract risk	Outcome of contract negotiations	2.5	0.0	2.5
Contract risk	Pay awards/contract uplifts	0.0	0.5	0.5
Addiitonal Cost Risk (capacity, pressures, winter)	ADHD/ASD Right to Choose activity	0.0	1.0	1.0
Total risk		**13.5**	**8.1**	**21.6**
Less Mitigations		-10.4	-5.5	-15.9
Net Risk		**3.1**	**2.6**	**5.7**



Return to Agenda

NHS Birmingham and Solihull

Capital

NHS Black Country

Capital Plan

The two ICBs have specific ICB capital plans which predominantly will be used for Primary Care improvements.

The total allocations for the ICBs are shown in the table and include £671k of in year allocations relating to additional roles reimbursement (ARRS).

At this stage of the year the expectation is that the overall allocation will be fully spent.

Capital Performance

The variances to forecast plan are relate to the spend relating to ARRs, due to a technical issue, as the additional allocation received by each ICB isn't reflected in the BAU/Strategic Capital performance table. This is a presentational issue within the IFR report. The ARRs funding relates to additional GPIT hardware refresh for those roles. The transfer to WMAS for made in M06 for £5m for patient transport capital funding.

Capital Risks

No capital risks identified to date

Table 1: Capital Plan

Capital Spend	Area	Business As Usual £000's	Strategic Funding £000's	Total BAU & Strategic £000's	UMF £000's	Total Start Point £000's	In year Allocations £000's	Month 06 Funding £000's
Black Country	Estates		495	495	1,250	1,745		1,745
	IT	2,151	1,606	3,757		3,757	301	4,058
Total Black Country		**2,151**	**2,101**	**4,252**	**1,250**	**5,502**	**301**	**5,803**
BSOL	Estates	1,100	1,400	2,500	1,500	4,000		4,000
	IT	1,150	2,417	3,567		3,567	370	3,937
Total BSOL		**2,250**	**3,817**	**6,067**	**1,500**	**7,567**	**370**	**7,937**
Total Capital		**4,401**	**5,918**	**10,319**	**2,750**	**13,069**	**671**	**13,740**

Table 2: BAU/Strategic Capital Performance

	Year to Date			Forecast		
Capital	Plan £000's	Actual £000's	Variance £000's	Plan £000's	Actual £000's	Variance £000's
Black Country ICB	-	-	-	4,252	4,553	- 301
Birmingham and Solihull ICB	1,658	-	1,658	6,067	6,437	- 370
Total Capital	1,658	-	1,658	10,319	10,990	- 671



NHS Birmingham and Solihull

Delivery of Standards

NHS Black Country

Delivery of Standards

Both ICBs are reporting:

- delivery of the Mental Health Investment Standard for core ICB MH, however the reduced spend in Specialised Commissioning following the forecast release of prior year benefit to ICBs has resulted in both ICBs showing an under-delivery on the Specialised Commissioning MHIS. Discussions at the Specialised Commissioning group have suggested that the guidance may allow the exclusion of non-recurrent spend/benefit from the MHIS actuals and, as such, this will be explored by the Specialised Commissioning team.
- delivery within running cost allocations



NHS Birmingham and Solihull

NHS Black Country

Agenda No: 14		
Report to:	Joint ICB Cluster Board	
Date of Meeting:	9 November 2026	
Title:	Joint Transition Committee Assurance Report: September and October 2026	
Presenting Officer:	Harry Hayer, Chair of Transition Committee	
Author(s) of Report:	Peter McKenzie, Associate Director of Governance and Corporate Affairs	
To be Withheld from Publication?	**Yes** ☐ **No** ☐	**(If Yes) Rationale:**
Relevant Conflicts of Interest	None Identified	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☐	Digital Transformation	☐	Treatment to Prevention	☐

Related ICB Aims	Check box
Improve outcomes in population health and healthcare	☐
Tackle inequalities in outcomes, experience and access	☐
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☒

ICB Focus					
Delivery	☐	**Transformation**	☐	**Transition**	☒

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☐	**Digital First**	☐
Financial Discipline/ 2% productivity	☒	**Advancing Health Equity**	☒	**Integrated Partnership**	☐

Purpose:
To provide the Joint ICB Cluster Board with the final assurance and escalation report from the Joint Transition Committee, covering business considered on 10 September and 8 October 2026; to confirm the closure and handover position; and to seek approval of the establishment and draft Terms of Reference of the Joint Remuneration and People Committee.

Decision Required			
Approval	☒	**Accept as Assurance**	☒



Return to Agenda

Endorsement	☐	**Noted**	☐

Report Journey:

The Report details matters considered at the Committee's meetings in September and October 2026

Executive Summary

The Committee received a closure report that confirmed that, whilst some activity still remained to be completed the Transition Programme has delivered its planned objectives and recommended closure of the formal Transition Programme and Transition Programme Board.

In order to maintain oversight of the remaining activity, the Committee is recommending that a Joint Remuneration and People Committee is established. This will take oversight of the remaining actions from the management of change process and maintain ongoing oversight of the ICB's broader staffing arrangements including workforce stabilisation, equality monitoring, and alignment of HR policies. Other elements that are still outstanding from the transition programme including oversight of financial arrangements (particularly CSU stranded costs), transfer of staff to the Office of Pan-ICB Commissioning (OPIC) and preparation for a future ICB merger will transfer to the appropriate ICB committees as required for ongoing oversight.

Appendices

Appendix 1: Terms of Reference - Joint Remuneration and People Committee.

Recommendations & Next Steps

The Board is asked to:-

- **Note the closure of the formal Transition Programme and the handover position reported by the Joint Transition Committee.**
- **Approve the establishment of the Joint Remuneration and People Committee and the Terms of Reference set out in Appendix 1.**

Assessments:

Impact Assessments Completed		**Yes**	**No**	**N/A**	**Details (N= explain why: Y= approval date)**
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	No critical operational failure preventing staff from undertaking their roles was identified at Day One. Continuing workforce resilience, service-transfer and operational matters require oversight through successor routes.
Legal and Regulatory	The Committee received assurance on Management of Change governance and equality duties, but accepted only partial assurance on specific EQIA outcomes relating to disabled staff pending further analysis and monitoring. Transfer and employment obligations remain subject to continuing governance.
Finance	CSU stranded costs and OPIC funding/hosting arrangements remained uncertain. Relevant financial exposure is to continue through Resource Allocation and Payer Committee arrangements.
People	Workforce stabilisation, retention, capability, wellbeing, staff experience, residual redeployment and equality monitoring remain active matters. JTC001 and JTC002 were approved for transfer to the proposed Remuneration and People Committee.
Reputational	Clear closure, ownership and handover are necessary to maintain confidence. The Committee required receiving owners to accept transferred responsibilities and asked the Board report to recognise colleagues' contribution.



Return to Agenda

JOINT TRANSITION COMMITTEE ASSURANCE REPORT
9 November 2026

1. Introduction

This report summarises the material assurance, risks, actions and decisions recorded by the Joint Transition Committee at its meetings on 10 September and 8 October 2026. The Committee has endorsed a closure for report for the Transition Programme and is recommending that the committee be stood down and replaced with a Joint Remuneration and People Committee that will have responsibility for oversee of the ICBs' workforce responsibilities.

2. ALERT- Matters of Concern or Escalation, and gaps in Assurance (Minimal or Partial Assurance)

Item	Key Risks, Issues and Opportunities	Recommendations and Agreed Actions
Equality outcomes	The October Equality Impact Assessment found no evidence of systemic discrimination from the available analysis, but the Committee accepted only partial assurance on the evidence provided in relation to disability. It was recognised that further data was required to fully understand the impact on this group of staff.	Complete end-point analysis of staff who left, continue disability and ethnicity monitoring, review reasonable adjustments and staff experience, and incorporate learning into future change. This action will be picked up by the new Joint Remuneration and People Committee
Financial and external dependencies	The Committee was only able to take partial assurance on the financial impact of CSU stranded costs and financial allocation/hosting costs in relation to OPIC. Provision has been made for the worst-case position, but further detail is still awaited from external organisations to fully understand these costs.	Continue financial oversight through Resource Allocation and Payer Committee who will escalate material exceptions if required.

3. ADVISE- Actions Commissioned and Progressed (For Information)

Item	Assurance Obtained	Further Recommendations and Agreed Actions
Risk transition	The Committee reviewed the risks on its risk register and suggested that two risks were transferred to be picked up by the proposed Remuneration and People Committee, closed a risk relating to financial savings from the transition as this overlapped with a similar risk held by the Resource Allocation and Payer Committee; and closed a risk relating to the a potential ICB merger now it has been confirmed that this is delayed.	Update the Board Assurance Framework to reflect revised ownership and closure arrangements; receiving committees to confirm ownership, review dates, actions and reporting.

Return to Agenda

4. ASSURE- Positive assurances to provide (Reasonable or Significant Assurance)

Item	Assurance Obtained Including Assurance Source	Level of Assurance	Further Recommendations/ Agreed Actions
Programme delivery and operational continuity	The committee reviewed progress against the agreed gateway framework which provided a structured evidence and decision trail that demonstrated:- • The overall transition programme had completed its progress • Day One was implemented without material operational failure preventing staff from undertaking their roles • New governance arrangements were being established and embedded • The Management of Change process was coming to a conclusion.	Significant assurance	Retain gateway evidence and closure records; continue residual stabilisation and benefits activity under successor governance.
Management of Change and workforce stability	In September, NHS England approval had enabled Phase One compulsory redundancy and Phase Two voluntary redundancy activity to progress. The programme was broadly on track, but retention, capacity, capability and staff experience required continued monitoring.	Reasonable assurance	Transfer continuing oversight of workforce stabilisation, retention, capability, wellbeing and staff experience to the proposed Remuneration and People Committee.
Equality governance across most protected groups	The Committee accepted assurance that available analysis had not identified evidence of systemic discrimination and supported continued analysis and monitoring.	Reasonable assurance overall; partial for disability as highlighted above.	Carry forward monitoring, staff experience evidence and lessons into future organisational change and People governance.

5. ASSURE- Decisions made

Item	Summary of Key Discussion	Decision
Closure of transition governance	The Committee considered the closure report, gateway evidence, residual risks and successor arrangements.	Approved formal closure of the Transition Programme and Transition Programme Board, and controlled transfer of residual risks, actions and work.
Joint Remuneration and People Committee	The Committee considered draft Terms of Reference and requested clearer Deputy Chair wording and formalised non-executive membership and substitute arrangements.	Recommended to the Joint Cluster Board establishment of a Joint Remuneration and People Committee, subject to the requested amendments.



6. Chair's Comments

The Chair, on behalf of the committee, thanked all of the leaders and staff across the ICB who had supported the transition process. The committee recognised the volume and quality of the work involved, at a period of high stress and often in the context of personal uncertainty whilst continuing to deliver the ICB's responsibilities for the population we serve.

7. Recommendations

The Board is asked to:-

- **Note the closure of the formal Transition Programme and the handover position reported by the Joint Transition Committee.**
- **Approve the establishment of the Joint Remuneration and People Committee and the Terms of Reference set out in Appendix 1.**



Return to Agenda

NHS Birmingham and Solihull

NHS Black Country

Joint Remuneration and People Committee

Terms of Reference

1. Constitution

The Joint Remuneration and People Committee (the Committee) is established by the ICB Joint Cluster Board as a Joint Sub-Committee of the Boards in accordance with NHS Birmingham and Solihull Integrated Care Board's and NHS Black Country Integrated Care Board's Constitutions and Section 65Z5 of the National Health Service Act 2006.

These Terms of Reference which must be published in the ICBs' Governance Handbooks on their websites, set out the Membership, the remit, responsibilities and reporting arrangements of the Committee and may only be changed with the approval of Joint ICB Cluster Boards, acting under authorities delegated by the two ICB Boards.

The Committee is a non-executive joint sub-committee of the Cluster Board and its members, including those who are not members of the Cluster Board and/or the ICB Boards, are bound by the Standing Orders and other policies of the ICBs.

2. Authority

The Remuneration and People Committee is authorised by the Board to:

- Investigate any activity within its terms of reference.
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the committee) within its remit as outlined in these terms of reference.
- Commission any reports it deems necessary to help fulfil its obligations.
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so the committee must follow any procedures put in place by the ICB for obtaining legal or professional advice.
- Create sub-committees and task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Committee's members. The Committee shall determine the membership and terms of reference of any such sub-committees and task and finish sub-groups in accordance with the ICB's constitution, standing orders and SoRD but may not delegate any decisions to such sub-committees and sub- groups.

3. Role of Joint Remuneration and People Committee

The purpose of the Committee is to assure the ICB Boards that the ICBs discharge their statutory, contractual and good-employer responsibilities effectively, consistently and transparently, while maintaining appropriate independence for decisions concerning senior remuneration, terms and conditions, severance and related matters.

The Committee will oversee the ICBs' People Strategy and Staff Experience Framework as key organisational enablers. Through these frameworks, it will seek assurance that the ICBs have the capacity, capability, culture, leadership and employment arrangements required to

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fulfil their statutory functions, operate as effective strategic commissioners and deliver their objectives for local populations.

The committee will assure that both ICB Boards exercise their responsibilities relating to paragraphs 17 to 19 of Schedule 1B to the NHS Act 2006. In summary: 'To confirm the ICB Pay Policy including adoption of any pay frameworks for all employees including Very Senior Managers/Directors (including board members) and Non-Executive Directors excluding the Chair are effectively discharged.

The Committee has no executive powers, other than those delegated in the SoRD and specified in these terms of reference.

In order to deliver these responsibilities, the Committee has delegated authority from both ICB Boards for the following responsibilities set out in the ICBs Schemes of Reservation and Delegation (SoRD):

- Approval of the terms and conditions, remuneration and travelling or other allowances for ICB/ICS Board Members, including pensions, gratuities and redundancy and settlement arrangements; with reference to guidance published by NHSEI.
- Approval of the terms and conditions of employment for all employees of the ICB (excluding the Chief Executive) including remuneration, pensions, fees and travelling or other allowances payable to employees and to other persons providing services to the ICB.
- Approval of any other terms and conditions of service for the ICB's employees and other persons providing services to the ICB.
- Approval of disciplinary arrangements for ICB employees, including the Chief Executive, and for other persons working on behalf of the ICB.
- Approval of any individual compensation payments.
- Approval of the arrangements for discharging the ICBs' statutory duties as an employer.
- Approval of the human resources policies for employees and for other persons working on behalf of the ICB

In the exercise of these responsibilities and to deliver its core purposes the Committee will be responsible for: -

Formal remuneration and terms of service responsibilities

- Determine and keep under review the ICB pay policy, including adoption and application of relevant national pay frameworks.
- Determine remuneration arrangements for Very Senior Managers, including salary and any permitted performance-related elements, pensions, contractual benefits and termination arrangements.
- Approve individual termination, redundancy, settlement, compensation or special payments within delegated limits, following appropriate scrutiny of business justification, calculation, value for money, equality impact, legal advice and national approval requirements.
- Receive assurance on compliance with national frameworks for executive and non-executive remuneration and take account of relevant benchmarking and national agreements.

People Strategy

- The Committee will oversee development, approval recommendations, implementation and impact of the ICBs' People Strategy, ensuring that it is aligned with the ICBs' strategic

objectives, operating model, financial plans and responsibilities as strategic commissioners. In particular, the Committee will:

- Seek assurance that workforce plans are based on robust demand, capacity, affordability, skills and capability analysis.
- Oversee workforce design, establishment control and organisational development so that the ICBs maintain an appropriately sized, skilled, diverse, resilient and productive workforce.
- Oversee recruitment, retention, talent management, succession planning, leadership development, learning and professional development.
- Oversee workforce transformation, redeployment, new ways of working and responsible use of digital solutions, ensuring appropriate staff engagement and change management.
- Monitor delivery of agreed people outcomes, milestones, benefits and performance indicators, including dependencies with finance, transformation, quality and commissioning plans.

Staff Experience Framework

The Committee will oversee the Staff Experience Framework as the principal mechanism for understanding and improving the experience, wellbeing, inclusion, engagement and contribution of the ICB workforce. It will:

- Receive and triangulate evidence from staff surveys, pulse surveys, sickness absence, turnover, vacancies, grievances, disciplinary matters, speaking-up themes, occupational health, health and safety, exit information and other workforce intelligence.
- Monitor delivery of actions arising from staff feedback and require clear ownership, timescales and evidence of impact.
- Oversee staff wellbeing, psychological safety, inclusion, compassionate leadership, employee voice and effective engagement with recognised trade unions and staff-side representatives.
- Monitor performance against the Workforce Race Equality Standard, Workforce Disability Equality Standard and other applicable equality and inclusion measures.
- Seek assurance that staff affected by organisational change, transfer, redeployment or redundancy receive timely communication, consultation and appropriate support.
- Ensure that staff experience is treated as an enabler of organisational performance, productivity, service quality, decision-making and delivery of the ICBs' statutory responsibilities.

Employer responsibilities and people governance

- Approve and periodically review the overarching suite of human resources policies for employees and others working on behalf of the ICBs, ensuring legal compliance, consistency, fairness and appropriate staff-side engagement.
- Oversee the people-related elements of major change programmes including transfers of functions, merger, organisational redesign and management-of-change programmes.
- Receive assurance on workforce implementation plans, consultation, selection, redeployment, retention, staff transfer and support arrangements.
- Agree and monitor a people and workforce assurance dashboard, including agreed indicators, trajectories and tolerances.
- Receive assurance on the effectiveness of disciplinary, grievance, capability, speaking-up, employee relations and employment case-management arrangements, without becoming involved in individual cases unless a decision is explicitly reserved or delegated to the Committee.
- Oversee compliance with employment law, equality duties, health and safety responsibilities relating to staff, data protection requirements and relevant NHS employment standards.
- Review relevant Board Assurance Framework and corporate risks, including legal, regulatory, financial, operational, reputational, capacity, capability, wellbeing and staff experience risks.

- Ensure workforce risks affecting finance, quality, performance, transformation, health inequalities or statutory delivery are communicated to the relevant committee.
- Escalate material risks and issues in accordance with delegated authority and the ICBs' risk appetite statements.
- Oversee the terms of reference and work programmes of any groups reporting to the Committee and evaluate whether they provide proportionate assurance.

4. Membership

The Members of the Committee shall be appointed by the Cluster Board in accordance with the ICBs Constitutions. The Cluster Board will appoint the following members of the Committee: -

- At least two Non-Executive Members from the ICB Boards
- The Director of Corporate Affairs and Chief of Staff
- The Chief Finance Officer
- *The Associate Director for People and Corporate Affairs*

Chair and Deputy Chair
The Cluster Board will appoint a Non-Executive Member of the ICB Boards to Chair the Committee. Another non-Executive Member of the Committee will be appointed as the Deputy Chair of the Committee and act if the Chair is absent or unable to act. When determining the membership of the Committee, active consideration will be made to diversity and equality.

The Chair will be responsible for agreeing the agenda and ensuring that matters discussed meet the objectives as set out in the Terms of Reference.

Attendees
Only members of the Committee have the right to attend the meetings, but the Chair may invite relevant persons to the meeting as necessary in accordance with the business of the Cluster Board. Meetings of the Committee may also be attended by the following individuals who are not members of the Committee for all or part of a meeting as and when appropriate. Such attendees will not be eligible to vote:

- Chief Executive or their nominated deputy
- The Chair of the ICBs
- The ICB's most senior Governance Advisor or their nominated deputy

No individual should be present during any discussion relating to:

- Any aspect of their own pay;
- Any aspect of the pay of others when it has an impact on them

5. Meetings, Quoracy and Decision making

The Committee will meet quarterly with the arrangements and notice for calling meetings as set out in the ICBs Standing Orders. Additional meetings may take place as required. The Cluster Board, Chair or Chief Executive may ask the Committee to convene further meetings to discuss particular issues on which they want the Committees advice.

In accordance with the Standing Orders, the Committee may meet virtually when necessary and members attending using electronic means will be counted towards the quorum.

Quorum
For a meeting to be quorate the following members must be present: -

- The Chair or Deputy Chair
- At least one other committee member

If any member of the Committee has been disqualified from participating in an agenda item, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Nominated Deputies
A Member of the Committee may nominate a deputy to attend if there are occasions when they are unavailable. This should be the exception rather than the rule. The deputy must be appropriately briefed and have decision making authority to adequately deputise for the member.

Any attendee wishing to nominate a Deputy for a meeting must notify the Chair of the name of their deputy in advance of the meeting taking place

Decision making and voting.
Decisions will be taken in accordance with the ICBs Standing Orders. The Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

Only members of the Committee who are present may vote. Each member is allowed one vote, and a majority will be conclusive on any matter.

Where there is a split vote, with no clear majority, the Chair of the meeting will hold a second and casting vote.

7. Behaviours and Conduct

Benchmarking and guidance

The Committee will take proper account of National Agreements and appropriate Benchmarking in reaching their determinations.

ICBs values

Members will be expected to conduct business in line with the ICBs' values and objectives, and the principles set out by the ICBs.

Members of, and those attending, the Committee shall behave in accordance with the ICBs' constitution, Standing Orders, Scheme of Reservation and Delegation, and Standards of Business Conduct Policy.

Declaration of Interests, Conflicts and Potential Conflicts

Where a member of the Committee is aware of an interest, conflict, or potential conflict of interest in relation to the scheduled or likely business of the meeting, they will bring this to the attention of the Chair of the meeting as soon as possible and before the meeting where

possible. The Chair of the meeting will determine how this should be managed and inform the member of their decision.

Any declarations of interests, conflicts, potential conflicts and arrangements to manage those agreed in any meeting of the Committee, will be recorded in the minutes.

Equality diversity and inclusion

Members must actively promote the principles of equality, diversity, and inclusion in all interactions and demonstrably consider these principles when considering and making decisions.

Confidentiality

Papers that are marked 'in confidence, not for publication or dissemination' shall remain confidential to the members of the Committee unless the Chair indicates otherwise. Members, representative or any persons in attendance shall not reveal or disclose the contents of these papers without express permission of the Chair. This prohibition shall apply equally to the content of any discussion during the meeting which may take place on such papers.

General Data Protection Regulations (GDPR) and Data Protection Act (DPA) 2018

Cluster Board members will give due regard to the responsibilities of the ICBs to comply with GDPR and DPA legislation.

Freedom of Information Act 2000

All papers are subject to the Freedom of Information Act. All papers that are exempt from public release under the FOI Act must be clearly marked 'in confidence, not for publication'. These papers may not be copied or distributed outside of the Committee membership without the expressed permission of the Chair. FOI exemption 41 (duty of confidence) applies.

8. Accountability and reporting

The Committee will have authority to make decisions in line with the ICBs' Scheme of Reservation and Delegation and Constitutions. The Committee is accountable to the Joint ICB Cluster Board and shall report to the Cluster Board on how it discharges its responsibilities at each Cluster Board meeting.

The minutes of the meetings shall be formally recorded by the secretary and submitted to the Committee for approval at the following meeting. If this Committee ceases, the minutes will be confirmed by the chair and at least three other attendees present at the meeting that was recorded, as an accurate and complete record.

9. Secretariat and Administration

The Committee shall be supported by a secretariat function, which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead.

- Minutes are taken in accordance with the standing orders and agreed with the chair and that a record of matters arising, action points and issues to be carried forward are kept.
- The Chair is supported to prepare and deliver reports to the Board.
- The Committee is updated on pertinent issues/ areas of interest/ policy developments; and
- Action points are taken forward between meetings.

10. Review

The Committee will review its effectiveness at least annually. These terms of reference will be reviewed at least annually or earlier if required. Any proposed amendments to the terms of reference will be submitted to the Board for approval.

Date of approval:

Date of review:

NHS Birmingham and Solihull

NHS Black Country

Agenda No: 15		
Report to:	Joint Cluster Board	
Date of Meeting:	9 November 2026	
Title:	Board Assurance Framework	
Presenting Officer:	Peter McKenzie - Associate Director Governance and Corporate Affairs	
Author(s) of Report:	Jodi Perks - Head of Corporate Services	
To be Withheld from Publication?	Yes ☐ No ☒	(If Yes) Rationale: N/A
Relevant Conflicts of Interest	None	

NHS 10 Year Plan Three Shifts					
Hospital to the Community	☒	**Digital Transformation**	☒	**Treatment to Prevention**	☒

Related ICB Aims	Check box
Improve outcomes in population health and healthcare	☒
Tackle inequalities in outcomes, experience and access	☒
Enhance productivity and value for money	☒
Support the broader social and economic development of Birmingham, Black Country and Solihull	☒

ICB Focus					
Delivery	☒	**Transformation**	☒	**Transition**	☒

ICB Strategic Anchors					
Commissioning for Value and Outcomes	☒	**Accelerating Neighbourhood Health at Pace**	☒	**Digital First**	☒
Financial Discipline/ 2% productivity	☒	**Advancing Health Equity**	☒	**Integrated Partnership**	☒

Purpose:
To provide the Joint Cluster Board with an updated Board Assurance Framework position following committee scrutiny since the Board meeting on 14 September 2026, setting out changes made, actions progressed and committee risk discussions requiring continued Board oversight.

Decision Required			
Approval ☐	☐	**Accept as Assurance**	☒
Endorsement ☐	☐	**Noted** ☐	☐

Return to Agenda

Report Journey:

Population Health and Strategy Committee - 28 September and 26 October 2026; Quality and Safety Committee - 15 October 2026; Resource Allocation and Payer Committee - 23 October 2026.

Executive Summary

The Board Assurance Framework remains the principal mechanism through which the Joint Cluster Board receives assurance on delivery of strategic objectives and management of the ICBs' six principal risks. Committee work since September confirms a broadly stable principal risk profile. Risk 101 remains the highest-rated risk at 16 with moderate assurance; Risk 102 remains at 8 with full assurance; Risks 103, 104 and 105 remain at 12, 9 and 12 respectively with moderate assurance; and Risk 106 remains at 12 with substantial assurance. The Population Health and Strategy Committee increased the score of the related committee risk PHS001 concerning analytics continuity. It did not propose a change to BAF Risk 103 and will review the principal risk at a future meeting. The Board is asked to accept the updated position as assurance, note the committee-level changes, and commission a review of Risk 106 and its future scrutiny route following dis-establishment of the Transition Committee.

Appendices

Appendix 1 - Updated Board Assurance Framework (separate document)
Appendix 2 - Change, action and committee discussion summary
Appendix 3 - Gap, discrepancy and traceability log

Recommendations & Next Steps

The Board is asked to accept the updated BAF position as assurance; note that PHS001 has increased while BAF Risk 103 remains unchanged pending future review; note the linked committee risk movements; and require a review of Risk 106, its controls, actions and successor scrutiny route following dis-establishment of the Transition Committee.

Assessments:

Impact Assessments Completed		Yes	No	N/A	Details (N= explain why: Y= approval date)
	QIA	☐	☐	☒	
	EQIA	☐	☐	☒	
	DPIA	☐	☐	☒	

Implications	Impact
Quality and Safety	Risk 101 remains materially above target and requires continued Quality and Safety Committee scrutiny of delivery evidence and control effectiveness.
Legal and Regulatory	The BAF supports discharge of Board oversight responsibilities. Controlled records should identify the authorised scrutiny route for each principal risk.
Finance	Risk 102 remains at 8 with full assurance. The merger-delay capacity risk may affect finance and corporate capacity and should remain linked to strategic risk review.

People	Risk 106 includes organisational maturity and transition dependencies. Dis-establishment of the Transition Committee requires a clear successor route for oversight.
Reputational	Prompt reconciliation of committee decisions, review dates and action evidence supports confidence in the Board's assurance arrangements.



Return to Agenda

BOARD ASSURANCE FRAMEWORK UPDATE

Joint Cluster Board – 9 November 2026

1. Why this matters now

The September Board accepted the Quarter 2 BAF as assurance on a stable profile of six principal risks and asked committees to continue testing whether controls are effective and supported by evidence. Since then, assigned committees have reviewed their linked risk registers and BAF extracts. This provides the first opportunity to bring the resulting movements, actions and assurance issues back to the Board. Committee work shows that linked risks are being created, closed, de-escalated or rescored as circumstances change. The increase to the analytics continuity committee risk, the effect of delayed merger arrangements and the dis-establishment of the Transition Committee now require clear follow-through in the controlled BAF and its governance routes.

2. What the evidence tells us

The evidence supports a broadly stable principal risk profile. Risk 101 remains extreme at 16 against a target of 8. Risk 102 retains full assurance, Risk 106 retains substantial assurance and Risks 101, 103, 104 and 105 retain moderate assurance. The Population Health and Strategy Committee increased the score of PHS001, the committee risk relating to analytics continuity and CSU transfer uncertainty. It did not propose a revision to BAF Risk 103. The Committee will review the principal risk at a future meeting, informed by evidence on the transfer timetable, interim and end-state costs and the future service model. The controlled BAF therefore continues to record Risk 103 at 12 against a target of 8.

3. The local picture

Committee scrutiny since September confirms: Risk 101 remains at 16, with closure or de-escalation proposed for linked risks where controls have strengthened and antimicrobial resistance retained as an ongoing managed risk; Risk 102 remains at 8 with full assurance, supported by strengthened cluster oversight and a proposed reduction of JFP005 from 12 to 8; PHS001 has increased because of analytics continuity concerns, while BAF Risk 103 remains unchanged pending future review; Risks 104 and 105 remain stable; and Risk 106 remains at 12, with the Management of Change action rephased to November 2026. Following dis-establishment of the Transition Committee, Risk 106 requires review to confirm its current assessment, controls, actions and successor scrutiny route.

4. Decisions required

DECISIONS:-

The Board is asked to make the following decisions:

DECISION 1	**ACCEPT AS ASSURANCE** **The Board is asked to accept as assurance the updated principal risk position at Appendix 1, enabling continued oversight of the six principal risks and their associated controls and actions.**
DECISION 2	**NOTE** **The Board is asked to note that the Population Health and Strategy Committee increased related committee risk PHS001 but did not propose a revision to BAF Risk 103, enabling the principal risk to remain at 12 pending future committee review.**

Return to Agenda

DECISION 3	**NOTE** **The Board is asked to note the committee-level changes at Appendix 2, including the proposed RAP006 risk, closure or de-escalation of linked risks and revised treatment of antimicrobial resistance as an ongoing managed risk.**

DECISION 4	**REQUIRE** **The Board is asked to require a review of Risk 106 and its future scrutiny route following dis-establishment of the Transition Committee, enabling continued accountable oversight of organisational maturity and transition dependencies.**

ASSURANCE:-

The Board is asked to accept the following assurance:

ALERT	**The increase in PHS001 indicates a developing risk to analytics continuity. Evidence on CSU transfer timetable, costs and the future service model remains incomplete, and BAF Risk 103 requires future review.**

ADVISE	**Committee risk movements and action updates demonstrate active scrutiny. Risk 106 requires review and reassignment of scrutiny following dis-establishment of the Transition Committee.**

ASSURE	**The six principal risks remain recorded in the controlled BAF, with Risk 102 retaining full assurance and Risk 106 retaining substantial assurance pending review.**

5. Risks of doing nothing

The Board should note the following risks if the identified governance actions are not completed.

Risk	Likely consequence
Risk 103 is not reviewed after the increase in PHS001.	The BAF may not reflect evidence about analytics continuity and population health capability.
Risk 106 has no confirmed successor scrutiny route.	Oversight of organisational maturity, transition dependencies and delayed merger impacts may become fragmented.
Undated or rephased actions are not controlled.	Accountability weakens and committees cannot distinguish acceptable phasing from delay.
Committee risks are closed or de-escalated without outcome evidence.	The Board may receive false assurance that controls are effective.



Return to Agenda

Risk	Likely consequence
Review dates and committee outcomes are not reconciled.	The Board may receive an outdated or internally inconsistent strategic risk position.

6. Proposed Workplan and Actions

Subject to the Board's decisions, the following actions are proposed. Governance route, evidence and review-cycle requirements are included within each action.

Owner	Action	By
Chief Strategy Officer	Review BAF Risk 103 in light of the increased PHS001 score. Governance route: Population Health and Strategy Committee -> Executive review -> Joint Cluster Board.	Next scheduled PHS risk review
Transition Director	Review Risk 106, its controls, assurance, actions and score following dis-establishment of the Transition Committee. Governance route: Executive review -> successor scrutiny committee assignment -> Joint Cluster Board.	Next scheduled risk review
Head of Corporate Services	Update the controlled BAF and committee-to-Board mapping to record the successor scrutiny route for Risk 106. Governance route: Executive review -> Joint Cluster Board.	Next scheduled risk review
Chief Finance Officer / Transition Director	Map RAP006 to relevant BAF drivers, controls and actions and determine whether merger-delay impacts require amendment to Risk 106 or another principal risk. Governance route: Resource Allocation and Payer Committee -> Executive review.	Next scheduled risk review
Risk owners	Validate that completed actions provide evidence of improved control effectiveness, not only activity completion. Governance route: assigned scrutinising committee.	Next scheduled committee cycle
Head of Corporate Services	Reconcile review dates, action dates and statuses across the live BAF and committee extracts. Governance route: Executive review -> Joint Cluster Board.	Before final Board pack

Appendix: Source Documents

This paper draws on the following source documents:

Final Board Assurance Framework report to the Joint Cluster Board, 14 September 2026 (approved Board report).

LIVE BAF Full document - in review (controlled working document supplied for this report).

Quality and Safety Committee agenda and papers, 15 October 2026.



NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

Board Assurance Framework

Update following committee review - October 2026



NHS Birmingham and Solihull

NHS Black Country

Agenda/contents

- Board Summary
- Risk Level by Corporate Objective
- Risk Summary View
- Actions and next steps



NHS Birmingham and Solihull

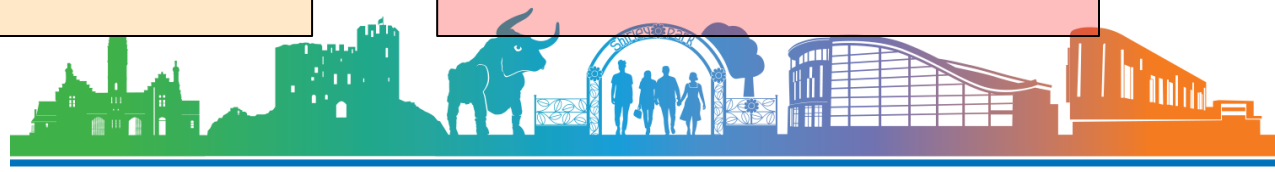
NHS Black Country

Board Summary

- **Assure**
- Risk 102 retains full assurance at 8 against a target of 6.
- Risk 106 retains substantial assurance at 12, pending review of its controls and future scrutiny route.
- No principal BAF risk score has changed since September.

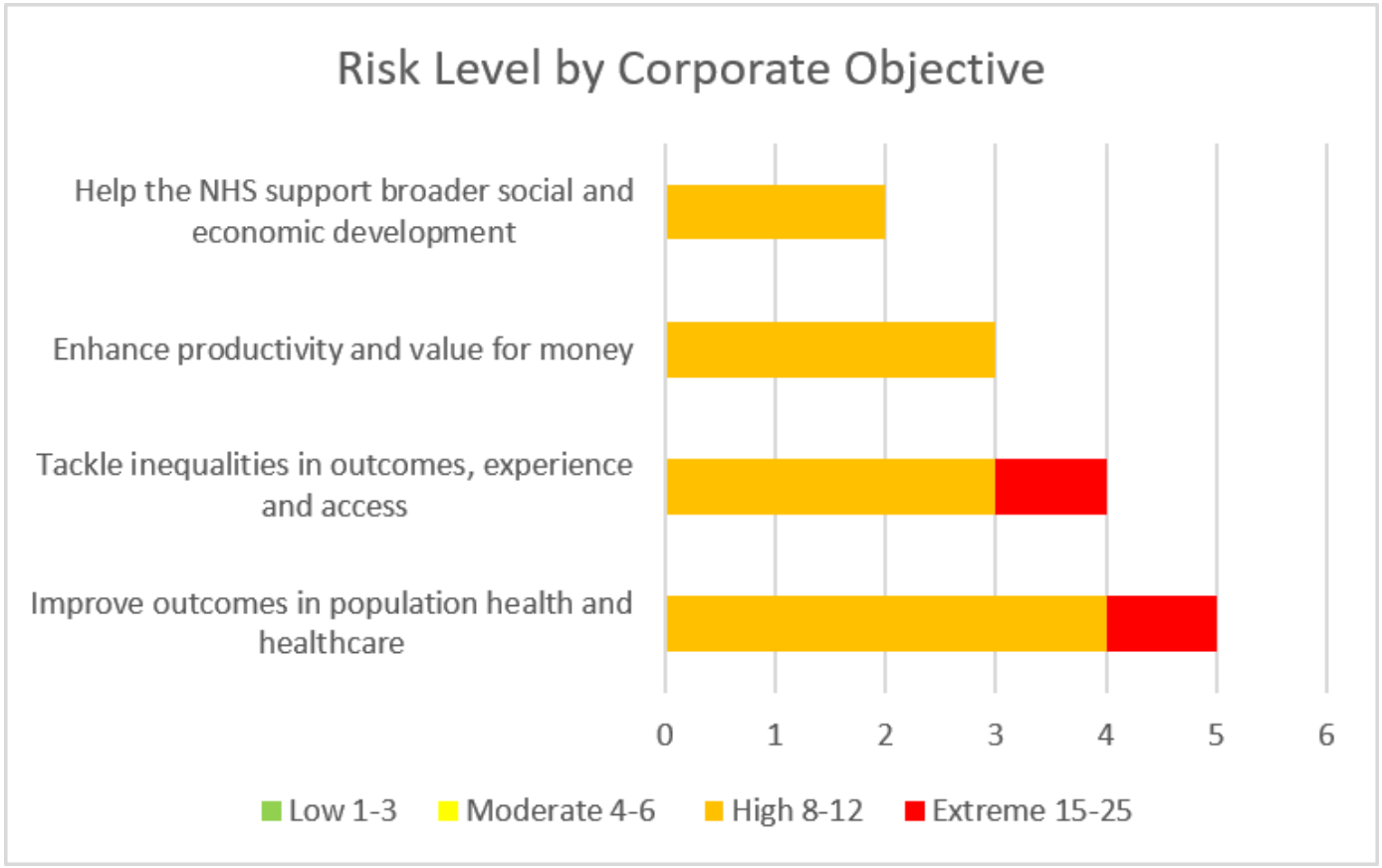
- **Advise**
- PHS001, the committee risk for analytics continuity, has increased. BAF Risk 103 remains at 12 and will be reviewed by the Population Health and Strategy Committee at a future meeting.
- Linked committee risks have been closed, de-escalated or rescored where controls have strengthened.

- **Alert**
- Risk 101 remains the highest-rated principal risk at 16 against a target of 8 and requires sustained oversight.
- Following dis-establishment of the Transition Committee, Risk 106 requires review and an approved successor scrutiny route.



NHS Birmingham and Solihull

NHS Black Country



Corporate objectives 3 and 4 continue to carry the highest overall risk exposure, each with one extreme principal risk.

Risk 101 remains the extreme-rated risk aligned to both objectives. No principal BAF risk score has changed since the September Board report.

Return to Agenda

NHS Birmingham and Solihull

NHS Black Country

Ref	Risk Title	1				Initial Rating	Responsible Committee/s	Risk Owner/ Lead Executive	Q1 Rating	Q2 Rating	Current Rating	Q4 Rating	Movement (since last report)	Assurance Assessment	Target Score	Variance to Target
		1	2	3	4											
101	Safe and Effective Services	x	x			16	Quality and Safety	CCQO	16	16	16		→	Moderate	8	+8
102	Financial Balance	x				10	Resource Allocation & Payer	CFO	8	8	8		→	Full	6	+2
103	Data Quality & Completeness	x	x			12	Population Health & Strategy	CSO / Dir. Comms	12	12	12		→	Moderate	8	+4
104	Digital Opportunities	x	x	x	x	12	Resource Allocation & Payer / Population Health & Strategy	CFO / CSO	9	9	9		→	Moderate	8	+1
105	System Influence and Delivery	x	x	x	x	12	Resource Allocation & Payer / Population Health & Strategy	CFO / CSO	12	12	12		→	Moderate	9	+3
106	Organisational Maturity			x		12	Successor scrutiny route to be agreed	Transition Director	12	12	12		→	Substantial	6	+6

NHS Birmingham and Solihull

Return to Agenda

NHS Black Country

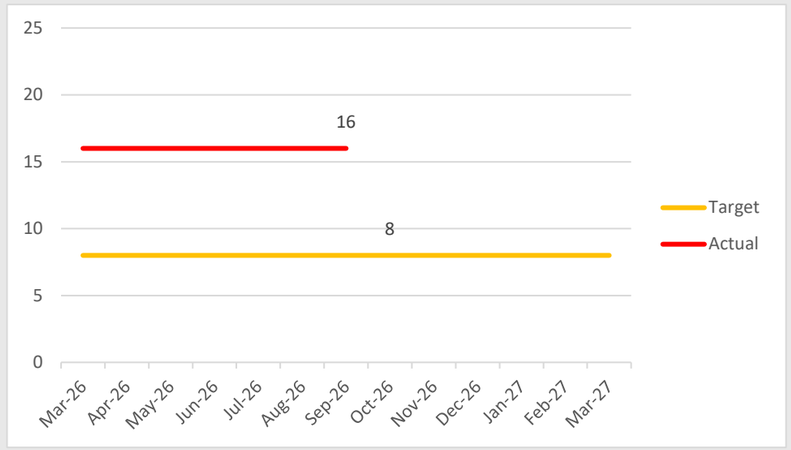
Board Assurance Framework

Principal Risk No: 101	The ICB may fail to utilise its levers to ensure consistently safe and effective services across all Places, leading to avoidable harm, worsening inequalities, and poor patient experience and resulting in non-achievement of the ICB statutory duties.

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	**X**
Date Last Reviewed	28/10/2026	Tackle inequalities in outcomes, experience and access	**X**
Risk Owner:	Chief Clinical and Quality Officer	Enhance productivity and value for money	
Scrutinising Committee(s):	Quality and Safety	Help the NHS support broader social and economic development	

Score Tracker

Overall Current Score		Target Score
16		8
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
4	4	Moderate
Associated Committee Risks		**Key Risk Impact Classifications:**
JQ&S001 JQ&S002 JQ&S003 JQ&S004 JQ&S005 JQ&S006 JQ&S007 JFP 005		• Health, Safety and Injury • Failure to meet statutory duties



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Delays & Access (Neurodevelopment pathway)	• Service Contracts based on National standards and local needs. • Monthly strategic oversight group- led by ICB in place • Improvement programme in place • Commissioning policies and procedures • £15m (3 year recurring) investment approved at Board to address waiting lists and pathway redesign	• Access waits • ADHD trajectories not secured	• Provider Commissioning Reports • Quality Committee Reports • Improvement programme oversight group reports • CQC Inspections •	X	X X	X	Partial assurance regarding ADHD access	• 1 • 3
Data Availability (Serious Violence Duty)	• Provider Safeguarding framework • ICB Safeguarding framework • Safeguarding Improvement plan • Safeguarding standards defined • Policies and procedures in place • CQRM review meetings	• Staffing pressures • Inconsistent training levels across providers • Access to key safeguarding performance data from UHB	• Provider Commissioning Reports • System Oversight Group Reports • Quality Committee Reports • CQC Inspections • Sexual Safety and Sexual Misconduct: Self Assessment Audit	X X X	X X	X	• Annual Health Checks (AHC) • Limited safeguarding performance and compliance reports within third sector providers.	• 4
Service Standards (Maternity and Neonatal)	• Provider Maternity Service Strategies and Improvement Plans (including Ockenden Compliance Framework) • Quality Surveillance Improvement Plans via LMNS including MNIPLMNS framework • Monthly Perinatal System Assurance & Oversight Group (PSAOG) taking place, co-chaired by ICB CNO & NHSE Regional CNO.	• Baroness Amos Review Highlighted cultural issues • CQC Sec 29a remains live	• Provider Commissioning Reports • Quality Committee Reports • PSAOG Reports • HSSIB/MNSI Investigations • CQC Inspections • National Enquiry (Amos Review)	X	X X	X X X	None identified	• 2
Service Continuity (Dudley childrens services)	• DGFT conducting due diligence • DGFT continue to provide the medical element • BCHFT is continuing to provide the specialist element of the service for the time being • Shropcomm continue to provide the additional Review Health Assessment support until the end of June • ICB contracts discussions continue on notice period.	• As yet DGFT have not confirmed that they will take the whole service for Children in Care	• Risk monitored through Q&S committee • Data available in relation to Children in Care Services for Dudley • Data reviewed by Corporate Parenting Board • Overseen by NHS E	x	x x	x x	None currently identified.	• 5

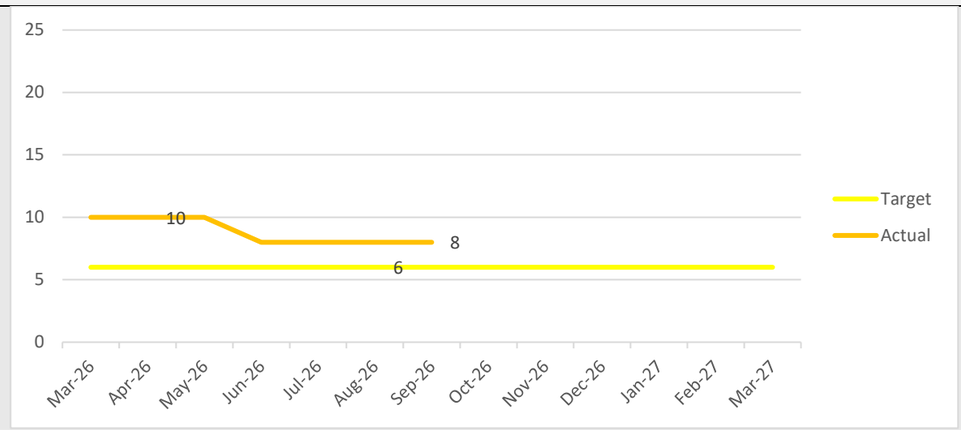
Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Improvement Plan for LD & Autism via Mental Health Provider Collaborative, including LeDeR Improve Plan and Strategy	CCQO	November 2026		**On Track**
2. Maternity Services Quality Improvement Event	CCQO	28/05/2026		**Complete**
3. Modelling on £15m (recurrent 3 years) investment in Neurodevelopment for pathway redesign & waiting list reduction		July Board		**Complete**
4. ICB's to write to NHS E regarding data collection at trust level	CCQO	TBD		
5. Agree plan for transfer of the elements of the Children in Care Service currently provided in Shropshire Community Trust and BCHFT to DGFT.	CCQO	August 26		**Complete**

Principal Risk No: 102	The ICB may be unable to deliver a financially balanced position, constraining our ability to commission high value care and exposing the organisation to regulatory intervention.

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	X
Date Last Reviewed	28/08/2026	Tackle inequalities in outcomes, experience and access	
Risk Owner:	Chief Finance Officer	Enhance productivity and value for money	
Scrutinising Committee(s):	Resource Allocation & Payer	Help the NHS support broader social and economic development	

Score Tracker

Overall Current Score		Target Score
8		6
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
4	2	Full
Associated Committee Risks		**Key Risk Impact Classifications:**
FP001 FP002 FP003 FP004		• Failure to meet statutory duties • Financial



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Unmet efficiencies	• ICB Efficiency Board • Programme management appproch followed for key efficiency programmes including completion of PID documents • Capital Programme framework and plans • Management processes of Independent Sector Contract through Activity Management • ICB financial plans Set	• Some efficiency documentation still to be developed • More work required to align oversight approach between 2 ICBs	• ICB Efficiency Reports • Finance and Performance Committee reports • Contract Oversight and Approval Reports • Provider Finance Oversight Committees • External Audits of Financial Accounts • NHS England review of ICB finance reports	x x	 x x	 x x	• Still have some elements of plans rate high risk or in development stage. Completion of MOC stage 2 to drive CHC and Meds Mgt programme	• 1, 2
Slippage of timescales (Running cost reductions)	• Process managed with Programme Board & dedicated PMO support • Finance engaged in timeline discussions with HR • Regularly updated monitoring to financial impact including reporting to NHSE and the ICB Transition Committee • Funding set aside in 25/26 provision and 26/27 budget to cover anticipated costs	• Transition Management Programme Board • Transition Committee	• ICB Transition Committee Reports • Finance and Performance Committee Reports • • • •	x x			Completion of MOC is required CSU stranded costs to be negotiated	• 3
Insufficient Capacity / Capability	• Organisattional structures designed for effective delivery of the ICB's requirement • Processes followed for match and slot / competitive match. • Process in place to fill remaining vacancies through suitable alternative employement • Transition Finance Lead in place for 26/27	• ISFE2 issues and requirement to complete 2 sets of ICB accounts continue to pull on resources • Vacancies remain in structure	• Transition committee reports • Finance and Performance Committee Reports • • • •	X x			Vacancies remain within the finance directorate. Loss of organisational knowledge and processes as staff leave under VR	• 4

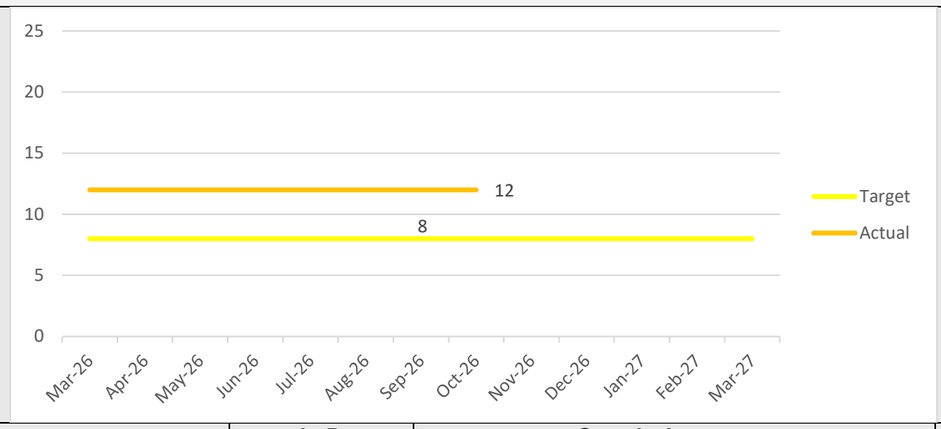
Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Efficiency monitoring and continued remedial planning to continue throughout financial year	CFO	31/03/2027		On Track
2. Alignment of cluster wide financial controls and processes to take place during Q1/Q2 26/27	CFO	30/09/2026		Completed
3. Reporting of financial impact of MOC to Transition Committee	CFO	31/12/2026		On Track
4. Vacancies to be filled though MOC & recruitment	CFO	31/12/2026		On Track

Principal Risk No: 103	Poor quality or incomplete data may lead the ICB to make wrong commissioning decisions, slowing improvement in outcomes and perpetuating health inequalities.

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	**X**
Date Last Reviewed	24/09/2026	Tackle inequalities in outcomes, experience and access	**X**
Risk Owner:	Chief Strategy Officer Director of Communications	Enhance productivity and value for money	
Scrutinising Committee(s):	Population Health and Strategy Committee	Help the NHS support broader social and economic development	

Score Tracker

Overall Current Score		Target Score
12		8
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
3	4	Moderate
Associated Committee Risks		**Key Risk Impact Classifications:**
PHS 001		• Regulatory • Operational • Quality • Financial



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Data Availability (Commissiong Data Provision)	• NHS England transition governance, with ICB and CSU stakeholder engagement routes. • West Midlands Chief Analyst Group engagement with NHS England to test and troubleshoot the national offer. • Regional provision being considered • Local provision being scoped as further contingency • Csu to continue current provision until Dec 26	Costing of the interim and end state service is not yet available CSU voluntary redundancies may affect capacity for continuation of service Regional specification and costings cannot be finalised until NHS E offer is known	• Population Health and Strategy Committee reports	x			No committed timelines for integration of data provision into FDP Lack of clarity for the post transition Service Specification	1
			• Transition Committee reports	x				
			• West Midlands Chief Analyst Group		x			
			• Monthly NHS E National Meetings/ updates		x			
			•					
			•					
			•					
Community/Patient Voice	• Community Engagement Programme • VCSFE Alliance, including NEM involvement • Healthwatch • BRIG 10 year Strategy • Community engagement, supported by healthwatch complete	Equality Monitoring dataset incomplete- Complaints POD Complaints dataset completeness CYP/VCSE MHSDS Data Quality Inclusion of Patient/Community voice reports in Board papers Futue of Healthwatch uncertain	• Communication Team activity reports	x			UTC programme continues with business case being developed for board decision.	2 4
			• Project engagement/co-production reports	x				
			• Population Health and Strategy Committee reports	x				
			• Annual Health Inequity Self-Assessment		x			
			• NHSE Annual Health Inequity Assessment			x		
			• UTC programme board	x				
Operational Data Quality	• Policies and procedures defining data standards (Data Quality Policy • Information Asset Register & Owners • Incident reporting policy and procedure • Data validation processes	ISFE2 data management and reporting Performance Data Lag Limited Accountability frameworks	• Function team reports	x			Data Quality Maturity Index Independent Audits	3
			• IGSG reports	x				
			• Incident reports	x				
			• Finance and Performance Committee Reports	x				
			• Audit Committee Reports	x				
			• Internal Audit Reports			x		
			• External Audit Reports			x		

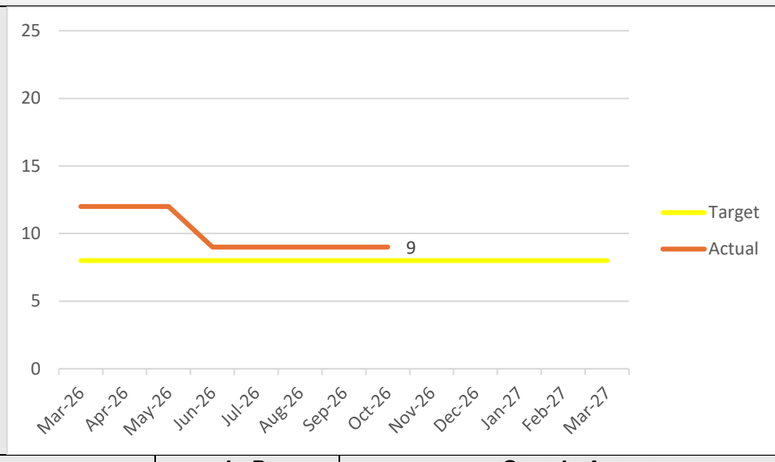
Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Continue engagement with NHS E regarding national service offering	CSO	Dec 26		**On track**
2. UTC Public Consultation, supported by Healthwatch	Dir. Of Comms	June 2026		**Completed**
3. Implementation of a unified IGSG across both organisations	CSO	01/03/2026		**Completed**
4. UTC business case to be developed and brough to cluster board for decision	CSO	November 26		**On track**

Principal Risk No: 104	Failure to exploit digital opportunities may lock the system into higher cost models of care, poorer experience, and reduced productivity.

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	X
Date Last Reviewed	28/09/2026	Tackle inequalities in outcomes, experience and access	X
Risk Owner:	Chief Finance Officer Chief Strategy Officer	Enhance productivity and value for money	X
Scrutinising Committee(s):	Resource Allocation & Payer Population Health and Strategy Committee	Help the NHS support broader social and economic development	X

Score Tracker

Overall Current Score		Target Score
9		8
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
3	3	Moderate
Associated Committee Risks		**Key Risk Impact Classifications:**
None		• Failure to meet statutory duties • Financial • Operational • Quality • Regulatory



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Opportunity Identification and Exploitation	• ICBNHSE digital standards included within policies and procedures (such as DTAC) • IG Policies and procedures- including DPIA • Digital plan included in Cluster Strategic commissioning plan • Cluster Digital SDIP established • Digital Policies and procedures • AI Policy and procedure • PSR and procurement policies and procedures • Caf aligned DSPT assessments • BC Digital Strategy and delivery plan	• ICB and provider digital capacity & capability • FDP full adoption • Alignment of current systems in use • Alignment of digital systems across commissioned providers • Lack of national or regional digital strategy • Closure of CSU services and the successful reintegration of functions.	• Digital programme oversight boards	x			• Digital system implementation • Digital Policies to be aligned across the cluster	• 1 • 2
			• Cluster Digital Programme Board	x				
			• Population Health and Strategy Committee (delivery assurance)		x			
			• Finance and Performance Committee (formal scrutiny and assurance)		x			
			• Place digital programme boards	x				
			• Audit Committee Reports		x			
			• Internal Audit Reports			x		
			•					
			•					
			•					

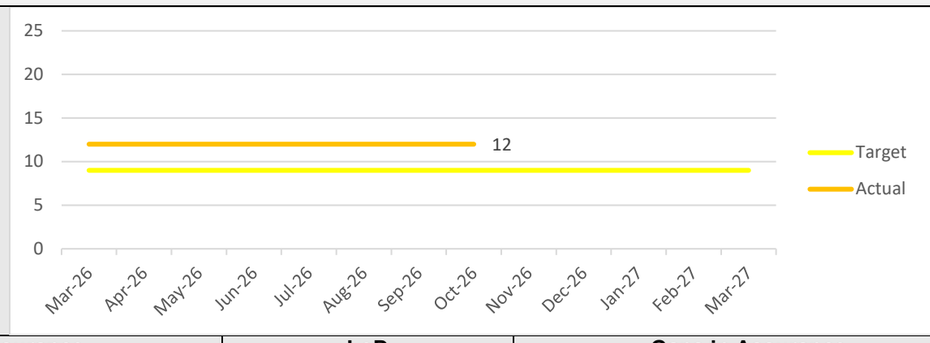
Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Implement and embed the Cluster Digital Governance Framework	CFO	December 2026		**Completed**
2. Approve and commence delivery of a single Cluster Digital Strategy and Roadmap	CFO	March 2027		**On Track**
3. Establish clear DDaT accountability and leadership arrangements across the cluster	CFO	December 2026		**Completed**
4. Implement a system-wide cyber assurance and DSPT improvement programme	CFO	March 2027	June 2027	**On Track**
5. Develop and implement a digital workforce and capability plan	CFO	June 2027		**On Track**
6. Deliver priority digital transformation programmes identified within the SDIP	CFO	March 2028		**On Track**

Principal Risk No: 105	The ICB may lack sufficient influence across the system to shape delivery, resulting in fragmented services, poor value for money, and failure to deliver against priorities

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	X
Date Last Reviewed	28/09/2026	Tackle inequalities in outcomes, experience and access	X
Risk Owner:	Chief Finance Officer, Chief Strategy Officer	Enhance productivity and value for money	X
Scrutinising Committee(s):	Resource Allocation & Payer Population Health & Strategy Committee	Help the NHS support broader social and economic development	X

Score Tracker

Overall Current Score		Target Score
12		9
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
4	3	Moderate
Associated Committee Risks		**Key Risk Impact Classifications:**
PHS 002 PHS 003 PHS 004 PHS 005		• Financial • Reputational • Regulatory • Operational



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Strategic Planning & Delivery	• Memoranda of Understanding (MoUs) with Places and Provider Collaboratives; Joint Committees/Section 75 where appropriate. • Clinical and Care Professional Leadership framework embedded at system and place. • System Operating/ financial Plan co-produced; • alignment of financial plans to organisational priorities. • Contracting approach incorporates outcome measures, collaboration incentives, and gain-share/risk-share • Stakeholder engagement plan across providers, LAs, Healthwatch, VCSE; patient and public involvement. • Healthwatch/peer reviews on patient experience and pathway agreements • Stakeholder engagement plan across providers, LAs, Healthwatch, VCSE; patient and public involvement	• Unaligned contracting models impacting collaboration and outcomes • Insufficient data/analytics to support population health management • (PHM) and outcomes tracking • Further Data Sharing/processing agreements to be implemented • ICB Management of Change Process to be finalised including the transfer of functions • Full establishment of the Strategic Commissioning Model	• Signed MoUs/Section 75 agreements; meeting minutes and decision logs.	x			• Clustering plans not yet audited and evaluated. Place structure evaluation	• 1 • 3
			• Contract documentation; contract performance reports.	x				
			• Transition Programme Reports	X				
			• Finance and Performance Committee Reports		X			
			• Strategic Commissioning Committee Reports		X			
			• Transition Committee Reports		x			
			• Internal audit reviews on governance maturity and controls.CQC assessment reports			X		
			• NHSE oversight meetings and segmentation; improvement plans and checkpoints.			X		
			• External audit observations on governance and VfM arrangements.			X		
			• Annual Health Inequity Self Assessment		X			
			• NHSE Annual Health Inequity Assessment			x		
Place Development & Delegation	• Phased implementation approach agreed • Progression based n readiness • 5 readiness domains defined • Financial instability identified as hard constraint on delegation • Memorandum of Understanding (MoUs) with Places and Provider Collaboratives; Joint Committees/Section 75 where appropriate	• Provider commissioning capability does not yet exist and will require development. • Detailed readiness assessment methodology is not yet fully defined. • Maturity and integration vary across providers and places. • Scope and sequencing of initial delegated functions remains to be confirmed • NHS ICB reform impacting on governance maturity of the system	• Place Based Delivery Model		x		• objectives and outcomes framework are still being developed through the clinical strategy. • Cross-place performance and outcomes monitoring is not yet fully defined. • Accountability arrangements between Place Committees, Anchor Organisations and the ICB require further clarification.	• 2 • 3 • 4
			• Oversight through SCC		x			
			• Joint Cluster board		x			
			• Finance and Performance Committee		x			
			• Transition programme reports	x				
			• Place Committees report to both Health and Wellbeing Boards and SCC		x			
System Capability	• System-wide transformation portfolio with prioritisation criteria tied to outcomes and inequalities. • Dedicated commissioning support identified for anchor readiness transition • Primary care transformation programme in place • Primary care access plan • Stakeholder engagement and scrutiny processes •	• Inconsistent adoption of shared outcomes framework and joint strategic needs assessment across Places • Sovereign organisational internal priorities and pressures • variable provider engagement in agreed pathways, data sharing, and clinical leadership. • Strategy Unit modelling and planning support	• Strategic Commissioning Committee Reports		X		• Provider commissioning capability limited (anchor arrangements) • No formal assessment of provider readiness during organisational change	• 2 • 3 • 4
			• Transition Committee Reports		X			
			• F&P reports		X			
			• Primary Care Board	x				

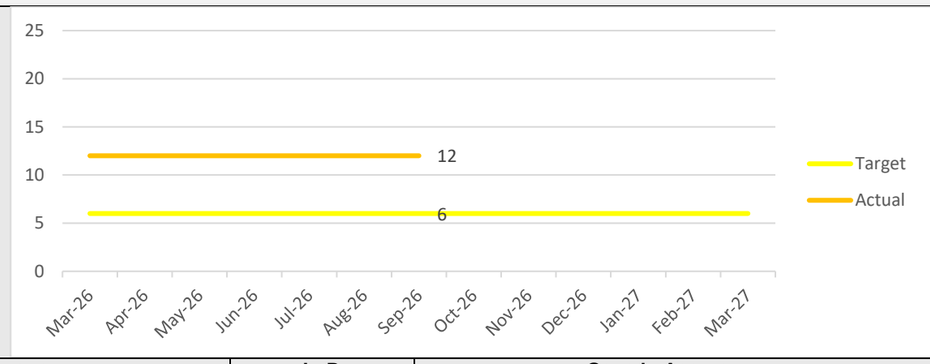
Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Place and Anchor organisation strategy	CSO	June 2026		**Complete**
2. Agree local place arrangements with all six local authorities.	CSO	October 2026		**Complete**
3. Develop the clinical strategy and consistent objectives framework	CSO	July 2026		**Complete**
4. Develop detailed commissioning support function	CSO	December 2026		**On track**

Principal Risk No: 106	The ICB may not have the organisational maturity to deliver the transition to operate as an effective strategic commissioner, leading to poor productivity, weak grip on outcomes, and reduced value for money.

Summary Data		ICB Aim/s:	
Date Opened:	01/02/2026	Improve outcomes in population health and healthcare	
Date Last Reviewed	28/10/2026	Tackle inequalities in outcomes, experience and access	
Risk Owner:	Transition Director	Enhance productivity and value for money	x
Scrutinising Committee(s):	Transition Committee	Help the NHS support broader social and economic development	

Score Tracker

Overall Current Score		Target Score
12		6
Current Consequence:	**Current Likelihood:**	**Overall Assurance Assessment**
4	3	Substantial
Associated Committee Risks		**Key Risk Impact Classifications:**
JTC 001 **JTC 002** **JTC 003** **JTC 004**		• Regulatory • Operational • Quality • Workforce • Financial



Driver	Controls	Gaps in Control	Source of Assurance	LoD 1st	LoD 2nd	LoD 3rd	Gaps in Assurance	Associated Actions
Workforce Stability	• New organisational structure defined • Organisational development strategy • ICB Vision, Values and Leadership Standards • Health & Wellbeing support • Trade Union representation • Freedom to speak up guardians • Staff mental health hub • All staff PDR's	• MOC not yet complete	• MoC documentation including staff feedback	X				• 1 • 2
			• Freedom to Speak Up Guardian Reports	X				
			• EDI reports	X				
			• Staff survey		x			
			• Freedom to speak up reports (audit committee)	x				
CSU tranistion	• CSU Service Transition Plan implemented with all workstreams mapped and SROs identified. • Regular communication with CSU and updates on the state of their service to minimise impact of an unplanned loss	• Some workstreams are under review to finalise transition plans • New workstreams are being identified such as the OWM IG support service	• CSU reports	X				
			• Function- Operational Risk register	X				
Cluster Maturity	• Policy alignment work has begun • SFI have been aligned • Constitutions aligned • Board and committee structures aligned • Joint NEMs in post • Joint Executives in post • Aligned Scheme of Reservation and Delegation • Audit committees meeting in common • Board forward plan developed • Unified Board Assurance Framework implemented • Cluster Risk Appetite statement approved • Internal Governance review underway	• MOC not yet complete • Some sub-committee structures are to be defined and implemented • Work is active to align key activities such as PSR and contract management • Committee forward plans are being developed • Outcomes of governance review not yet known • Long-term merger readiness criteria not yet fully defined	• Management/ Functional reports	X				• 3 • 6
			• Committee oversight and escalation reports		X			
			• Board Assurance Framework		X			
			• Scheme of Reservation and Delegation Approved by CiC Boards		X			
			• Governance structure approved by CiC ICB Boards		X			
			• Constitution approval by CiC ICB Boards		X			
			• Constitution approval by NHSE			X		
			• BAF and Risk management Internal Audit			X		
			• NHSE ICB reviews			X		
National Dependencies	• Continuous engagement with NHS E regional team •	• ICB has no control over release of NHS E guidence	• Executive oversight	X				• 4 • 5

Further Planned Actions	Action Owner	Original Due Date	Amended Due Date	Progress Status
1. Management of Change Process to be completed and finalised	Transition Dir.	Sept 2026	Nov 2026	**On Track**
2. Organisation OD Strategy to be finalised	Dir. Corp Affairs	April 2026	July 2026	**Complete**
3. Governance review outcomes to be shared for information and any subsequent action	Dir. Corp Affairs	Sept 2026		**Complete**
4. Maintain active engagement with NHS England regarding national requirements and timescales.	Dir, Corp. Affairs / Transition Dir.	April 2027		**On Track**
5. Develop contingency plans should national guidance, implementation expectations or timelines change	Dir, Corp. Affairs / Transition Dir.	Jan 2027		**On Track**
6. Develop a formal Merger Readiness Framework and assessment process	Dir, Corp. Affairs / Transition Dir.	Jan 2027		**On Track**

Assurance Assessment		
Limited Assurance	The committee has limited assurance that controls are effective, with significant gaps in controls and/or evidence. Controls are weak, incomplete, or absent Evidence is insufficient or indicates controls are not working Risk is not controlled and may be increasing Significant further action is required	
Moderate Assurance	The committee has some assurance that controls are in place; however, there are gaps in evidence or effectiveness which mean the risk is not yet sufficiently controlled Controls exist but are: • partially effective, or • recently implemented, or • not yet evidenced Evidence is incomplete, inconsistent, or still developing Risk may be stable or reducing but not yet close to target Further controls and/or evidence are required	
Substantial Assurance	The committee is largely assured that controls are effective, with good evidence in place; however, some gaps remain which prevent full assurance being confirmed. • Controls are well designed and mostly operating effectively • There is strong but not yet complete evidence • Risk is reducing and close to target, or on a credible trajectory • Remaining gaps are known, limited, and expected to close	
Full Assurance	The committee is assured that the controls in place are effective and that there is sufficient, robust and sustained evidence that the risk is being managed within the target level. • Strong, consistent evidence that controls are working in practice • Risk is at or clearly moving within target range • Any remaining gaps are minor and actively managed with clear actions • Evidence covers multiple lines of defence	

Return to Agenda

Board Cycle and forward plan.
Strategic Themes for the year

Each Board cycle will intentionally draw on one or two of these pillars as its anchor. Health Inequalities will be referenced as a ‘golden thread’ throughout all of the strategic themes.

1. Developing and delivering strategy and understanding our population health
2. Commissioning that delivers high levels of performance, quality effectiveness, improvement and statutory duties and measurable outcomes
3. Using public money efficiently to secure the best health and health services for our residents
4. System development and leveraging partnerships for population health.

Principles:

Principle	***What this means in the context of the board***
Strategic focus	Every formal Board agenda should have no more than three strategic items and a tight assurance section.
Complementarity	Development sessions prepare the Board for major decisions, strategic direction-setting, or system shifts expected in the next formal meeting.
Integrated thinking	Every meeting blend NHS, Local Government, and wider system perspective including citizen/patient/colleague voice.
Time well used	We prioritise discussion, debate, and judgement over presentation. Consent items and escalation reports are grouped together to allow time for a focus on strategic discussions.
Longitudinal flow	The board follows a clear annual narrative – vision, strategy, delivery, assurance, planning, reset. Value is captured including demonstrable impact and measurable outcomes.

The revised forward plan applies the established Board principles to the integrated ICB delivery plan, NHS Oversight Framework readiness requirements, strategic commissioning maturity priorities, BAF risks and merger or successor-readiness programme.

The annual flow is designed so that workshops shape choices before formal decisions, committees provide evidence and recommendations, and the Board receives one integrated account of delivery, maturity, risk and benefit. The November reserved-decision gateway for UTC and the January and February review of Neighbourhood Health Readiness Assessments are explicitly reflected in the revised cycle.

Quarter 3:

Quarter 3 moves from review to forward design. The Board shapes the workforce, planning, resource and partnership assumptions that will govern the 2027/28 plan and successor operating model.

November 2026 - Formal Meeting

Strategic Purpose: ***Planning, Future Shape and Strategy Refresh***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Chair Report	Chair	N/A										X	Summary report.
Chief Executive Report	Chief Executive	N/A										X	Integrated update on national, system and organisational developments.
Committee Escalation Reports	Committee Leads and Chairs	Population Health and Strategy Resource Allocation and Payer Quality and Safety Remuneration and People Audit						X			X		Exception-based assurance, decisions and matters outside committee risk appetite.
BAF Q2 and First NOF Performance / Capability Dashboard	Director of Corporate Affairs / Chief Finance Officer	Audit All Risk-Owning Committees	End-of-quarter risk position and first integrated view of NOF metrics, capability and recovery.	X	X	X	X	X			X		Assesses progress against objectives, risk appetite and readiness actions.
Place Prevention Fund	Chief Strategy Officer / Medical Director	Population Health and Strategy Resource Allocation and Payer Quality and Safety	Review Place, neighbourhood and provider-collaborative options alongside the first Commissioning Opportunity Assessments for CVD and frailty/UCR, including population need, variation, options, outcomes, quality, inequalities and affordability.	X	X	X	X	X		X			Provides direction on Place and provider roles and endorses the first strategic commissioning priorities, conditions and benefit-review dates.
UTC Decision: Consultation Outcome and Decision-Making Business Case	Chief Strategy Officer / Chief Nursing Officer / Chief Finance Officer	Population Health and Strategy Quality and Safety Resource Allocation and Payer	Consider the consultation outcome, final Decision-Making Business Case and integrated clinical, quality, equality, financial and implementation assurance for the urgent treatment centre pathway.	X	X	X	X	X	X		X		Reserved Board decision in November 2026. The paper should state the recommended option, consultation influence, conditions, implementation milestones, benefits and review dates.

Return to Agenda

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Neighbourhood Health Readiness Assessment: Framework and Baseline Position	Chief Strategy Officer	Population Health and Strategy	Review and endorse the assessment framework, evidence requirements and baseline position across Places, including primary care and Care Management Service readiness, workforce, data, governance, contracting and payment dependencies.	X	X	X	X	X		X	X		Board to confirm the framework, material evidence gaps and conditions for completion of Place assessments ahead of the January review and February challenge.
Fit and Proper Person Test	Director of Corporate Affairs	Remuneration and People	Confirmation that ICB has assessed whether Board Members comply with the requirements of the Fit and Proper Persons Test	X	X	X	X				X		Annual Report for Assurance

December 2026 - Board Workshop

Strategic Purpose: ***Planning, Future Shape and Strategy Refresh***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Voluntary Sector Compact Arrangements	Director of Communications and Involvement	Population Health and Strategy	Discussion on arrangements for Voluntary Sector Partnership Board and principles for working with the sector				X	X	X	X			Potentially agreeing terms of reference for Partnership Board
Resource Allocation, Investment and Disinvestment	Chief Finance Officer / Chief Strategy Officer	Resource Allocation and Payer Population Health and Strategy	Review expenditure mapping, Prevention Fund, investment and disinvestment options, capital forecast and neighbourhood payment model.	X	X	X	X	X		X			Provides direction for budgets, contracts and commissioning mandates.
Successor Governance, SFIs and SoRD	*Director of Corporate Affairs / Transition Lead*	*Remuneration and People* *Audit*	*Shape successor governance, delegation, committee architecture, SFIs, SoRD and opening-control requirements.*		*X*	*X*	*X*			*X*			*Establishes design principles before final approval in March.*
Oversight Response and Recovery Framework	Chief Finance Officer	Resource Allocation and Payer Quality and Safety	Define escalation, intervention, consequences and recovery where delivery, quality or financial standards are not achieved.		X	X	X			X			Strengthens payer capability and links recovery to contracts and assurance.

Quarter 4:

Quarter 4 closes the annual cycle by testing whether governance, people, resources, intelligence and delivery arrangements are ready for the next strategic cycle and successor operating environment.

January 2027 - Formal Meeting

Strategic Purpose: ***Review Delivery Architecture and Accountability***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Chair Report	Chair	N/A										X	Summary report.
Chief Executive Report	Chief Executive	N/A										X	Integrated update on national, system and organisational developments.
Committee Escalation Reports	Committee Leads and Chairs	Population Health and Strategy Resource Allocation and Payer Quality and Safety Remuneration and People Audit						X			X		Exception-based assurance, decisions and matters outside committee risk appetite.

Return to Agenda

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Anchor Contract Frameworks	Chief Strategy Office/ Chief Finance Officer	Population Health and Strategy Resource Allocation and Payer Quality and Safety	Agreement of Behavioural frameworks, Quality and Risk and Financial Delegation arrangements		X	X	X		X				Opportunity for the Board to agree a final framework for Anchor Contract arrangements.
BAF Q3	Director of Corporate Affairs	Audit All Risk-Owning Committees	Quarter 3 risk position	X	X	X	X	X			X		Identifies final-quarter assurance gaps and
Draft 2027/28 Operational, Financial and Commissioning Plan	Chief Strategy Officer / Chief Finance Officer	All Principal Committees	Integrated draft plan covering clinical priorities, outcomes, budgets, contracts, efficiency, workforce and delegated functions.	X	X	X	X	X		X			Board direction before final assurance and approval.
Freedom to Speak Up Annual Report	Director of Corporate Affairs	Audit & Governance Remuneration and People	Annual Report from Freedom to Speak Up Guardian supporting Board conversation on Organisational Culture	X	X	X	X			X	X		Board to identify system-wide actions, Place-specific conditions and matters to be incorporated into the final 2027/28 plan, commissioning mandates and successor operating arrangements.

February 2027 - Board Workshop

Strategic Purpose: ***Review Delivery Architecture and Accountability***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Risk Appetite and Principal Risks for 2027/28	Director of Corporate Affairs	Audit All Principal Committees	Review the effectiveness of current risk appetites and principal risks and identify changes required for 2027/28.	X	X	X	X	X		X			Shapes the risk parameters used in the annual planning and approval process.
Integrated Challenge of the 2027/28 Plan	Chief Executive / Executive Team	All Principal Committees	Challenge affordability, outcomes, quality, inequalities, workforce, digital, contracts, delegated functions and merger readiness.	X	X	X	X	X		X			Specifies the final amendments and conditions required before approval.
NHS IMPACT, Strategic Commissioning Maturity and Neighbourhood Readiness Review	Chief Strategy Officer / Medical Director / Director of Corporate Affairs	Population Health and Strategy Quality and Safety Resource Allocation and Payer	Review the NHS IMPACT and strategic commissioning maturity assessments, the Neighbourhood Health Readiness Assessment conclusions, the improvement roadmap, benefits method and evidence required for opening-year assurance.	X	X	X	X	X		X	X		Tests whether the final plan and operating model close the material capability and neighbourhood-readiness gaps before March approval.

March 2027 - Formal Meeting

Strategic Purpose: ***Review Delivery Architecture and Accountability***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Chair Report	Chair	N/A										X	Summary report.
Chief Executive Report	Chief Executive	N/A										X	Integrated update on national, system and organisational developments.
Committee Escalation Reports	Committee Leads and Chairs	Population Health and Strategy Resource Allocation and Payer Quality and Safety Remuneration and People Audit						X			X		Exception-based assurance, decisions and matters outside committee risk appetite.
2027/28 Principal Risks, BAF and Risk Appetite	Director of Corporate Affairs	Audit All Principal Committees	Approve principal risks, risk appetite and opening BAF for 2027/28.	X	X	X	X	X	X		X		Links the annual settlement to explicit control and assurance expectations.

Return to Agenda

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
2027/28 Operational and Financial Plan, Budgets and Commissioning Mandates	Chief Executive / Chief Strategy Officer / Chief Finance Officer	All Principal Committees	Final annual plan, budgets, JFP refresh, contracts, resource allocation, outcome trajectories and benefits register, incorporating the agreed Neighbourhood Health Readiness actions and UTC implementation requirements.	X	X	X	X	X	X				Formal approval of the annual settlement, commissioning priorities, Place and neighbourhood conditions and implementation controls.
Agree Hosting Arrangements for Office of Pan ICB Commissioning (OPIC)	Chief Executive Officer	Population Health and Strategy Resource Allocation and Payer Remuneration and People	Agree arrangements for transfer of NHS England Direct Commissioning Services (subject to Legislation)										Th Cluster will be the host for staff transferring from NHS England
Governance, Delegations and Successor Readiness	*Director of Corporate Affairs / Transition Lead*	*Remuneration and People* *Audit*	*Approve governance framework, SFIs, SoRD, committee plans, people readiness, opening controls and residual-risk handover.*		*X*	*X*	*X*		*X*		*X*		*Records conditional gateways and successor responsibilities.*
ICB Capability Review	Chief Strategy Officer / Director of Corporate Affairs / Executive Team	Population Health and Strategy Quality and Safety Resource Allocation and Payer Remuneration and People	Review the findings of strategic commissioning maturity and NHS Oversight Framework capability review, Neighbourhood Health Readiness Assessments across Places.	X	X	X	X	X		X	X		Board to identify system-wide actions, Place-specific conditions and matters to be incorporated into the final 2027/28 plan, commissioning mandates and successor operating arrangements.

2027/28 Quarter 1:

The first quarter of 2027/28 focuses on safe mobilisation, early control effectiveness and rapid resolution of people, service, finance and information exceptions.

April / May 2027 - Formal Meeting

Strategic Purpose: ***Mobilisation Assurance and Early Operational Grip***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Day-One and Day-30 Mobilisation Assurance	Chief Executive / Executive Team	All Successor Committees	Assurance on statutory functions, services, people, finance, contracts, data, controls and transferred responsibilities.		X	X	X	X			X		Board decides recovery action and whether conditions or delegations can be released.
Opening NOF, Finance and Quality Position	Chief Finance Officer / Chief Quality Officer	Resource Allocation and Payer Quality and Safety	First integrated post-mobilisation position, including metric exceptions, service continuity and financial forecast.		X	X	X	X			X		Establishes early grip and confirms assumptions remain valid.

June 2027 - Board Workshop

Strategic Purpose: ***First 90 Days, Value and Capability***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
First 90 Days and Strategic Commissioning Maturity	Chief Executive / Chief Strategy Officer	All Principal Committees	Review 90-day delivery, maturity evidence, Place and neighbourhood mobilisation and lessons from transition.	X	X	X	X	X		X			Identifies corrective action and capability priorities for the remainder of the year.
Clinical and Value Dashboard	Medical Director / Chief Finance Officer	Population Health and Strategy Resource Allocation and Payer Quality and Safety	Review clinical outcomes, utilisation, payment behaviour, benefits, quality and inequalities.	X	X	X	X	X		X			Tests whether commissioning and contracting changes are altering outcomes and provider behaviour.

Return to Agenda

July 2027 - Formal Meeting

Strategic Purpose: ***Q1 Assurance and Corrective Action***

Item	Lead Officer	Committee	Description	Strategic Themes				Health Inequity	Decision / Discussion / Assurance / Information				Additional Information
				1	2	3	4		De	Di	As	In	
Chair Report	Chair	N/A										X	Summary report.
Chief Executive Report	Chief Executive	N/A										X	Integrated update on national, system and organisational developments.
Committee Escalation Reports	Committee Leads and Chairs	Population Health and Strategy Resource Allocation and Payer Quality and Safety Remuneration and People Audit						X			X		Exception-based assurance, decisions and matters outside committee risk appetite.
Q1 BAF, NOF and Integrated Delivery Assurance	Director of Corporate Affairs / Chief Finance Officer	Audit All Principal Committees	Quarter 1 position across risks, metrics, finance, quality, people, benefits and inequalities.	X	X	X	X	X			X		Provides an integrated view of delivery and capability after the first quarter.
Q1 Resource, Quality and Outcome Decisions	Executive Team	Population Health and Strategy Resource Allocation and Payer Quality and Safety	Agree material corrective actions, investment or disinvestment and programme changes arising from Q1 evidence.	X	X	X	X	X	X				Reallocates action and resource where benefits or outcomes are off trajectory.
People, Culture and Leadership Stability	Director of Corporate Affairs	Remuneration and People	Quarterly review of engagement, sickness, vacancies, succession, capacity and leadership stability.		X		X				X		Confirms people actions needed to sustain delivery.

This document is accurate at the time of production but remains subject to change through the controlled delivery tracker and agenda commissioning process.